

Apollo Hospitals Enterprise Ltd. (APOLLOHOSP)

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Apollo Hospitals Enterprise Limited
Q1 FY23 Earnings Conference Call Transcript
August 12, 2022

Moderator: Ladies and gentlemen, good day and welcome to the Q1 FY23 earnings conference call of Apollo Hospitals Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you and over to you, sir.

Mayank Vaswani: Thank you, Mr. Garet. Good afternoon everyone and thank you for joining us on this call to discuss the financial results of Apollo Hospitals for the first quarter of financial year 2022-23, which were announced yesterday. We have with us on the call today the senior management team, represented by Mrs. Suneeta Reddy, Managing Director; Dr. Hariprasad, President of the Hospitals Division; Mr. A. Krishnan, Group CFO; Mr. C. Chandra Sekhar, CEO of AHLL; Mr. Obul Reddy, CFO of the Pharmacy Division; and Mr. Sanjiv Gupta, CFO of Apollo 24/7.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide # 2 of the investor presentation that has been shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks.

Thank you and over to you, ma'am.

Suneeta Reddy: Good afternoon, everyone. Thank you for taking time to join our earnings call. I trust all of you have received our earnings documents which we shared yesterday.

We have commenced fiscal year 2022-23 on a very strong note. After two financial years marked by disruption, the start of this financial year is characterized by a more stable environment leading to normalized economic activity.

We have seen an increase in volumes and occupancies accompanied by the return of planned high-end surgeries. This has enabled the healthcare services business to register a strong recovery in Q1FY23, with patient volumes and revenues higher as compared to Q1FY22 and quarter Q4FY22.

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Q1FY23 occupancy across the group was at 60%. IP volumes increased 9% quarter-on-quarter. The occupancy must also be viewed against the backdrop of ALOS of 3.38 days, down from 4.81 in Q1FY22, where COVID dominated, and 3.57 in Q4FY22. ARPOB also improved sequentially from Rs. 48,510 in Q4FY22 to Rs. 51,999 in Q1FY23, an increase of 7%. In a significant shift in payer mix, insurance revenue now contributes 41% of our overall revenue, up by 34% from pre-COVID times. Cash and insurance segments registered a quarter-on-quarter improvement of 15% in revenues.

Our focus on Centers of Excellence has resulted in a quarter-on-quarter growth of 7% in Cardiac, 17% in Oncology, 12% in Neurosciences, 28% in Orthopedics, 19% in Gastrosciences and 18% in Transplants. Surgical revenue now is 68% of the total revenue, up from 60% during COVID times. We also have a promising uptick in the International patient volumes and revenues. We have reached 85% of our pre-COVID levels on volumes and are par on revenues.

Against this backdrop, let me walk you through our consolidated financials for the quarter:

Consolidated revenues grew 7% quarter-on-quarter to Rs. 3,796 crore. Healthcare services grew by 9% quarter-on-quarter to Rs. 2,023 crore. Mature healthcare services revenue grew by 13% to Rs. 1,147 crore, while the New hospitals, they grew by 1% to Rs. 577 crore.

Apollo HealthCo revenues grew 8% quarter-on-quarter to Rs. 1,479 crore. Within Apollo HealthCo, the digital business recorded a GMV of Rs. 215 crore for the quarter, which is a sequential growth of 21%, and 34% on a like-to-like basis excluding the Omicron impact in January 2022. 232 net New Stores were opened this quarter, taking the number to 4,761 stores.

AHLL revenues stood at Rs. 293 crore, a decline of 5% on a quarter-on-quarter basis. This was primarily because of COVID and COVID allied testing revenues that increased during the Omicron wave in quarter four last year. Non-COVID diagnostic revenues, however, grew 12% quarter-on-quarter. Consolidated EBITDA stood at Rs. 491 crore, registering a quarter-on-quarter growth of 6%. Within this, healthcare services EBITDA grew 19% quarter-on-quarter to Rs. 484 crore. Healthcare services margins were at 23.9%, a total of 6 basis points improvement over the sequential quarter. Margins in Mature Hospitals were strong at 26.4%. Margins in New Hospitals stood at 17.7% for the quarter.

The pharmacy distribution segment in Apollo HealthCo recorded an EBITDA of Rs. 112 crore at a steady margin of 7.6%. Operating costs at Apollo 24/7 came in at Rs. 135 crore. Therefore, AHLL EBITDA post Ind AS was Rs. 29 crore compared to an EBITDA of Rs. 37 crore in Q4FY22.

Consolidated PAT is at Rs. 317 crore after deferred tax liability reversal of Rs. 147 crore. Consolidated Healthcare Services' PAT was at Rs. 222 crore, as compared to Rs. 176 crore in Q4FY22, a quarter-on-quarter growth of 27% in PAT for Healthcare Services.

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I am pleased to share that we recently acquired a hospital asset in Gurugram from Nayati Healthcare for a consideration of Rs. 450 crore. The asset has a potential of 650 beds across 7 lakh square feet. We are excited about strengthening our footprint in the national region and look forward to taking our unique brand of world-class clinical outcomes to the fast-growing and discerning city. We expect the facility to take pole position in attracting medical value travelers from around the world. In addition to building strong centers of excellence, we also plan to create a comprehensive ecosystem of care

across pharmacies, clinics, stay surgery, birthing centers and primary care clinics in the region to complete our offering. The integrated healthcare complex at Gurugram is expected to be commissioned in a span of 24 months.

We would also like to provide a few operating metrics on Apollo 24/7. The platform now has 17 million registered users; and is completing 35,000 transactions per day, up from 25,000 a quarter ago. The GMV run rate is now on track to deliver Rs. 1,500 crore for FY '22-'23 against the earlier guidance of Rs. 1,000 crore. To sustain the growth momentum, we do expect to incur 20% higher expenditure than guided earlier. We are already seeing a rationalization in loyalty costs and discounts and expect them to trend lower going forward. Overall, we are on track to become the number two digital player in the country during the current fiscal year.

Looking ahead, we believe that the momentum in the Healthcare Services business will continue. Occupancy at a group level will improve from 60% to 70% over the next 12 to 15 months, an improvement of 15%. This will result in a revenue growth of 15% to 20%, unlocking operating leverage, which will lead to an EBITDA expansion of around 35%. We have demonstrated our ability to actively optimize our payer mix and our case mix, and these efforts will continue to drive margin expansion up to 200 basis points.

The diagnostics business within AHLL can scale to Rs. 1,000 crore top line within the next 3 years. The digital business within Apollo HealthCo has shown significant traction and is tracking faster than planned numbers. The pharmacy distribution business will continue to grow at 20%, with our focus on private label products complemented by an accelerated store expansion plan. We expect margins in this business to hold steady.

The deep capabilities and mix of offerings that we provide to consumers through our omnichannel, multifaceted engagement model which is differentiated across the value chain is indeed very unique and forms the basis of our belief in our strong, sustained growth potential. We view the three verticals as the engines of growth and remain committed to our core value system of building solutions and offerings that will deliver the best clinical outcomes and are integrated and meaningful for the consumer. We intend to deepen our relationships with them over a longitudinal time frame and create lasting value for our stakeholders.

On that note, I would like to hand over to the moderator and open the line for questions and answers. I have Dr. Hariprasad with me, Dr. Hariprasad, Krishnan, Obul Reddy, Chandra and Sanjiv with me to take all your questions. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Nitin Gosar from Invesco. Please go ahead.

Nitin Gosar: I wanted to understand the reason for increase in operating costs in 24/7 ?

Suneeta Reddy: Sanjiv?

Sanjiv Gupta: Yes. So I think a couple of important points that we need to understand, as far as 24/7 is concerned. One is that, the last quarter, we had only 16 days of numbers. It included the overall expenditure for the last quarter was in the range of about Rs. 100 crore. We took a conscious call to achieve the number two position in the country given the fact that there is a lot of traction from the customers. We are seeing a lot of intake of various services that we are providing to the customers. And what we decided is that we will pull a little bit of expenditure from Q2 or rather, Q3 to Q1 and Q2. And what this helps us is to get not only the number two spot, but also it helps us to provide more services to the existing customers or the new set of customers. So

this is a conscious decision for Q1. And I do expect that similar numbers will be there for Q2 as well, but beyond that, we do not expect expenditure to go up sequentially. I think the guidance will be that 50% growth overall in the year from Rs. 1,000 crore GMV to Rs. 1,500 crore GMV, which will place us to the number two spot in the country, will be managed within 20% you know overall increase in the expenses.

Another two, three things that we are doing internally now is that, one is that the discount on the pharmacy vertical is tapering down and some due to the external factors and some because of the kind of continuum of care that we provide in 24/7 already reduced by 1%. And we do hope that another 0.5% to 0.75% we should be doing it in current month, apart from the fact that we have got certain more optimization initiatives. So I think with this I will only say that taking the second spot in the country is equally important. The companies which are there in this industry for last 7 to 8 years and we have been only 2 years old and one quarter down in third year. Taking that spot is not easy. You need a lot of efforts. You need some bit of investment. And I think this investment is what we see in Q1. This is slightly higher than Q4, but it is important.

Nitin Gosar: Got it. And I just wanted your thought process around this 24/7 cost structure. So Rs. 400 crore was the earlier called-out number which is going to be the investment which will go through P&L. Now that number has gone up and it may stay elevated for FY '22-'23. Incrementally, as we progress, these numbers are going to stay elevated, keeping in mind especially the environment which was very conducive for e-commerce until last year. The liquidity was high at that point of time, and liquidity is low at this point of time. It only allows people like us to sustain the business model with a lower cash burn. How do we look through going forward on the cash burn, especially for incremental quarters post the second quarter, onwards?

Sanjiv Gupta: As we said earlier, the last quarter we had, we kind of provided the guidance of about Rs. 450 crore of annual expenditure in digital business. I think 20% higher is what we are looking at, at this stage. And we strongly believe that we would be able to do a meaningful job to get to the number two spot, to deliver 50% higher volumes instead of Rs. 1,000 crore to Rs. 1,500 crore GMV and well within the guided numbers.

Nitin Gosar: Okay. And for FY '23, this number should not be at Rs. 550 crore, it should come down in FY '24. In FY24 the number should be less than Rs. 550 crore?

Sanjiv Gupta: Yes, all efforts are in that direction, sir.

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead. Due to no response, we will move to the next question, which is from the line of Lavanya Tottala from UBS. Please go ahead.

Lavanya Tottala: So going a little bit Apollo 24/7, I just wanted to understand on the numbers reported in results note. So how the GMV will be different from the revenue reported from 24/7 digital? And the sales through the online channel will be considered as part of pharma distribution, I just wanted some clarification here?

Sanjiv Gupta: Yes. So ma'am, I think you gave the answer in your question itself. You are right. So the entire online pharmacy GMV would be part of the front-end and back-end sales. So that will be the first part of it. And second, how GMV to revenue get reconciled is, typically in the e-commerce companies, GMV is the platform transactions that are done and revenue is the commission that we charge to the service providers. So, this entire GMV gets into various companies. So fo

For example, the entire virtual consultations will be the revenue in Apollo Hospitals. The entire pharmacy GMV will be part of the front end, back end. And the entire diagnostics services will be part of the AHLL.

Lavanya Tottala: Got it. So the take rate for this each segment will be in the range of ?

Sanjiv Gupta: So take rates are different for different verticals and depending upon the kind of volumes that we have. For example, on the virtual consultations, we have a take rate of between 5% to 10% depending upon the specialty. And on the diagnostics, we have a flag-based rate. So currently we are at about 12% to 13%. As volume goes up, there is another slab that kicks in. And accordingly, all these take rates are marked to market from that range.

Lavanya Tottala: And on the hospital business, I have seen in the presentation that, excluding vaccination in the base quarter, it is the growth is 7% YoY, but in the base quarter we have COVID-related occupancy also which could be impacting, if I understand right. So I need some clarification here.

Krishnan A: So first quarter was a 13% if you exclude vaccines. So Q1 versus Q1, excluding vaccines, is 13%. Q1 versus Q4 is 9%.

Moderator: Thank you. The next question is from the line of Sameer Baisiwala from Morgan Stanley. Please go ahead.

Sameer Baisiwala: Sir, just on ALOS, what is really helping us drive it down so much, excluding the COVID quarters? I mean it used to be around 4, now come down to 3.4. And what is the outlook for this?

Suneeta Reddy: So Sameer, what we have really done is to focus on really driving down the ALOS, recognizing two things. One is that we are using robotics and minimally invasive, and this enables us to discharge much faster. The second is that we really want to make it cheaper for the patient. So, any extra day means that they would have to pay one more day room rent, and for us, the earnings are higher in the first 3 days. So, we

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have taken clinical initiatives as well as operational initiatives to see that discharge happens much faster. We track these metrics on a month-to-month basis. So currently I think that we have really done well this quarter. And considering that most of our work is now surgical, we can definitely look forward to moving towards keeping it at 3.4. We have also introduced new procedures, which is TAVI as one of the cardiac procedures versus CABG. In knee transplants, we also have robotics. And we have 20% of our business is now baked with laparoscopic.

Sameer Baisiwala: Okay, great. So, this has released a lot of your capacities, so that is a good job done. And the second question is on Gurgaon project. Krishnan, how much capex do you need to do for this project? And my understanding is Gurgaon is a fair bit competitive market, so how will you attract the best medical talent? And can the road to profitability be a lot longer?

Suneeta Reddy: So what we plan now is we paid Rs. 450 crore for 5.67 acres. We think there is a building that is existing there, so maximum, we will invest another Rs. 350 crore, taking the project costs made up to Rs. 900 crore maximum. And with regard to competition, I think that we have a clearer plan on how to attract talent. We also bring talent from overseas. And I am sure that, you know we spoke about an integrated network, so we will have a funnel. One funnel will be part, 24/7 consumers that will send patients to Gurgaon. The second that we have is our relationship with corporates, which is really, really high. And I think the whole Apollo reputation in Delhi with what we have started with in the first. I think the trust in Apollo is very strong, and doctors continue to be very happy to join an Apollo facility. And finally, I have to say

that we will get a lot of international patients because we are working on that channel, and Gurgaon is probably the best place to be in if you are focused on that channel. So really, I think for me the most important thing is improving the clinical work that is existing now in Delhi. It is to take it to another level and therefore attract both doctors and patients and make the right investments in infrastructure and in technology.

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: My first question is on Apollo 24/7 spend. So Sanjiv mentioned about some initiative investment which you have taken to increase our standing in the market, so can you please a bit elaborate? Like where is majority of spend happening? Whether it is on the infrastructure, IT or on the discounts, if you can provide some better clarity.

Sanjiv Gupta: Yes. So about 25% is the expenditure that is getting into the product and the technology side of it. And when I am talking about the technology, it is the entire app

experience. It is the entire web experience and the entire CDSS and CIE engine that we are working on. So that is about 25%. We are also ensuring that the adequate capacity with respect to the knowledge and manpower is there pan -India, at the right verticals at the right places. So about 30% to 35% is getting into the augmenting of resources. We have about 20% to 25% of the costs which is getting into some sort of a branding and this is to ensure that the communication to the customers with respect to the various services that digital line of the business as well as the entire Apollo ecosystem gets communicated. And about 10% to 15% expenditure is getting into user acquisition and that is to ensure that we will get new set of users, chronic

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users specifically. And the digital spend is within this 10% to 15%. And the remaining 8% to 10% is towards the other infrastructures, primarily the support side of it.

Damayanti Kerai: So that is helpful. And what is the discount on the orders at this point of time compared to peers? You mentioned you have already taken some reductions, but where do you stand?

Sanjiv Gupta: Yes. So last quarter, we saw discounts in the range of about 18%. And that was the whole of the entire Q1. July, we started tapering down the discounts. And much of the things have to happen from how exactly the external forces are working, external forces in the sense of the competition out there. And secondly, what is the mix of repeat customer, repeat cohorts and the chronic cohorts? Depending upon all this, we take the calculated decision. And in July month, we have reduced discount by 1%. I think I also see another headroom for 0.5% to 0.75%. Something in this range should happen during current month or maybe by end of the next month.

Damayanti Kerai: So broadly, you will be standing at 16%, 17% discount in near term; and then going ahead, depending on how market evolves.

Sanjiv Gupta: I think, that would be fair. I think 16.5% should be or 16.75% that will be the numbers that we can look out for Q2. And as far as the external forces or the environment is concerned, given the fact that we have got the very high Apollo internal base of customers, we do not get much impacted as far as the competition out there is concerned. So I think we should be able to hit this number of 16.5% to 16.75 %.

Damayanti Kerai: Okay. And my last question is on ARPOB, very strong growth, 27% year -on-year. So ma'am mentioned payer mix change obviously helped, but how much of contribution is there from price hike in this rise? And whether this should be sustainable going ahead. So ma'am, my question was on ARPOB, very strong number. You

mentioned

it was helped by the payer mix improvement, but my question is does it include a component of price hike and whether this kind of growth is sustainable ?

Krishnan A: Yes. So ARPOB, there was a small price hike of 2% that we have done, and it is inclusive of that. And this is something, you know the mix, the advantage, there are 2, 3 levers on the ARPOB. One was the surgical mix, if you look at the surgical mix its rebounded significantly. And we are now at 68% of our revenues comes from surgeries, so that is one important lever which has impacted our ARPOB positively. We are continuing to see also a good case mix aided by insurance and walk -ins. So insurance itself, which was 34% has now become 41%. And so clearly there are good levers that are helping us get the ARPOB, and this is sustainable.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Motilal Oswal. Please go ahead.

Sumit Gupta: I just want to know regarding the capex plans, if any and regarding the hospital segment, like where all do you plan to expand ?

Krishnan A: So, the routine capex you are meaning or our project capex that you are asking about ?

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Sumit Gupta: Routine as well as projected, sir.

Krishnan A: So routine capex, which is part of our cash flows, et c. from our every year, it is approximately around Rs. 300 crore. That is the number that we have as routine capex at a consolidated level and that will continue for now. And the free cash flows even after that would be in the region of around Rs. 800-plus crore, which is what is the capex, routine capex, that we have. And the free cash flows is what we are going to be using for deployment in new projects, et c. as you know, we already have free cash in our hand. And even Nayati has, the Gurgaon asset acquisition happened in Q2, which is the Rs. 450 crore that we spent towards Gurgaon. So clearly the free cash flow plus the cash that we have in our hands of almost around Rs. 900 crore,

is something that we can use towards some of our expansions. We have announced two expansions, one in Chennai and the other one which is in Gurgaon. And we have said that we are looking at Bangalore as well as we are looking at North. So, we are comfortable we have a combination of this plus there is a receivable from Apollo 24/7 of Rs. 1,200 crore which we hope that we will get over the next one year, if not in the coming round, the next round or whatever. There is an amount that will come to us. So over the next three years, the free cash flow will be used plus this Apollo 24/7 can help us in our expansions. So current expansions that we have announced is the Chennai one and the Gurgaon one, Rs. 900 crore in Gurgaon and maybe Rs. 800 crore in Chennai.

Sumit Gupta: Yes, understood, sir. And sir, regarding ARPOB, so at the end of financial year '23 and like '24, what kind of ARPOB that you are targeting?

Krishnan A: So the ARPOB that we are targeting, is it?

Sumit Gupta: Yes.

Krishnan A: We do not have any specific targets. I think this is a good ARPOB that we are focused on. You should appreciate that our ARPOB numbers that we give are net of fee-for-service doctors. So if you look at it, our ARPOB is really not driven by price. It is driven mostly by case mix and ALOS and day surgery, etc. That is what we have been focusing on, increasing our ARPOB. And this is an ARPOB that we have which is a combination of Tier-1 and Tier-2, right? If you look at it, we have a lot of Tier-2 hospitals across our system and networks, and this is after that. So if you actually gross up for the fee-for-service doctors, this ARPOB will actually go closer to the Rs. 60,000, from a gross ARPOB perspective. So this is a good ARPOB. We

will continue

to focus on this ARPOB and grow from here.

Moderator: Thank you. Our next question is from the line of Harith Ahamed from Spark Capital.

Please go ahead.

Harith Ahamed: So I am looking at the combined pharmacy business. There has been a decline in EBITDA margin by around 180 basis points quarter-on-quarter from 9.3% to 7.5%, but when I look at the back-end pharmacy margins, it is been flat quarter-on-quarter, so can you help me understand this discrepancy?

Obul Reddy: Yes. As you rightly said, the back end pharmacy margins remain the same at the earlier level. And in the front-end retail pharma business, there are two, three developments that have happened during the quarter which brought in onetime

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costs. Like we have been working on aligning the Amazon systems for a pan-India operationalization of their deliveries, and that online technology costs we completed that online alignment and that costs have come into this quarter. Also, we have created necessary infrastructure for Amazon deliveries to enhance their numbers. We have created about 16 dark stores in Mumbai city alone and about another 12 dark stores in other parts of the country to see that Amazon reaches the entire country with our networks available in other states. And also, that we have now enhanced our rollover around the pharmacy. We have about 230, 240 pharmacies opened during the quarter against our normal run rate of about 140. All these onetime events brought about Rs. 25 crore to Rs. 30 crore additional costs. Out of that, about Rs. 20 crore is onetime, so we expect margins to come back to normal in the next quarter.

Harith Ahamed: And next one is on the diagnostics business which is in AHLL. So there is been a decline in revenues and margins over the last few quarters. So I understand that you had RT-PCR revenues in the previous quarters, but still the EBITDA margin decline for this quarter is at 5%. So the decline seems to be a bit sharp, so any one-offs that you would call out there?

Suneeta Reddy: Chandra Sekhar?

Chandra Sekhar: Yes. On the margins, yes. The COVID, the change in case mix has one part of an impact on the margin, but primarily the others are in the nature of investments that we are making in the beginning of the fiscal year. These are in the nature of tech investments upgrading our abilities to serve digital logistics side. We also have seen a slight increment in consumable costs because of certain freight and other logistics-based inflation. We are doing another, we have made a lot of investment, which leads to higher opex in upping our technical capabilities. We are moving to super specialized areas such as oncogenomics, reproductive genomics and a few others, which has called for both enhancement of a doctor infra, scientific sales force and product teams. So this is something which we believe is going to even out as revenues start coming from the super specialized areas of work and is in the nature

of the investment. So we should get back on a mature level to, as EBITDA margins are at a better level than we have.

We also have lastly a cost overhang. This is on account of the large infra, doctors and technicians that we have created for RT-PCR testing across the country. This cost overhang remained unutilized during this quarter. So also, we are going ahead of time in terms of our (Inaudible) expansion, yes, so that we are beefed up to cater to demand that is coming from home collection. This (Inaudible) expansion also leads to a higher of investment and operating expense, but all of this will get evened out in the coming quarters.

Harith Ahamed: Okay. And last one from my side, with your permission. So when I look at your SEBI disclosures, there is

a revenue of Rs. 63 crore under 24/7 digital. And versus the GMV that you disclosed of Rs. 215 crore, this is close to 30%. It seems to be on the higher side versus the take rates that you mentioned, 5% to 10% for consultations and 12% to 13% for diagnostics, so can you help me understand this revenue number that you have disclosed for 24/7?

Chandra Sekhar: Sanjiv, you will have to answer that.

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Sanjiv Gupta: I am not kind of clear over the questions. You might need to, do you want to repeat that?

Harith Ahamed: Yes. So in your SEBI filing, there is revenue disclosed for 24/7 digital under the segmental disclosures. That is Rs. 63 crore. That is 30% of your GMV of Rs. 215 crore, so it seems to be higher the take versus the take rate that you mentioned which is around 5% to 10% for consultation and 12% to 13% for diagnostics. So that is what the question is.

Sanjiv Gupta: No, sir. The number that you are reading is in rupees lakhs, so what you are reading as the Rs. 63 crore is actually Rs. 6.33 crore.

Moderator: Thank you. The next question is from the line of Shaleen Kumar from UBS. Please go ahead.

Shaleen Kumar: Two questions. One, is it possible to get some sense on contribution from COVID vaccine at EBITDA level in a base quarter?

Dr. Hariprasad: Only Rs. 11 crore of revenue in the first quarter. The EBITDA will be somewhere around 12% to 13%.

Shaleen Kumar: Base quarter, last year base quarter, I mean first quarter of last year?

Dr. Hariprasad: For base quarter, it was Rs. 211 crore.

Shaleen Kumar: That is revenue, right, sir?

Dr. Hariprasad: That is revenue. EBITDA will be around 15%.

Shaleen Kumar: And so ma'am, you alluded that, external fund raise, you were pushing by six months in the last call. Is the time line intact for that? I mean which effectively means that you would be looking at fundraise in probably third quarter. Or is there any change of plan over there?

Suneeta Reddy: No, we are on track to close for December.

Shaleen Kumar: Okay, by calendar year-end you should be able to close an external fund raise?

Suneeta Reddy: Yes.

Moderator: Thank you. The next question is from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal: On the hospital business, we have seen a good margin traction; and so is the case with the peer set reported so far. I am just trying to understand, in our case, how do we see the margins tracking ahead given that we are adding some hospitals? So what is the margin trajectory in the hospital space that we expect over the next one to two years? And pardon me if you already discussed. I joined a bit late, so I was on other call.

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Krishnan A: Sure. So first point, yes, at the first level, our occupancy is 60%, as you know. And we are looking at increasing this to 70% over the next 12 to 15 months. That itself should give us a good kicker because a 60% to 70% occupancy means a 15% increase in volumes. And that should increase our revenues by 25% and, hopefully, our EBITDA by 35%. So that is what we are trying to work on. And with that, if that works, and that should also ensure that our margins go from the current 24% to at least 26%, 27%. So that is at one level. The expansions that we are talking of will come after that, right, because all of our expansions, the Gurgaon, which should be the first which should come in, will be 24 months from now, followed by Chennai, so

which is how we are looking at this. And I guess we do not see any problems with the margins for now.

Prakash Agarwal: Okay, lovely.

And what about some other expansion plans? You have talked about Mumbai. Is there any update there? And in the past call and on the annual report, you talked about adding a few more hospital beds. If you could update us, what is the plan there?

Krishnan A: Continuing to work on them. So these two together itself is 1,000 beds now, right, between Chennai and Gurgaon, 1,000 beds is something that is there, which is Gurgaon can, the phase 2 can go up to 650 beds even if they started at 500. So it can go to 650 beds potentially. This is 1,000 beds now that we are focusing on. You will hear from us. We are looking at the North for expansion. We are looking at Bangalore. So all of this will come between the next 2, 3 years once we start. So that is how we are looking at it. And we are looking at bolt-on also. If there are any bolt-on acquisitions which come our way, we would look at it.

Prakash Agarwal: And sir, is there any update on Mumbai?

Krishnan A: Working on it still. Still working on it. I do not think there is any further update that we have on Mumbai.

Prakash Agarwal: Okay. And moving to the 24/7. So there is a change in guidance in terms of the investments that we plan to do. If you do not mind, if you could if you already said, then if you could repeat the same, please. Why the change? What is the thought process? Are we seeing more traction, or are we seeing less traction and we need to invest more?

Krishnan A: Sanjiv? In fact we are seeing very good traction and that is why we are investing, but Sanjiv will add onto it.

Sanjiv Gupta: Yes. So yes, so we are seeing very high traction into the business. In fact, we are, well, almost there to become number two online player in the country. And the Rs. 1,000 crore of guidance that we gave the last quarter earnings call, I think we are well within there to reach a Rs. 1,500 crore mark. So this is one side of it. As far as the overall investment into the Company is concerned, for the current year, which we assume to be higher by 20%, I think much of the investment is kind of investment ahead of this kind of a thought process. We are building up certain capacities, taking infrastructure. It is taking some bit of expenses, but yes, very much we are looking at very high traction into the business as of now.

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Prakash Agarwal: Okay. And any color on the pharmacy business? We have earlier guided for 15% to 20% growth. And I see that we are flattish, so has the competitive intensity gone up?

What are we feeling now? And what are we expecting for the remaining nine months?

Obul Reddy: So we expect our growth for the year to be in the range of 20% to 22% for the offline network. And adding from the online, we expect to be anywhere about 27%-plus growth for the current year. So Q1, generally as you know, we have, Q4, some COVID impact. And then generally Q1 is a sluggish quarter with April, May lesser sales. We are seeing the growth coming back. And we are on track for about 20%, 22% offline, coupled with online about 27%, 28% growth for the year.

Krishnan A: So last year, combined pharmacy business reported was around Rs. 6,800 crore and the target is over Rs. 8,500 crore.

Prakash Agarwal: So this includes the Amazon piece as well?

Obul Reddy: Yes, it includes Amazon, but we expect to see the tractions next year because we have set up the system alignment, system integration and the infrastructure. We expect that to grow.

Moderator: Thank you. The next question is from the line of Shaleen Kumar from UBS. Please go ahead. It seems we have lost his line, so we will move to the next question, which is from the line of Amit Ganatra from Invesco. Please go ahead.

Amit Ganatra: Just wanted to understand this entire

long-term thought process on Apollo 24/7. Now this year, there is a GMV that is expected to be around Rs. 1,500 crore and there certain investments that you are planning to make. Now next year, if the GMV doubles and becomes Rs. 3,000 crore, so how do we then view the investments? I mean investments also keep on going up or they remain capped at whatever we are doing annually this year. How to link these investments to GMV from a long-term perspective?

Sanjiv Gupta: Yes. So I think the way to look at this is that where exactly this investment is going on. In one of the previous questions, I gave the answer that about 25% to 30% is

happening into the products and technology side. And about 30% to 35% is into the operations, building on the capacity around operations; and the other, support, 10% to 12%. So essentially about 65% to 70% is the investment that is getting into building up the Company or the network or the capacity. Now obviously, when you double the next year or grow three times, I do not need to kind of double or triple all these costs because of your platform, whether it is Android or iOS platform or the West side. That would not require same kind of investment even if the traffic doubles or goes to 5x or 10x. So, as we ramp up year-on-year, I do not see that our numbers or the expenses to go in the same fashion. In fact, we would get a large-scale benefit across the expenses as well as we will have a lot of benefits with respect to the commission structure. So, I think the way to look at this is that any e-commerce Company, once gets a substantial portion of the business or the transactions becomes substantial, in our case, we look at something like about Rs. 250 crore to Rs. 300 crore of a monthly GMV would be the goal spot, so to say. Until that, whatever capacity that we have built up today is good enough for us to continue. I do not see that we will be raising our bar, as far as the expenses side is concerned.

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Some 10%, 15% or 20%, expenses will go up, but that is a natural phenomenon year-on-year but not a direct proportion.

Amit Ganatra: So correct me if I am wrong. What you said was that, till the time that you can reach Rs. 200 crore to Rs. 250 crore of monthly run rate, you are creating capacity for that kind of an outcome. Is that correct understanding?

Sanjiv Gupta: Yes.

Moderator: Thank you. The next question is from the line of Vihang Subramanian from Zaaba Capital. Please go ahead.

Vihang Subramanian: Just one from my side. On the pharmacy distribution piece, if I am correct, there is a lot of unorganized stores, right, single stores which are sort of losing their competitiveness. So instead of a store expansion strategy or in addition to a store expansion strategy, have you considered an acquisition strategy as well? Because there is a lot of regional familiarity that you can get by acquiring smaller stores, right?

Suneeta Reddy: Yes. I will ask Obul to answer.

Obul Reddy: We have our network partners doing the distribution bid for us. And we do not get into the pharma distribution acquisition. As far as the front-end network is concerned, we are aligned and our expansion is across the country.

Suneeta Reddy: So just to add some color to that. We did do an acquisition of Hetero, which happened 4 to 5 years ago, but I think the team has become very efficient in starting their own stores, and the cost of creating a new store is better than acquiring the small mom-and-pop stores.

Vihang Subramanian: And just another thing, on the 24/7, I think you had mentioned to an earlier participant about the sales and the GMV. I was not really clear on that. So if the GMV is like Rs. 1,500 crore, I was under the understanding that, then

at is sales plus GST, right? So

sales should be like 70% of GMV, right? Or is that wrong?

Krishnan A: That is correct, but that gets captured in the back end, which is the pharmacy distribution. That is what Sanjiv explained.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Motilal Oswal. Please go ahead.

Sumit Gupta: Sir, I just want to get a clarity on the Amazon deal. So like what kind of cost sharing is there between Apollo and Amazon? And what kind of discount is borne by like Apollo takes all the discount, or what?

Sanjiv Gupta: Yes. So I think, on the second part, first, as far as the discount side is concerned, what has been agreed with Amazon is a 15% discount. And so that is the benchmark that we are looking at, but they are allowed to offer discounts in this case to, say, Amazon Pay or the third-party financial instruments provider. So, it could be any bank offering, let us say, another 5% cash back so on and so forth, but such deals are in any case are there in every e-commerce platform. So, I think that part is on to

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them to bring the right sort of deals to the customers, but from the baseline discount is concerned, it is 15%. That is one.

Secondly, the first question. What are the economics between the two, between the partners? I think both the companies are playing to their strengths. Amazon is playing to the strength of the customer experience, the customers acquisition, putting up the

branding, marketing and then delivery side of it because Amazon has got a wide network. And they can easily serve that. I mean millions of orders a day they serve every day. So they are playing to their strengths. Apollo is, we are playing to our strengths. Our strengths is to have a right set of assortment at the right place, the entire picking and tracking, the billing, all that. So these strengths is being played at our end. So this is how we have kind of constructed the entire deal.

Moderator: Thank you. As there no further questions from the participants, I now hand the conference over to the management for closing comments.

Suneeta Reddy: Good afternoon, everyone, and thank you for joining this call. I believe that at Apollo Hospitals we have always taken a long -range view on financial and operating sustainability and value creation, and we believe that this is the only reliable path to institution building. As you have seen over the past years, we have created several verticals and all of which are growing ; the hospital vertical, the pharmacy with the digital and Apollo Health & Lifestyle . So I look forward to a continued engagement with all of you. And thank you for joining this call.

Moderator: Thank you. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility of such errors, although an effort has been made to ensure high level of accuracy .

Call: Nov 2022

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Apollo Hospitals Limited
Q2 FY 2023 Earnings Conference Call Transcript
November 11, 2022

Moderator: Ladies and gentlemen, good day, and welcome to Apollo Hospitals Limited Q2 FY23 earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you, sir.

Mayank Vaswani: Thank you, Rutuja. Good afternoon, everyone, and thank you for joining us on this call to discuss the financial results of Apollo Hospitals for the second quarter and H1 of FY23, which was announced yesterday. We have with us today the senior management team represented by Mrs. Suneeta Reddy, Managing Director; Mr. A. Krishnan, Group CFO; Mr. C. Chandra Sekhar, CEO of AHLL; Mr. Obul Reddy, CFO of the Pharmacy division; and Mr. Sanjiv Gupta, CFO of Apollo 24/7.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note, the disclaimer mentioning these risks and uncertainties on Slide #2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on our corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Thank you, Mayank. Good afternoon, everyone, and thank you for taking time out for this call. I hope you have received the documents, which were shared yesterday. We have reported a strong all-round performance in Q2FY23 with each of our distinct verticals, building on the momentum of quarter 1 and demonstrating a very healthy growth and trajectory.

Healthcare Services IP volumes were 19% higher year-on-year and 13% quarter-on-quarter. While medical discharges increased during the quarter, reflecting the seasonal nature of the business, surgical volumes also grew in which H1 FY 23 IP

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volumes registered a growth of 24% year-on-year. Overall, Q2FY20 occupancy across the group was 58% compared to 60% in Q1FY23.

The occupancy in mature hospitals was at 70% in new hospitals at 64%. Our payer mix continued to improve with cash and the insurance segments registering a quarter-on-quarter improvement of 16% and a year-on-year improvement of 10% in revenues. Overall, ARPOB was stable at 50,353 versus Q2, 51,999. ALOS was at 3.44 days.

Against this backdrop, let me walk you through the consolidated financials for the quarter. Consolidated revenues grew by 22% on a year-on-year basis to INR 4,251 crore, after normalizing for COVID vaccination revenue in Q2FY22. Healthcare Services grew by 12% year-on-year and quarter-on-quarter to INR 2,264 crore. Mature Healthcare services grew by 15% and 10% quarter-on-quarter to INR 1,592 crore, while new hospitals grew by 4% year-on-year and 17% quarter-on-quarter to INR 673 crore.

Revenue for Apollo Health and Lifestyle was INR 318 crore for the quarter, a 12% growth year-on-year after normalizing for COVID related revenue and 9% growth quarter-on-quarter. Apollo Diagnostics recorded its highest ever revenues this quarter and crossed the landmark of INR 100 crore, Q2FY23 revenues at INR 104 crore, 52% year-on-year growth after normalizing for COVID related revenue and 28% growth quarter-on-quarter.

Apollo Health Co revenue grew by 43% year-on-year and 13% quarter-on-quarter to INR 1,668 crore. Private label sales contributed to 11% of the revenue in the Pharmacy distribution business. We opened 241 net new stores this quarter, taking the total number of stores to 5,004. The GMV of our digital platform, 24/7 was at INR 294 crore, a growth of 38% quarter-on-quarter.

EBITDA. Q2 consolidated EBITDA post Ind AS excluding 24/7, operating costs stood at INR 740 crore. This is a 12% year-on-year and 17% quarter-on-quarter

improvement. Healthcare Services EBITDA post Ind AS was at INR 571 crore, a year -on-year growth of 13% and quarter -on-quarter growth of 18%. Healthcare Services EBITDA margins were at 25.2%, a year -on-year expansion of 190 basis points and a quarter -on-quarter expansion of 129 basis points. Mature Healthcare Services margins are at 28.1%, an improvement of 312 basis points year -on-year. New Healthcare Services margins were at 18.4%. Apollo Health & Lifestyle EBITDA was at INR 38 crore, a 28% growth on quarter -on-quarter. Pharmacy Distribution EBITDA post Ind AS 116 was at INR 131 crore and margins at INR 7.84 crore. Apollo 24/7, operating cost was at INR 174 crore for Q2FY23. This includes a non-cash ESOP charge of INR 22 for the quarter. Consolidated reported EBITDA was at INR 565 crore due to a n impact of 24/7 operating costs as compared to the EBITDA post Ind AS of INR 615 crore in Q2FY22 and INR 491 crore in Q1FY23. PAT, consolidated PAT is at INR 220 crore prior to adjustments of a onetime capital gains charge of INR 16 crore . Operating metrics of 24/7 , Our digital health platform 24/7, continues to witness strong momentum. It now has 20 million registered users and completes over 50,000

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omni -medicine deliveries per day, in addition to the 5,000 virtual consults and Diagnostic sample collections. The GMV run rate is firmly on track to deliver INR 1,500 crore for this fiscal, and we will be the number 2 digital player in the country. We continue to see steady rationalization and loyalty costs and discounts and expect them to trend lower going forward.

Looking ahead , on our Healthcare Services business, there is headroom for gr

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and occupancy, and we hope to see occupancy closer to 70% over the next 12 months. This will unlock operating leverage and will further strengthen margins. We expect payer mix to consolidate, International business to grow with our focus on our centers of excellence, which will drive high -end clinical work. Our cost and productivity initiat ive will improve margins by 100 to 150 basis points over the next 18 months.

On retail health, our specific focus is on primary care and Diagnostics. We expect the Diagnostics business to reach INR 500 crore of revenue by the next fiscal and achieve INR 1 ,000 crore of top line over the next 3 years. We believe we have the depth of clinical understanding to deliver the high -end Diagnostic testings and will, therefore, be a reference unit for hospitals from around the country.

Our Pharmacy Distribution busi ness will sustain its momentum and margins. Apollo 24/7 is already the fastest -growing health care platform in this part of the world and is poised to break new ground in terms of a seamless continuum of clinical care that it will offer to the consumers. I n our view, we have created 4 robust pillars, each one with an independent value proposition and positive forward trajectory. But most importantly, we are a 360 -degree health care ecosystem and the pillars are interconnected in a way that offers limitle ss potential for synergies and to integrate the health care offering holistically for the consumer.

We believe strongly in the network effect and that our work on touching so many lives in our hospitals, pharmacies, retail and digital health business will create a strong understanding of our consumers and help forge deep relationships with all. Above all, we have an unparalleled track record on being clinically oriented at our core and we believe that this is what differentiates us significantly and set s up a positive outlook.

On that note, I would like to hand over to the moderator. I have our CFO, Krishnan with me; Obul Reddy from the Pharmacy; Chandra from Apollo Health & Lifestyle and Sanjiv from 24/7 with me to take your questions. Thank you.

Moderator: The first question is from the line of Nikhil Mathu r from HDFC Mutual Fund. Please go ahead.

Nikhil Mathur: My first question is on Apollo 24/7. Can you also share the revenue number book as per the GMV that has been recorded in the quarter?

Sanjiv Gupta: So, for the quarter 2, INR 294 crore on the GMV that we did and the revenue we got recorded is INR 158 crore. This is a part of those two things you know, one will be online Pharmacy distribution and plus the service revenue that we earn on the Consultation and the Diagnostics side. So, INR 158 crore which is INR 294 crore of GMV for the quarter.

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Krishnan A: So, this INR 158 is what we recorded in Apollo Health Co. If you look at the overall , if you look at APL, that would be a higher number. Because what we record is only

the distribution part of it, which is around 80% of what we typically record in Apollo Pharmacies Limited.

Nikhil Mathur: So, this INR 158 crore revenue, that includes a stake rate on the direct sales booked plus also the distribution in association with those revenues. Am I reading it right?

Krishnan A: That's right.

Nikhil Mathur: And also, can you share what is the broad breakup of the INR 150 crore of operating costs with regards to 24/7 in this particular quarter?

Sanjiv Gupta: Yes, sure. So typically, what we, I mean, the scale that we have today, which is a INR 1,500 crore run rate analyzed on based, primarily, we are spending on two important elements. One is the product intent development. And I think this will continue for another two to three quarters, which is about 25% to 30% of the cost

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which goes into building up the project and tech. Secondly, we are also building up the operations network or let me put this way capacity, something like Mumbai, we are putting up our large stores. And to this operational capacity takes around 30% to 35% of the cost today. And apart from that, we are also putting money into clinical engine support systems and various other you know the new verticals, the new revenue segment, which are going to come, it will be in range of about 10% to 15%. Apart from that, another 10% goes into the support cost and the balance gets into the user acquisition and the branding.

Nikhil Mathur: Right. And what is the difference in discounting between online and offline projects?

Sanjiv Gupta: So as far as the online discounting is concerned, we maintained what we discussed in the previous call also that today, today we are at about 18 % as well as the online discount is concerned, although in the market, you've got very high numbers, which are being pushed by various competitors, but we continue to maintain our position of providing service and continuum of care versus playing the discount card. As well as the offline is concerned, the discounts are in the range of about 13% to 13.5%.

Nikhil Mathur: So, the difference between the two discounts, if sales is booked on an online channel that is also part of this operating cost of INR 24/7 right, which is the customer acquisition cost in some sense?

Sanjiv Gupta: Yes, the entire customer acquisition cost is part of the expenses that we are talking about.

Nikhil Mathur: Understood. And then one final question if I may, please. What's the time line on the Gurgaon possible acquisition that has been done, I mean, how far are we from commercialization of that facility?

Suneeta Reddy: It will take 18 months. We have started work.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

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Shyam Srinivasan : Just the first one on the new hospitals. Just in terms of the Y-o-Y performance, excluding vaccination also around 4% when the otherwise it's 12% for the group. So, what's happening there? Is it just occupancy seems to be down, but I just wanted to get your color on the slower growth rate and when will this likely start inching towards more corporate level?

Suneeta Reddy: So, in terms of the new hospitals, I think some of them witnessed a slowdown, but they are actually, if you look at October, they are back on track. We do have, we have put more clinical specialties there. So initially, we did not have the type of offerings that we would have in a Tier 1 city. So, we've added this. And this will take a little bit more time to pick up. In the prior years, we did have COVID occupancy, which is why if you remove the COVID occupancy, there is growth. And we are quite hopeful that with the pickup of insurance that more people now have access into Tier 2 hospitals. So, we firmly believe that the 18% margin that the new hospitals delivered will improve significantly by another 200 basis points in the next 12 months.

Krishnan A: So, in the base also, what happens in the new hospitals is because there was this COVID, we didn't exclude COVID revenues at all, right? So, we only excluded the vaccination to show you the like-for-like growth. COVID revenues also had a lot of inbuilt hospital-based Pharmacy component. So, if you look at the volumes that these hospitals are doing now, we are quite, we know that the volumes are doing well. And even the surgical volumes are picking up, as Ms. Suneeta said. You have the rebate that for that in the base year. The base year had this hospital-based Pharmacy element in the COVID because COVID had more medications even on medical

discharges than some of the others. Which is why it looks at 4%, but as you go forward, you will see that it will start doing better.

Shyam Srinivasan: Krishnan, just the guidance of 70%. So, we also expect new hospitals to kind of reach that number over time, right?

Krishnan A: Yes.

Suneeta Reddy: Yes.

Shyam Srinivasan: Second question, and I can get back in the queue after that, is in the Diagnostic services. Non-COVID revenues have grown 52%. So, if you see some of the other listed peers have been struggling, right, 6%, 7% for them organically. So, can you also comment about the general chat around competitive intensity in the Diagnostic services space? Where are you able to grow? Is it network expansion that's helping you? Is there any sense of same-store growth for you as well, will be helpful?

Suneeta Reddy: Chandra Sekhar?

Chandra Sekhar : Yes, ma'am, I'll answer that. As you rightly pointed out, I think we are growing from a slightly lower base. So hence, we continue to have network expansion-led growth.

On a same shop basis, while it will not be exactly accurate, but thereabouts, the growth is over 15% at this point of time, 15% to 20% kind of range at this point of time. And we continue to keep growing our network, which will add because we are in the mode of market saturation. On competition intensity, I guess the competition intensity does affect us in the routine testing book, where we are seeing the fixed sale but then our own journey has clearly been on moving across the curve in terms

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of specialized and semi-specialized testing. And towards that, we have made arrangements to our testing menu apart from the routine basket. So, it does affect us, but we are having other strategies to reconstruct.

Shyam Srinivasan: And this 15% same-store growth, would it be volume price? Any color there?

Chandra Sekhar : Volume, we are holding prices, we are not dropping prices, but we are also not increasing prices.

Moderator: The next question is from the line of Harith Ahamed from Spark Capital. Please go ahead.

Harith Ahamed: For the combined Pharmacy business, you've disclosed posting the EBITDA of INR 167 crore. Will you be able to share the pre-Ind AS EBITDA for the business for the combined Pharmacy business?

Krishnan A: Sorry. We'll come back to you. We don't have it with us now.

Obul Reddy : We will share with you offline.

Harith Ahamed: Yes, sure. And on the Amazon partnership, can you provide an update on the current number of orders per day or the GMV from that alliance? And the revenue with Amazon, where exactly does this fit? Is it entirely in the front end? Or is there an impact of this on the back-end Health Co numbers as well?

Sanjiv Gupta: So, Amazon, I think we are on the path of building of the entire tech integration between their systems and our system. So, we are still hoping to complete this by December. So, I don't see that any challenge with respect to any delay over there. So, I think once the entire customer journey is integrated between the two platforms, then probably the numbers will also, or the orders will also start going up because Amazon will also start doing a little bit of (inaudible) and the other digital initiatives. At this stage, we approximately get about 1,200 to 1,500 orders per day. And this is essentially in places like Mumbai and Kolkata where we are doing the pilots. These pilots are to just ensure that the integration has happened well and the experience on end-to-end from booking to deliveries is as for the standard. So, this is a pilot stage, number 1. Number 2, yes, as far as the sales is concerned or the revenue side is concerned, revenue gets booked into front end, which is the Apollo Pharmacy Limited. And

to the extent, 80% of that gets booked into Pharmacy distribution in Apollo Health Co.

Harith Ahamed: And the revenue share with Amazon. That is entirely at APL or is there a component of that in Apollo Health Co as well?

Chandra Sekhar : Yes. So, revenue share is only between Apollo Pharmacy Ltd. and Amazon.

Harith Ahamed: And last one from my side on AHLL. The INR 100 crore revenue that we have in the Diagnostics business for the quarter, is this reported net of revenue share with Apollo 24/7?

Sanjiv Gupta: Yes.

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Harith Ahamed: And then there's a decline in margins on the primary care side, the quarter-on-quarter decline, which is quite sharp. We used to have mid to high-teens margin run rate in that segment and this quarter, it's at 8%. Any particular reason that you would want to call out?

Obul Reddy : No, there are some one-offs, and this particular fixed cost enhancement is not going to remain. So, we will revert back to the earlier trends. Usually, fully to this quarter, but we will, we have the visibility on getting that correct.

Moderator: The next question is from the line of Kunal Randeria from Nuvama. Please go ahead.

Kunal Randeria: So last quarter, your surgical revenue, I think, was around 68% of the total revenue, and I think it was around 60% during COVID time. So, could you share what would it be in this quarter? And what is the headroom that you see?

Krishnan A: It would be in the same range; it would be a bit lower this time because we had some medical volumes also. There was a surge in some medical percentage overall. But it will be around the 65% range.

Kunal Randeria: And should we maybe in the next 1 or 2 years, should we expect it to be around this level?

Krishnan A: Yes, yes. It will continue to be at this level. There could be an upward bias also on this, especially when the new hospitals in particular, the surgical mix goes up, which is the focus. Because there are most mature clusters, if you look at places like Chennai, they would be closer to 75%.

Kunal Randeria: And as far as the payer mix goes, has there been any sort of material improvement from pre-COVID days?

Krishnan A: From pre-COVID days, yes, it has definitely been, it has improved significantly because both the retail, which is a cash payment as well as insurance has seen a good uplift. Insurance is over 42% of our revenues now, which was closer to 35%, and that is a good uplift on that. The overall institutional business has also come down. So, which is what is visible from the pre COVID days. And now we are stable versus Q1.

Kunal Randeria: And just lastly, I think you did mention what the online discounts for medicine are currently. But we have seen your online payers discount levels at around 18% to 20%. So maybe let's assume 6 months or 12 months down the line, this is the norm for the industry. So, should we also expect your discounting to go up?

Sanjiv Gupta: No, I think more than discount, the customer is also looking at the other aspects of the order, which is to do with no split, which is to do with getting the entire SKUs in one go and the right quantity, and it's the right time. Apollo 24/7 is the pioneer in delivering the entire order thing to us, and that is what got copy-pasted by competitors out there. We strongly believe, and this is what customer retention ratios, and the data also suggest is that while we are low on discounts, the competition gives 20%, 22%. We are managing between 17.5% to 18%. Customers are not walking away from our platform. And I think, we do not believe and that our discounts will go up. In fact, given that our service levels are extremely well, and customers are loving it,

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customers have, there's a very high repeat. There is a possibility to maintain or lower the guard by maybe another 50 basis points. That is how we look at as far as the discounts are concerned.

Moderator: The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: My first question is, can you update us on the International patient segment. How has been the pick up, where it has come back compared to pre COVID level, etc.? And how do you see it moving ahead?

Suneeta Reddy: So, it has not yet reached the levels of pre COVID. What has happened is that in some cities, it's back. But having said that, I have to say that connectivity has just about opened up, and we hope to see in quarter 3 and quarter 4, much better. Currently, it's at 6% of revenue and our target was 10% of revenues for this year, moving on to 15% for next year. But in terms of revenues, it is ahead by 27%.

Damayanti Kerai: 27% compared to what level?

Suneeta Reddy: Pre-COVID. In terms of revenue.

Damayanti Kerai: My second question is on your AP in Telangana cluster. So there, what we have seen in the past two quarters, performance has been muted. This quarter, revenues declined 15% and ARPOB around 20%, whereas the IP volume continued to grow at 15% and occupancy also improved. So, can you just elaborate like what is happening on that particular segment?

Krishnan A: So there has been a pruning on some of the institutional cases there in that segment because in that region, it's where we have more bulk of the State Government cases, etc., there. And we have been pruning it down to improve on the profitability overall. That has been part of our plan also, which is why you're seeing that come off.

Damayanti Kerai: Okay. So, when you are likely completing this pruning process and then coming back on track?

Krishnan A: It's already done now. It's already done. If you look at Q2 overall net revenues, net revenues have gone up versus the Q1. In this sector, it was INR 315 crore in Q1, now it's INR 356 crore in Q2. So clearly, now it's well behind even the ALOS is back in the Company level of 3.6 approximately.

Damayanti Kerai: And my final question is on your cost trajectory for 24/7. So, did you mention you will continue at around INR 1.5 billion for next few quarters? So earlier, you mentioned this quarter, somehow here costs will be INR 5.4 billion. So, it will likely exceed, right? And then if you can comment on the ESOPS also like what are these charges related to?

Sanjiv Gupta : I think as far as the expenses side are concerned, if you just refresh what we discussed last time, last quarter we had a INR 1,000 crore of our outlook for the year, which we increased to INR 1,500 crore of GMV. And we marginally increased our expenses by about 15% to 20%. We thought that this is the time to build the sales , this is the time to invest a little bit more behind people as well as the technology. And

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I think , we strongly believe that we should be able to close the year with less than INR 600 crore as far as the overall expense line is concerned. So, I think between INR 575 crore to INR 600 crore that should be the number for the expenses as far as 24/7 operating cost is concerned. Second, on the ESOP s, so this is a phenomenon with all the startups where the leadership team as well as some of the key resources in the Company have got ESOPs, the team was approved, and we gave ESOPs on the 1st of July. And this cost is continuing to the 2 months. This is a s per accounting standard; this is a noncash expense and INR 22 crore is the charge for the last quarter.

Damayanti

Kerai: And will similar level will continue or this is something one -off?

Sanjiv Gupta : So, this will continue. This is a charge over a period of the vesting period. So, I think this will continue quarter -on-quarter.

Moderator: The next question is from the line of Lavanya Tottala from UBS. Please go ahead.

Lavanya Tottala: Thanks . Good set of numbers. And my question is on Apollo 24/7 and the overall GMV. So, can you help us understand what is the split of Pharma and Di agnostics and Consultation in the overall GMV for?

Sanjiv Gupta : For the quarter? Yes, that's okay. So out of INR 300 crore or INR 290 crore, we have INR 240 crore for the Pharma and Diagnostics and Consultation is about 25% and 30%.

Lavanya Tottala: So here, the revenue of INR 158 crore, it includes online distribution for Pharma and the take rate for all the segments or only Pharma for 24/7?

Sanjiv Gupta: Yes, the first one. So, it includes the take rate for all the sectors plus the online Pharmacy distribution.

Lavanya Tottala: And also on the employee cost, there is a sharp hike in employee cost. So, is there any one -off in this particular quarter? Or is it something like the same level which is going to sustain in the upcoming quarters also?

Krishnan A: Looking at the consolidated numbers, is it?

Lavanya Tottala: Right. For the consol numbers.

Krishnan A: Consolidated numbers two things, right . One is we have the increment setting in, in the current quarter from July across the Company . So typically, that is when the increments get factored in. So, the results that we have given is after factoring in the impact of the increments, et c. And also, the 24/7 ESOP charge is part of the employee cost, which is the INR 22 crore, which is a noncash charge, which we have already told you, which will continue to be there beyond this quarter for at least a couple of years.

Lavanya Tottala: So, this ESOP related cost that will be there for how many quarters, any guidance there?

Krishnan A: It will continue for 3 years.

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Lavanya Tottala: Okay. For 3 years. Okay. So, employee cost will sustain at this level overall for the consolidated level, right? So, there's no one -off?

Krishnan A: Yes.

Lavanya Tottala: So AHLL, you mentioned there is some one -off related to the primary care business. So, can you quantify that? Any broad number? And how do you see the margins moving for the overall AHLL segment in the coming quarters?

Chandra Sekhar : The current aggregate AHLL level, we are seeing the margin at about 12%. If you look at it and break it across various segments, Diagnostics is in the 12% to 14% mark. And primary care would be around the 4% to 5% mark at this point of time if

you exclude those one-off additional expenses that we've incurred. And these are on account of primarily a ramp-up of network, which essentially drew some, we have set a very robust network growth, which resulted in some additional hiring and additional costs, which will even out in the coming quarters. So, we will get back to the 5%, 6% mark and hopefully increase it up to the 10% mark in the primary care level. Specialty Care is delivering in the return of 11% to 15% and will continue to do so at an aggregate level, we're hoping to inch up the 12% to the 15% mark and onward. Our margins in the diagnostics are slated to continue to be at this level for a while as we keep growing and investing. So, but then we will aspire to reach the industry benchmarks in the 20% plus in the years to come. But we are currently in the next 2 to 3 years will be consistently growing our network and hence, there will be some evening out on our EBITDA margins.

Lavanya Tottala: So, if I understand right, at the overall AHLL level, the margin is likely to remain at

the same?

Chandra Sekhar : 12% rate will improve a little bit to the 15% mark, we are hoping to improve it to the 15% mark.

Lavanya Tottala: And on the health care business, how are you seeing this current ongoing quarter going on? Do you see any seasonal weakness? Or how is it going like overall hospital business?

Suneeta Reddy: So, the trend generally is you know that the holidays, the 4 holidays that were in October, I mean 4 days, so October was somewhat muted, but we're getting back on track in November and December should do really well.

Lavanya Tottala: And my last question is on institutional patients. You mentioned that you're consistently reducing that like in Telangana and AP. So, is it completely done? Or you are still working on reducing institutional patient proportion on the overall base?

Anywhere else is any work going on or reducing?

Krishnan A: It's an ongoing process across the Company. We will keep looking at that options across because, especially with some of these newer perspective from CGHS where even Pharmacies are getting discounted, it may be difficult for us to sustain that in some of the places where we give CGHS prices even for some of the PSUs. So, we keep looking at it and seeing whether there are opportunities to prune that. In AP and Telangana, bulk of it behind us is what I meant.

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Suneeta Reddy: There's only 1% of our revenue that is coming from CGHS currently.

Lavanya Tottala: So overall institutional, what is the percentage of revenue at this point of time?

Suneeta Reddy: CGHS is 1%. State Government scheme is 2%, which is also like Ayushman Bharat public sector is 7%.

Moderator: The next question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: One on the mature hospitals margin. You were already at 27%, 28% margin, if I look at the first half numbers. Is this number sustainable? And we've managed to grow it year-on-year quite well. So incrementally, how much of this is dependent on ARPOB improving given where capacity utilization already is for the mature hospitals?

Suneeta Reddy: So, I think there are three factors that come into play. The first is case mix, and as we move to a higher-end case mix, ARPOB improves. The second, I mean, if you are thinking about ARPOB and margins improve. The second is that when you look at margins, when we increase institutional patients, we will continue to see an improvement in margins. The third, of course, is cost. And we are looking at 100 to 150 basis points improvement coming from cost over the next 18 months. So, there will be a structural improvement. And all this will contribute to definitely a growth in a 150 basis points growth in margins over the next 18 months. And of course, occupancy matters because it certainly leads to better margins and better revenue and better profits.

Neha Manpuria: And second question on the ARPOB, our metro ARPOBs of over 60,000. Is there, other than case mix, does this improvement include any price adjustments that we have taken during the year. Is that done for the year?

Krishnan A: Yes. It's taken and done.

Neha Manpuria: So that's factored into the numbers that you're seeing in the quarter?

Suneeta Reddy: Yes.

Neha Manpuria: And my last question on Apollo Health Co. Ma'am in your interview this morning, you mentioned about fundraising in Apollo Health Co by the end of this calendar year. If you could give us an update on that?

Suneeta Reddy: The financial year for, not the calendar year, but it would be more of the calendar year, which is March 31st, we will be able to do it.

Neha Manpuria: So, there would be fundraising in the digital business by the end of this fiscal year?
Suneeta

Reddy: Yes, fiscal year.

Neha Manpuria: And that would mean our ability to increase the spend in the digital business in FY24? Would that understanding be right?

Suneeta Reddy: Sanjiv?

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Sanjiv Gupta: No, I do not think that the fund raise will mean that we need to spend more. I think we have our own path to profitability, and I think we are on that course. By no means the fund raising is going to increase our expenses. With the funds we might look into some kind of an acquisition because that is what we have been thinking through to speed up the process. But yes, no increases in expenses side.

Moderator: The next question is from the line of Sameer Baisiwala from Morgan Stanley. Please go ahead.

Sameer Baisiwala: Just taking from the previous participant, any change in the earlier planned fund raise, which was roughly \$500 million kind of an inflow?

Suneeta Reddy: Yes, Sameer, I mean, across the world, no one is raising that much money at the current market price, which does not factor in the true value of the assets or the revenues. So, we are only looking at \$200 million right now.

Sameer Baisiwala: And what happens to the slump sale INR 1,200 crore flow back into the parent Company?

Suneeta Reddy: That will be in the next tranche at a better valuation.

Sameer Baisiwala: And the second round would be any time in the foreseeable future? Or that's something for the later?

Suneeta Reddy: You know, I'll give you a number now, you'll hold me to it, a number and a date. But let's cross this first threshold. And then you will see value build up in 24/7 because it is the fastest -growing health digital platform. And we have a lot of interest. Having said that, I do have to say we have a lot of interest. It's for us to take it at the right valuation and at the right time.

Sameer Baisiwala: And the second is on the brick -and-mortar expansion in Pharmacies. So there, I think you've done 450 plus kind of store adds in first half. I think our full year target was 500. So, if you can just talk about it. And thanks for the math that you have provided. So, looks like the Central India is something that you have not tapped at all. So, your thoughts on that?

Obul Reddy: We don't, we are now planning about 800 stores expansion for the current year. And part of the new source, we will move into, except in two States we are there everywhere. Madhya Pradesh is the only place where we don't have presence. And in other states, we just started, and we will ramp up in the next two years.

Sameer Baisiwala: And just on the hospital side, what's the, I mean, good to see ALOS now stabilizing around 3.4%. So that's really commendable. And do you think you're going to sort of maintain at this level? And second is on occupancy. Practically speaking, at a hospital level, what's the sort of upper limit to where you can take it?

Suneeta Reddy: So, first on ALOS, I think 3.4 is a reasonable ALOS. But because we've introduced robotic surgery, we're seeing ALOS come down. So, and 25% of our business, it's only about less than 2 days. So as this grows, as the segment grows, we do believe there will be a further reduction in ALOS. But this will take another two years to see significant improvement or anything that we can really talk about. Occupancy, I think

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Apollo is probably the only one that has headroom to grow. We, you can easily take it up to 82%, 83%. And we believe that we are working towards this. We have 2,000 beds that we can fill. And like I said earlier, whatever we do is margin accretive profit, good for EBITDA, good for re

venue and profitability. But our target this year is 70%

on a group -wide basis, and I think we're moving close to achieving that target.

Moderator: The next question is from the line of Shaleen Kumar from UBS. Please go ahead.

Shaleen Kumar: Yes. I think most of my questions have been answered.

Moderator: The next question is from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal: First question is on, there was a news flow on the CCI for few of the hospital names.

Could you confirm that Apollo also received that? And if yes, what is our cost correction here? And if not, I just wanted to understand how should we build ARPOB?

You gave a good clarification on case mix, payer mix, et c. But how should we build in ARPOB for the next 2 years?

Suneeta Reddy: So, on the CCI thing it's only relevant for enterprise staff where Apollo Hospitals holds 25% shareholding. With regard to ARPOB, definitely, the way that we look at it is it's a function of case mix. It's a function of payer mix, and we've shown an improvement in payer mix. And the third is ALOS. So, I think these three levers are what we use. International patients coming back will definitely show an improvement in ARPOB.

Prakash Agarwal: And in line with inflation, what is the price hike we...

Krishnan A: 7% in that was a part. And I think with the inflation and this broadly around the 7% is fine to move it.

Prakash Agarwal: But I was trying to understand on the price hike that we annually take it. So, inflation is now high single digits. So, do we take it in line with inflation or ours is a gradual 3%, 4% kind of annual hike on the pricing part?

Suneeta Reddy: On the health care inflation, it's 5%, and we've absorbed that in the price.

Prakash Agarwal: And secondly, some more color on the brownfield and greenfield. So, you talked about Gurgaon which is 24 months out. But what are the mid - to near -term kind of opportunities we are looking at, especially on the brownfield side, if any?

Krishnan A: For us, unlike some of the others, we clearly have headroom for growth in our existing businesses itself. With the creation that we have already done on the assets and 68% occupancy that we have; we can clearly take that to the 72% and 75%. Proton is a big opportunity. It's doing well. We can take it, we can double the revenues in Proton over the next couple of years. We still have high potential in taking New Bombay higher. So, all of that itself is significantly higher. While brownfield is one that we are looking in Bangalore, in particular. But with that and in Mysore, and also Bhubaneswar. Some of those are opportunities which will come earlier. But with that more than that, our focus is to increase the revenues in the existing hospitals, look at seeing how we can grow oncology further. We are the largest oncology revenue in the country today. We would like to further grow that. So, we are focusing on some

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of those specialties to see how growth can come within the health care services revenues itself at a faster pace.

Suneeta Reddy: So just in addition to that, we are adding, I mean if you look at Gurgaon, Bangalore, what we're doing in Chennai and you know what we plan to do in cluster, we are adding 2,000 beds over the next 3 to 4 years at a cost of INR 3,000 crore.

Prakash Agarwal: And some more color on this 24/7 kind of funding, etc. You talked about plans for around \$200 million now. So, is it that the stake sale on the entity would be around 10% or it is now reducing?

Krishnan A: So, it would be roughly around that or lower, right? Because if you look at it, our focus is to get, we continue to be focused on the \$2.5 -plus billion valuation that we have been talking of. And even

even though valuations across have come down, there our delivery has far outperformed what we had in our plan.

Prakash Agarwal: That's what I was trying to get. So, it would be around 2.5% plus, right?

Krishnan A: Yes, that's the plan.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: On the off line Pharmacy business, since we've upped the expansion target now, how should we look at the network size, say, in another say by FY25?

Obul Reddy: We should continue to add at least 500 stores per annum, and we'll see that we'll be about 6,500 stores by '25.

Nitin Agarwal: And sir, by that time, what kind of EBITDA margin should be there for the off line business?

Obul Reddy: As long as the expense continues, we will have some impact on the cost impact on the profit. We expect to be around 6% on the combined business, 6% to 6.5%.

Nitin Agarwal: And just pure on the off line piece itself?

Obul Reddy: Yes, I'm talking of offline.

Nitin Agarwal: So, we are currently at about 7.9%.

Obul Reddy: That is on the back end, what you are talking. What I said is about combined business. If back end continues we'll be at the same margin. Whatever impact we have on account of expansion will be on the front -end side.

Nitin Agarwal: And likewise, on the online business, the GMV target that we have for this year, about INR 1,500 crore thereabout that we are well set to exceed, how should we look at this number over the next couple of years?

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Sanjiv Gupta: Yes. So, I think what we are targeting is that the next year should be in the range of about something like 3,000 to, I mean at least we should do 2x, if not 2.5x to 3x of the current year numbers. The idea is to get to about \$1 billion in 2 to 3 years' time.

Nitin Agarwal: Okay. And Sanjiv, what does that mean for the EBITDA of the Apollo 24/7 the whole , the piece on the online Pharmacy distribution on 24/7, when you start hitting \$1 billion number in say, 2 to 3 years?

Sanjiv Gupta: I think 2 to 3 years, once we hit this particular , okay, let me just give you a little broad sense. One is that at this scale, when you hit about INR 7,000 crore to INR 8,000 crore, you are able to make the digital business, which is one section of Apollo Health Co. This business becomes breakeven or in fact, it starts giving you a little bit of profit. While the back -end business, which would be roughly INR 4,000 crore in the Apollo Health Co . that should give you about 8% to 9% as , or maybe 10% at best as the EBITDA. So, I think those are the , that is the mental model in which we are working that next year, we should make Apollo Health Co as near breakeven and FY24, '25 should be the year when we should strive to hit a scale of about INR 7,000 crore to INR 8,000 crore of GMV and make the digital segment also breakeven. And obviously, the Pharmacy business continues to grow and continues to show a healthy EBITDA margins of somewhere 9% to 10%.

Nitin Agarwal: Secondly, on the hospital piece, what kind of capex are we looking at the health care services business? Obviously from a greenfield perspective, the only project we've discussed so far has been the Gurgaon project. What are the other projects and the kind of investments we're looking at on the health care services part of it?

Krishnan A: The recurring capex that we have, which is already known to you is around INR 350 crore a year. That is what we will continue to work on. This is the recurring capex . And the basis, the 2,000 -3,000 beds that we 2,000 beds and the INR 3,000 crore that we are saying, broadly, it would be split across 3 years.

Nitin Agarwal: But sir, in that baring Gurgaon, which are the other projects we're talking about?

Krishnan

A: So, we are looking at OMR, which is in Chennai. We are looking at , hopefully, we will come soon to you on a couple of other projects that we are working on. So, we are looking at Bangalore, as we have said. We are looking at Bombay. So, we will come back on some of those soon.

Nitin Agarwal: And if I can squeeze in one last one on the diagnostics, what is the geographical spread for a diagnostic business? So, when it goes to INR 1,000 crore number in the next couple of years that we talked , about 2 to 3 years. I mean, what , is it going to be a very South India focused business predominantly South India focused business from a geographical footprint perspective or broadly pan -India?

Chandra Sekha r: Yes. I will answer that. The current revenues are largely between South and East and getting into other zones minutely. We have made entries into the Bombay and the Delhi markets over the last 12 to 18 months only. So, their contribution to the overall current revenues are miniscule in small single -digit percentages. We expect South and East to be a large contributor. South and East should be around the 70% mark with 30% coming from other markets, including some Central India, West in terms of Bombay and North India. That will be the mix in the next couple of years.

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Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Sir, if you could share EBITDA for Proton?

Krishnan A: EBITDA for proton, we don't have it offhand now, but it will cross over INR 60 crore, INR 65 crore in this year.

Tushar Manudhane: So, if I kind of do some kind of math, back of the envelope calculation. So, the new hospitals' EBITDA margin has been very stable for the past three quarters, while there has been a good improvement on the mature hospitals. So that is where I'm coming from that the new hospitals' margin, how do we see the improvement in that segment?

A. Krishnan: You're right. And Proton will be one of the big contributors there, too, because clearly, Proton, the potential of EBITDA is almost INR 200 crore, which is why I did say that over the next couple of years, you will see good fillip coming from Proton itself apart from some of the other hospitals that we are working on in New Bombay and Vizag. These are two hospitals that have good potential compared to where they are delivering today.

Tushar Manudhane: And on the Pharmacy side, the private label share has also been very stable at about 10%, 10.5%, 11%. So how do we plan to ramp up there, scale up there?

Obul Reddy: We will see that scaling up because this is coming on the background of COVID, in COVID time, there were a lot of products which were contributing to higher sales, and it was about 12.5%. We are at now 11%. We could see now growth coming back in the normal volume.

Tushar Manudhane: So at least in a year's time, we should be back to pre -COVID levels and then

subsequently the further improvements?

Obul Reddy: Definitely, yes. I mean next 12 months; you'll see that we'll be at about 12% to 13%.

Moderator: The next question is from the line of Naushad Chaudhary from Aditya Birla Sun Life.

Please go ahead.

Naushad C: Most of my questions have been answered, some clarifications, sir. Firstly, on the discount trajectory last quarter, if I remember it correctly, we had indicated that in last quarter, it was around 18%. We had indicated that we had taken 1% cut in July month, and we were expecting some 0.5% cut. And eventually, we had guided around 16.5% to 17% in Q2FY23, but as you said in some of the question that it was at 18%. So, we see this number in next coming quarters or can you comment on this?

Sanjiv Gupta: Yes. So, see the discount

it has a factor of lot many things in the , and then obviously you understand there is an external environment, which is kind of decide at some point of time, what should be offered. I think, yes, we wanted to , we brought it down for some period to be honest with you, we brought it down for some period to the level that we mentioned in the last earnings call. But see, whatever is the best for the organization and for the consumer side, I think we all should do all the time, right?

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So, while lowering it down for some time, we realize that at least from the chronic patients, chronic patient is something which is a valuable asset to the organization and 0.5% here and there does not kind of , I mean, is it that chronic customer moves out of the system is much more harmful to us versus 0.5% here and there. And you've got the opportunities for cross pollination and various other channels which are coming into the Company . So, while we lowered it down a little bit, we further increased it to maintain at 18%. As I said earlier, 0.5% we can still play, but we'll be mindful to the external environment and we take call, which is good for the customer. You may expect that we retain 18% for another two months. At this stage, we are still doing 18%. So , but yes, I mean, it is something which is a moving needle. We don't know what , how exactly the cohorts and the various other things behave. But 18% or at best 0.5% down.

Naushad C: Secondly, just a suggestion, sir. This time, we have you know though we have given the 24/7 numbers in the presentation, but as last quarter, we reported in our segment reporting rate given the separate line for 24/7. And just wanted to understand what is the rationale this time , we have not given and we have clubbed it into a Pharmacy distribution segment?

Krishnan A: This is , what we have given now is the full online Pharmacy distribution and the Apollo 24/7. This is the correct representation of the 24/7 business because the 24/7 business is getting the INR 294 crore GMV. And what we have now represented as INR 158 crore is what gets reported under the Apollo Health Co for the businesses generated by 24/7. So, this would be a combination of the Pharmacy back end, and it will have the service lead gen fees also, which is part of this. So that is how it will keep increasing, and this is the right representation of the 24/7 business to see how the profitability moves. And this is what clearly even Sanjiv guided as to how this will, over a period of time, the GMV can go from INR 294 crore, which is an annualized INR 1,500 crore to INR 1 billion and how that will be 65% Pharmacy and services, etc.

Naushad C: All right. And lastly, sir, if I'm reading it correctly, if I see sequentially in Pharmacy distribution, we have seen some margin decline on a , because last quarter, we had indicated around INR 20 crore, we had some onetime costs in some infra building for Amazon deal, which I'm assuming would not be in this quarter and be removed 24/7 losses. So, net-net on the core business, we have some sequential margin decline. If I'm reading it correctly, can you explain on that part, sir?

Obul Reddy : If you see that Q2 EBITDA level versus Q1, it's just about 10, 15 basis points, as Mr. Akhileswaran Krishnan said, we also have the employee cost coming in slightly additional in Q2 being the, where we give increments. And then we have about 10, 15 basis points, which is negligible, and we will be back with improvement in the volume.

Moderator: The next question is from the line of Yash Shah from Investec India. Please go ahead.

Yash Shah : So, my first question was regarding the GMV. In first half, we've done approximately INR 5 billion of GMV, and we are looking , we are target

ing about INR 15 billion , INR

1,500 crore for the whole year. So, the rest INR 1,000 crore in second half, could you give me some idea of like how are we targeting double of what we've done in H1?

Sanjiv Gupta: Yes. So, as you mentioned in the earnings call, there are new business lines that we're creating, which is around the complication led to hospital IP OP services and the entire integration of software and touching that 24/7 will be hospital (inaudible) management system. So, all that was done during Q2 and then some part of Q1. Now IP OP is one particular service, which we started in that as part of the Company was (inaudible) require that. And the current funding that happened in October or what it was in October and some part of November, suggest us that I think this particular vertical itself, will give about INR 400 crore to INR 500 crore of additional (inaudible) for the balance part of the year. And I think the current set of three verticals, which is the ePharmacy, diagnostic and consultation, they would contribute to the group. We grew about five times in Q2 versus Q2 of previous year and about 36% versus the previous quarter. So, and then lot many levers that we've still not checked, we do something like we thought hyperlocal delivery in top 6 cities, which are increasing to 17 cities that would fuel the growth. And there is a lot of work that is happening on the other segment. So, I guess the October as well, October GMV as well as the current run rate suggest that you would easily get INR (inaudible) crore mark.

Yash Shah: My next question was regarding take rate in Pharmacy and Diagnostics business. As our volumes have increased, sir have we been able to increase the take rates on both these, in both Pharmacy and Diagnostics as compared to previous quarter?

Sanjiv Gupta: So, the Diagnostic take rate is basis the slab that we hit. So, this is any other commercial discussion that happens between the two organizations. You have a slab-based take rate. The minute you start in the upper slabs, you start getting more and more commission. And yes, the diagnostic business is increasing quarter-on-quarter. So, it is governed by the actual performance, and we already locked in the entire commercial deals till FY23. And based on the run rate in February of '23, we will discuss the take rates for the next financial year.

Yash Shah: Sir, one very small clarification question regarding the capex. As we've guided 2,000 beds in the next 3 years, of which 675 beds will be in Gurgaon and 500 beds will be Chennai the multispecialty. So, between Mumbai and Bangalore, which we are planning right now, it will be another 1,000 beds. Is the understanding correct, sir?

Krishnan A: So, 675 would not be Gurgaon. Gurgaon will be more like 500, 550 beds to begin with. And 500 of OMR is broadly what we are looking at. Bangalore, we have the brownfield Bangalore expansion, which should help us get another 250, 200 to 250 beds. And the one in the other hospital in Bangalore, we are looking at 400 beds.

Yash Shah: So that leaves Mumbai with 400 beds?

Krishnan A: Bombay would be another 400 beds in our plan.

Moderator: Ladies and gentlemen, this will be the last question for today, which is from the line of Rishabh Tiwari from Allegro Capital Advisors. Please go ahead.

Rishabh Tiwari: I had a clarification question regarding the Proton EBITDA. It was quoted around INR 60 crore. Is it for the quarter or for the half year? Because the last reported number that I can see for Q4 was around INR 14 crore.

Krishnan A: So, it

will be for the full year, it would be almost around, so for the half year it is I think it would be trending at around INR 65 crore I said that didn't include the Pharmacy, so over INR 75 crore to INR 80 crore is what the overall target for the year will be.

Rishabh Tiwari: For the Proton, right?

Krishnan A: That's correct.

Rishabh Tiwari: But the current, can you give me a ballpark number for the current that we are trading around half year? It would be half of Rs. 85 crore?

A. Krishnan: INR 40 crore. That's correct.

Moderator: Ladies and gentlemen, this was the last question for today. I now hand the conference over to the management for closing comments.

Suneeta Reddy: Thank you, ladies and gentlemen, for taking time out for this call. We hope that we have convinced you that Apollo is the only integrated health care player in this part of the world. Our hospital business is on a very strong growth trajectory and will continue to grow. We will improve not only in revenues but margins but most importantly in the type of clinical offerings that we give to our patients.

Apollo Health and Lifestyle, which is our primary care vertical, the focus on diagnostics and primary care clinics will continue. And this will give us, it will enlarge our patient base, which will increase revenues and therefore, margins. Apollo Pharmacies, which is our retail play is something where we've demonstrated strong

growth, and they're on track for a INR 10,000 crore revenue target. All of this comes together with 24/7. And I think that the losses that we made here, which are close to INR 170 crore, you must think of it as an investment into the future because there can be no health care institution without a digital platform. And I think that we have made the right decisions, and we continue to be optimistic and hopeful in our mission to really move health care closer to the patient to create accessible health care and to be sustainable. So, thank you very much for taking time out for this call. Good afternoon.

Moderator: Thank you. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

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Call: Feb 2023

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Apollo Hospitals Enterprise Limited
Q3 FY23 Earnings Conference Call Transcript
February 15, 2023

Moderator: Ladies and gentlemen, good day and welcome to Apollo Hospitals Limited Q3 FY23 earnings conference call. As a reminder, all the participants' lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you and over to you, sir.

Mayank Vaswani: Good afternoon everyone and thank you for joining us on this call to discuss the financial results of Apollo Hospitals for Q3 and the 9 M of FY23, which were announced yesterday. We have with us today the senior management team comprising Ms. Suneeta Reddy – Managing Director; Dr. Hariprasad – President of the Hospitals Division; Mr. A. Krishnan – Group CFO; Mr. C. Chandra Sekhar – CEO of AHLL; Mr. Obul Reddy – CFO of the Pharmacy Division; and Mr. Sanjiv Gupta – CFO of Apollo 24/7.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide # 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated and these have also been uploaded on the corporate website and the websites of the respective stock exchanges.

I would now like to turn the call over to Ms. Suneeta Reddy for her opening remarks.

Thank you and over to you, ma'am.

Suneeta Reddy: Good afternoon everyone and thank you for taking time out for this earnings call. I hope that you have received the earnings documents which we had shared yesterday.

We have delivered a steady performance in Q3 FY23 with the Healthcare Services business growing at 10% year-on-year excluding vaccines. Growth was driven by an increase in IP volumes which were 7% higher year-on-year and supported by pricing and case mix gains. ARPOB has increased 12% year-on-year and it is currently at Rs. 51,482 in Q3. It is important to note that we had reported 41% revenue growth last year Q3 FY22 which was aided by COVID revenues and vaccination revenues, which was ahead of the industry growth. In the light of the high base, the 10% year-on-year growth in Healthcare revenues is meaningful and adjusted for nonrecurring COVID revenues and vaccine revenues on a like-to-like growth in the Healthcare Services business is at 17% year-on-year.

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Q3 FY23 occupancy across the group was 65%. The Q3 FY23 occupancy in mature hospitals is 66% and new hospitals is 62%. We witnessed an improvement in the payer mix too as cash and insurance segments registered a year-on-year improvement of 28% in revenues.

Against this backdrop, let me take you through the consolidated financials for the quarter :

Consolidated revenues grew 19% on a year-on-year basis to Rs. 4,264 crore after normalizing for vaccine revenues in quarter 3 last year. Healthcare Services revenue grew 10% to Rs. 2,194 crore. HealthCo revenue grew by 34% on a year-on-year basis to Rs. 1,758 crore. Offline pharmacy distribution revenue grew 21% year-on-year to Rs. 1,581 crore. Online pharmacy distribution and 24/7 revenue was at Rs. 178 crore.

Combined pharmacy revenue grew by a healthy 31% on a year

-on-year basis. AHLL

revenue stood at Rs. 311 crore. Excluding COVID vaccination and testing, it grew by 30% on a year-on-year basis. Diagnostics core revenue grew by 62% on a year-on-year basis to Rs. 92 crore in Q3 FY23.

Healthcare Services EBITDA was at Rs. 543 crore, a year-on-year growth of 9%.

This was supported by volume, price, and case mix improvement. Healthcare

Services EBITDA margin was at 24.7%, a 15 basis points improvement over last year. Margins in mature hospitals were at 27.5% as against 25.6% in Q3 FY22. Margins in new hospitals stood at 18.2% for the quarter.

EBITDA for the offline pharmacy distribution business was at Rs. 124 crore for the quarter, representing a growth of 28%. The online pharmacy distribution and 24/7 business reported an EBITDA loss of Rs. 186 crore, resulting in a net loss of Rs. 63 crore for Apollo HealthCo. AHLL recorded an EBITDA of Rs. 25 crore for the quarter. Overall consolidated EBITDA was at Rs. 505 crore, reflecting the investments that we have made in the online business.

At the PAT level, Healthcare Services PAT was at Rs. 261 crore, a year -on-year growth of 30%. Consolidated PAT, however, was at Rs. 153 crore.

I will now summarize the performance for the 9 months ending December 2022 :

We closed the 9 months of FY23 with a consolidated revenue of Rs. 12,310 crore , a growth of 16% over the 9 months of FY22, excluding revenue from vaccination. This includes a revenue growth of 12% year -on-year in the Healthcare Services business and in AHLL and 23% growth in Apollo HealthCo. EBITDA stood at Rs. 1,561 crore. Within this, Healthcare Services EBITDA was Rs. 1,598 crore. EBITDA from mature hospitals increased by 25% year -on-year, with an increase in the EBITDA margin to 27.4% from 24.3% in the 9 months of FY22. PAT for the 9 months of FY23 stood at Rs. 675 crore.

In operating terms, the offline pharmacy business added 194 net new stores for the quarter, taking the network to 5,196 stores, with private label sales at 11.03%. The online pharmacy distribution in 24/7 platform recorded a strong GMV growth of 85% quarter -on-quarter to Rs. 543 crore. The platform added 3 million users this quarter, and its total users as of date is 23 million users. The platform witnessed around 42,000 transactions per day compared to the 25,000 transactions in March 2022.

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This has been a good quarter for our Healthcare Services business. Our occupancy and volume levels are robust and our margins have expanded. There is still room for growth in both the mature and new hospitals as surgical cases increased and international patients returned. We continue to focus on the centers of excellence, case mix, and payer mix as levers for revenue and EBITDA growth.

AHLL has demonstrated resilience in a quarter that was muted for the sector. Our penetration and growth plans for primary care and diagnostics will continue, and we will see improvements in EBITDA and margin profile.

Apollo HealthCo has done really well on the pharmacy distribution revenue and GMV growth and operating metrics. It is on track to exceed Rs. 1,500 crore of GMV this fiscal and well ahead of the information memorandum to deliver 2x growth in FY24. We believe we are at the peak burn rate this quarter and expect losses to moderate from here on. Margins in Apollo HealthCo will improve with the offline pharmacy distribution margins improving as well as discount rationalization, digital health services revenue improvement, and expansion of private label sales. We expect HealthCo to directionally be PAT neutral by the end of FY24 and become the fastest digital player to achieve segment profitability by FY25.

In the larger context, while individual business verticals are all delivering to plan, the

largest opportunity for us is the network effect. It's really leveraging the synergies across all the formats in both the physical and digital touch points that we have created. We are the only 360 -degree integrated healthcare ecosystem and our clinical proposition remains strong. I believe that we have put a lot of work into our strategy not just for this year but for the coming 24 months and we look forward to our next call to see it play out over the next 24 months.

Meanwhile, I have Dr. Hariprasad; our CFO – Krishnan; Obul Reddy; Chandra Sekhar from AHLL; and Sanjiv from 24/7 to take questions from all of you.

Moderator: We will now begin the question & answer session. The first question is from the line of Anuj Suneja from ICICI Prudential. Please go ahead.

Anuj Suneja: I have a couple of questions mostly related to 24/7 expenses. One, I would like to understand what is the breakup of the expense at Rs. 134 crore that we are doing. Is it just going to discounts or is there any other strategic initiative that is also being driven by this number? And going forward, you have given a guidance of Rs. 200 crore for H1. If you are giving the breakup right now, what would be the breakup six months down the line? That's the first question.

Sanjiv Gupta: Let me just talk about the breakup of expenses that was your first question. As we spoke in the last earnings call, we talked about that the Company is investing in the resources and tech and products side for the new line of businesses which it intends

to start. As you recall, pharmacy, diagnostic, and consultation are something which are the firing engines as of now. And during Q3, we invested into product and tech essentially and some bit on people resourcing for the new line of businesses. Essentially, that is where most of the incremental expenditure has happened. If I look at broadly about 30% of the expenditure is into product and tech side. We have about nearly 20% to 22% on the operations side. We do have marketing for acquisition of new customers to the tune of about 15% of expenditure, and something to do with the call centers and sustenance of communication and SMS and all that stuff another 15% to 20%, and the remaining expenditure happens towards the support side. So

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that's a broad breakup of the expenditure. And some bit of additional expenditure in Q3 is accounting for only related to the new line of businesses wherein we are putting money.

Anuj Suneja: And the second question is, we are reporting somewhere around Rs. 1,500 crore of GMV. What is the GMV to revenue bridge? And going forward, how would the EBITDA look like for this Rs. 1,500 crore of GMV? If we can have some thoughts on that.

Sanjiv Gupta: If you look at it, for the last quarter, we did a GMV of about Rs. 543 crore. And the revenue that comes to the books which is including of the online pharmacy and the take rate for the other services is about Rs. 177 crore. That is the ratio that we are looking at it. And yes, Rs. 1,500 crore, I think we should be doing more than Rs. 1,500 crore. Idea or rather the estimate at this stage is to hit around Rs. 1,600 crore, with another 2x growth for the next year. As far as the overall investment into the project is concerned, I think ma'am suggested this during her opening remarks also that we're looking at somewhere peak of the expenses as far as digital side is concerned. And next year in the Apollo HealthCo, somewhere in 3rd or 4th quarter, we are looking at coming to near breakeven.

Anuj Suneja: Any guidance on the EBITDA front? Say Rs. 1,500 crore to Rs. 177 crore of revenue. And beyond that, on EBITDA any guidance?

Sanjiv Gupta: As we get into the annual operating exercise for the next year and during this month

and first half of the next month, there will be a detailed exercise done on this. And I think in the next earnings call, probably we can guide you on the digital side of the EBITDA also for the next year.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Thank you for taking my questions; the first one on the Hospital business. Occupancies on a blended basis have come off 300 basis points. I just want to understand how should we look at the quarter 4? And I think we had an earlier guidance of reaching higher occupancies. If you could just reiterate or at least observe what those occupancies are and what are our targets for the next, say, 12 to 18 months?

Suneeta Reddy: October to December are generally holiday months and I think the seasonality of the business is something that we have to consider and which is why our occupancies were around 64%. But having said that, they have picked up in February and March, and we expect to see a higher occupancy rate. Our target was 70% for the year. We will be close to 70%, especially for the last quarter.

Shyam Srinivasan: Ms. Suneeta, I think we had a longer-term goal also, right? I recollect because....

Suneeta Reddy: Yes, what we said was that our intention is to take it up to 75%. We do have a target for the next 18 months where we will do that. In fact, it is baked into next year's AOP. And hopefully, we will be able to commit to absolute numbers after the first quarter of next year is over.

Shyam Srinivasan: The second question is to the previous participant's question on revenue to GMV. While it is 33% conversion on the 24/7 this quarter, last quarter was 55% and the

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quarter before was 56%. Is there any change in how we now get our revenue or the conversion from GMV? Is there any marketplace that makes the take rate lower relative to previous quarters? Is it like are we now starting to recognize revenue from, say, the Amazon platform that is making this take rate lower?

Sanjiv Gupta: That's a good call out. This quarter, we started the entire IP/OP business for 24/7 users, and typically, it's a question of mix. What I would broadly say is that between 30% to 35% should be the revenue to GMV ratio which we see in this quarter should be the percentage that we can see in the next 1 or 2 quarters. And as we get into more of the digital therapeutics and insurance distribution side of it, probably this

percentage will further go up. But for the current quarter Q4 as well as for Q1, let's assume in the range of about 35%.

Shyam Srinivasan: And Sanjiv, what is the mix of the revenue? If you could help us refresh that as well? How much from pharmacy, what is from consult? Because, the commentary seems to suggest that the growth has largely come from IP/OP, at least in the commentary on the presentation. If you could just give us a qualitative color, please.

Sanjiv Gupta: Yes, that's true. Most of the growth has come from the IP/OP business, this is the new vertical that we started in Q3. If you read through the Q2 transcript, we said that we would be investing into the customer journeys for the users who can come from Apollo 24/7 and then get the physical bookings as well as get the IP and the other things done. For Q3, a little distortion because a new vertical whenever it comes and starts firing, the percentage mix changes. But for Q3 if I talk about, pharma is about 50% and consultation together with IP/OP is about 45% and the rest is Diagnostic and VC, virtual consultations.

Shyam Srinivasan: And this mix for 1H was what?

Sanjiv Gupta: Mix of H1 was, about 80% is pharma and then we had about 10% of consultation and about similar 10% for diagnostic and the subscription

on and pay model.

Shyam Srinivasan: My last question is on the pharmacy business. Both in terms of network addition, just a simple observation; we used to add 350 stores annually in the past years, we have now added 757 in 9 months, but now the number of closures is also equal to like one-third of how much we were adding earlier. Is there any change in the philosophy? Why are we closing more stores? Maybe it's a percentage of new stores added, but I just wanted to get the philosophy there. And also a data point on what is the combined pharmacy EBITDA margins? I think you were disclosing it until last quarter, I can't see it in the presentation for this quarter.

Obul Reddy: This year, we embarked on additional stores and expect to open for the full year about 950 stores, and we closed around 100 stores which is more on the basis of identifying them as nonperforming, not that it is as a percentage to the store additions. And on the EBITDA, it is about 7.38% on the pharmacy business, slightly about, say, 50 to 60 basis points lesser than last quarter and largely because of the establishment costs on the addition of stores. And the growth coming from these new stores in the next 2 quarters, that will considerably change.

Moderator: The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

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Damayanti Kerai: My first question is on your Hospital business. How much is International patient contribution during the quarter? And how do you see this piece moving, say, next few quarters?

Suneeta Reddy: We have moved from 5% last quarter to 7%, and our focus is that it will be 10% in the last quarter.

Damayanti Kerai: 10% by the end of this fiscal?

Suneeta Reddy: End of this fiscal, yes.

Damayanti Kerai: Ma'am, my second question is on hospitals again. We have seen improvement in the AP & Telangana cluster which has seen a good pickup in the third quarter.

Among your all cluster hospitals, which are the segments where you see enough headroom to improve on the payer mix, etc.? I think last quarter you talked about AP & Telangana. Apart from this, which are the segments where you see you are not yet targeted payer mix payable?

Suneeta Reddy: So we are looking at some of the new hospitals which are now ready to ramp up. I think we had moved the whole strategy from being primary and secondary care to tertiary care. And with this offering of tertiary care, we should be clinically differentiated in these markets. So, you will see improvement in Vizag, you will see improvement in Madurai and Trichy, and Navi Mumbai coming through in the last quarter.

Damayanti Kerai: Ma'am, my next question is on your cost improvement plan for your business. You earlier mentioned some 100 to 150 basis point improvement over the next 18 months. If you can provide an update on that. And which are the key drivers for this cost improvement?

Suneeta Reddy: I think we are pretty consistent with what we hope to achieve. You will most probably see it at the end of 2023-24. What are the drivers for this; - clearly, we are looking at creating surgical packages, kitting it out. We are also looking at admin costs. We are looking at HR costs as well. But more importantly, I think the whole initiative is about improving the overall efficiencies within the system, so conversions happen at a better rate. It's also about growing revenues as well as cutting costs. So, I think it's a very crucial program for Apollo, and we will deliver on both revenues, volumes, and

EBITDA margins.

Damayanti Kerai: My last question is, can you specify at your network level, what is the contribution from cash, TPA as of third quarter in terms of your payer mix? If you can call out a number?

Krishnan A: Almost

around 80% is from cash and insurance.

Damayanti Kerai: And within that, I assume around 50% is cash and another 50% TPA. There is equal contribution or how is the status?

Management: Yes, equal contribution.

Moderator: The next question is from the line of Shaleen Kumar from UBS Securities. Please go ahead.

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Shaleen Kumar: Since, on 24/7, the nature of GMV has changed a bit, is it possible to get a like -to-like comparison of GMV? If I have to see my previous Q2 GMV versus Q3 GMV, how the growth has been?

Sanjiv Gupta: Q2 GMV was Rs. 295 crore and like -to-like comparison would be about Rs. 340 crore; that would be up by about 15%. That is like -to-like. Like -to-like here means pharmacy, consultation, and diagnostics.

Shaleen Kumar: But then, basically the new GMV contribution to revenue is quite less because if I look at the revenue growth, it's also somewhere around that, right? 12% to 15% kind of a revenue growth is there sequentially.

Sanjiv Gupta: Yes, the observation is right. And this was one of the questions asked previously also. Whenever a new engine fires, the entire mix undergoes change and that leads to change in the mix. I think it is all temp from that sense. But as we see another one or two quarters, we are coming out with new lines of the businesses, this will continue to be slightly different before it starts maturing on 1 %.

Shaleen Kumar: So, we intend to increase our contribution margin from that or there is a scope for that?

Sanjiv Gupta: Absolutely. That's the constant endeavor to reduce expenses and increase your top line as well as the margin.

Shaleen Kumar: Just one bookkeeping question. There is a cost related to JV and associate. Just trying to understand what is it pertaining to? Is it one -off or it will be sustained or something like that, around Rs. 21 crore?

Krishnan A: That is the consolidation of the front -end pharmacy business. There is this establishment cost also, which as Obul said, as part of those costs which is getting consolidated.

Shaleen Kumar: Will it be at this level?

Krishnan A: No, it will come down as some of that starts maturing.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed : I was looking at AHLL and within AHLL the diagnostics segment. We have seen a sequential decline in margins this quarter at around 5% which is a sharp decline versus the second quarter. Are there any one -offs? And what's the number that we should expect for this segment going forward?

Obul Reddy: On an even basis, I think this YTD number stacks to the range of 8% to 8.5%. These are turnover discounts and such other one -offs that have spiked the Q2, but they pertain to both the Q1 and Q2 overall. So, we sequentially record them as and when such credits come our way. The overall outlook this quarter has been muted in line with the overall industry trend. There is a Rs. 10 crore approximate drop between quarters, and that contributes to about Rs. 6 crore to Rs. 6.5 crore of EBITDA for us considering that the fixed costs are covered. So, on an even basis on a steady state, we are hoping to be in the 8.5% to 10% mark this year. We are continuously growing. So, our move from 10% onwards is something that we are planning to achieve next

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year as we start realizing the front -loaded costs and thereof the revenues from areas such as specialized testing and such. That's the broad growth path.

Harith Ahamed : And to a previous participant's question, you mentioned that the like -to-like GMV for the quarter is around Rs.

350 crore while what you disclosed is around Rs. 540 crore.

What exactly accounts for the difference between these two numbers?

Sanjiv Gupta: I think this is pertaining to 227 wherein they want to check on the increase in the new admissions. So, during Q3, we started opening up the customer journeys for booking of physical appointments for hospitals across the full ecosystem as well as helping

the customers to get the radiology, pathology, and the other IP -related items. And I am happy to share some broad numbers. Monthly, we have on an average 2 lakh leads that we get for the OP/IP business and about 22% is the current conversion leading to the OP side of it and about 6% to 7% conversion from there towards the IP side of it. That is the line of business that we started. Q2, we spent some money into it building up the tech and products side of it, and Q3 essentially came realizing the entire GMV. Put together, about Rs. 200 crore worth of GMV what we got for the Q3. That's the incremental versus Q2.

Harith Ahamed : For the combined pharmacy revenue that you have disclosed for the quarter, around Rs. 2,175 crore, is there an offline and online breakup for this number that you can share, like we have for the pharmacy distribution?

Obul Reddy : It's about 9% of the revenues coming from online, rest is offline. As a number, it is Rs. 1,963 crore from offline, Rs. 204 crore from online. Together, it is Rs. 2,167 crore.

Harith Ahamed : And last one with your permission. Can you help us with the timelines around some of our bed additions expected over the next 2 or 3 years? How should we think about the number of beds from the current 7,855, let's say for FY25 and FY26?

Suneeta Reddy: In terms of bed addition, I think that our short-term challenges on the current bed addition, let's move to 70% occupancy. Over a 24-month period, we are looking at adding beds. There will be 2 brownfields in Bengaluru, which will give us another 400 beds. Adding on to this, which is more of from FY25 onwards, you will see beds added in both Chennai OMR, a large hospital, as well as Gurgaon which will come in the year 2025. With that, we will have an additional 1,000 beds in that year. Our plan is really to go to 2,000 beds in the next 3 to 4 years with a capital outlay of Rs. 3,000 crore.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Again, on the GMV, while the rate has been quite exponential over the past year or so and you have given the guidance for FY23 as well, but if you can take this forward for the next 12 to 15 months in FY24, how do you see this GMV moving?

Sanjiv Gupta: Rs. 1,600 crore roughly that we are going to hit this year, truly 2x growth in any case we will have in the next year. That would be anything over and about Rs. 3,200 crore to Rs. 3,300 crore. However, as I said previously that our annual operating plan exercise just initiated and from now to till 15th of March, we have detailed workings around it. Probably a bit more color we will have in the next earnings call. But for now, you can think as 2x growth for the next year.

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Tushar Manudhane: And on the offline pharmacy side, while the store addition has been quite aggressive over 9 months, but particularly in 3Q, the offline pharmacy sales growth has been much higher at 22%. It was just 8% for the first half of FY23. Anything in particular in this quarter to highlight?

Obul Reddy : We have also added a similar number of stores in the Q1 and Q2, which have started contributing to the sales and the growth is there.

Tushar Manudhane: On the current base, even to grow by 15% to 18%, almost 1,000 stores at least to be added every year. Is that the

number to go by in the coming years?

Obul Reddy: No, with the 1,000 stores, we are growing at 24%. At 21% to 22%, we will have normal 500 stores addition. And the growth coming from this will be sufficient for us for the next 2 years.

Tushar Manudhane: Lastly, the private label as a portion still remains relatively low. Any efforts out there to improve that?

Obul Reddy: Number 1, the stores are increasing and the new stores, it will be slightly less for some period. Number 2, if you go back to the earlier period, there was a lot of COVID portfolio which is not there currently. So, we are seeing like -to-like on these SKUs volume growth. So, maybe it will take another two quarters for us to show the growth on this from hereon.

Moderator: The next question is from the line of Kunal Randeria from Nuvama. Please go ahead.

Kunal Randeria: Sir, my first question is on the 24/7 business. If I were to look at your revenues, it has grown from around Rs. 120-odd crore in Q1 to around Rs. 175 crore in Q3. But at the same time, the OpEx that you are doing has grown in almost exactly the same proportion, which is at 100% of your revenues. I am just wondering, going forward, assuming that the revenues will be more than double in 1 to 1.5 years' time, which cost items can be controlled so that as ma'am mentioned, you can also turn profitable?

Sanjiv Gupta : I think what we need to actually look at is that now we are in the fourth year; on 5th of February, we stepped into fourth year. It is something like one of the fitness

centers have this traditional line that Today's Pain, Tomorrow's Gain. I think more or less, we are off the mark of putting up more investment into the tech and the products side. I think our app, websites, our individual customer journeys within, and the entire integration with the service providers, all that is more or less done. 1) That means going forward, as we scale up and as we get more GMV and more revenue, my cost with respect to product and tech will not be in that proportion. 2) On the resources side also, we are adequately staffed, so does the office infrastructure and the other things. I think what we can clearly save further on the existing costs is the discretionary expenditure on the marketing side of it, which is to do with the branding. And I would just recall this point a couple of earnings calls back I had talked about this is that the users were also to be talked about various benefits that they can get from the Apollo 24/7 app. To that extent, we had to do a lot of campaigns and marketing to make them aware about the fact that you can now easily put your medicines on the app, you can get the diagnostics done through the app, you can get the consultations, physical bookings, so on and so forth. All that is something which is now passed and all that expenditure is kind of a discretionary expenditure today. So cut short, moving forward, we would get more scale on a very low

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incremental cost, and we would have higher margins. So, we should be able to do a better job as far as the EBITDA side is concerned. So, the possibility is very high to do a pretty good job in the next fiscal year for the EBITDA line for the digital side.

Kunal Randeria: Sir, I need to understand this, maybe, a bit more. Would it be fair to assume that you are now at a tipping point where, let's say, for the next Rs. 100 crore in quarterly revenue, you need not spend Rs. 100 crore and spend maybe Rs. 40 crore or Rs. 50 crore to achieve the Rs. 100 crore revenue?

Sanjiv Gupta : Absolutely. I don't know about 40 or 50, but absolutely. Now the scale that we have it, we don't spend a lot of dollars to fulfill the upside on the revenue. The system itself, the engine itself will take care of the

additional revenues without any additional cost.

Kunal Randeria: And this we should see from the coming quarter or it will be in the next fiscal?

Sanjiv Gupta : No, I think some benefit we will start seeing it from Q1 itself and those benefits will start coming down as we move forward beyond Q1.

Kunal Randeria: Just one quick question on the Hospitals side. Your ARPOBs are close to Rs. 60,000 in metros. Just wondering what the headroom over here is, especially considering that you already have 80% of patients from cash/TPA, which is almost at an optimum level. So, what kind of ARPOB growth we should assume in the coming years?

Krishnan A: We will continue to go with what we have been doing in the past. I think a combination of case mix and price increase is what you should factor in. You are right, we have not been guiding you for towards anything specific around change in payer mix. We will continue to work on the payer mix, but as Ms. Suneeta said, the payer mix opportunity still exists in non-metros, which is what we are focusing on. In the metro cities, we would not have much of payer mix opportunities. So, you should look at the ARPOB growth of around 8% CAGR. This is a combination of 4% to 5% on pricing and the balance on case mix.

Moderator: The next question is from the line of Rishabh Tiwari from Allegro Capital. Please go ahead.

Rishabh Tiwari: What was the combined pharmacy EBITDA number for this quarter? I think it was not mentioned in the presentation this time.

Krishnan A: Yes, that was missed in the presentation. We will put up the presentation with the number; 7.4% is the number.

Rishabh Tiwari: Also, I am not sure if this is a miss. The number of new hospitals has been brought down to 14 from earlier 15.

Krishnan A: There was one hospital which got transferred. There was a Karapakkam Center in Chennai, which got transferred to Apollo Health & Lifestyle. This is the hospital which was within the AHEL books, which got transferred to Apollo Health & Lifestyle. That's a Cradle business that we had within Apollo, which got transferred to Apollo Health & Lifestyle. That's why it has gone lower by one.

Rishabh Tiwari: And since you mentioned AHEL, does the management see any specific guidelines regarding the margins going down in AHEL?

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C. Chandrasekhar: The quarter 3 margins at a consolidated level, we are at about 10% at this point of time on a YTD basis. On Quarter 3, I think we are lower essentially because of the aberration on account of in October, which actually dragged us into a negative EBITDA zone.

And November was a muted recovery. So, it is not very representative of where we stand in terms of how the businesses are performing. Having said that, despite these seasonal fluctuations, I think the revenue buildups have been, especially in the 4 businesses minus the vaccines and the COVID testings which were one-offs, continues to be a robust growth. So, we are hoping to have a sequential improvement quarter-on-quarter on our margin profile going forward because we are also controlling some of our costs as we are on a growth path both in primary care and diagnostics. These are likely to remain a little muted for some more quarters in comparison to the industry.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Sir, two questions. One is on the pharmacy business. On the offline pharmacy piece, what kind of growth rate can we assume for the next, say, 2 to 3 years? And do we see incremental operating leverage on the margins on this revenue growth?

Obul Reddy: Very difficult to guide you for the next 2 to 3 years. At least next 12 months, we expect to be in the range of 22% to 24%

given that we have invested in the growth during the current year.

Nitin Agarwal: And sir, on the margins, we have been kind of flattish to a little muted on the margins for the business versus our trend that we have been consistently increasing these margins in the past.

Obul Reddy: The margins as we have last guided you, we expected that customer discounts will be in the range of about 13.5%. But we could see a big round. It is moving to 14.5%. So, we are largely muted on the margin. Whatever increase in the customer discounts, we are able to manage through the additional procurement margin, but still there is about 30 to 40 basis points impact on the EBITDA. We will be managing that at this level because now we are at about 14.5% on the customer discounts, which we consider very decent for the off market to sustain the growth.

Nitin Agarwal: And secondly, Sanjiv, on the online business, we have talked about the HealthCo being PAT positive or EBITDA neutral by the second half next year. When does the online piece on its own start to become EBITDA neutral in your assessment?

Sanjiv Gupta: That would take a little bit of time. I don't see that to happen in the calendar year of 2025 but somewhere in the fiscal year of FY25, maybe in the last quarter. That should be the time when we should see this segment as becoming profitable. There are a couple of engines that we need to fire. We are working on them. Potential is huge. And if you are able to execute well, which we believe that the current set of team members would be able to do well. Once we do that, I think we should be able to give you a better guidance on the timeline, but FY25 should be the year where we should think that digital would also be, as a segment, EBITDA neutral.

Nitin Agarwal: This is towards the end of FY25?

Sanjiv Gupta: That's right.

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Nitin Agarwal: And we talked about the doubling of revenues on this business next year. And I think earlier we talked about \$1 billion in revenues in this business in the next, I think, 2 years or 3 years. So if you can refresh on the revenue targets for the online business, by when do you get to the \$1 billion number or thereabout?

Sanjiv Gupta: I think FY25 should be the year when we should look at numbers closer to \$800 million to \$1 billion. And that is the scale which is required to ensure that you have a high repeat rate, you have a high net margins and you are able to leverage on your fixed expenses and so you are able to do justice to the entire economics. So, I guess, as I said that next year the numbers are still to be worked upon, but if you are looking at roughly 2x growth and another similar growth for a year thereafter, I think FY25 should be the year to get near the \$1 billion mark.

Nitin Agarwal: What kind of margin this business you should be ideally making at the size?

Sanjiv Gupta: I think anything around 20% should be the margin that you should get it as a blended margin. It all depends upon the mix that comes. A little difficult for me to give you one number, but yes, anything between 20% to 25% should be the mark.

Nitin Agarwal: Our operating cost, you said, is probably getting to a point where it will start to plateau out beyond a point after some time versus the current base that we have built in.

Sanjiv Gupta: Yes, absolutely. Pretty much it's a fixed expenditure. Once you have a tech product done, then it is just a maintenance cost. And digital platforms help you to get to the scale faster. But yes, in the initial years, there is a pain with respect to putting up expenses and putting up lot of infrastructure there. But at that scale, from next year onwards, I don't see that expenses going up substantially. They will start getting plateaued and we would

Id also do a certain bit of rationalization given the fact that we understand the external environment and what are the right decisions to be taken for the business. And I think those business decisions would also be implemented and executed. So, I think a mix of everything, but yes, expenses are plateauing there.

Nitin Agarwal: On the Diagnostics business, I think we have had, from an industry participant perspective, a lot of mixed commentary around the outlook for the organized business in the near to medium term. Mr. Chandra Sekhar, how are you looking at the dynamics of the business? Are there some challenges on the organized sector volume growth or that's sort of come up over the last few quarters?

C. Chandra Sekhar: I think there has been more than normal competitive intensity for the digital aggregators. But the way I see it is this is on the top 20 to 25, if not 30, frequently used tests which form the bucket of routine. I guess the competition is intense there and so I believe that it's going to remain that way for a while. But I think the focus of the more organized larger players and rightly so will shift to more specialized and more high -end testing and also towards packages with an emphasis on quality because at the end of the day, I think 95% of all clinical decisions are taken on the basis of these diagnostic tests. So, it's important to also bring up the topic of quality. There is a cost to it. So, I think pricing for organized players in terms of responding to such competitive pricing from digital aggregators is not likely. There could be some short -term dips in volumes in the routine basket, but the endeavor for the organized sector will be to make up for that through other semi -specialized, specialized, and super -specialized buckets. That's I think the outlook that I think we will see. But this will settle down.

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Nitin Agarwal: There has been a lot of emphasis on selling bundled packages versus specific illness tests. Is this a strategy that we are also following in terms of trying to increase our share of bundled tests?

C. Chandra Sekhar: We are following that. In another business which is not reported in the Diagnostics actually is the Clinics business which is in the Primary Care. I think we will do a lot of health checks. I think 60% of the business there comes out of health checks, which essentially also has a radiology component but essentially a lot of pathology. But within the Diagnostic business like -to-like in comparison to other larger organized players, our wellness packages are now around 7% to 8% of our mix. We are hoping to make this go to 20%, and that will offset this very stark comparison in terms of a -la-carte testing.

Moderator: The next question is from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: My first question is on discounting levels. You mentioned that discounting level has increased from 13% to 13.5% to 14.5%. Can you give us a split of your discounting level in offline and online pharmacy? And if this increase is happening in online pharmacies, do you see an increasing discounting pressure again in this segment, which is causing the higher discounting level? And also, can you update on the Amazon partnership?

Obul Reddy: The online discounts are at about 18% to 18.5% like last quarter. It is only offline which moved slightly. And on the Amazon, Mr. Sanjiv will comment.

Sanjiv Gupta: Before Amazon, I think we guided on the online discounts that we would retain our 18% mark. YTD December has been 17.6% although the last quarter was 17.8%.

We will continue to maintain discounts near 18% and we will also time our discounts in such a way that wherever opportunities are there to reduce it, we will continue to

ue to

do that or sustain at this level. On Amazon, we are now live with 15 stores and current rate is around 3,000 orders per day. We couldn't increase the run rate in the middle of the year due to tech integration issues which we had between us and Amazon, but now those have been resolved. And there has been a little bit of network expansion also to take care of Amazon operations. And in the next 4 weeks, we would see that Amazon customers would be able to see breadth and depth of inventory across the top 6 cities to start with. And there is a little bit of marketing activity also in Amazon app that has started over the last 3 to 4 weeks. And what we believe that we should be able to hit a \$10,000 per day somewhere in July or August of the current year.

Sayantan Maji: Are we doing a revenue -sharing arrangement here? Or is it on a profit -sharing basis?

Sanjiv Gupta: It's a revenue share.

Sayantan Maji: My second question is on a little clarification on the new GMV. This hospital IP/OP segment, is it the same customers who are coming, they are basically being converted online or is this a new set of customers that you are targeting here? Can you explain a bit about this IP/OP segment?

Sanjiv Gupta: Yes. This is a new set of customers. Obviously, there is no point in having the same customers both sides; it is a new set of customers. The way the entire operational side of it works is that; one is that we have integrated our 24/7 app with the entire

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Apollo ecosystem where we have about 55 participating hospitals which in the current quarter will go to 70; and second, there is an engine that works behind all this. People who search out surgeries, say suppose say knee surgeries or some kind of surgeries online, googling and all that stuff. Those are the data points that we collect. And then those are the curative leads that we use to further engage with them. As I said, the monthly average for Q3 was about 2 lakhs leads that we got from various, I would say, websites or the places where digital customers were trying to find out about those surgeries. And those 2 lakh leads were then kind of, we engaged with them through tele calling and through various nudges that we do it from Apollo 24/7 and finally we converted about 22%, 22.54% to be very precise, for the last quarter and about 45,000 customers went on during the month of December to the hospitals and they got the OP consultations done, and from there, about 2,500 people got the surgeries done. This is all is well managed and a curated engine and then you have got many apps out there in the market who does this kind of work wherein they identify people who are digitally looking out solutions around surgery and those are the leads that we get, those are the leads we further cure and then we further kind of engage with them to get into this side of the business. It's all very different. I can discuss a lot more, but in the interest of time cannot discuss much about it, but happy to engage with you one-on-one whenever you want a bit more information about this.

Sayantan Maji: No, that's very clear; that explains. And finally, just wanted to check what will be the loss run rate for Apollo 24/7 going ahead now that we have peaked?

Sanjiv Gupta: First of all, we are cognizant to the fact that somewhere we need to convert the entire thing to EBITDA positive. And we are also looking at how we build our plan for the next year, which are the segments that need to fire more, and where exactly we need to do incremental investment. And all this will result into a detailed exercise of building our budget for the next financial year. And accordingly, we will get to know about the expenses that we feel appropriate for the business. And accordingly,

y, we

will have a little more color on this. Maybe next earnings call will be the right time to discuss about this by quarter; in fact, for the entire year, we can talk a lot more in the next earnings call. So, I would just request you to wait for some more time before we dialogue on this.

Moderator: The next question is from the line of Bhagwan Chodhary from Sunidhi Securities.

Please go ahead.

Bhagwan Chodhary: Sir, one question on the Healthcare side. Can you comment something on the future capex side how we are going to do this, as we are going to generate a lot of cash, Rs. 1,500 crore to Rs. 2,000 crore kind of or so? Can you comment something on that?

Krishnan A: Clearly, the plan, as Ms. Suneeta has already articulated, is to get to an additional 2,000 beds over the next 3 years. And we have a clear plan on our road map and where we are adding that. And the free cash flow, as you already know, is around Rs. 800 -plus crore that we have in this segment across, and we will deploy this into some of them. We don't think we are going to be requiring significant incremental debt. Whatever is required, we will take some debt. Debt/ EBITDA is also very comfortable, as you have seen.

Bhagwan Chodhary: And this will be organic or inorganic? How it is going to be?

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Krishnan A: There will be some inorganic, some bolt-on acquisitions that we are looking at and also greenfield.

Bhagwan Chodhary: And secondly, can you talk about this performance of Proton, how that is doing?

Krishnan A: Proton is doing much better now. We are on track to get to Rs. 50 crore of EBITDA in this year. And we would like it to double in the next year. That's our plan.

Bhagwan Chodhary: Finally, an observation on the Slide # 19 where you have detailed these pharmacies across India in all the States. But there is no pharmacy in Madhya Pradesh. Any particular reason for that?

Obul Reddy: We don't have logistic support. We are planning in the next year. We need to set up a back-end support system before we open the retail pharmacies. Just working on that. You could see next year some pharmacies there.

Moderator: The next question is from the line of Neha Manpuria from Bank of America. Please

go ahead.

Neha Manpuria: Sir, on the GMV increase that we have seen; I understand that obviously a lot of this gets monetized in other parts of the Apollo value chain. But for Apollo Digital itself, is it fair to assume that these would just be leads which help the Hospital business and Diagnostic business and not so much the Apollo 24/7 and therefore the take rate would remain in the mid -30 level?

Sanjiv Gupta: No, there's no free lunch. There is a highest board governance and the things at various entities of Apollo and we got Deloitte as auditors and we got PwC who is helping us on the arms' length pricing. As I said, there are no free lunches. As we start ramping up our business, soon the take rate will also go up. And classic example is, Diagnostic; we started our journey with about less than 10% discount. Today, we are near 17% to 18% discount, and as we grow business, more commission will come to us. Everything is dependent upon how much business and what are the commercials around it. It's not like that our take rates are not going to go up. Our take rates would be dependent upon the kind of efforts that we are putting on the cost that we are incurring and the net EBITDA that the other participant group companies are getting out of it.

Krishnan A: Neha, just to be on the same page, these are only for new customers. It's not for the existing customers, point #1. Point #2, these are only for customers which are coming to us through the Apollo 24/7 platform. Digital leads which come directly through

our marketing efforts into the Apollo website and us come directly to us and also the call centers come directly to us. It's only the Apollo 24/7 where as Mr. Sanjiv said, there are significant efforts which are happening into the customers of Apollo 24/7. And which is where if you remember, a couple of years back, we did say that even if you look at the pharmacy customers of Apollo and if we look at the number of customers that they have and look at the number of those customers who come into Apollo, it is less than 1%. That is one of the big opportunities which are there because he is already an Apollo customer in some sense and to really work on getting them into Apollo is what Apollo 24/7 is working on. We should see benefit of that with time.

Neha Manpuria: I get that from an Apollo consolidated perspective, but I was just trying to understand how will Apollo 24/7 monetize this incremental GMV that we are getting over time?

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Krishnan A: Yes, as we said, he will get a percentage of the revenue for the incremental numbers that he has done.

Neha Manpuria: Suneeta ma'am or Krishnan, any update on the Apollo 24/7 stake sale, strategic sale that we have?

Suneeta Reddy: I think we have seen peak losses that 24/7 has incurred, but we have also seen an 84% growth over last year. And we believe that in this current environment where we as a Company are looking for growth, but we are also looking at profitability. We understand that this is a very difficult environment to get the valuation that we deserve, because 24/7 is differentiated in many ways. It's just not pharmacy online. It connects all of Apollo's physical offerings and creates the digital offerings. So, keeping in mind the value proposition of 24/7, we believe that there is investor interest which will take a little bit more time to convert. I wouldn't really like to put a time horizon here. But I would like to say that at Apollo, we believe that this is the future; it is necessary to invest in the business. Fortunately, we have capital to enable it to grow in the next 6 months. And by which time, we are quite sure that we will have an investor there.

Moderator: Due to time constraint, we will take one last question from the line of Shaleen Kumar from UBS Securities. Please go ahead.

Shaleen Kumar: Sir, just one thing. If I heard correctly, you said the discount in offline pharmacy has increased a bit?

Obul Reddy: That's right. Keeping the market in the view and what is happening and consistency between the two offerings, we have definitely introduced some schemes which increased the loyalty cost to the customers. And as I said, largely, we are able to get that compensated through the procurement improvement margin. And net-net we are about 30 to 40 basis points impacted; and with the growth coming in, we will be able to take that in the cost.

Shaleen Kumar: So, it's more of an initiative from our side? Is it to kind of parallel the scheme and not driven by the....

Obul Reddy: It is an initiative from our side to see that we don't want to lose our offline customers. So, we have introduced some schemes, something called Circle plan where they get some extra benefits, thereby there is a cost increase.

Shaleen Kumar: Sir, just last one observation. In Diagnostics when I was looking at, while the revenue has understandably grown only by 2%, but our profitability has come down significantly. Any specific reason over there?

C. Chandra Sekhar: You are comparing Q3 FY22, and that had a very significant component of COVID and COVID -allied testing's , which we have mentioned. The non -COVID revenues are to be seen and the margin profile there is definitely lower, and not the ones that we got in the COVID testing margins; that'

s the reason. You are comparing Q3 FY22 to Q3 FY23.

Moderator: I now hand the conference over to the management for closing comments.

Suneeta Reddy: Thank you, ladies and gentlemen for taking time out today. As you can see, at Apollo, we firmly believe that we have established 3 strong engines of growth. Of course,

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the core Hospital business continues to grow , and we believe that the fourth quarter will be a fairly good quarter for us. The pharmacy business, again, the offline pharmacy business continues to grow, and we are seeing about over 20% growth in revenues, and we will see an improvement in margins in the future. Our primary healthcare business which is Diagnostics and Clinics is an area that we are focusing on. And again, we are seeing strong growth in these three verticals, while keeping our core clinical proposition as our core differentiator. More importantly, I think the network effect of having these verticals and bringing them together through 24/7 is something that has not been attempted so far in this part of the world. And we believe that investing in the future and making us digital is very important for us to retain our position as the #1 healthcare provider in this part of the world.

Thank you, ladies and gentlemen, for supporting us on this journey, and we hope to hear more from you in the fourth quarter. Thank you once again.

Moderator: On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility of such errors, although an effort has been made to ensure high level of accuracy .

Call: May 2023

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Apollo Hospitals Enterprise Limited
Q4 FY23 Earnings Conference Call
May 31, 2023

Moderator: Ladies and gentlemen, good day, and welcome to Apollo Hospitals Limited Q4 FY23 earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you, sir.

Mayank Vaswani: Thank you, N eerav. Good afternoon, everyone, and thank you for joining us on this call to discuss the financial results of Apollo Hospitals for Q4 and FY 23, which were announced yesterday. We have with us on the call today, the senior management team comprising Ms. Shobana Kamineni – Executive Vice Chair, Ms. Suneeta Reddy, Managing Director, Dr. Hariprasad – President of the Hospitals Division, Mr. A. Krishnan – Group CFO, Mr. C. Chandra Sekhar – CEO of AHLL; Mr. Obul Reddy – CFO of the Pharmacy Division, and Mr. Sanjiv Gupta – CFO of Apollo 24/7. Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on slide two of the investor presentation that was shared with all of you earlier. Documents relating to our financial performance have been circulated and these have also been uploaded on the corporate website and the websites of the respective stock exchanges. I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Good afternoon, everyone. Thank you for taking time off to join our earnings call. I trust that you have received our earnings document, which we shared yesterday. Before we get into the details of our quarterly and full year performance, I would like to take a few minutes to reemphasize our vision and journey towards creating Asia's largest integrated healthcare ecosystem with the consumer at the centre.

While high-quality healthcare has always been our core offering, we have systematically, over the years, created several thousand touch points of care through offline pharmacies, multi-format clinics, diagnostics, and now a comprehensive digital health platform, Apollo 24/7. This health infrastructure is formidable in terms of footprint and scale and key to us being able to be the most trusted healthcare partner for discerning customers across the value chain.

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We are confident that the investments we have made and the solutions that we are working on, whether it's AI for clinical decision-making or digital therapeutics and condition management, will result in providing consistent and seamless hybrid customer journeys. This will be a key differentiator, and it will give us the right to win a disproportionate share of the health wallets spends of affluent households. All our efforts are oriented towards unlocking network effects, and we are sure that we are building strong upstream and downstream funnels. Against this backdrop, let me take you through our financial results, which reflect the strength of our core business and the potential of our investments for future growth. We have reported a very strong fourth quarter with the healthcare services business witnessing a robust 18% year-on-year growth in Q4FY23, driven by growth in volume and supported by pricing and case mix gains.

During Q4FY23, occupancy across the group was at 5,041 beds or 64%, mature hospitals were at 65% and new hospitals were at 62%. This was planned and achieved after a sustained reduction in low-paying institutional business over the last six months. This has resulted in a lower reported occupancy but has visibly strengthened our margins. Alongside the trend of increasing insurance business, which is currently 43% of revenues and 40% of volume contributed by this channel in Q4. This was less than 30% pre-COVID.

ARPOB increased 10% year-on-year to INR 53,232. Consolidated financials for the quarter, our consolidated revenues grew by 21% on a year-on-year basis to INR 4,302 crore. On a year-on-year basis, Healthcare Services revenue grew by 18% to INR 2,195 crore. Mature Healthcare services grew by 18% to INR 1,519 crore, while new hospitals

grew by 16% to INR 676 crore . Revenues from the Apollo HealthCo stood at INR 1,799 crore in Q4 against INR 1,375 crore , representing a growth of 31%.

AHLL revenues stood at INR 309 crore in Q4FY23, with the core revenue growth, excluding COVID and vaccination, of 23% in Q4. Consolidated EBITDA stood at INR 488 crore without the effect of the operating cost of Apollo 24/7, the EBITDA was actually at INR 706 crore . Within this, Healthcare Services EBITDA grew by 31% to INR 535 crore . Healthcare Services margins expanded by 249 basis points to 24.4%. Offline pharmacy distribution EBITDA was at INR 121 crore , the margin remaining very healthy at 7.8%. Net EBITDA loss in Apollo HealthCo was at INR 72 crore . AHLL recorded an EBITDA of INR 26 crore compared to an EBITDA of INR 37 crore . The decline was owed to a drop in COVID-related revenues and increased cost to expand the high -end testing and the phlebotomy network. Healthcare Services PAT was at INR 257 crore , a growth of 46%. Consolidated PAT was at INR 145 crore , a 60% growth on a year -on-year basis.

We closed FY 23 with consolidated revenues of INR 16,613 crore , a growth of 13% over FY22. EBITDA was at INR 2,050 crore and PAT was at INR 819 crore after accounting for higher operating costs of Apollo 24/7 and due to the nonrecurring revenues from COVID and vaccination.

The financial performance of the core business is robust. The margins are healthy, and the operating indicators continue to improve . Our focus on the tower specialties of

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cardiac sciences, oncology, neurosciences, gastroenterology, and orthopaedics have ensured that they now contribute 60% of the hospital revenues. Our ALOS was at 3.39 days for Q4. It is the best -in-class considering the severity and acuity of our mix.

In terms of inorganic expansion, we have made significant progress on implementing our plans to add 2,000 beds at an outlay of around INR 3,000 crore over the next four years in key metros. Alongside this, we are on track to improve hospital occupancy to 70% by the end of FY 24 and unlock margins by revenue and cost optimization. We serve over 7 lakh footfalls a day in our offline pharmacies. Over 15% of revenue in this vertical now comes from private label and generics. We are therefore making sustained efforts to increase this number consistently.

Our investments in the future are also yielding results. I am happy to report that Apollo 24/7 achieved a GMV of INR 1,643 crore for FY 23, delivering on our commitment to exceed a GMV of INR 1,500 crore this fiscal. The platform has over 25 million registered users. The committed trajectory of GMV and revenue growth is on track and breakeven achievement at an entity level in Q4 FY24 is well on target.

In AHLL, the non -COVID di

agnostic business has grown by 45% year -on-year and is currently at a run rate in excess of INR 100 crore for the quarter. We expect to grow this to INR 500 crore for FY 24 and to INR 1,000 crore in three years. This should be accompanied by an expansion in EBITDA margin from 5% at the present to about 10% to 12% in the next two, three quarters as we focus on expanding the test menu to include specialty high -end testing modalities and genomics, while recording an increased utilization of our phlebotomy network.

The structural strength of healthcare services and offline pharmacy combined with the potential of our growth verticals gives us immense confidence in the impact that we can make in shaping the healthcare ecosystem of this country and delivering high -quality care to our consumers in a more seamless, ubiquitous way and value -(inaudible) way. We acknowledge the short -term concerns around the cash burn in Apollo HealthCo. Several measures have been initiated that will reduce cost by INR 125 crore to INR 150 crore in this fiscal.

We would like to assure our stakeholders that we are on the cusp of change, and we are building a platform that is sustainable and value -accretive with fiscal prudence and distinctive consumer value proposition as our twin goals.

On that note, I would like to hand it over to our moderator and open the line for questions. I have Shobana, Dr. Hariprasad, our CFO - Krishnan, Obul Reddy, Chandra Sekhar from AHLL, and Sanjiv from 24/7 with me to take all your questions. Thank you.

Moderator: The first question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: My question is on Apollo 24/7. So , you recorded GMV of INR 5.9 billion for fourth quarter, which has seen 9% sequential growth. So , although like, you have beaten your earlier expectation for full year. But for next year, I guess, you have earlier indicated that you would like to double up your GMV compared to FY 23 level. So , from this level, like, how do you see GMVs picking -up in coming quarters so that you can almost double

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up GMV in FY 24 versus the FY 23 level? Because fourth quarter growth seems a bit low, I'll say.

Shobana Kamineni: I am happy to take this question, this is Shobana Kamineni, which is on growth. We'd like to tell you that we've initiated many programs. So the plan here is very clearly is that we have tailored our growth to achieve a 100% increase from the last year. So we will finish this FY 24 with INR 3,000 crore. Most of this will come from pharmacy and while keeping our marketing spends at the same level. And what we've seen in Q4, which is the early shoots of how we do the following year, is that we've been seeing a lot of retention. And then our monthly, the monthly customers that keep coming back to our platform has actually increased, people who make three (inaudible) transactions. It used to be in the range of less than 500,000 people last year. Now this is inching up towards 1 million-plus. And I think that this is very significant of people as we continue to get traction of doctors that continue to come on. And please understand that what we've done is that we're also (inaudible) Suneeta said, our journeys within, especially within the offline and online, all these years, we've just been creating the pathways. And now we are actually seeing traction where in community-by-community we're able to get more-and-more people coming back to our pharmacies that shop omni. And the thing is that we've actually opened up a new age group of customers that it is not of healthcare users, many of who are digital first. So the 25 registered who are digital first, many of them, almost 2 million of them, they shop not for themselves

s, they shop for

someone else in their family. These are very interesting cohorts that we've seen. We've also accelerated the ability, like if you look at our figures, we have now 1 in 4 of them actually doing another service. Mostly it's a diagnostic service that they take. And I think that this kind of traction is what we'll be able to build throughout this year. Yes, 9% was muted, but that was largely because if you see during that period, everyone pulled back in terms of discounts and that short period of growing 3% less in discount, all the high-value seekers just moved out and which is good for the business because now we have much more traction of serious customers engaging.

Damayanti Kerai: Okay. Ma'am, this discount part, you said you are offering a 3% less discount versus most of other peers, is that?

Shobana Kamineni: No, no. Most of them have moved down. But yes, we continue to stay at least 1% less than the others.

Damayanti Kerai: My second question is AHL spend seems to have plateaued during the quarter. So, are we going to see similar run rate ahead? And if you can elaborate where all the spend has gone in various buckets? So that will be helpful.

Shobana Kamineni: Sanjiv, please.

Sanjiv Gupta: So, I'll take this question. I think there are two, three things. If you look at in Q4, our AHL overall EBITDA is at INR 72 crore negative versus Q3 of INR 63 crore. There is a one-off marketing intake integration tie up cost with insurance and corporate health benefit platform. This is for the distribution and servicing medical insurance policyholders. So, I think with this one-off tech integration, we would be well positioned to start taking B2B, which is a corporate side of the business as well as the insurance

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side of the business.

As far as the spend side is concerned, I think as stated in the last earnings call also, we don't see any difference as far as Q2 versus Q3 is concerned. About 20% of the expenditure goes into product and tech side. We've got around 25% of expenditure into the op side. The marketing, which is the brand communication, connecting with the customers, content and so on, that is roughly about 20%. The call centre and the other lines, tele-sales, and all, that takes about 15%, and the remaining 10% is towards support and other things.

And I would also like to state one more point that we have a line of sight for at least INR 20 crore to INR 25 crore of reduction in losses by the end of H1, and we should be seeing this INR 72 crore of Q4 losses to come down sub 50 by H1.

Damayanti Kerai: Sorry, Sanjiv, you said INR 25 crore to INR 30 crore cost reduction by H1. That's what you're aiming for?

Sanjiv Gupta: That's right. So we have a line of sight for 25 (inaudible) of EBITDA improvement or the loss improvement, but more work is going on. We'll update you in the next earnings call with respect to where we are.

Moderator: Thank you. Next question is from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal: Just carrying on with the last participant, the voice quality was not clear. You were saying your spend from INR 189 crore would come down by INR 25 crore, INR 30 crore, which means you'll be doing INR 150-odd crore per quarter starting first half, Q1

onwards. I could not get this.

Sanjiv Gupta: I think what we suggested is that as a Company, we are into the endeavour of ensuring that Q4 is breakeven for the Company. From Q4 of the current fiscal year should be breakeven for the Company. As far as the last quarter is concerned, we made a loss of INR 72 crore. The line of sight that we have for the next two to three months suggests to us that by H1, this INR 72 crore of loss should

be around INR 45 crore to INR 50

crore. So, there will be a reduction in losses by about INR 25 crore from the current level in next three to four months.

Prakash Agarwal: That is clear. But this loss reduction is by higher sales or you plan to reduce this INR 189 crore burn rate also?

Sanjiv Gupta: Absolutely. So that's a good point. And we have taken a couple of areas where we can and as suggested by Shobana ma'am that there is a reduction in the discount by about 3%. We were at about 18% to 19% in Q4, now down to, in the current quarter, we are at about 15% apart from the fact that we've got 20% reduction in the logistics. There's a lot of headcount rationalization and productivity coming out of the automation, which would also support us in the expense reduction. So, there are certain areas where action has already been taken and those actions were taken in the first half of Q4 and early April. That is the reason we couldn't see anything in Q4. But indicative numbers for April and May suggest that there will be a reduction in the operating cost as well.

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Prakash Agarwal: Understood. And one more, if I may, on the HealthCo. So, if you see the offline pharmacy Y-o-Y is pretty soft, 12%. Any particular reason, was it due to any COVID base last year, or how do you think about that?

Obul Reddy: Yes. You are right. Last year, first Q1 and Q2, we had a huge COVID sale. If you adjust for that, our offline growth is in the range of about 19% to 20%.

Prakash Agarwal: Q4, sir, not Q1 and Q2. Q4 of last year.

Obul Reddy: Q4 of last year to Q4 of this year, there's a 21% growth.

Prakash Agarwal: On the offline pharmacy platform, right?

Obul Reddy: Yes, that's right. And just Q1 of last year, there's a 19%.

Prakash Agarwal: And second question on hospital business. So glad to know we are on track of this 70% occupancy levels. This quarter, I heard on that the ARPOB and case mix actually have been, the focus has been to improve that. Typically, the season is strong Q4. So, did we see any seasonally soft quarter and what is the bridge to move from 65% to 70% by end of this year?

Suneeta Reddy: So, if you look at occupancy, we've reduced the number of institutional people, the patients that we usually get. And with this, the occupancy dropped by at least 3%. So, this is why you also saw the ARPOB increased by 10%. Yes, our case mix has also improved, but it is a combination of both fees that has led to an ARPOB increase.

Prakash Agarwal: So, despite this thing improvement in the case mix and institutional going down, you expect 65% to move to 70%.

Suneeta Reddy: Yes. And this will come from the program that we have to recruit new doctors, which is already happening across the system. The second is to really to look at what our network funnel is going to bring us. We have started getting patients from 24/7, and we believe that we have created a strong funnel. The third lever that we're using is really to look at how we leverage our corporate relationships, our corporate relations as well as International. So, we are looking at this. Currently if you look at our payer mix, 45% is retail and another 45% is private insurance. So that is really going to contribute to an increased occupancy. And finally, for the Tier 1 cities, international patients, currently we're at about 7%. We're moving this to 10%, so that should add another 3% in terms of occupancy.

Prakash Agarwal: And lastly, you mentioned there is a reasonable progress on your inorganic initiative of adding 2,000 beds. Last time you mentioned three, four locations. If you could just highlight any particular location, any particular development, I would be happy.

Dr. Hariprasad: We should be in a position

to come back to you in the coming quarter, hopefully on this,

Prakash. We have line of sight across all of these. We are progressing on Delhi and Chennai, as you already know. We are working on Bangalore. We are working on Mumbai as well and Calcutta. So, Hyderabad also we're working on. So, there are multiple options. By the coming quarter, you should get a clarity.

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Moderator: Thank you. Next question is from the line of Gina Kim from Schroders. Please go ahead.

Gina Kim: I don't know if this is a minor point, but on Page 15 of your presentation, is the financial

performance for the fourth quarter of AHLL. Everything makes sense but specialty care side, EBITDA, EBIT sales is all growing, I guess, except for PAT. So, PAT losses have ballooned, it seems, in the fourth quarter of last year. Can you explain why?

Chandra Sekhar: So, we have higher depreciation because of new centres. Specialty care includes a lot of the fertility centres which were opened in the year before. And all of these depreciation and the effect of depreciation in the Ind AS treatment also has effected that.

Gina Kim: Sorry to interrupt. On the depreciation, that would have been if reflected in EBIT, right?

So, EBIT grew from INR 1 million to INR 64 million if you're looking at the line above.

But only PAT went the opposite direction.

A. Krishnan: There was also a deferred tax asset reversal in this quarter, which is a one-off, which will not recur from the next quarter. There was a INR 12 crore deferred tax asset reversal.

Chandra Sekhar: Yes.

A. Krishnan: So that's a charge to the P&L, which will not recur from next quarter.

Gina Kim: That's for the specialty care.

Chandra Sekhar: That's a one-off. Yes, correct.

Gina Kim: And just generally, I guess, I mean, you've mentioned some sort of guidance in bits and pieces, occupancy of 70%, your capex plan, HealthCo revenue growth. But can you provide an overall picture of what you expect for this year, FY 24, as a group?

Suneeta Reddy: Healthcare Services, we are looking at 15% growth coming from higher occupancy and therefore better EBITDA. This quarter, we showed an increase in EBITDA margin by 249 basis points. We believe that we can improve this by another 100 basis points. And this will come from pure, from occupancy covering fixed costs and making sure that higher occupancy is better EBITDA and therefore, higher profitability. Moving on, I think 24/7, Sanjiv or Shobana, would you like to talk and Chandra Sekhar, on AHLL.

Sanjiv Gupta: I can take it up for Apollo 24/7 on the digital front. On the digital front, we are looking at about 100% growth on platform transaction. We did roughly INR 1,640 crore last year.

We're looking at doubling it. On the pharmacy side, we are looking at about 29% growth, 28% to 29% growth in the current fiscal year versus the previous year. Over to Chandra Sir, for AHLL.

Chandra Sekhar: Yes. On AHLL, I think we have had a base reset kind of figure. So, the one-offs on COVID, etcetera, completely reduced this year. So, an optical view shows a flat year to a minor de-growth. But then AHLL core revenues on a year-on-year basis have grown by 28%. Diagnostics continues to grow at a pace, but again, optically it shows 4% de-

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growth, but that is because of a very high component of COVID testing the previous year. Non-COVID diagnostics, excluding COVID testing and COVID allied testing actually grew 63% year-on-year. So that's, I think, the flavour that I think we are carrying forward. The overall growth at AHLL level from INR 950 crore

odd number to INR 1,280

crore number approximately is a reflection of about, of a continued growth path. The growth has been driven by the core business.

Now on a future outlook; Primary Care had a one-off significant INR 195 crore revenues on account of vaccination in the previous year, which has subsequently obviously has fallen off. So, these were the flavour of the year and the overall year's performance. So we remain bullish about the continuing the growth at a 40%-odd growth rate that we have demonstrated in the non-COVID segment. For the previous year, it was approximately INR 250 crore, which has moved up to the INR 384 crore number, representing about a 44% growth in terms of the non-COVID testing. We are making investments on adding and changing our mix from routine basic to semi-specialized, specialized, and super specialized. Towards this, we are taking additional cost by way of both skilled doctors, specialized sales forces. We're also investing continuously, and we have yet to receive the complete benefit of the ramp-up of our phlebotomist capacity because the consumer demand and something that we are very committed to is to serve the home collection requirements. These will compress our margin for a little while, there in diagnostics. However, our growth rates will continue to be at the 35% to 40% level, and we would like it to be even higher further. So that is the plan that we are taking on the growth part.

On the other segments, we continue to have growth rates, which I think we will sustain and add a few centres and the Cradle segment are being added, which will again cause a slight gestation-based depression in margin but will continue to give us benefits.

Cradle is operating on a high 18% to 20% EBITDA margin at this level, and that will continue to be useful.

IVF, which is fertility, is a more newer line of business for AHLL. We have made

investments during the midst of the COVID years. The last year has seen a significant growth year -on-year, but we are yet to come to a level of completely utilizing the potential of the infra we have created. So without any new centre additions, we are going to grow the utilization at IVF. So that's setting the broad plan.

Our diagnostics division margin has been at the 7.5% on an aggregate annual level, and we are making efforts to take it up 10% to 12% and progress we'll be aiming to take it to the 15% level. There can be quarters of slight fluctuation because we are making investments. And most of the investments for expansion are operating expense in nature. So that's the broad outlook for AHLL.

Moderator: Thank you. Gina, I'll request you come back for a follow-up question. The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just some of the metrics that we track on Apollo 24/7. A little confused. So daily active users, I am looking at the last four quarters. So, June 2022, 6.5 lakh, daily active users. It became 10.6 lakh in September, December - 7 lakh, and now fourth quarter again 7 lakhs. So, some of the comments that have made on us dropping discounts and some

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of these value customers moving away. So did that happen two quarters back or, I'm just trying to map it to the trajectory of daily active users.

Sanjiv Gupta: Yes. So I think as far as the upward movement from June to September and some bit of October was on account of the kind of campaigns that we were running across pan - India and those campaigns were more of a broad base than of a kind of a very, very particular opportunity for select catchments. So one is that we shifted our campaigns from broad-based to more data analytics oriented. So that, in any case, we believe will

result into a lot of active users, which only come for kind of a comparison between various websites, (inaudible) come in. On the value seekers, will start dropping in. And in any case, we never wanted to have a value seeker coming into our platform. So, this was one of the conscious decisions to ensure that value seekers does not land into the platform and more of a chronic, more of a customer who's looking out for continuum of care and various others opting for Apollo ecosystem. So, this is one conscious decision. Now between December and March, I think one of the reasons, and you rightly pointed out, could be us trying to the discount being lowered. So, discounts in Apollo 24/7 have always been about 2% to 3% lower than the market. You would recall that somewhere in FY 22 discount was 22%, 23%. And last year, they came down to 18%, 19%, 20%, and we were always less. And somewhere in March itself, we have brought it down to sub-15%, apart from the fact that; so I think mild deductions will also be attributed to lower discounts being offered by platform plus we are also not looking at those orders which are less than INR 200 because that distort the entire economics of the pharmacy delivery. I think these are the two reasons, plus value seekers going out of the platform would be the third reason resulting into daily active users to be constant between December and March. But current quarter, we see upliftment as far as the now is concerned.

Shyam Srinivasan: That's helpful, Sanjiv. Just, I'm just a little worried about like active as a percentage of registered users, right? Registered users have gone from 14 million last year to 25 million, and we are at the same level of active users. So, am I reading it too much, you think, or?

Sanjiv Gupta: I think the metrics which we need to look at it is that to what an extent repeat percentage is flowing through the platform between the two endpoints. So one is that the repeat percentage is as high as 40% for the last year for the Company, which was probably around 22% to 25% in FY 22. So, one is that more and more repeats are coming into the platform. As far as the overall base versus daily active, please note that chronic customers or as a matter of fact, any customers will open the app only when our service is needed, not necessarily, we never expect that 25 million customers to open the app every day and become a daily active user. We would have only as many active users onto the platform were coming to consume either a product or a service and service could be content also of care. So, I think this is to be seen from this perspective that whether your active user base is increasing, your repeat percentage is increasing, and your total platform transactions are increasing year-over-year. So, if I look at it year-on-year, we have about 266% growth in the platform transaction. So, I think this is how I would sum up that we should look at the numbers.

Shyam Srinivasan: Last question for me. Just the divergence in GMV Q-o-Q growth, I think, 9%, like I think

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one of the other participants asked. But revenue has grown 44% to about INR 2,539. So just trying to understand, say, if I use revenue to GMV ratio, that's pumped up. And

also, if you could help us with the split of the GMV in the different services that we share?

Sanjiv Gupta: Okay. So, in the last earnings call also we had a question regarding GMV to revenue ratio and to what an extent because we could look into as a stable number. Last quarter, we've had 42% as 42.9% to be precise. So revenue as a percentage to GMV, I think pretty healthy. And we continue to look at this number in the range of about 45% to 50% between current quarter to the next quarter. So, it should

stabilize somewhere

there. You also checked on what the distribution of INR 593 crore between various verticals. In pharmacy, we did about INR 375 crore, which is roughly 74%, Diagnostic we did INR 22 crore, which is 16% and remaining 10% is on the consultation side.

Moderator: Thank you. Next question is from the line of Tushar Manudhane from Motilal Oswal. Please go ahead.

Tushar Manudhane: Sir, just on this number of offline stores for the quarter, there has been bit of slowdown, while for a full year basis the number has been quite aggressive compared to FY 22. So if you could throw some light on the number of stores that would be added in the coming years.

Obul Reddy: So last quarter, we added about 300 stores, but there is a little bit of higher closure, that is why net addition is less. And we are on track with the new opening.

Suneeta Reddy: And for the year, 1,100...

Obul Reddy: For the full year, we have opened about 1,050 stores. Next year, we are planning to be in the range of about 500, 600 stores and then see what's next.

Tushar Manudhane: So any particular reason for this, why, in terms of slower pace of addition of stores?

Obul Reddy: We want to focus on the productivity, and that is the reason we want to add lesser stores.

Tushar Manudhane: And at the same time, certain incremental cost has come up. So, for FY 24, how to think about the offline pharmacy distribution margin?

Obul Reddy: So, we expect to be some offline about 7%, that's the structured business, which is the range of 7%, about 0.5% maybe around 7.5% to 8%. That will be the range.

Tushar Manudhane: And on the hospital side, the operational beds has been pretty stable in FY23 as well. So, while the occupancy can increase to 70%, but the bed addition will happen only after a major capex layout, or is there a scope of adding beds at the existing sites?

Dr. Hariprasad: There are some beds which will be added, but it will probably be in the year after next. It is particularly in Bangalore and Mysore, which is where we are looking at some additions. So mostly after next year.

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Moderator: The next question is from the line of Aneesh Deora from Nomura. Please go ahead.

Aneesh Deora: So firstly, a clarification on the private label contribution that we've reported. So this quarter, it was 15.5% of the total pharmacy revenue, whereas in the previous quarter, it used to be in the range of around 11% or so. So, have you like club private label in trade generics in this quarter as was reporting?

Obul Reddy: That's right. Pure FMCG private label is about 11%, with generics, it is about 16%.

A. Krishnan: There were a set of people who are asking for this number after the last call, and which is why we decided to include this as well.

Aneesh Deora: So the generics is basically of the other players. It's not Apollo private, I mean, Apollo manufacturer or like it's the trade generics of other places?

Obul Reddy: We have a few SKUs, Apollo manufacturer and we'll be adding further into that, even on the generic side.

Aneesh Deora: And secondly, on the hospital's ARPOB basis, the clusters. So, if you look at the ARPOB increase, it has been a bit diverse across the clusters like certain clusters have seen a very healthy ARPOB growth during the year while certain have been flat. So, any particular reason and any particular trend that you can see out of this particular thing?

Suneeta Reddy: So, let me first say that it is a function of case mix as well. And I think we have done some tariff increases across the board. So, across the hospital division, there was an increase in ARPOB Q1 too.

Dr. Hariprasad: So we are you know, we would be, we have seen a good increase in ARPOB. And as we go

into the next year, we are also, the one division that didn't see some increase was the AP, Telangana region because we also have Vizag, et cetera, which was there in this region. So while Hyderabad the ARPOB has increased, given that it's the region that you're looking at, there is, it's not showing an increase there because given that it's not a Tier 1 metro, that ARPOB is a bit lower and that occupancy as it goes up, it impacts the overall ARPOB more from a mix perspective. Otherwise, all the other

regions, we are fine. Other is also something that we are hoping that in this coming year, we should see a 5%, 6% increase in ARPOB.

Aneesh Deora: And if I can just squeeze one last in, just a clarification on the share of profit from the joint ventures and associates that we report. Last quarter, there was a loss of about INR 20 crore that was reported and in this quarter, there is a INR 3 crore profit. So, it has been fluctuating quarter-to-quarter. So any particular reason for this? And how should we be looking at this number going forward?

A. Krishnan: Let me come back to you on this because there was nothing specific that. I'll come back to you offline on this because I have to just look at that reasoning, reason for the same.

Like maybe there was a one-off last quarter. Let me come back to you.

Suneeta Reddy: Because all of them are currently profitable.

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Moderator: The next question is from the line of Shaleen Kumar from UBS. Please go ahead.

Shaleen Kumar: So as we have improved our payer mix, moved out of CGHS kind of (inaudible). I think actually our margins should improve. So, is there any one-off cost etcetera lying in the fourth quarter?

Suneeta Reddy: Just margins should improve on hospital services, you were not clear in the beginning, is this hospital services?

Shaleen Kumar: Yes, ma'am, hospital services. I mean, sequentially margin, I mean, so I'm just wondering if there is a scope for margin to improve here.

Suneeta Reddy: Payer mix work was started only barely five months ago, and you will see that there has been a 249-basis points improvement. We are at 249 basis points improvement, at 24.6% EBITDA margin. So clearly, there is an improvement.

Shalin Kumar: I was basically comparing last quarter, Q3 versus Q4. So just wondering if there is more that we can have just in case there's any one-off costs here in Q4?

Management: Nothing specific.

Shaleen Kumar: Nothing specific. Okay, sir. And just sequentially, again, the physical pharmacy revenue has not grown or rather marginally declined. Is it because of rationalization of the store and it should pick up?

Obul Reddy: Physical stores, in the back end, it's a structured margin, which stays around 7.8%.

Suneeta Reddy: Growth is 31% sequential growth.

Obul Reddy: Sequential growth, this quarter, we have a slightly slower growth compared to, generally our Q2, Q3 will be the best. And Q4, we generally have a muted growth, and that is the reason. Otherwise, even we are at a growth of 2% sequentially.

Suneeta Reddy: No, I think I have to state that across the board that January is traditionally a month of many holidays in the South. So therefore, if you're comparing sequentially, yes, there was in terms of both occupancy and hospital utilization as well as maybe pharmacy, there could have been a lower growth. If it's sequential, there was also a mild COVID wave, which was only resulted in people taking Paracetamol. So, you would not have seen huge growth in the fourth quarter because of this mild COVID wave and because the South hospitals and pharmacies in the South really didn't see much traction because of the many holidays.

Moderator: Next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: On the Hospital Services EBITDA, just reconfirming, you guided to a 15% growth in the EBITDA for the hospital services for this year?

Dr. Hariprasad: So yes, it should be higher. The growth in EBITDA is what you're saying, right?

Nitin Agarwal: Yes.

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Dr. Hariprasad: Yes, it should be higher than that because we are hoping that the overall growth is closer to that 13% to 15% on revenue and EBITDA should be higher than that because of the expansions that we are planning on both operating leverage as well as cost cuts.

Nitin Agarwal: And sir, when you look through beyond the first year, I mean, beyond FY 24, if I take like a 3-year view of the hospital services business, at what rate, I mean, what's the broad range in which we think we can grow our EBITDA from FY 23 base?

Dr. Hariprasad: I think we should continue to look at the same numbers because it is possible to grow at a similar number.

Suneeta Reddy: So, the advantage that we have is that we have existing capacity. So without investing too much, we are going to increase occupancy. Post, in '25, '26, we will have new beds coming up.

Nitin Agarwal: Ma'am, you talked about the occupancy going to 70% for FY 24 versus 64%, which is a very large increase. I mean it should ideally lead to a much disproportionate increase in profitability for these hospitals, if we can get this kind of occupancy increase for the year.

Suneeta Reddy: See, I think we should set our targets high. And really, we have the potential, and this 70% is by quarter 4. So, like I said, we are working on these initiatives, including improving our international patient mix. And with this, we should be able to get 70% occupancy by quarter 4. It won't happen overnight, but definitely in quarter 4.

Moderator: Thank you. The next question is from Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: So, my first question is on the EBITDA for Pharmacy segment. So, our back end, when we had done the deal, so the guidance was that 80% of the EBITDA will be captured in back-end pharmacy. But in over time, it has been sort of almost equal or higher. So, this quarter as well INR 148 crore was captured in the front end, out of a INR 145 crore was captured in the back end. So, when does it go to 80%? Or is it something that our back-end EBITDA margin will always be at 7.5% to 8%? And what are the additional costs that are being incurred at the front-end pharmacy?

Obul Reddy: As you appreciate, the back-end pharma business is a structured business. As a EBITDA percentage, it will be in the range of about 7.5% to 8%, depending upon the cost and other sales movement during that quarter. It remains largely in the range of about 78% to 82% versus the front-end business. As you are aware of it, this year, we added about 1,000 stores in the front-end network. We have higher onetime and infrastructural costs. So to that extent, as a percentage, it might look a little slightly different, but we will be back in the current year, by end of the current year, we expect that to be in the same range as we explained earlier and as we planned.

Sayantan Maji: For an online order, what are the additional costs that are being incurred at the front end?

Obul Reddy: We don't have any additional costs other than the delivery costs associated with us

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because the inventory is coming from the store inventory and only logistics handling costs. And there is a set of dedicated employees. So, we are largely manageable within the overall

cost structure.

Sayantan Maji: And my second question is on 24/7. So, in 24/7, we have guided for INR 20 crore to INR 25 crore, say, deduction in costs. So, this INR 189 crore, if I just look at the operating cost, so that is expected to go down to, say, INR 170 crore. And what is the next set of investments that we have to do in FY 24? So, are we mostly done with the product? So, what is the next set of investments that you envisage in 24/7 in FY 24, even at say, INR 170 crore quarterly run rate?

Sanjiv Gupta: So I think as far as the platform is concerned, I think more or less the entire second product investment has happened. We don't see that you know while we have a target to grow our business by 100% in current year, I'm not seeing any potential investment into the business, other than the fact that we had a onetime tech and product development for the insurance and B2B corporate side of it. There is a clinical intelligence engine, CI, which is taking a bit of investment, but I guess that, that is something which we can absorb within the various cost initiatives that we have planned for the Company.

So there should not mean incremental cost, but having said so, the way we look at is that what is our expense as a percentage to GMV. If you look at in Q1, start of the previous financial year FY 22, FY 23, we were at somewhere at 66%. So, our expenses as a percentage to GMV, started coming down to 52% in Q2 and by Q4 at the end, it is 39%. So basically, INR 189 crore divided by INR 593 crore, which is about 32%. Now the target for the current year, which is FY 23, FY24, is to bring this down to a range between 20% to 23%. And so, I think from that sense, we are into a well-defined, the entire activity in such a way that if there is an incremental cost on any of the technology that we need to put in, that could be only digital area where investment will go. But at this stage, we don't see any material investment into the product and tech side, apart from ongoing ones.

Moderator: Ladies and gentlemen, we'll take the last question from the line of Rishabh Tiwari from Allegro Capital Advisors. Please go ahead.

Rishabh Tiwari: My question is regarding the hospital services business. Once we reach the occupancy level of, say, 70% by this year, what are the number of beds that we are looking at adding in over the next two, three fiscal s? Because as a group, my understanding is we are seeing AHIL and HealthCo as a growth lever. But within the healthcare services, what are the short term, you know few years, what is the growth that we are looking at?

Suneeta Reddy: We have 2,000 beds planned, by 2027. Every year we'll add a new hospital into the system. And close to around 700 new beds after '24.

Rishabh Tiwari: So just to confirm, we are adding around 700 beds per year after FY 24.

Suneeta Reddy: Yes. Minimum. Yes. We continue to look at brownfield and we continue to look at

acquisitions.

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Rishabh Tiwari: And these acquisitions would be focused on Tier 1 cities?

Suneeta Reddy: Yes.

Moderator: Thank you. I now hand the conference over to the management for closing comments.

Suneeta Reddy: Thank you all for taking time off today. It was a pleasure answering your questions.

Krishnan, Sanjiv, and the entire team would be happy to take any other further questions. You may e-mail them. Let me promise you that we are committed to not only growing this enterprise, but towards delivering outstanding world-class clinical healthcare, which is the crux of really what Apollo means for everyone. And let me also reassure you that we continue to improve in terms of market share, in terms of clinical leadership, and our commitment to our patients. Thank you all.

Moderator: Thank

you very much. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines. Thank you.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility of such errors, although an effort has been made to ensure high level of accuracy .

Call: Aug 2023

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Apollo Hospitals Enterprise Limited
Q1 FY24 Earnings Conference Call
August 11, 2023

Moderator: Ladies and gentlemen, good day, and welcome to the Apollo Hospitals Limited Q1 FY24 earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone telephone. Please note that this conference is being recorded. I now hand the conference over to Mr. Devrishi Singh from CDR India. Thank you, and over to you, sir.

Devrishi Singh : Good evening, everyone, and thank you for joining us on this call to discuss the financial results of Apollo Hospital for the first quarter of the financial year '22-'23, which were announced earlier today. We have with us on the call today, the senior management team comprising Mrs. Suneeta Reddy – Managing Director; Mr. A. Krishnan – Group CFO; Mr. Sriram Iyer – CEO of AHLL, Mr. Ashish Maheshwari – CFO of AHLL; Mr. Obul Reddy – CFO of the Pharmacy division; and Mr. Sanjiv Gupta – CFO of Apollo 24/7. Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide number 2 of the investor presentation that was shared with all of you earlier. Documents relating to our financial performance have been circulated and these have also been uploaded on the corporate website and the websites of the respective stock exchanges. I would now like to turn the call over to Ms. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy : Good evening, everyone. Thank you for taking time for this earnings call. I trust that all of you have received the earnings document.

We are pleased to commence FY 24 on a positive note with a strong first quarter, characterized for continued growth in topline, improved volumes and payer mix, meaningful network additions, and further growth in the user base for our digital offerings. Our Healthcare Services witnessed a robust 13% year-on-year growth in Q1FY24, even as IP volumes were 6% higher year-on-year. Within this, the self-pay and insurance volumes grew by 10%, while revenues grew 21% year-on-year. Insurance revenues now contribute 44% of our total IP revenues. Overall, occupancy across the group was at

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62%, mature hospitals at 63%, new hospitals at 60%. ARPOB on an overall basis has increased by 11% year-on-year to 57,760.

Against this backdrop, let me walk you through the consolidated financials :

Consolidated revenue grew by 16% on a year-on-year basis to INR 4,418 crore .

Healthcare Services grew by 13% to INR 2,294 crore , Mature Healthcare services grew by 10%, while new hospitals grew by 23 %. Revenues from Apollo Health Co stood at INR 1,805 crore in Q1FY24, a growth of 22% year-on-year. Combined pharmacy grew by 24% on a year-on-year basis. Apollo Health and Lifestyle revenues registered 19% year-on-year growth at INR 319 crore in Q1FY24, excluding the COVID impact.

Consolidated EBITDA was at INR 509 crore , registering an increase of 4% year-on-year. Within this, the Healthcare Services EBITDA grew by 12%, and Healthcare margins were at 23.6%. Margins in mature hospitals were at 26.8% as against 26.4% in Q1FY23. Margin at new hospital was at 16.7% for the quarter as against 17.7% in

Q1FY23. The

drop in margin was account of a few factors, an increase in overall surgical discharges resulting in higher material consumption, increase in marketing expenditure for the new hospitals, and focused investment in clinical talent in these hospitals for future growth. The pharmacy distribution business in Apollo recorded an EBITDA about INR 125 crore , year-on-year growth of 13%. Operating costs and ESOP charges were at INR 204 crore . There is a INR 13 crore lower than cost in trailing quarter of Q4FY23. EBITDA loss in the

company was at INR 57 crore, down from INR 72 crore in the trailing quarter. This is in line with - keeping in mind our commitment to reduce costs in this vertical, the business is on track to achieve operational breakeven in Q4FY24.

AHLL recorded an EBITDA of INR 23 crore as compared to an EBITDA of INR 29 crore in Q1FY23. The drop in EBITDA due to investments in network growth and specialized manpower. The margin profile is expected to improve with revenue ramp-up from the network as well as focus initiatives to drive process efficiencies.

Consolidated PAT was at INR 167 crore. Healthcare Services PAT was at INR 264 crore, a like-for-like growth of 19%. In substance, this was a strong quarter across several dimensions and metrics, which we have identified in areas of focus.

Within the Healthcare Services business, we have delivered an ROCE of 25% with balanced ROCE across all our geographies, the Metro, the Tier 1, and the Tier 2. We believe we have the most diversified footprint in the portfolio and significant headroom for growth.

We have continued to improve our payer mix with visible results in our ARPOB. The diagnostics vertical with AHLL recorded core revenue growth of 48% on a year-on-year basis and surpassed the revenue run rate of INR 100 crore for the quarter.

Clinics core revenue grew by 23% year-on-year. Margins were impacted due to investments in network expansion as well as specialized manpower. Several initiatives are underway to improve diagnostic margins, including platform changes and improved

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asset utilization.

Private label and generics business contributed 16.02% of total pharmacy revenues. This was an improvement of 168 basis points over the same quarter last year. Our digital platform 24/7, added 2 million new users this quarter. The platform GMV was at INR 623 crore, a growth of 189% on a year-on-year basis. We have achieved this GMV while reducing cash burn and are well on track to achieve operating breakeven in this vertical in Q4FY24.

This quarter has signalled that the investments in strategies adopted by us are gaining traction and are largely playing out to the plans that we had. We remain firmly committed to building the strongest integrated omni-channel health care network with the consumer at the center, ensuring access to high-quality health care across the value chain. We will do this while achieving a healthy EBITDA margin and strong ROCEs. We believe we are building a sustainable and value-enhancing platform, guided by fiscal responsibility, and a unique consumer value proposition as our dual objective.

I would like to take this opportunity to introduce our new CEO of Apollo Health & Lifestyle, Sriram Iyer. He was previously Chief Revenue Officer at Metropolis and brings 25 years of retail experience to the table.

On this note, I would like to hand over to our moderator and open the line for questions.

Moderator: Thank you. We have the first question from the line of Harith Ahmad from Avendus Spark.

Harith Ahmad: So, my first question is on Apollo 24/7. I am looking at the GMV for the quarter, there's an 11% quarter-on-quarter growth. But when I look at the revenue that you've disclosed for the

online pharmacy distribution of Apollo 24/7 segment, there's almost 20% quarter-on-quarter decline. So, can you help us understand this disconnect?

Sanjiv Gupta: Thanks for the question. My name is Sanjiv, I'll pick up this question. I think there are two important things that have happened in the quarter. And I would first refer to the Q4 earnings call wherein we discussed about that how in the platform we've reduced our discount to sub 15%, and at that point of time, January and February months were the time when the discounts to the pharmacy vertical was as high as about 17% to 18%, and we did bring it down to sub 15%.

So, one impact that has happened during the quarter is that a lower discount of roughly 13.7% that we're running in Q1 has impacted second top line. And I think it is a conscious decision by the management to let go those value seekers onto the platform who are looking out for only the discount. In fact, as we have been continually working on the model, our discounts have always been lesser than the market.

So the first impact that has happened to the Q1 impact on a lower 5% growth Q2 versus Q1 on the GMV or the lower revenue-to-GMV ratio is on account of discounts being lower, that reflected into lower pharmacy sales. Second thing, what we did is that we also looked at those orders, which are not profitable at all. And orders which were less than INR 200 of bill value are those orders, which we have let go from the system, and we are not

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fulfilling them, and apart from the fact that there are certain PIN codes where logistic costs or the fulfillment cost of the last mile is so very high, that it does not make any sense to take those orders.

So next thing what we did is that while we improved our unit economics and the cost line and majority of that benefit will also accrue in Q2 and some benefit has accrued in Q1, this has resulted into about 7% to 10% dip in our pharmacy revenue. So that is the reason you would see that revenue -to-GMV ratio, which was about 42%, 43% in Q4 has come down to about 33% to 34%. So, slowly and steadily, as we see July month and as we see this quarter, I think this will further improve. But the essential reason behind a lower revenue -to-GMV ratio or a 5% growth in GMV in Q1 versus Q4 is only to make sure that the business is more viable, more profitable.

Harith Ahmad: Is this revenue -to-GMV ratio that we should be factoring going forward, around 20%, 30%?

Sanjiv Gupta: I think it should be better. What happens is whenever you do a structural change to the business, there is a fall that we see on an immediate basis. July, we witnessed a little bit, but I think August is tracking very well. And I think we should be moving up from this 33% to 35% to 40% in this quarter at least.

Harith Ahmad: Okay. Got it. The second question is on your guidance of INR 10,000 crore of revenue and 6% EBITDA margins for the combined pharmacy business. So firstly, I wanted to clarify if this 6% is a post -IndAS margin guidance that we have given.

Suneeta Reddy: Sanjiv, will you take this or Obul is also here? Obul?

Obul Reddy: In line with our combined business which we have seen, this can still be sustained.

Harith Ahmad: Yes. So there is obviously, when you're thinking of the post -IndAS margins, there is IndAS benefit as well. And that's, in the past, been around 300 to 350 basis points.

Obul Reddy: This is at the business level.

Harith Ahmad: Yes. Understood. So when I deduct the IndAS benefit here, the combined pharmacy EBITDA margins are maybe in the 2.5% to 3% range, and this is lower than what we used to have in the past. So, I understand there's a higher level of discounting, but you talked about some additional costs in the front -

end pharmacy level in the past, in the last

few quarters, how are we tracking on those? And how should we think of margins in the combined pharmacy vertical?

Obul Reddy: So, we have the same cost continuing. In fact, if you look at it, the discounts have slightly moderated versus last year and Q4. However, the network cost in terms of the store additions and almost about 20% of our stores are yet to reach breakeven. It may take a quarter or two more. So in Q1, we have that costs coming in and impacting the profits. Otherwise, we will be back to normal level.

A. Krishnan: And you have to remember, this is the combined business, which includes the online as

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well. So online is where there is higher discounts. So if you look at offline, this is online, offline would be higher in this mix.

Moderator: We have the next question from the line of Saurabh Kapadia from Sundaram Mutual Fund.

Saurabh Kapadia: In your region -wise operation parameters, if you look at Tamil Nadu and the other region, the inpatient volume as well as outpatient volume has been muted. So, any specific reason for this?

Suneeta Reddy: So, there was a 6% growth in the inpatient, outpatient as well. So, in Tamil Nadu region, the reason for that is clearly, one, it was holiday season. The second is that, let me say, travel to Tamil Nadu was restricted because of train logistics as well as some of the airlines cancelling that route. But now it's reinstated and July is looking very good.

Saurabh Kapadia: Okay. And similar case for the other market where we saw the outpatient volume down by (inaudible) %?

Suneeta Reddy: The previous year had the RT -PCR test, which was COVID test, and therefore, the OP looked high. But now without COVID, it has grown. On a like -for-like basis, there is growth.

Moderator: We have the next question from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just one on overall occupancy. So, 62%, I know you're comparing with 60% a year ago, but that may have been impacted because of COVID and other stuff. So, I just want to understand the level of occupancy, and this is not only you, right, even other companies, I know there's a seasonal issue here in Q1, but has been below expectations, right? So, is there an element of either pent-up that's not coming back, or you generally think this is just a seasonally slowest quarter in the year and things will pick up? So just a color on yourself as well.

Suneeta Reddy: So, it's definitely picking up. This has happened more in the South; I think we were impacted. It was very hot summer. It was vacation season. And like I said, travel was restricted to one of our networks, which is Chennai. But I think it is seasonal. And July is looking extremely good. So, it's clearly because of some of the things that happened in the first 3 months.

Shyam Srinivasan: When we had an aspiration to reach 70% and over time, 75%. Is there any impediments to reaching that numbers? Or do you think over time, our network will be able to get there? Is there an issue of, say, cannibalization in the sense that people who were earlier traveling to Chennai are no longer able to travel or they're going to another network hospital of yours. Are you seeing those dynamics, which will prevent us from reaching historical high occupancies, you think?

Suneeta Reddy: So Chennai is clearly, is there now in terms of, so people are still traveling to Chennai for quaternary care. What is good is that our Tier 2 hospitals also are doing much better. And there is a 17% growth in Tier 2, which I think is a very good signal for, and it's

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rewarding to see that our geographic footprint and the impact on ROCE has validated this strategy.

Shyam Srinivasan: Okay. And my last question, I'll keep it brief. We added, I think, 900 stores last year. We have added 30 stores in the first quarter. So, I just want to understand what's our target for this INR 10,000 crore, don't we need additional stores, or we think we're going to have a much lower run rate this year?

Obul Reddy: We have factored about 500 to 600 stores to be added during the year. Q1, consciously we slowed down because in the Q4 last year, we added about 370, 380 stores. And in the last month of March, about 150 stores. So, we want to give that space to stabilize those stores. And Q1, we are back to our normal store openings, and we expect to be there between 500 stores and 600 stores for the year.

Moderator: Thank you. We have the next question from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: My question is again on Apollo 24/7. So Sanjiv, you mentioned unit economics are improving, you're reducing losses, etcetera. But GMV growth has been, I'll say, slow compared to what we are expecting in last two quarters. And then earlier, you have set a target to double up GMV in '24 compared to '23 level. So, do you think you can still achieve that or you're focusing more on reducing losses?

Sanjiv Gupta: So I think it is both the things. First important thing is to ensure that the business is; only those transactions onto the platform are run which are making some economic sense to the business, while also looking into the fact that we have a target in our minds. We did about INR 1,650 crore in the previous year and then obviously, we guided INR 3,000 crore. We did INR 622 crore in Q1 and I think the run rate itself is INR 3,500 crore. And while we tapered down certain volumes in Q1 started March to till end of June. But I think, we've got various other initiatives to ensure that we fill the gap quickly with new set of customers, new offerings and we increase the average order value. In fact, our average order value has also gone up in spite, if you could look at it, INR 845 previous year versus INR 935. So, I think we should be able to reach the target of INR 3,000 crore; very early in the stage to kind of say that we'll not be able to hit. I think INR 622 crore is a decent number for Q1, and we should be meeting our target. This is what I would say at this point of time.

Damayanti Kerai: Okay. I want to understand the discount part better. So do you suggest that there has been some like seasonality here also? You mentioned there were higher discount in February and March, that's why a bit of more volumes, but now it has reduced. So can you talk a bit like, how your discount work for, say, offline orders and online orders across the year? What are the patterns or how do we see things moving there?

Sanjiv Gupta: I think as far as the discount strategy if I just talk about; discount strategy has always

been a delta between offline and online to the extent of about 150 bps to 200 bps. At no point of time, this gap was higher if I look back in the last six months. Prior to that, obviously, it was a very different ballgame where we were also giving a discount of about 18% to 20% and offline was steady at above 10% to 11%. As we move more into omni

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play as more -and-more customers have started transacting on both the platforms, obviously, we have to, at some point of time, rationalize the entire thing looking into the scale, looking into the right time, and this is what we did somewhere in March month, that more -and-more omni players started happening across the channels, and it is

to the

interest of the customer that we should not confuse him or her with differential pricing, and that is where we curtailed down on our discount. So, this is a good move as far as the customer side is concerned. Obviously, it does give a benefit to the Company. As far as the current year is concerned, we are running at about 13.7% for Q1. I think this will continue to be in this range for Q2. And I don't see any reason that we should be inching up in Q3 or Q4, maybe 10 basis points, 20 basis points here and there. It could be max 14%, if not 13.25% to 13.5%. So, something in between. And I guess, as far as the offline business is concerned, they will continue to be around 12% to 12.5%, only a gap of 100 basis points to 150 basis points. So, I think directionally, all of us should look into these discounts for the rest of the year.

Damayanti Kerai: So somewhere you said 12% to 14% will be the broader range for discount. Just want to...

Sanjiv Gupta: Yes. That's right. Yes.

Damayanti Kerai: And my last question is, if you can provide, this question is for Suneeta ma'am. Volumes are a bit muted, but you have continued to perform well on ARPOB growth, etcetera. So how should we look at ARPOB growth outlook for, say, next one year to two years?

Suneeta Reddy: So just one thing on the ARPOB growth has been over 10% improvement in ARPOB 11%. And this is a function of both case mix and tariff increase. So, I think this year, this 11% is; this INR 57,000 moving to maybe up to INR 60,000 is something that we can sustain. In terms of how do we look at occupancy, I think that is the important question that we have headroom for growth and that in places like Chennai, we're already 41% of total market share. And we see, we are growing this. So, with this, I think it's going to be, as we go forward, this was asked earlier, it's going to be higher occupancy, resulting in higher EBITDA and also higher EBITDA margins coming from the cost, fixed cost being met. And therefore, we are quite confident that, that this is the way forward.

A. Krishnan: The other thing which has also happened is that if you look at the case mix, the case mix has also improved for the better. We have seen that the Congo case mixes are going up, which is all the high-end cases that we are doing, which is the other reason that ARPOB is high. Secondary care cases have also gone up because, clearly, with the insurance and the payer mix helping us, there is a tailwind in insurance, which we believe is here to stay for the next several years. We have just started seeing the traction in insurance, which is good. So clearly, now we are seeing that a lot of people also are coming for secondary care into the hospital, which is why ALOS is down. So, ALOS being down is also a matter of, even if you look at the overall volume, volume growth is 6%. But if you look at insurance and self-pay, the volume growth is actually 11%. So, we had let go, as you know, last year, by Q4, we had let go of lot of CGHS patients across the system, which is why when you compare Q1 to Q1, you look at Q1, which has CGHS and

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institutional volumes, but now the volume doesn't have that, but still we have grown 6%. So with that, which is why insurance and self-pay when you look at the 11% is the volume growth and that on revenue growth is actually 20% on the IP side. We are showing a 13% overall revenue growth for the quarter. But we know that the overall mix of the cases that we have are very good. This occupancy is something that we are working on, and we will see that also go up because it is, as Suneeta said, Chennai is higher. It is some of the business, some of the units outside of Chennai, like Madurai, Trichy where we have some lower occu

panies, but ROIs are all good. So, if headroom for growth exists, we have to figure out how to fill it up now. So, we are working on that plan as well. So broadly, I think structurally, if you look at the P&L, structurally, the P&L is quite strong, and it results in higher ROIC also because of that. Occupancy is something, I guess, over the next two years, it's like, we look at it from an opportunity perspective. Damayanti Kerai: Okay. So ARPOB, we should be sustaining at a healthy level. Yes, I'm just clarifying. Okay.

Moderator: Thank you. We have the next question from the line of Kunal Dhamesha from Macquarie. Please go ahead.

Kunal Dhamesha: So the first one on the 24/7, it seems that in order to kind of improve the profitability, we have put some filters in terms of our strategy to refine our user base. But how would that have impacted our TAM when we look at what our strategy a year back versus now been? So like how much reduction we would have seen in TAM because of this?

Sanjiv Gupta: You're referring to CAC, right, cost of acquisition?

Kunal Dhamesha: Not cost of acquisition, just because, let's say, we are putting a filter that below INR 2,000 order you would generally take. In that case, that would also reduce our total addressable market based on the affordability, etcetera, right? So how would TAM calculation would have changed for you?

Suneeta Reddy: It's INR 200.

Kunal Dhamesha: INR 200. Okay. I misheard that. Okay.

Sanjiv Gupta: Yes, that's the INR 200. And I think, see, we also need to understand, at times what happens is that customers have a habit of breaking their entire ; normally, any e-commerce player would like to have a basket shopper into the platform versus the top-up shoppers. Now chronic customers are supposed to be taking medicine at least for the whole of the month. And if not, at least it should be for two weeks or three weeks. Over a period of time, we also saw certain twin quotes and certain customers behaviour given the fact that you've got the analytics behind, now we are in the fourth year. We've got three years of experience there. We found out that there are certain geographies, there are certain set of customers who are good shoppers, but they have a tendency to reduce their shopping cart by as low as INR 200, INR 150, and they continuously order. So, what we have actually done is that certain customers, who were actually only giving us a total value of about INR 600 to INR 700 in the entire month, those are the ones which we believe that, and if they are not chronic, are those set of customers to whom we let go.

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And major impact has started coming in from the value seekers. The people who had a tendency to take the orders where the discounts are very high. And I think that is the time that we are now no more in that particular stage, or rather phase 2 of the Company is very different, where we would like to have those set of customers which are chronic and are associated with the Company for a little longish and also cross-pollinating between the other verticals, be it the consultation or the diagnostics.

I think as far as the overall canvas is concerned or a future potential is concerned, I think online business has got a vast thing, in a market. And in a true spirit, omni is actually giving us a larger wallet share of any customer. The Aventus suggested that customers are becoming omni during 30% more wallets share versus when we were not omni. So from the strategy point of view also, it's good to have those set of customers which are chronic, which gives us a high repeat. Our repeat for Q1 has been about 27% versus 21% in the previous year, same quarter. So that is also a very good metric. So, I think we are running

into a right direction from the strategy and the execution point of view.

Kunal Dhamesha: Sure. And the transacting user number that you have given for this quarter and comparable quarter, would you be able to share the number for quarter 4?

Sanjiv Gupta: Which specific number are you looking at it?

Kunal Dhamesha: Transacting user, right, which is INR 11.4 lakh, which is what is in returning, transacting user base?

Sanjiv Gupta: Yes, that's right. So, in the previous quarter, Q4 if you are referring to, it was about INR 9.9 lakhs.

Kunal Dhamesha: And the second question is on the payer mix. I think we have shared that insurance is around 44%. But would we be able to share the other channels as well? And let's say, we have been doing this payer mix refinement for last one year. But let's say, when I look at it from two years to three-year perspective, pre-COVID let's say, FY19 or FY20, how would have our payer mix looked? And from there, how much you would have done in terms of refinement?

A. Krishnan: I think pre-COVID self-pay was almost around 45%. Now if you look at self-pay as a percentage of revenue, it is 40%. Insurance is where we have had significant increase, where it was close to 25% to 28% pre-COVID, that has come to 44% of our revenues now. So clearly, between insurance, private pay and self-pay and private sector, we have almost around 80% of our revenues is coming from that, 80%, 82%.

So what has happened is we have definitely been able to focus on bringing down some of the Government and some of the low-paying cases across. And this is the new normal that we would like to believe, we will continue to go into as we proceed as well. IPS is an area that will further grow because IPS growth for the quarter has been good. Hopefully, once we get our presence in Delhi also in the next 18 months, we should be able to get higher IPS growth because Delhi is one of the places where IPS is high. So, in our consolidated results, we don't provide IPS because otherwise, IPS is today 7% of our revenue here, whereas in Delhi, it is 15%. So that is the other area that we hope that we

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should be able to tap into as we move forward, which should change a bit as later. But otherwise, this is the new normal.

Moderator: We have the next question from the line of Lavanya from UBS.

Lavanya Tottala : I just wanted to check that healthcare revenue growth is driven by ARPOB largely. But sequentially, our margins are broadly similar for both mature and new hospitals. So why is the ARPOB is not translating to higher margins?

A. Krishnan: So in both, there are different answers for both mature and for new. In mature, one of the things that we have seen is there has been a lot of increase in some of the ; we have seen good increase in the surgical mix, which actually augers well from a longer perspective . So, the surgical mix increase has resulted in a higher cost of materials in the mature hospitals. This is obviously aided by higher insurance, as I said, someone who's covered more by insurance has the propensity to come to a better-quality hospital or is definitely choosing us over some of the others and this is one of the reasons that we are seeing high-end cases as well as secondary care cases go up. So, cost of , surgical material costs have gone up. But what we have also seen is that the doctor's fees related to that went up a bit in this quarter. But as we move forward, we , there is , we had taken some tariff correction in the middle of the quarter. We think that will start paying off. And into the next quarter, etcetera, we will see the margin go up on mature.

In the new, we have added doctors because one of the things which we have also done is that to focus on the

utilizing occupancy, we had seen doctor gaps in places like Navi

Mumbai and some of the others like Vizag, where we have added doctors on guaranteed money fees. This, we believe will start paying off over the next 2 quarters, and then that margin should also come back. So these are the reasons for both these segments. Some bit of marketing cost has also gone up. We did spend a bit of marketing in the new hospitals as well.

Lavanya Tottala : Okay. Got it. So , this should sequentially also we should see improvement in margins of both mature and new hospitals?

Management: That's correct.

Lavanya Tottala : On 24/7, can you give the split of GMV for this quarter, like in terms of pharma and diagnostics?

Sanjiv Gupta: Yes. So , Pharma, we did roughly INR 350 crore for the quarter and diagnostics was roughly INR 30 crore , and the remaining was coming in from consultations and the entire IP/OP business.

Lavanya Tottala : Okay. What would be for the split for a previous quarter, like Q4?

Sanjiv Gupta: Q4 for pharmacy was INR 375 crore and diagnostics was INR 42 crore . Diagnostics went up by about 16% to 18% for the quarter. And pharmacy, as I said, in the previous question that we took certain decisions to let go those volumes which are not profitable to the organization and that is the reason we see that gets settled in Q1.

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Lavanya Tottala : If I may ask last question, what is the tax rate?

A. Krishnan: I'll just answer that question, which she actually was raising. The tax rate is 25% overall

for the Company. And in the standalone, it is 25%. What happens in consol, you see it at a higher number. That's because there are losses which comes from Apollo HealthCo and Apollo AHLL, where there is no tax, which actually skews the tax rate to close to 32%, but the tax rate at the Company is today operating in the standalone is 25%. So one or two of our subsidiaries is still at that older tax regime, like Bangalore will move into the new taxes next year. And I think the same is the case with Lucknow. So these are the two which are in that older tax regime. They will also move to the newer tax regimes in the coming fiscal year.

Moderator: We have the next question from the line of Rishabh Tiwari from Allegro Capital Advisors.

Rishabh Tiwari: In the previous quarter, you issued both combined pharmacy EBITDA and someone previously asked that it was not quoted as well as we are expecting some ramp-up from the recently added stores. So if you could please tell the combined pharmacy EBITDA? And second question is regarding the IndAS impact on the EBITDA, there used to be a slide on this. If you could give a ballpark guidance on that.

Obul Reddy: On the combined pharmacy EBITDA, as I explained to you that last year, we added about 1,000 stores, and it will take about 1 year, you know 12 to 15 months for the breakeven. So those higher number of stores contributing to the losses has impacted, and we will be back in the next 2,3 quarters. And...

A. Krishnan: On the IndAS to post-IndAS, I'm sorry, that was a miss in the slide. I think we have not sent that. We will add that and put it back there. I think that's the same number as what was there last quarter, pre-IndAS to post-IndAS, I think INR 30 crore at the Healthcare Services level and INR 20 crore at the AHLL level. We'll put it back out there and that slide, we will add and sent it back. We will upload it on the website. Sorry about that.

Rishabh Tiwari: Just a clarification. So, there is no EBITDA improvement on the combined pharmacy.

Obul Reddy: So, it will be through the volume and cost-cutting next 2 quarters, and our improved profitability on the new stores.

Moderator: We have the next question from the line of Naushad Chaudhary from Aditya Birla Sun Life Asset Management.

Naushad Chaudhary: Firstly, on the occupancy side, the target which we have 70%, can we reach and sustain that 70% for the long period of time? Or do you think it will be like touch and then plan for a sizable capex?

Suneeta Reddy: No that 70% will not require any capex, but this will take us a little time to get to 70%. We're already trending. Probably, we'll reach close to 70%. But while we do have a target of 70%, we will not be able to deliver it and it will require no capex.

Naushad Chaudhary: No, I'm asking can we sustain; once we reach there, can we sustain 70% for a long period

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of time?

Suneeta Reddy: Yes, we can in those regions. So overall, as a Company, maybe not. But I think in each of the regions where we touch 70%, we should be able to sustain.

A. Krishnan: Yes. And to also add on to your point, as we said about the new hospital brands, which we are doing you know the 2,000 beds and INR 3,000 crore over the next 3 years, the first stock of beds are coming in the two regions where we have more than 75% occupied, one is Kolkata and the other is Bangalore, which we are looking at. So these are the 2 that will start off. Then we are looking at Gurgaon, which will follow in the year after that, where again, our occupancy is over 75%. So, we are quite clear about how we are looking at the strategy of expansion. Those are new markets. These will not impact our existing occupancies at all.

Naushad Chaudhary: I understand. I'm just trying to understand on the blended level, 70% is doable on a sustainable basis or not, on a blended level?

A. Krishnan: Yes, at this level, but the new hospitals will take time, right, to get to 70%, any new hospital that would come.

Suneeta Reddy: Up to 2026, yes.

Naushad Chaudhary: Without compromising on the future growth.

Suneeta Reddy: Yes.

A. Krishnan: Yes.

Naushad Chaudhary: And lastly, in terms of the scalability of your health care business, if you hypothetically think of doubling your health care revenue from current base, what are the challenges comes to your mind?

Suneeta Reddy: I think when we are looking at doubling, yes, we are adding beds to ensure that we have beds to fill. But more importantly, again, it's the fact that we still have, by increasing 20%

occupancy, we're actually taking up our revenue by another 40%. So clearly, there is headroom to grow, do have plans. And I think that there is strong traction in the Hospitals division. This quarter should be a very good quarter. And I think you could look at it next quarter.

Moderator: We have the next question from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Sir, on the 24/7 in terms of the operating losses, which are really expenses which are there at INR 175 crore for this quarter. I mean what is the relation that we should keep in mind the GMV for these expenses?

Sanjiv Gupta: Yes. So, I think this is how we also internally look into, and if I look at Q1 FY24, we (inaudible) GMV about 28%, which was roughly 66% in the same quarter previous year, and Q4 was about 32%. And the target for the current year, which I talked about in the

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previous earnings call was also to having between 20% to 22%. This is one thing.

And secondly, I think many of the initiatives have started working in the organization, which we had thought through and started executing. We had INR 189 crore of expenses in Q4 and INR 15 crore already we used in Q1, which is down to INR 175 crore and this is a structuralized change. So obviously

y, INR 60 crore worth of cost deduction has

already happened. And as I see July month numbers, I am sure that Q2 numbers are also going to be lesser than INR 175 crore that we see next year during the time.

Nitin Agarwal: So I guess the way to look at this thing is this INR 175 crore number is a top-ish number. It keeps probably coming down with the cost reduction as we go forward. And the EBITDA to GMV conversion, the EBITDA, especially with the GMV growth keeps going up, which starts to; I mean that's how the losses begin to narrow in this business.

Sanjiv Gupta: Absolutely. That's the direction that we have started working on, too.

Nitin Agarwal: And in the next couple of years, when say the GMV goes to close to \$1 billion and all that you talk about, I mean at what level do the operating expenses begin to peak out or sustain at around when you start to get to \$1 billion in GMV?

Sanjiv Gupta: This is a little tricky. But I think the way we look at it is that whatever level of expenses we have it in the digital segment itself, those expenses we used to, we (inaudible) get out of the (inaudible) or the (inaudible) rate that we have it in, on the GMV. So yes, the expenses have started going down. In the future, depending on which particular vertical we intend to invest, there will be a little bit of delta over there. But otherwise, the thought process that the digital alone itself \$1 billion could be able to take care of the expense (inaudible). But as far as the current quarter is concerned, I think we did this one segment being down to INR 175 crore this quarter. And current quarter, which is Q2, I think will also be, let's say, at INR 175 crore for sure.

Nitin Agarwal: And if I get last one on that. And then how should we see the EBITDA percentage improving with relation to GMV over the next couple of years?

Sanjiv Gupta: I think today, if I look at the margin mix, my margin mix is also steadily going up. It used to be about 6% to 7%, 6.6% of the margin to the revenue in Q1 FY23. It went up to 7%, 9% and 9.7% over the previous quarters. This quarter, it is 10.89%. I think the next 2, 3 years, for sure it is going to be more than (inaudible) %.

Moderator: We have the next question from the line of Siddharth, an investor.

Siddharth: So, my question was regarding our associate Company, Indraprastha Medical. So last couple of quarters, we've been seeing improved business performance. So, what has led to this turnaround? Is there any operational changes that we have taken?

Suneeta Reddy: Yes. Clearly, we have got very good doctors onboarded, and this has resulted in better performance and high occupancy.

Siddharth: Okay. Can you highlight what is the current occupancy right now?

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Suneeta Reddy: 72%.

Siddharth: Okay. And regarding the lease, which is due this year, so what is the progress on that front?

Suneeta Reddy: We'll inform you at the next quarter.

Siddharth: Okay. And since our occupancy is around 70%, 72%. So, are we looking to do further

capex on that land?

Suneeta Reddy: There will be some capex on the land, but Apollo Hospitals is also looking at expansion in the region, leveraging the networks that we've built.

Siddharth: On the same line on which Indraprastha is there?

Suneeta Reddy: There is some expansion on Indraprastha itself. But beyond Indraprastha, we are looking at expansion in the region.

Moderator: Ladies and gentlemen, that was the last question. I now hand the conference back to our management for closing comments. Please go ahead.

Suneeta Reddy: Thank you, ladies and gentlemen, for joining this call. As I conclude, I would like to emphasize that everything that we do in Apollo Hospitals centers around the consumer
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and the patients. Our deep focus on clinical care as shown in our CONGO case mix; our technology absorption, the fact that we have now 25 robots that are being used, demonstrates that we are filling down the all-India network. And it also demonstrates that we have the ability to meet all challenges that may come in our way of future growth. So we are quite confident that this could be a very good year for us. Thank you, ladies, and gentlemen.

Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility of such errors, although an effort has been made to ensure high level of accuracy .

Call: Nov 2023

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Apollo Hospitals Enterprise Limited
Q2 FY24 Earnings Conference Call
November 10, 2023

Moderator: Ladies and gentlemen, good day and welcome to Apollo Hospitals Limited Q2 FY24 earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touch tone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you and over to you.

Mayank Vaswani: Good afternoon, everyone, and thank you for joining us on this call to discuss the financial results of Apollo hospitals for the second quarter of financial year 2023-24, which were announced yesterday. We have with us on the call today the senior management team represented by Mrs. Suneeta Reddy – Managing Director; Dr. Hariprasad – President of the Hospitals Division; Mr. A. Krishnan – Group CFO; Mr. Sriram Iyer – CEO of AHLL; Mr. Ashish Maheshwari – CFO of A HLL; Mr. Obul Reddy – CFO of the Pharmacy Division, and Mr. Sanjeev Gupta – CFO of Apollo 24x7.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on slide 2 of the investor presentation, which was shared with all of you earlier. Documents related to our financial performance have also been posted on our corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks.

Thank you and over to you ma'am.

Suneeta Reddy: Good afternoon, everyone. Thank you for taking time out to join our earnings call. I hope you have received the earnings document which we had shared earlier.

We are pleased to report that we are able to sustain and improve our momentum in FY24 with a robust second quarter characterized by continued growth in top line, improved volumes and payer mix, meaningful network additions and further growth in the user base for our digital health offerings.

Our Healthcare services business witnessed strong 12% year-on-year revenue growth in Q2 FY24. Within this, the insurance revenues grew by 18% and contributed 43% of total IP revenue. Revenue from International patients also grew by 20% year-on-year.

Within IP volume growth of 4%, surgical volumes grew by 7% year-on-year, representing a strong surgical performance in a quarter usually characterized by seasonal medical admissions.

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Overall occupancy across the group was at a healthy 68%. This was after the planned calibration of low-paying business and despite a delayed onset of expected seasonality, and also the reduction in ALOS by 5% from last year.

ARPOB on an overall basis increased 14% year-on-year to Rs. 57,379.

Against this backdrop, let me walk you through the financials:

Consolidated revenue grew by 14% on a year-on-year basis to Rs. 4,847 crore.

Healthcare services grew by 12% to Rs. 2,547 crore.

Revenues from Apollo HealthCo stood at Rs. 1,945 crore in Q2 FY24, a growth of 17% year-on-year. However, combined pharmacy grew by 21% on a year-on-year basis.

Apollo Health and Lifestyle revenues registered 11% year-on-year growth at Rs. 354 crore for Q2 FY24.

Consolidated EBITDA was at 628 crore, registering an increase of 11% year-on-year. Within this, the healthcare services EBITDA was

at Rs. 634 crore, a growth of

11% and EBITDA margins were at 24.9%, marginally lower than Q2 last year on account of increased investments in clinical talent and enhanced focus on advertising and marketing.

The pharmacy distribution business in Apollo HealthCo recorded an EBITDA of Rs. 130 crore, a year-on-year growth of 9%. 24/7 operating costs were at Rs. 197 crore

against the operating cost of Rs. 204 crore in the trailing quarter . EBITDA loss in the Company was at Rs. 39 crore , of which non -cash ESOP charge accounted for Rs. 35 crore . This loss is down from Rs. 57 crore in the trailing quarter , in keeping with our commitment to reduce cost s in this vertical . The business is on track to achieve operational break even in Q4 FY24 .

AHLL recorded an EBITDA of Rs. 32 crore as compared to EBITDA of Rs. 38 crore in Q2 FY 23. The EBITDA was subdued on account of ongoing network expansion as well as the relocation of 2 Spectra units . The margin profile is expected to improve with the ramp -up from the new network .

Consolidated PAT was at Rs. 233 crore . Healthcare Services PAT was at Rs. 314 crore , a growth of 14%.

We are happy to announce a new 250 bed s hospital in Pune, which is expand able to Rs. 425 crore in the next two years at an overall cost s of Rs. 675 crore . The hospital is expected to be commissioned in Q1 FY25.

We have already announced the expanded footprint in Eastern India with the acquisition of a 225 bed s hospital in Kolkata ; expandable to 325 beds.

We are on track to add 2 ,300 beds across eight locations at a cost of Rs. 3,400 crore in the next three financial years.

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Within this pipeline for growth in place, there are several highlights within the quarter's performance , which signal a healthy trajectory for all of the verticals going forward.

Within the Healthcare Services business, we have delivered a ROC E of 28% with a balance ROC E across all geographies , the Metro , tier 1 and tier 2. The metro ROC E is at 27% and today , we have the largest non -metro presence in the country with 3,195 operating beds delivering a ROC E of 23 %.

Within the pharmacy, the private label and generic business contributed to 16.52% of total pharmacy revenues. This was an improvement of 92 basis points over the same quarter last year. Our digital platform 24 /7 added 2 million users this q uarter and now has 29 million users overall . The platform GMV was at Rs. 726 crore , a growth of 16% on quarter -on-quarter basis and in line to deliver Rs. 3,000 crore of GMV this fiscal.

The diagnostics vertical within AHLL recorded a revenue of Rs. 124 crore for the quarter, well on track for an annualized top line of Rs. 500 crore with an improved margin profile.

All these indicators validate our intent to create Asia's largest integrated healthcare ecosystem with high quality healthcare at the core, supported by multiple consumer touch points such as offline pharmacies, multiformat clinics, diagnostics and our comprehensive digital health platform 24 /7.

This ecosystem will be underpinned by a focus on achieving the best possible clinical outcomes accentuated by investments in cutting edge technology and AI for clinical decision -making . By providing each guest a seamless journey which accords them with consistently high standards of healthcare at each touch point , we ensure that we cross -pollinate our offerings while unlocking the network effects resulting in robust upstream and downstream funnels.

On that note, I would like to hand it over to moderator and open the line for questions and answers . I have with me D r. Hariprasad , Krishnan , our CFO, Sriram Iyer - COO, AHLL, Ashish Maheshwari - CFO , AHLL and O bul Reddy - CFO , Pharmacy and Sanjeev from 24 /7.

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oderator: We will now begin the question -and-answer session. We have a first question from the line of Kunal Dhamesha from Macqu arie. Please go ahead.

Kunal Dhamesh a: My first question is on in -patient volume growth in our core markets of Tamil Nadu, AP and Telangana, which has kind of remained muted for first half. In Q1, we suggested there was some holiday season . But any particular reason why the trend is not picking up in Q2 as well?

Dr. Hariprasad : Actually, just looking at the Q2, it is a quarter where we have a high volume of medical cases particularly because the monsoon related cases like deng ue and stuff like that which we had in the last year second quarter. But I guess because of a delayed monsoon or whatever may be the reason , we did not have that influx of medical cases during the second quarter of this year, specially in the Southern

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region. And that's why you would see a growth, but it is not in -line with the other regions that we have seen a significantly higher growth.

Kunal Dhamesha: Would you say that similar seasonality played out in other regions because there the in-patient volume growth is quite significant?

Dr. Hariprasad : Yes, some of the things if you actually notice , our surgical have gone up in volume s too compared to the medical volumes and the same thing explains the other reasons also.

Kunal Dhamesha: The second question is on the ARP OB growth ; when I see across all the regions, it's in the double digit in Q2 as well as for H1 also. So , is it driven by decent rate hikes that you would have taken across the network ? And if you can also provide the current payor mix, I know ma'am has said 43 % is insurance, but probably self -pay, International , etc., also if you can provide?

A. Krishnan: So, what's happening in the overall case mix is there are three trends that we are witnessing. One is , we are seeing that there are higher category of bed s which there are lot of patients who are opting for including private beds , etc., So, you could have seen that we have reconfigured some of the beds across the system and we have brought down some of these general wards and in crease semi -private and private, because we are seeing that there is a higher demand that we are witnessing for both semi -private and private which is one of the reasons that the average ARP OB has also gone up across because clearly the AR POB of some of those beds are better . Pricing wise, we have not taken any price increases in this quarter, and these are the price increases which have always been there t wo quarters back and that is what continues to prevail. We are getting better realizations from insurance. We continue to work with insurance companies because some of the insurance companies' contracts are still over two years old and we are working with them to see how we can get better yields as the contracts come up for negotiation.

Kunal Dhamesha: And on payor mix?

A. Krishnan: Payor mix, there is an 18% growth . While we have said that overall revenues have grown by 12%, our insurance revenues have actually grown by 18% in the same YoY this quarter versus last year's same quarter and 4 3% of our revenues now comes from insurance and 38% comes from self -pay.

Kunal Dhamesha: And on International growth ?

A. Krishnan: International is 20% and we around 7 .5% of our revenue is now coming from International .

Kunal Dhamesha: Sir, just one suggestion . I highly appreciate more detailed segmentation for our business in the latest investor presentation. But if you could provide probably one year of historical data as well to the investor community, I think it would help us.

A. Krishnan: You can come offline. Any information we are willing to provide , you know tha

t we

are open to all of that and you can get offline . This time if you have seen that we have even given a regional cut as well across regions so that you get all the regions

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clearly now. You also see that both metros and non -metros are doing well for us. So that's the other slide that we' ve given. Happy to help you build your model.

Moderator: We'll take our next question from the line of Nikhil Mathur from HDFC Mutual Fund. Please go ahead.

Nikhil Mathur: My questions are centered around the hospitals business. When I look at the expansion plan, the projects that are coming onstream in FY25, they are towards Q4 FY25. On the Pune side, the phase -1 is coming up in Q1, but I am not sure what the stage of development there is ? But it seems that a lot of the expansion plan is quite back -ended towards FY25 perhaps in FY26. So , till the time these expansion activities come onstream, how do you see growth for the hospital business especially with ARPOB growth being so strong for the last few quarters and occupancy also reaching 68%, does it mean that the growth without the new expansion is kind of coming to a plateau side or some sort of a stagnation now?

A. Krishnan: No, that's not true. In fact, at 68% we are seeing a good increase overall this year . This quarter if you look at the volume growth, we had a 4% volume growth , but even within the volume growth of 4%, if you look at the mix, we had a 7% growth from surgeries. So , clearly there is a focus on surgeries. We have added a lot of guaranteed money d octors across our system in the new hospitals, etc ., which has also enabled the growth and we are seeing the new hospitals really delivering well also on the growth . 68%, still there is headroom for growth , we can take it to 70% , 72% next year and we still have headroom for growth over the next couple of years as you will. And we are also seeing that the average length of stay even now if you

look at it last year to this year, the average length of stay has come down by 5% to 3.26 days. So, while we have said that there is not much room that we have to bring down the ALOS, our focus is on in-patient volume growth, a lot of minimally invasive work, and a lot of robotic work that we are doing across the system. We are seeing definitely average length of stay is trending lower as you know with better infection control, better discharges that we do, etc.. So, I think from a headroom growth from the existing hospitals itself, we still have a headroom that is minimum there as we can see. And from our perspective, we continue to work on the cluster approach even in this, right. So, doesn't mean that we cannot actively add some asset-light models in some of these clusters as we have opportunities; like Calcutta is a classic example of something that we have brought onstream. For your information, the Royal M udhol Hospital, Pune, it's a new hospital, it's a new asset built in the Swargate area. It's an asset which has already been brought into place. Five floors is already built. They are close to get the occupation certificate and which is the reason that we are quite comfortable (inaudible 17:48 min.)

Moderator: I am sorry sir, we are unable to hear you.

A Krishnan: So, in the Pune market, the Q1 FY25, we are quite sure we will be able to get there because that's a new asset already constructed, been under construction by the other partners for the last three years, we are now acquiring this and we will be in partnership with them, it's a full 100% acquisition by us, it's a leased asset is how we are working on it.

Again, the one in Calcutta where we are 75% + plus occupied, it's again an asset which is already constructed. So, the construction of the asset is done. We ac

tually

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bought it in an asset sale from a trust which is why we are quite confident of bringing it also onstream by Q4.

The one in Gachibowli, again it's a greenfield. The building is already there. It's an asset-light model which is where we are comfortable. So, I guess Q4 FY25 is something that we are comfortable in both Calcutta and Hyderabad as we speak. And I think this expansion plans as we have laid out, as of now, we are quite comfortable of achieving those timelines.

Nikhil Mathur: Just a quick follow up on this. So, I don't know if you can give a number or not, but can it be expected that even without expansion plans a low double-digit kind of a growth is achievable on the base that you are at currently in 2Q?

A. Krishnan: Yes.

Nikhil Mathur: And the second question is again centered around the expansion plan. So, the expansion that I see is kind of more skewed towards the metro cities, especially in FY25, Hyderabad, Bangalore, Kolkata, in FY26 you have Gurgaon and Pune. But if I look at your non-metros, your non-metro breakup that you've given which is quite helpful. The return on capital is pretty strong at 23%. And we also see that in metro, there's a lot of competitive intensity, in the South there are so many hospitals expanding capacity in the North as well. So, given your early mover advantage in many non-metros, shouldn't the focus be a bit more on expansion in non-metros rather than metros where already the supply is pretty strong over the next three, four years, which is coming onstream?

Suneeta Reddy: No I think we have a very calibrated expansion plan and the cities that we have selected is actually the one-year strategic study where we looked at, one - the size of the market, the ability to pay, second - the ability for us in those institutions and really I think it's based on that, and the fact that these hospitals that we chose are they become regional centers as well. But we continue to look at the assets across, I think, wherever there is a penetration of private insurance, those are the markets that we would continue to look at.

Moderator: We have our next question from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: If I look at the pharmacy distribution business, I see that the store addition we have is lower than what we did last year. What should be the average store addition we should be looking at and the growth for the ...

Moderator: We can't hear you.

Neha Manpuria: So, I was wondering on the store addition of the physical distribution base. We're seeing a slower pace of store addition and probably I don't know, is that why the growth rate is lower than what we have done historically for the pharmacy business. What's the pace of growth that we should see here? Second, on the 24/7, we've reduced the cost. I know you've mentioned a break even in fourth quarter. But is there

scope to materially reduce opex more from FY25, '26 perspective or should we assume a large part of the profitability in Health Co comes from revenue growth rather than opex reduction ?

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Obul Reddy : First, the number of stores as explained to you during Q1 call, we have planned to open lower stores in the current half year. Our plan to add about 500 -plus stores is on. In H1 you will see more number of stores. On the operating cost of Apollo HealthCo, Mr. Sanjeev will answer.

Sanjeev Gupta : If you look at it, we had Rs. 190 crore of operating expenses excluding the ESOP non-cash expenses during Q4 of FY23 . From Rs. 190 crore we are down to Rs. 162 and I st

rongly believe that another Rs. 10 to Rs. 15 crore will further come down as we move forward. So , I'm looking at anything as a run rate of Rs. 150 crore or slightly lesser than that in Q3 , Q4. To be fair to this question that , would we see some savings in the next financial year ; - I think we'll continue to look into all the opportunities. It is a fine balance between the growth and the cost plus the investments that we're doing in two system technologies . This cost also takes care of the clinical intelligence excellence that we have built up , some of the AI tools that we are working on. Those are the investments that we are doing it. But yes, we can expect some changes in the next fiscal year. But depending upon to what an extent we put money into technology, we'll be able to guide you much better in Q4 than today. But yes, current costs will continue to come down as we move into Q3 and Q4.

Suneeta Reddy: Also, a rationalization discount .

Neha Manpuria: Isn't that largely done, ma'am because I remember you mentioned in the last call that now we are below peers ?

Suneeta Reddy: Yes, we continue to be below peers .

Neha Manpuria: And second question, I also saw we announced some sort of a tie up in Rourkela. Is this an existing hospital that we are taking over and therefore not part of our expansion plan and what's the thought process there because it's very different from the cities that we've chosen for the expansion plan?

A. Krishnan: We're not investing capital in that asset. Incidentally, it's a fully managed asset -light model that we have . We will be able to commission this in the coming quarter itself and this is something that we have hence not put in our expansion plans here . But it's a full bed, it's something that has been developed by Steel Authority of India and they have given it on operations management contract to us. We hope to start, this commissioning in the coming quarter fully . And this is a profit share model that we have such that 1/3 of the profits go to them , the balance comes to us. I think it's not easy to get such kind of models. But I think given our reputation and our clinical prowess , I guess the Steel Authority wanted us to operate that asset and which is how we've got that.

Neha Manpuria: So, this will be run as in Apollo Hospitals ..?

A. Krishnan: The revenues and profits will be consolidated with 1/3 of the profits go to them .

Moderator: We have our next question from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

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S Srinivasan: Just the first one on the hospital margins . I missed or I couldn't fully get the opening comments around a higher cost s in terms of talent acquisition and the margins being lower for the hospitals. So , if you could please elaborate that again?

Suneeta Reddy: So, our margins were at 24.9% . There was a 75 basis points decrease because we have spent on acquisition of guaranteed money for doctors. In fact , we bought a 100 new doctors into the pipeline and this has led to an increase in cost s plus the 0.5% increase in marketing cost s.

S Srinivasan: So how s should we look at the margins going forward, in the sense , are these done and in the numbers, or do we plan to keep adding these guaranteed doctors even in the second -half?

Suneeta Reddy: So, I think the guaranteed doctors are something where had planned last year and I think we've created a pipe line of new talent for our hospitals. So , you should not expect to see an increase in that. But what you will see is that these doctors when they start having started working, their payoff will be very good and therefore over a period of time margin improvement.

S Srinivasan: My second question is on the difference in growth rates between

n the platform at 21%

versus when I look at the pharmacy ; when I look at disaggregated, it's lower, right , what explains that?

Obul Reddy : Pharmacy distribution growth is independent of the platform growth because in the front-end, the 21% growth is for both omni -channel, which consists of offline -online, whereas pharmacy distribution reported in the investor presentation only relates to the offline, which is about 17%. So , both are independent numbers.

S Srinivasan: But I'm talking growth rate here . Those numbers should ideally be aligned , right?

Obul Reddy : If you look at omnichannel front end growth, which is at 21%.

Moderator: We take our next question from the line of Bin o Pathiparampil from Elara Capital.

Please go ahead.

B Pathiparampil : Just a couple of questions from my side . In Karnataka region, I can see a significant reduction in the number of operating beds. Why has that happened?

Suneeta Reddy: We're doing some refurbishment in Karnataka and Chennai. So , there has been a reduction. I think earlier we mentioned that we are replacing some of the general wards with semi -private because of the penetration of insurance where people are willing to pay for a semi -private.

A. Krishnan: And in this particular case, it was Mysore where this happened predominantly. And Mysore was the place where we had more of these beds which were general wards, etc., where we shifted them to semi -private and private.

B Pathiparampil: You commented on the hospital margins. Can I also get some comment on why the pharmacy and the AH LL margins are also down Y -o-Y?

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Obul Reddy : The pharmacy margins have remained stable in the last two quarters because the pharmacy distribution is a very stable business where we are maintaining the margins... (inaudible 29:58 min.)

Moderator: Sir, we can't hear you.

Obul Reddy : ...from about 4.6% to 5.8% .

A. Krishnan: So, if you look at the sequential quarter, the margins are better. If you look at from last year to now, if you look at it, we have increased the costs, there were higher costs in the online business, and which has all started to come off now and you will see better margins. As you see Q1 to Q2, which is a sequential quarter , we have done well and we are hoping that as we move forward, the margins will even go back better. Overall , on the consolidated numbers, we have said we are in track to achieve Rs. 10,000 crore of revenue this year on pharmacy with 6% EBITDA between the pharmacy distribution and APL.

B Pathiparampil: One final question on this 24/7. When you say this operating cost was Rs. 162 crore for the quarter, I assume that it is the total costs. There would be some revenues associated with it, right?

A. Krishnan: This is on top , right, the associated revenues are there on top what we have shown that Rs. 231 crore is the total revenues that we are getting out of it and there is an EBITDA which is coming because the EBITDA is a bit higher than the offline pharmacy distribution because they also get take rates on the diagnostics and on the hospital feeders which is there on the top as you can see.

B Pathiparampil: If I have to calculate the net EBITDA loss because of Apollo for 24/7, then I should take Rs. 231 crore minus 162, is that correct?

A. Krishnan: Rs. 28 crore is only the profit that I'm making out of it at one level higher if you look at it EBITDA. So, if you look at the loss which is what we are reflecting there of 169 after ESOP charge or maybe close to 130 without the ESOP charges how you should think of the cash loss there .

Moderator: We have our next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Just processing on the pharmacy question. So , when we

say this Rs. 10,000 crore

of omnichannel revenues with 6% EBITDA margin, what is the EBITDA margin for H1?

Obul Reddy : H1 is 5.6%.

Nitin Agarwal: What is the total revenue for H1 ?

Obul Reddy : We have Rs. 4,723 crore .

Nitin Agarwal: So, we're talking of higher revenue in H2 as well as higher margins for the omnichannel business ?

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A. Krishnan: Yes.

Nitin Agarwal: And then beyond the number that you reported in the HealthCo the omnichannel represents a front end of the business , that's not consolidated , that's the difference between the two?

Obul Reddy : That's right.

Nitin Agarwal: Typically, what proportion of our profitability is sitting in the front end of the EBITDA , what proportion of the EBITDA typically sitting in the front end, which we are not consolidating right now?

Obul Reddy : 80% of the profitability is sitting in the back end and about 20% of the profitability is in the front end.

Nitin Agarwal: So, Is this structure continuing the way in this current form ?

Obul Reddy : Yes, that is what it has been, and it will continue for some more time until we relook at that.

Nitin Agarwal: On the hospital s business , what is the guidance for EBITDA that one can look at over a two-year period vs. from the H1 level that we have ?

A. Krishnan: So, I think there are two things , right. One is that the operating leverage definitely should kick in as we move in from 68 to higher, which is our target for next year , definitely move it above 70 , 72 is something that we are working on . As you have seen ALOS has still come down even from last year to this year , the ALOS has still come down by 5% . If the ALOS was not lower , adjusted for a higher ALOS, the occupancy would have been 72, but that's okay, I t's good for us. So , as it goes to 72%, etc ., we should be looking at least a 200 basis point s increase in the overall EBITDA margin.

Nitin Agarwal: Something like 26.5% margin for the hospital s business ?

A. Krishnan: Yes.

Moderator: We have a next question from the line of Madhav Marda from FIL Research . Please go ahead.

Madhav Marda : Very interestingly that you all mentioned this shift from the general wards to semi and private rooms as insurance patients picks up. This seems like a very structural trend which can happen in the country in the next like medium term, right, because as insurance picks up, people would want better quality service wherever they can go to. So just wanted to understand is this a structural trend then as some of the patients shift to semi and private rooms, is that marginal -accretive for us , just wanted to get that sense ?

A. Krishnan: Yes, if you look at it, yes, it is margin -accretive overall from that bed perspective clearly. But as we are now looking at some of the newer hospitals, we are now bringing in the standard beds or the sharing beds , even the general wards are all less than 15% of the overall beds. So , that's how we are now configuring some of

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the new Hospitals that we are currently planning with more focus on private deluxe and semi -private. In some of our earlier hospitals , we have this opportunity to reconfigure, which we will keep looking at depending on market -to-market, because in some markets we still see there is still a good offtake on the standard beds and the general wards.

Madhav Marda: So basically, as we shift the mix towards this thing on an absolute EBITDA basis, it adds for the hospital unit basically that's what I wanted to check?

A. Krishnan: Yes, it is.

Moderator: We'll take a follow -up question from the line of

Bino Pathiparampil from Elara Capital.

Please go ahead.

B Pathiparampil : Just wanted to continue my discussion. So , we were discussing Rs. 162 crore , on which there is a net EBITDA made of Rs. 28 crore. So, if I say that the net loss because of 24/7 operations is 162 minus 28, is that a correct way to look at it?

A. Krishnan: It's a correct way of looking at it.

Moderator: We have our next question from the line of Prashant Kshirsagar from Univera Corporate Research. Please go ahead.

P Kshirsagar : Just wanted to ask you what is the progress on the Delhi expansion front? And a question on your associate Company , Indraprastha Medical , can you give us the capital expenditure figure plan for that hospital?

A. Krishnan: On Delhi expansion, there was a stay which has been vacated in the Gurgaon court and we are now proceeding with the expansion already and anyway the asset was already bought and secured and processed by us already. There was one excess stay which was there because of an M oU that the seller had with another buyer for which there was no money which was paid by the other buyer or for other person to the seller. So , there was a stay until the whole thing was done. But that stay is now

vacated and we are now proceeding with the expansion.

Suneeta Reddy: So, the expansion plan is 150 , that will be funded by cash flows and a little bit of debt.

P Kshirsagar: That is for Indraprastha you are talking ?

Suneeta Reddy: Yes, Indraprastha .

P Kshirsagar: When should that get completed?

Suneeta Reddy: That should take two years to complete.

P Kshirsagar: So, there will be some capacity addition or it 's just the quality improvement in the hospital because the...?

Suneeta Reddy: So, there is a capacity addition. There is a center of neurosurgery being established. So, there is a capacity addition of about 75 beds plus the multi -level parking.

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P Kshirsagar: So, there will be multi - level parking plus addition of 75 b eds which will take approximately to around 800 b eds and central neuros wing established for the hospital . And what sort of revenue ac cretion do you see after the capital expenditure in that hospital ?

Suneeta Reddy: We continue to see strong revenue growth of 15%.

P Kshirsagar: Can you tell us the mix of the International patients in that hospital ?

Suneeta Reddy: That hospital is high ; it's between 15% and 20%.

Moderator: We'll take our next question from the line of Damayanti K erai from HSBC. Please go ahead.

Damayanti Kerai: My question is regarding the margin impact as you are adding on quite a lot of beds . So, can you tell us, like in cities like Pune and Gachibowli , Hyderabad, what kind of breakeven time you are expecting for the hospitals and whether there will be any impact on margins due to addition of these new newer facilities?

A. Krishnan: See our base of EBITDA now is a very significant base across if you look at the consolidated numbers, we are over Rs. 2,000 crore now, and we have a calibrated expansion plan between the next three years adding 7 00, 800 beds every year . And in places like Pune, in fact even when we have made this announcement, we've got a lot of doctors who wanted to join us , etc. It's a market where there is not much of penetration of quality corporate hospitals like ours. So , we typically believe that we should be able to do well in some of these markets. Hyderabad is a lon e market. The reason we are putting up this expansion there is because that side of the market is definitely having a lot much capacity and we have a set of doctors who are wanting to join us there. So , I think 18 months is what we typically look at for breakeven on the EBITDA. I think we think in both these markets between 12 an

d 18 months would be the answer.

Damayanti Kerai: So, in 1, 1.5 -year typically both these centers should turn around ?

A. Krishnan: That's correct .

Damayanti Kerai: For funding your capex , you are relying on your internal cash flows or you're looking to add on some debt also because your balance sheet ...

A. Krishnan: This will be from internal a ccruals. If there is a small debt required at the third year, we would probably take some debt.

Damayanti Kerai: My last question is on the International patient traction . Right now , it's 7.5% of your hospital revenues. How do you see this picking up, say, in two years or we expect major traction to come once your program facility comes onboard?

Dr. Hariprasad : Yes, w e expect a major traction both the Gurgaon, particularly the Gurgaon facility coming up because lot of patients there have a direct access to Delhi and this facility is very accessible from the Delhi airport . So, we see a significant impact on International business once th is facility is up .

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Damayanti Kerai: Any markets where you are yet to see normal level of patient coming back from say some countries for this medical tourism or most of the markets are seeing normal flow as we anticipated after COVID?

Dr. Hariprasad : In our existing markets, we are seeing the regular flow from the pre -COVID levels and above pre -COVID levels , and we're also looking at the new markets and the new markets we hope to leverage , we've invested the resources and time into new markets and we hope to see results coming in from these new markets in the next one, two years.

Moderator: We have our next question from the line of Lavanya from U BS Investment Banking. Please go ahead.

Lavanya Tottala : So, just one question from my side is that if I look at EBITDA margin of offline and online pharmacies, online pharmacy margin is increasing on sequential basis like last two quarters as well, but offline pharmacy EBITDA margin has declined.

Anything specific for these two in a different direction?

Obul Reddy : There is no difference between the two. We look at omni channel level and it's growing. It's only one quarter we had some dip because of the sales and we are back into normal level and Q2 it stands at about 5.7% and we have guided to be at 6% plus for the year.

Lavanya Tottala : And anything on the fundraise at HealthCo level, like, how do you see what's the timeline and where is it struck?

Suneeta Reddy: Regarding the fund raise at 24 /7 level, I think we've always maintained that as you know as long as we have enough cash for growth, we should be good. They had a loss of Rs. 38 crore of EBITDA, out of which Rs. 34 crore was the non -cash loss due to the ESOP. So just going forward I think the back end with Rs. 130 crore of EBITDA is able to support the growth. Having said that, we will look at an investment where we believe the valuation is appropriate because we're quite sure we don't want to go below our dilution of 10% and this cash will be used like Sanjeev said to develop to invest into AI and to offer new services. So , right now, let me just say that we have access to investment , we're looking at the right valuation and this could happen I think in the next 6 to 12 months, but there is no pressure unless we get the right valuation.

Moderator: We have a next question from the line of Rahul J eewani from IIFL Securities. Please go ahead.

Rahul Jeewani : Regarding th is target of 70 % to 72% kind of occupancies for next year, can you help us understand in terms of what would help us to drive this occupancy improvement given that in first half of this year we are at around 65% kind of an occupancy and if I see even in FY20 our peak occupancy used to be 68% , and at that poin

t in time,

the ALOS was also higher , and as you have indicated, the ALOS numbers are also down, let's say from FY20 levels , the ALOS numbers are down 20% , So, do you think that you can hit this occupancy target of 70 % , 72% for next year?

Suneeta Reddy: Next year, yes, we are looking at the funnel and really looking at how we can do O P to IP conversion , we are looking at different markets, which is the corporate market,

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the retail market, the International market and the insurance market . We are seeing growth in all these segments and we're quite hopeful that we should reach 72%. I think the only disclaimer here is that ALOS b eing what it is , we should be able to achieve it and hopefully we're not expecting a reduction in ALOS . What is happening in Apollo is that we're doing a lot of day care surgeries using the robot. And this has made the system very efficient and therefore decrease in ALOS. But we are looking at expanding the funnel and making sure that occupancy picks up because all of our units now have doctors in place , and like I said, we had spent 5% more on marketing this year.

Rahul Jeewani: So, with this decline in ALOS , do you think that occupancies is the relevant parameter to be tracking, so instead shouldn't we be tracking, let's say the growth in IP volumes or occupied bed days rather than tracking the occupancy metric ?

Suneeta Reddy: No, no, we do both.

Rahul Jeewani: Second question on this margin expansion for the hospital business, which you talked about 200 basis point s, can you talk about the net margin expansion which we would see after accounting for the capacity expansions which we are doing?

A. Krishnan: So, this is before the capacity expansions that we are talking of because we will from the next year once we add the new expansions, we will give you the split of the current and the new ones. But this is at the existing is what we are looking at the 200-basis point s expan sion. The new ones, I don't have a number to just give you an EBITDA now, but as I said, most of the new is between 12 to 18 months is what we are expecting the EBITDA margins to become neutral and some of the new ones that we're adding next year. So , that is going to be a separate number. I don't have that number of f hand .

Rahul Jeewani: So, would it be fair to (inaudible 49:27 min.) basis point margin that from these new hospitals for the initial two to three -year period?

A. Krishnan: I'll give you the number offline. We will give you the anticipated EBITDA loss from each of this expansion.

Moderator: We have a next question from the line of Sh yam Srinivasan from Goldman Sachs. Please go ahead.

S Srinivasan : Just two quick ones. First on ARPOB. When we look at your investor presentation historically over like a 10 -year period, I think ARPOB growth has been 7%, maybe 8%. We are doing ARPOB growth of 14 %, 15% now . Do you think at some point of time, say maybe one year, two year, three -year later, that this comes back to that 7%? On the price and case mix , how long do you think this kind of higher than historical run rate last?

A. Krishnan: I think 7% to 8% is the right number to model and we are not expecting it to be more than 7 % to 8%. As you rightly have assessed, this change in ARPOB in the last three years is something which enabled through a significant mix in the payoff , as you know from 25 %, 30% on insurance pre -COVID , we are now at 43 %. And that is what is enabling us to get a better ARPOB. I think going forward, you should look at 7 % to 8% only.

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Suneeta Reddy: I think the

case mix is also definitely improved , it's matured . So cardiac , onco , neuro , ortho, contribute to a significant portion of the revenue with high margin.

S Srinivasan: Second question is on the GM V split. If you could get that in terms of pharmacy diagnostics and IPOP ?

Obul Reddy : So, currently pharmacy is running at about 45% GM V and diagnostic is another 5 % to 6%. Overall consultation is between 10 % to 12% and the remaining is IPOP and certain subscription packages that we sell to our customers.

Moderator: We have our next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : On the pharmacy business , in terms of the EBITDA that you report 8.2% margin which is Rs. 159 crore of EBITDA , prior to the 24 /7 operating losses how should we see the trajectory of this number over the next couple of years on an annualized basis?

Sanjeev Gupta: If you look at the margin for online pharmacy distribution, Apollo 24/7, which is the combined one, it is at 12.2% for Q2 and quarter -on-quarter it is increasing. So , I believe that more and more of pharmacy contribution as well as the new verticals that we're bringing in, which is digital therapeutics and insurance is to start in this quarter , our margin profile would be better and margin profile for Q4 it was 9.7%, now it is 12.2 %, so roughly 2.5% increase in six months , another 3 % to 4% is what I believe that should go up in another six months and going forward it should be near 20% is what we look at it. So, that's on this . On the operating cost of Rs. 162 crore, I think , as I was responding to one of the question s, first, there is a reduction that's happening on this expense and that reduction is attributable to certain automation related things as well as certain reduction in discretionary expenses. We are looking at anything like Rs. 150 crore of run rate expenditure for a quarter, and this year we'll complete about Rs. 3,000 crore of GM V and next year we should be doing anything between Rs. 4,500 crore to Rs. 5,000 crore . As we step up on the margin profile and look into the cost structure, I think overall losses what we see today and what we're going to see in the next year is entirely going to be very different. So , we have both the things in our place, increasing our margin profile as well as reducing our expenses.

Nitin Agarwal: Going forward, what GM V to revenue conversion should we work with?

Sanjeev Gupta: So, the revenue to GMV today is at about 32 % in Q2 and I think they should be in the range of about 40% as we step into the next year, that should be the ideal number to work on from the analysis point of view.

Nitin Agarwal: So, if I take a two-year view a 40% revenue to G MV conversion and a 20% margin is what we should aim for prior to the operating cost?

Sanjeev Gupta: Yes, sir, to start with this and as we progress forward, we'll have a little bit more insights about.

Moderator: We take our next question from the line of Nikhil Mathur from HDFC Mutual Fund. Please go ahead.

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Nikhil Mathur: In the non -metros , can you share what percent of revenue is derived from the insurance channel and how has that grown over the last year or two years or three years or so?

A. Krishnan: Overall it is 18% growth. metro, non-metro I'll have to get back to you on this.

Moderator: Ladies and gentlemen, that was the last question. I now hand the conference over to the management for closing comments. Over to you , please.

Suneeta Reddy: So, thank you, ladies and gentlemen for joining us on this call. This first half has been eventful and FY24 is shaping up to be an exciting year during which we will achieve meaningful progress on strategic plans. As I conclude, I would like

to emphasize that

we remain confident that the investments made by us and the solutions being worked upon will continue to differentiate us and provide us with a disproportionate share of the wallet from this earning customers. With occupancies in the high 60s across the network and capacity expansion underway and facilities that are registering notably higher than network occupancy, we continue to position ourselves with adequate headroom for growth .

Before I close, I would like to wish all of you a Happy Deepavali and a prosperous New Year . Thank you, ladies and gentlemen.

Moderator: Thank you, members of the management team . On behalf of Apollo Hospitals Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility of such errors, although an effort has been made to ensure high level of accuracy .

Call: Feb 2024

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Apollo Hospitals Enterprise Limited
Q3 FY24 Earnings Conference Call
February 09, 2024

Moderator: Ladies and gentlemen, good day, and welcome to Apollo Hospitals Limited Q3 FY 24 earnings conference call. As a reminder, all participant lines will be in the listen - only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing 'star' then 'zero' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you.

Mayank Vaswani: Good afternoon, everyone, and thank you for joining us on this call hosted by Apollo Hospitals Limited to discuss the financial results for the third quarter of financial year 2023 -24, which were announced yesterday. We have with us on the call the senior management team, represented by Mrs. Suneeta Reddy – Managing Director; Dr. Hariprasad – President of the Hospitals Division; Mr. A. Krishnan – Group CFO; Mr. Sriram Iyer – CEO of AHLL; Mr. Obul Reddy – CFO of the Pharmacy Division; and Mr. Sanjiv Gupta – CFO of Apollo 24/7.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Thank you, Mayank. Good afternoon, everyone. Thank you for taking time out to join this earnings call. I believe that all of you have received our earnings documents, which we shared earlier today.

We are pleased to report a strong performance in the third quarter of the financial year 2023 –24. While there was undoubtedly been some impact from seasonality, holidays and the cyclone in Chennai, we have reported double -digit growth in revenues and ARPOB this quarter on a year -on-year basis. Further, there has been continued progress in strategic imperatives such as further improvement in specialty mix , an enhanced payer mix, clinical augmentation and digitization. Our Healthcare Services business witnessed a strong 12% year -on-year revenue growth in Q3 FY 24, sustaining the pace of growth from the start of the fiscal year. Volume growth has contributed about half of the revenue growth, while the rest has come through pricing, payer and case mix improvements.

Transcript of AHEL Q 3 FY24 Earnings Call Page 2 of 14 Within this, the insurance revenues grew by 16% and contributed to 43% of total hospital IP revenue.

Overall occupancy across the group was at a healthy 66%. This was achieved despite a reduction in ALOS and a planned calibration of institutional volumes.

ARPOB on an overall basis increased 10% year -on-year to INR 56,368.

Against this backdrop, let me take you through our financial results :

Consolidated revenue grew by 14% to INR 4,851 crore . Healthcare Services grew by 12% to INR 2,464 crore .

Revenues from Apollo HealthCo were at INR 2,049 crore in Q3 FY 24, a growth of 17% year -on-year. Revenues from Apollo Health & Lifestyle registered a growth of 8% year -on-year at INR 338 crore in Q3 FY 24. The diagnostic vertical within AHLL recorded revenue growth of 19% year -on-year and was at INR 1

12 crore .

Consolidated EBITDA was at INR 614 crore , an increase of 21% year -on-year.

Within this, Healthcare Services EBITDA was at INR 586 crore , registering an 8% growth year -on-year. Healthcare Services margins were at 23.8%.

The margin impact was due to the reduced share of elective and surgical revenue in the revenue mix due to the holiday and festive season , as well as investments in clinical talent that have been made to strengthen our clinical profile, which is yet to be fully optimized.

The pharmacy distribution business in Apollo HealthCo recorded an EBITDA of INR 134 crore , a year -on-year growth of 8%. In a significant milestone, Apollo

HealthCo has reported a positive EBITDA of INR 2 crore , registering a break -even performance for the quarter, one quarter ahead of guidance.

AHLL recorded an EBITDA of INR 26 crore in Q3 FY 24.

Consolidated PAT was INR 245 crore , a growth of 56% year -on-year. Healthcare Services PAT was at INR 287 crore , up by 10%.

Within the Healthcare Services business, we have delivered a ROCE of 26.5% with a balance ROCE across all our geographies, metros, Tier 1 and Tier 2.

Looking ahead, we believe a healthy mix of metro and non -metro bids will lead to further improvement in ROCE.

We have also carefully studied the key markets and micro markets in the country, the demand -supply gap , and have set in motion a plan to add 2,000 beds over the next 4 years at a cost to INR 3,000 crore . This plan is well underway, and we would operationalize our hospitals in Pune, Hyderabad and Kolkata as well as the brownfield expansion in Bangalore in FY 25.

Private label and generics business contributed 16.55% of total pharmacy revenues with an improvement of 55 basis points over last year.

Transcript of AHLL Q 3 FY24 Earnings Call Page 3 of 14 The platform GMV of 24/7 was INR 658 crore , a growth of 21% on a sequential quarter basis.

We are committed to achieving break -even for Apollo 24/7 digital business within AHLL in the next 6 to 8 quarters.

We have begun to see the effects of our integrated network , with traction on net new footfalls coming into the system as well as more people consuming our services across the formats. We had over 4 ,50,000 new registrations this quarter within hospitals and over 1 ,60,000 incremental footfalls in the AHLL vertical compared to the same quarter last year. Our digital platform 24/7 added 2 million new users this quarter.

We believe that this comprehensive platform , with opportunities for improving our offering on consumer experience and lifetime value , offers us a competitive moat and headroom for accelerated growth . With each of our verticals delivering on the strategic promise , the network effect will deliver a holistic solution for the consumer and lasting relationship.

I would like to highlight that Apollo Hospitals has emerged as the first private hospital group in India to have successfully completed CAR -T cell program ; the group will now provide access to 'Made in India ' CAR -T cell therapy, a state -of-the-art treatment option for cancer patients. This is in keeping with the spirit of clinical pioneer ship that we have always stood for. Our efforts to offer outstanding clinical programs and technologies and deliver the best clinical outcomes for those who trust us has always been at the core of our DNA.

On that note, I would like to hand over to our moderator and open the line for questions and answers. I have with me Dr. Hariprasad ; our CFO, Krishnan; Sriram Iyer from AHLL, Obul Reddy and Sanjiv from Apollo HealthCo with me ready to take all of your questions. Thank you.

Moderator: Thank you very much. We'll take a first question from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: My first question is on margin t

rajjectory for hospital segment. So ma'am, obviously, you mentioned you are investing in clinical talent for new facility, etcetera. And we have seen margins falling off from, say, mid -20s to around 23 % , 23.5% or so. So , can you elaborate like how should we look at this margin profile over next few quarters? Because you have a couple of hospitals commencing operation in FY 25.

So that's my question on hospital margins.

Krishnan Akhileswaran: So, two points. As we said, this quarter has been, as you know, a base of a seasonal low quarter. So going forward, our focus clearly is to work on three levers.

The one lever is the inflation volume increase that we have been speaking of. And clearly, there are plans as part of our annual operating plan discussions, etcetera, to push each of the units on increasing the in patient volumes across their facilities, and that's what we are working on. That should help us get closer to the 70% as close to possible or even 70% next year, which is our internal target.

Second is clinical programs. If you look at the clinical programs that we are now focusing on, there is significant focus on oncology, neurosciences, some high -end

Transcript of AHLL Q 3 FY24 Earnings Call Page 4 of 14 cardiac, etcetera. Some of them, we believe will further increase our utilization in

some of our units, and that should also flow -through through the margins. So ,

these are two significant levers on the revenue side.

And on the cost side, as we already have spoken, one is this whole lever that we have of the doctors' fees because having invested a bit ahead of the curve, next year, we should be able to see that getting rationalized a bit and that should also help us see our margins go up. Internally, we would like to believe that there is an opportunity to increase the margins by at least 200 basis points over the next few quarters. And that's our internal way of looking at it.

Damayanti Kerai: So, this 200-basis point margin improvement is for existing set of beds or whatever like new beds will come cumulatively, you're talking about the sort of...

Krishnan Akhileswaran: New beds will come more by Q4 of the next fiscal. So, I think first is to get this done by the next fiscal, and we will obviously have to then take. But the base of the EBITDA that we have is so big now that some of the new beds should not alter and they are all the new beds that are coming if you look at it, Calcutta, Hyderabad, the way we are looking at Bangalore. Bangalore is very close to our existing hospital. So, you should look at it more as adjacent to our existing hospital, which should not impact our EBITDA significantly.

So, I think next year, focus is to increase it by 200 basis points. At the end of next fiscal, we hopefully get all these hospitals there, and we will show that separately. I wouldn't like to guide for now on what would be their EBITDA losses, but it will be very miniscule.

Damayanti Kerai: Okay. That's helpful. My second question is on 24/7 platform. So obviously, with improving profitability you are now expecting breakeven for this in next six to eight quarters. So what is plan for this? Like are you still looking for fundraise? Or do you want to put out separate IPO, etcetera? So, any plan for 24/7 in say next two to three years?

Suneeta Reddy: So currently, 24/7, in terms of cash is it's self-sustaining, so does not require a cash infusion, but we are looking at maybe at some time, if we require some cash, we are open to the idea. Let me just leave it at that, because we are looking at growth coming from 24/7 and achieving profitability between the sixth and eighth quarter of operations.

Damayanti Kerai: Okay. Thank you. I'll get back in the queue.

Moderator: Thank you. We have a next question from the line of Kunal Dhamesha from

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Macquarie. Please go ahead.

Kunal Dhamesha: So, the first question on the 24/7 GMV guidance that we had for FY 24 of around INR 3,000 crore. We have roughly done INR 2,000 + crore year to date. So, what is our expectation for full year and the next year? So, while we have achieved the breakeven at HealthCo level a quarter earlier than guided, that is good, but has that impacted the growth prospect of this business?

Sanjiv Gupta: So I think INR 3,000 crore was the guidance that we had given it for FY 24 as far as digital GMV are concerned. I think we need to hit something like INR 2,750 crore to

Transcript of AHIL Q 3 FY24 Earnings Call Page 5 of 14 about INR 2,800 crore. So that will be the final number versus INR 3,000 crore

mark. And this would mean about 75% to 80% increase in GMV for FY 24 versus FY23. We expect to continue to show a robust growth anything between 60% to 70% for the next year also versus this year. That's on the first question.

The second question is that on six to eight quarters profitability. Absolutely, we've got the clear (inaudible 15 :20 min.) with respect to increasing the GMV, increasing the margin line for each of the firing engines that we have today, which is the pharmacy, diagnostic, consultation. And as well as doing the right thing for the fixed expenses that the Company has it. And whatever right things are required to be done, they would be taken care. And as we step into the annual operating plan for the next year, I think maybe when we meet next, we'll have a little more understanding about this. But as we see, we have a detailed action plan to turn digital division also in the next six to eight quarters.

Kunal Dhamesha: But sir, for, let's say, for 60% to 70% growth next year, you should have at least some sequential growth, right; so versus this quarter sequential there is a decline of around 10%, right? So, I mean, is it a top task or is there something one-off in this quarter, which has...?

Sanjiv Gupta: Yes, this is something one-off. Q3 has been a transition quarter for the Company. And because of the holidays and the festivals, we have seen a slight dip in the GMV. But if I look at the January numbers and Q4 numbers, we are back into the growth, and you will notice this once we publish our Q4 numbers. So, nothing to worry. This is just one-off.

Kunal Dhamesha: Thank you. And the second question is on the Healthcare Services business.

Again, just going back to our guidance of around 15% revenue growth for FY 24 versus we have done 13% year -to-date. Now I think seasonality would not matter on a year -to-date basis, right? So where are we seeing this growth for FY 24 now for the Healthcare Services business? And what is the outlook for the next year?

Suneeta Reddy: I think we are targeted to grow at 15%. This quarter was because of the holidays. So, leaving out holidays, I think we are confident of achieving at least 14% growth for the Healthcare Services.

Kunal Dhamesha: For the full year. And for next year, any broad idea would like to know?

Suneeta Reddy: No, I think broad idea is that this trajectory has been good. So we are looking at 15% revenue growth.

Kunal Dhamesha: Thank you.

Moderator: Thank you. We have a next question from the line of Bino P from Elara Capital.

Please go ahead.

Bino Pathiparampil : You commented a bit on the pharmacy business margin improvement that is excluding the 24/7 cost s. I can see that the other expenses have , the overall expenses have come down Q -o-Q. The margin is up compared to last few quarters. Anything changing there?

Transcript of AHEL Q 3 FY24 Earnings Call Page 6 of 14 Obul Reddy: I think we have , you know last

year we have informed you that we have invested on the infrastructure required to create online services and the costs were a little high. And as business matures and we have planned those controls and then you can see that improvement in the margins. And lesser discounts also contributed to the margin.

Bino Pathiparampil : Okay. Second on this Rourkela SAIL Hospital, which you have inaugurated. Under which cluster will you add the beds and the revenue?

Krishnan Akhileswaran: So, Rourkela is just added very recently now. So, it's part of the existing , it's a 50 - bed that has just got commissioned. It's a 250 -bedded capacity hospital, but only 50 has got operationalized. So, it will get operationalized over the next three quarters. So , it's part of the overall Healthcare Services. It's a small number yet.

Bino Pathiparampil : My question was the healthcare you divide into Tamil Nadu, North, East, West, etcetera, right? Where would that ?

Krishnan Akhileswaran: East.

Bino Pathiparampil : Okay. And sir, in this quarter, I see there is an increase of about 42 beds operational beds in East and about 100 beds in North. Where is that coming from?

Krishnan Akhileswaran: East is what we said, right. Rourkela is also adding to that. And in North, it is Indore. Indore, we have added a brownfield, we have done a 100 -bedded expansion.

Bino Pathiparampil : In Indore, we have done a 100 -bed expansion. Okay.

Moderator: We have a next question from the line of Neha Manpuria from Bank of America.

Please go ahead.

Neha Manpuria: My first question is on the expansion plan. If I look at the timelines for Pune. From what I remember from the previous presentation, this was mentioned in the first quarter, but we have seemed to have delayed it to the later part of the fiscal. Any specific reason, I mean, of the significant delay in commissioning of that project?

Suneeta Reddy: Yes. So, the current hospital project, you know we have 220 beds, but we have the capacity to add another 200 beds. So, we are building the superstructure, so that we can gradually add 100 beds every year post this year.

Neha Manpuria: Okay. So, it is just to complete the superstructure for the additional beds.

Suneeta Reddy: Yes.

Neha Manpuria: Understood. And my second question on 24/7 and the comment that you mentioned of achieving break -even in 6 to 8 quarters. Would that be possible with the existing services that we have in 24/7? Or would that target require us to add more services to the platform that we have talked about in the past, subscription, insurance, etcetera. Would that be key to getting to that break -even in 24/7 because from the existing business, given your GMV guidance, I am not able to get to the math of break even in 6 to 8 quarters?

Transcript of AHEL Q 3 FY24 Earnings Call Page 7 of 14 Sanjiv Gupta: No, I think , see it has to be a mix of the new set of verticals also, which provides

you better margins. And we talked about the entire digital therapeutics as one of the offerings . And some time back, we also talked about the insurance distribution.

In fact, the insurance distribution is something which we started properly from January this year. And apart from these two verticals, obviously, the entire monetization of the app and the website is still not being done as much as we would like to do it.

So, I think these three particular offerings, which is digital therapeutic, insurance and app monetization, these are the three segments, we could give you a better margin line. In fact, the entire earnings that you earn out of these three verticals would go back or flow back down to EBITDA. So, I think, coupled with the existing offerings of pharma, diagno and consult, these are the three things that we should be seeing as we move forward. And together, al

I the 6 months should help us achieve this target.

Neha Manpuria: And sir, for digital therapeutics sir, app monetization, what sort of timeline should we start seeing this being, let's say, a driver? Would it take a couple of quarters for that to happen? Like for insurance, you mentioned you've started module from this month?

Sanjiv Gupta: Yes. So, app monetization is something which we can expect to start from Q1. And as far as the digital therapeutics is concerned, I think that is also work is getting done on the product and the tech side of the solutions. And I think something like in Q1 and Q2, we should also be seeing certain subscription -led module for digital therapeutics to also be ruled out of 24/7.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just first one on AHLL. Apart from diagnostics, I think the other segments of it have been like slow growth for the 9 months, even for quarter 3. Anything that we need to keep in mind? Is there a ; would we say an outlook where it goes back to a double -digit growth for the other segments as well?

Sriram Iyer: So on the specialty formats, we had a lot of centers that were coming up this year, and most of them have got commissioned in quarter 3 and some of them are expected to get commissioned in quarter 4. So, since we are transitioning from the older center to the newer center, there was obviously a transition time and because of which you see a muted growth in the YTD this year. And we are confident that we should go back to a double -digit growth starting the next financial year.

Shyam Srinivasan: Yes, Sriram, but when I look at like footfalls, I still see volume growth. So, what explains that discrepancy between volume growth and revenue growth being much subdued?

Sriram Iyer: See what has happened is that the volume growth is primarily to be with OP footfalls, but specifically in our Cradles and Spectras, these are the centers where we have gone, moved into different centers. We didn't have enough rooms available. So that is why the entire challenge that you see with respect to the volume growth. Also, we had a significant challenge in quarter 3. Because of Chennai as well, so we had a couple of centers in Chennai. There was an

Transcript of AHEL Q 3 FY24 Earnings Call Page 8 of 14 extended delay for us to recuperate back to the business. So that also caused this.

So those are primarily the two reasons. And as I said, we'll be back to a double -digit growth, and we are expected to commission some of the centers in this quarter, in the quarter 4.

Shyam Srinivasan: Got it. Helpful. And just last point on the AHLL, on diagnostics, right? So, we have done about 19%, 20% growth, but the variation just looking at from an industry seems to be all over the place, right? So have low single -digit growth to companies like you're growing 20%. So, anything you can speak from, you know obviously, you are starting from a lower base and maybe your margin trajectory is lower. But just want to understand what's happening there? And is it the case that hospitals are able to keep or hospital chains are able to keep their patients more within themselves?

Sriram Iyer: Yes. I can talk about what we are really doing. So as you rightly said, the industry, all the listed players are growing anywhere between 10% to 12%. And out of that 10% to 12%, the volume growth is pretty much in single digits. So, most of the growth has been driven by price. The good thing for us is that our entire growth has been led by volume. So, the focus on really driving expansion, opening centers and investing in new labs over the period of years is giving this kind of results. So, we are bullish that we'll be able to sustain a 20% kind of growth levels

. And we

will be hitting this year at INR 470 crore of top line. And as I said, our outlook for this quarter also is a 20% kind of growth levels. And with respect to the industry, I think at some point of time, I think the industry is also getting mature in that sense. There is some price play that is coming into this one. So, we've seen some bit of price rationalization happening in the market from across the industry.

So, I think all that is also coming into play where the volume growth is slightly becoming challenging. But as I said, we are positioned well, and we'll continue to invest on diagnostic business, and we have an outlook of 20% growth in the coming year as well.

Shyam Srinivasan: Understood. Helpful. Just a second question on Apollo 24/7. I'll be brief. We had 2 million registered users add up, but our daily active users have kind of come off. Is there a way in which we represent this number? Is it average versus period end? If you could help us, please?

Sanjiv Gupta: Yes. That's the average numbers and what happens is that typically in the e-commerce when you get to the registered users, it takes a little bit of time before they start transacting into your system. And this is a little cyclical in nature. But as you move forward, you would see that the impact of adding new customers into the daily active users and weekly active users will start doing that.

Shyam Srinivasan: Just a request here because when we want to compare with historical numbers, it becomes very difficult. Like, last time, I think we had 8.8 lakhs or 8.5 lakhs. Now it is 6 lakhs. And now you're saying it's average. So, for us to see whether traction is happening because the registered users are going up. But if the active users are not following, it's a request because if you change these parameters, then it's difficult for us to compare with historical numbers?

Transcript of AHEL Q 3 FY24 Earnings Call Page 9 of 14 Sanjiv Gupta: No, it's the same apple to apple. It's just as far as the Q3 is concerned, as I said

earlier also, this is the period where we saw the seasonality impact because of the holidays and festivals. But otherwise, the corresponding numbers are similar. And as I said previously, when you add customer base, which is about 2 million for the entire quarter, the impact of that will start happening from the current quarter onwards. And maybe going forward, we'll start presenting certain key metrics also, so that you all are well informed about those numbers and then compare it versus the previous quarters.

Shyam Srinivasan: Perfect. Last question is on the GMV split. If you could give us some broad on what are the key line items in the GMV?

Sanjiv Gupta: So, INR 658 crore to GMV for Q3 is about 50% is on the pharmacy, about 35% is the IP, OP, which is the funnel to the hospitals, and the rest about 20% is towards the diagnostic and the consultation.

Shyam Srinivasan: And versus Q2, is it very dramatically different?

Sanjiv Gupta: Not exactly, there's a slight drop in the hospital side of the business. I think that's to do with the seasonality. But otherwise, I think nothing worries us from that point of view, just the seasonality impact of Q3.

Moderator: We have a next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: On the off line pharmacy, can you give us a nine-month growth number for the offline business and the outlook that we have for the growth and profitability for this piece for the next couple of years?

Obul Reddy: Pure offline business has grown at 19.4%, and we continue to grow around 20%, 22%. This is slightly because of the lower number of pharmacies we added during the year, which we are going to ramp up going forward.

Nitin Agarwal: And Mr. Reddy, how do you see that playing

ing over the next two years, 20% CAGR we are on?

Obul Reddy: We will be between the 20%, 22% as we guided always. And this year, because of the lower number of stores and as I said, we'll be going back to the regular normal increase, and we assure you that we'll be around that percentage.

Nitin Agarwal: And how does one look at profitability on this piece now over the next couple of years?

Obul Reddy: Profitability, we have improved a lot. It will improve further. But I can't guide you the exact number. You have seen this quarter a substantial improvement. We further have head room to improve on that with the cost rationalization and growing the matured stores where we are focusing now.

Nitin Agarwal: Okay. So, if I would, it's fair to say that with the guidance that we have, the digital business will be breaking on its own over the next six to eight quarters, will effectively the offline EBITDA should pretty much start, should be equivalent to the overall HealthCo EBITDA as you go forward six to eight quarters.

Transcript of AHEL Q 3 FY24 Earnings Call Page 10 of 14 Obul Reddy: I agree with you, but this will have independent growth, whereas they will be achieving with their own revenues and cost rationalization.

Krishnan Akhileswaran: But you're right, Nitin.

Obul Reddy: You are right. On a comparative basis but...

Krishnan Akhileswaran: It should flow through to Apollo Health Co.

Nitin Agarwal: Yes. So we're looking potentially at INR 800 crore to INR 1,000 crore EBITDA on the overall Health Co business over the next couple of years?

Obul Reddy: Couple of years, yes. Yes, you can take that about 20% year -on-year growth.

Moderator: Thank you. The next question is from the line of Dheeresh Pathak from White Oak. Please go ahead.

Dheeresh Pathak: I'm referring to Slide no. 24 of the deck. So there, on right -hand side, you mentioned omnichannel pharmacy revenue right, which is INR 2,583 crore . And on the left -hand side, the total health revenue in the grid is INR 2,049 crore . So, the difference is INR 534 crore . So obviously, there is some 24/7 revenue also in here, but the bulk of the difference is the front -end Apollo Pharmacy, right . That is the way to understand, right . When you say omnichannel pharmacy, that is what you refer to, right? But in the omnichannel pharmacy, do you also include 24/7 revenue, or do you exclude the 24/7 revenue?

Obul Reddy: We exclude the 24/7 revenue.

Dheeresh Pathak: How much is that sir?

Krishnan Akhileswaran: Sanjiv, what was that revenue for Y -T-D? Would you have that number o fffhand?

Sanjiv Gupta: That would be in the range of about , for the quarter, we are roughly INR 300 crore – INR 325 crore .

Krishnan Akhileswaran: No, revenue.

Dheeresh Pathak: Revenue of INR 24/7?

Sanjiv Gupta: Yes, in the front end.

Dheeresh Pathak: No, not front end.

Krishnan Akhileswaran: Only 24/7 he is saying –

Obul Reddy: Digital revenue.

Krishnan Akhileswaran: Digital. Only digital, Sanjiv.

Sanjiv Gupta: Yes. So digital revenue is in the same slide, we can refer to digital revenue of INR 225 crore . And if you are referring to...

Transcript of AHEL Q 3 FY24 Earnings Call Page 11 of 14 Dheeresh Pathak: No, sir. That is online distribution as well as 24/7. I want you to split the online distribution and 24/7 in to two.

Krishnan Akhileswaran: He is asking only IP, OP and diagnostics and consults related revenues that you have.

Sanjiv Gupta: Yes, understood. So that would be in the range of about INR 15 crore for the quarter.

Dheeresh Pathak: INR 15 crore for the quarter , okay. And when you refer to on track to achieve INR 1,000 crore revenue, 6% EBITDA. This i

s the omnichannel pharmacy business, including the front -end Apollo Pharmacy, but excluding this 24/7 INR 15 crore a quarter and the 6% is the pre AS 116, right, margin?

Obul Reddy: That's right , where we are currently at about INR 5.85 crore .

Dheeresh Pathak: Okay. So, this INR 15 crore on a GMV of INR 657 crore , it looks like less than 3% take rate, is that the right way to look...

Krishnan Akhileswaran: INR 657 crore includes the pharmacy also. So, the INR 657 crore includes the pharmacy as well. So, you have to exclude that pharmacy and then look at the take rate.

Dheeresh Pathak: Okay. So, take out INR 200 crore roughly, so INR 467 crore . So INR 15 crore on INR 467 crore , that will still be...

Krishnan Akhileswaran: 50% is at pharmacy, 25% plus 25%, we said it's IP, OP and diagnostics. So maybe you can get offline to understand this from Sanjiv.

Moderator: The next question is from the line of Arkoprati Pal from Sanjay Agarwal Broking. Please go ahead.

Arkoprati Pal: My first question is , could you please share industry report on average occupancy rate within your hospital network for the month of December 2023? And how much it is growth as compared to December 2022?

Suneeta Reddy: That's 66% monthly numbers...

Krishnan Akhileswaran: You will have to come offline and take it from our Investor Relations, Krishnakumar. You can reach out to him, and he can provide you some guidance around that. The occupancy rate is also to do with the average length of stay, which has come down across the system. So, if you look at year -on-year, we have been growing volumes at almost around 7%, excluding institution. We brought down some of the institutional cases. So , while we are showing a 5.5% or 5. 6% growth on inpatient volumes, if you look at the inpatient volumes December -to-December, only on the

institutional and retail, which constitutes around 80%, 85% of our business, that has grown by a healthy 7.5% at volume.

Arkoprathim Pal: Okay. I understand. And my second question is based on current industry trends and your understanding of the healthcare landscape, what is your expectation about the demand scenario of the current and coming quarters compared to last quarter?

Transcript of AHEL Q 3 FY24 Earnings Call Page 12 of 14 Suneeta Reddy: So, I think the structural demand is intact. And what we're seeing is that more

people now have access to private insurance. And that's why we're seeing a significant growth in this segment. So, definitely more people who can afford corporate health care, and therefore, demand is increasing. And this is happening not only in the metros, but in Tier 1 and Tier 2 cities as well.

Moderator: Thank you. We have a next question from the line of Siddhant Kanodia, an Individual Investor. Please go ahead.

Siddhant Kanodia: I have a couple of questions. My first question is regarding , like in last couple of years, medical tourism in India has seen quite a good uptick, especially in the Delhi -NCR region. So , like we are coming up with the Gurugram facility, so why aren't we looking to do brownfield in our existing Delhi facility?

Suneeta Reddy: So, as you know that existing Delhi facility is a joint venture with the Delhi Government , but we are looking at an expansion there. Our International patient revenue there is 15 % of total revenues. We do believe that Gurgaon, which will be 100% owned by AH EL, will definitely offer an opportunity for us to increase our International patient revenue because of its location and because of the doctors and the clinical portfolio that we will offer there.

Siddhant Kanodia: All right. I'm just asking about the brownfield, if the ramp -up happens a lot faster in brownfield, plus if we have...

Suneeta Reddy: Yes, we are adding , currently, we are addin

g 50 beds. We are adding a new , a huge neuro department, neurology. So, simultaneous brownfield in the old hospital at Indraprastha and new hospital coming up in Gurgaon.

Siddhant Kanodia: Okay. So, there are plans to do brownfield as well. And my second question is that there was this article involving some kidney racket, which accused Apollo. So , we haven't heard anything , have we gotten any clean chit or what is the status so far?

Suneeta Reddy: So, the status is there has been no negative finding. And this much I can say with confidence because we were fully compliant with Board , with all Government regulation. So , there has been no finding.

Siddhant Kanodia: So, there hasn't been any negative finding. Okay.

Siddhant Kanodia: Thank you ma'am. That's it from my side.

Moderator: We have a next question from the line of Kunal Dhamesha from Macquarie. Please go ahead.

Kunal Dhamesha: Just one on the broader industry side. This proposed move to the 100% cashless health insurance, what could be the impact on the hospital industry and specifically for Apollo?

Krishnan Akhileswaran: For us, I don't see any impact coming out of this, any negative impact coming out of this because all our insurance businesses are all cashless. We have all empanelment across all large insurance companies at cashless. So, working capital continues per se, as such. Of course, we are also ensuring that I think the one good thing which can come out of this is as everything becomes cashless, I think the larger industry will expect greater, faster payments from the insurance

Transcript of AHEL Q 3 FY24 Earnings Call Page 13 of 14 companies also. And we are hoping that some of that should hopefully come faster,

and it should ideally for people like us should shorten the receivables period also.

But otherwise, for us, nothing significant for now.

Kunal Dhamesha: Sure. And the second question on the INR 100 crore incremental doctor expenses that we have incurred this year. Can we say that it is primarily attributable to the 150 beds, which we have added in Indore and Rourkela or some Eastern region and Northern region?

Krishnan Akhileswaran: Not only that, it is something that we are planning across the system because we clearly have 66% occupancy to move to 70%, and we have even opportunity to grow some of the clinical programs as we spoke, right . So clearly, it is a combination of this and some of the clinical programs that we are planning.

Suneeta Reddy: So, there was a growth in oncology of 17%, urology of 14%. And all this growth is attributable to the new doctors that we've hired.

Kunal Dhamesha: Sure. And the last one, if you can share the take rates for the service lines for 24/7,

the consultation, pharmacy delivery and diagnostics?

Sanjiv Gupta: So, on the pharmacy side, as you know that the entire pharmacy comes back to us as a revenue, as pharmacy distribution to get the same revenue to us. As far as the consultation fees is concerned, consultation moves anything between 5% to 7% depending upon the category of the doctor and whether it is clinic or the hospital. On the diagnostic side, it is roughly between 15% to 18%, depending upon the volumes as well as the class of diagnostics.

Kunal Dhamesha: Sure. Thank you and all the best.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments. Over to you.

Suneeta Reddy: Ladies and gentlemen, thank you for participating in our conference call. We truly believe that as we look ahead, we are confident that in the upcoming quarters, there would be very strong growth. We have delivered ahead of time on EBITDA break-even for Apollo HealthCo. And I am sure we will continue to deliver on our strategic intent.

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With our network maintaining strong occupancies and ongoing expansions in key facilities, we are well prepared to seize emerging growth opportunities.

I would also like to emphasize that our unwavering confidence in the investments we've made and the solutions that we're working on. These efforts will surely differentiate us because they are anchored in a strong clinical core with convenience and access as key enablers.

Once again, thank you for joining the call, and we look forward to connecting with you in the future.

Moderator: Thank you, ma'am. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Transcript of AHIL Q3 FY24 Earnings Call Page 14 of 14 Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility of such errors, although an effort has been made to ensure high level of accuracy.

Call: May 2024

Transcript of AHEL Q 4 FY24 Earnings Call Page 1 of 15
Apollo Hospitals Enterprise Limited
Transcript of Q4 FY24 Earnings Conference Call
May 31 , 2024

Moderator: Ladies and gentlemen, good day and welcome to the Apollo Hospitals Limited Q4 FY24 earnings conference call. As a reminder, all participant lines will be in the listen -only mode. There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal an operator by pressing star and then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Mayank Vaswani from CDR India.

Thank you, and over to you, sir.

Mayank Vaswani: Thank you, D orwin. Good afternoon, everyone and thank you for joining us on this call, hosted by Apollo Hospitals to discuss the financial results for the fourth quarter of financial year '23 -'24, which were announced yesterday.

We have with us today the senior management team, represented by Mrs. Suneeta Reddy - Managing Director; Mr. A. Krishnan - Group CFO; Mr. Sriram Iyer - CEO of AHLL; Mr. Ashish Maheshwari - CFO of AHLL; Mr. Obul Reddy , CFO of the Pharmacy business; and Mr. Sanjiv Gupta - CFO of Apollo 24/7. Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risks and uncertainties. Documents relating to our financial performance have been circulated earlier and will be referenced during this call.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Thank you, Mayank. And good afternoon, everyone. Thank you for taking time to join our earnings call. It has been just over a month since our previous investor interaction, and it is my pleasure to interact with all of you once again. I trust that each of you have earnings document , which we had shared earlier. FY24 has been an eventful year for Apollo Hospitals. We have reported strong double -digit growth in consolidated revenue and profit after tax with a solid traction on all of our business lines. Continued solid operational progress in the healthcare service s business across parameters of case mix, payer mix and the ARPOB. Investments in high -end technology, cutting -edge treatment for patients are gaining traction with increasing number of robotics and minimally invasive surgeries and advanced procedures like TAVI and TAVR with outstanding clinical outcomes. We have maintained the pace of network expansion with the addition of beds in Indore and Rourkela for the Hospital business, a net addition of 489 pharmacies and several additions across the Apollo Health and Lifestyle facilities and diagnostic centers.

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Against that backdrop, here are a few highlights for our Q4 FY24 performance. Our Healthcare Services business witnessed a strong 17% year -on-year revenue growth, picking up the pace of growth from a relatively slower start at the beginning of the fiscal year.

Within this, the cash and insurance revenues grew by 17% year -on-year and contributed to 82% of our IP hospital revenue. Revenue from international patients also grew by 24% year -on-year. These represent the outcome of our focused efforts to optimize our p ayer mix.

IP volume grew by 6.1% year -on-year. Within this, surgical volumes grew by 9% year -on-year, maintaining a healthy trajectory. Tertiary care specialties like Oncology, Neuro Sciences and Gastro Sciences grew at a healthy rate. Higher secondary specialties too witnessed a robust growth due t

o patients preferring

the organized sector care enabled by insurance products.

Overall, occupancy across the group was at 65%. This has been achieved despite a marginal reduction in ALOS year-on-year and after recalibrating some beds for focused specialty clinical requirements and optimizing realization.

ARPOB on an overall basis increased 12% year -on-year to INR 59,523.

Financial results : Our consolidated revenue grew by 15% on a year -on-year

basis to INR 4,944 crore . Healthcare Services grew by 17% to INR 2,563 crore . Revenues from Apollo HealthCo were INR 2,027 crore in quarter 4, growing at 13% year -on-year. Revenues from Apollo Health & Lifestyle registered a growth of 15% year -on-year at INR 355 crore in Q4 FY24.

Consolidated EBITDA was at INR 641 crore , registering an increase of 31% year-on-year. Within this, the Healthcare Services EBITDA was at INR 593 crore, registering a growth of 11% year -on-year and Healthcare Services margins were at 23.1%.

The marginal dip in margins was due to curated investments in new doctors and enhanced sales and marketing costs. We believe that these investments in clinical talent would lead to traction in volumes in FY 25 and our increased focus on discretionary costs will help us restore and grow margins.

The pharmacy distribution business in Apollo HealthCo recorded an EBITDA of INR 134 crore , year -on-year growth of 11%. 24/7 operating costs were INR 151 crore , lower than the operating costs of INR 156 crore in the trailing quarter of Q3 FY 24. Apollo HealthCo has, therefore, reported an EBITDA of INR 12 crore , sustaining a trajectory of positive EBITDA.

AHLL recorded an EBITDA of INR 36 Crore , 40% year -on-year growth and an improved margin of 10.1% as compared to 8.3% in Q4 FY23.

Consolidated PAT was at Rs. 254 crore, a strong growth of 76% year -on-year.

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Within the Healthcare Services business, we have delivered a ROCE of 26.1% with balanced ROCEs across all our geographies; the metro, the Tier 1 and the Tier 2.

The Private Label and Generic business of pharmacy was at 16.1% of total pharmacy revenues. Our digital platform, 24/7, added 2 million new users. The platform GMV was at INR 681 crore , representing a growth of 35% over the same period in the previous year.

The outlook for FY25 is clear and exciting. We are pursuing multiple vectors to drive growth and improve profitability and ROCE.

These include a plan to operationalize 4 new hospitals with 1,500 beds in the existing markets, is well on track. We will go live with our new hospital in Gurgaon as our anchor hospital for NCR as well as Kolkata, Hyderabad and Pune within the calendar year 2025 to 2026.

In the Healthcare Services Delivery business, there is a concentrated effort to enhance surgical volumes across key Centers of Excellence. These will be driven basically by the augmented medical team, higher -end procedures, innovative therapies such as CAR -T cells and aided by tailwinds provided by steadily increasing proportion of insurance patients in our payer mix.

We are also implementing cost optimization measures, which accompanied by growth in surgical volumes should help improve Healthcare Services EBITDA margin by 150 basis. We are committed to achieving breakeven for Apollo 24/7 Digital business within AHLL in the next 6 to 8 quarters. We will continue to grow the platform GMV for Apollo HealthCo in FY 25, while progressing on the transaction and integration. We will deliver a healthy growth on the diagnostics vertical with an expanded test menu, and therefore, increasing margins.

On that note, I would like to hand it over to our moderator and open the line for questions and answers. I have Krishnan, our CFO; Sriram Iyer: from Apollo Health and Lifestyle, as

well as Obul Reddy and Sanjiv from Apollo HealthCo with me to take all of your questions. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session.

The first question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: While we have said that there are multiple vectors of growth, can you provide more quantitative details on the growth outlook for each of our top 3 businesses; Healthcare Services, Pharmacy and AHLL for FY 25?

Suneeta Reddy: So let me start with Healthcare Services ; so, we are looking at a growth of beyond 15%. This growth will be driven by volume, and we say this with confidence because as we look at part by our network expansion and better asset utilization, we also looked at our payer mix within these markets. There is a significant growth in the penetration of private insurance. And this is enabling people who would, otherwise, go to smaller nursing homes to come into the Apollo system, and I think it's creating access, and this is really driving volume s.

The second aspect of this is we have been focused on international patients. However, all of the connectivity was not as good last year as it is this year. And with that, international patient revenue is currently at 7% of total revenue. But in Delhi, it's at 20% of total revenue. So, we do hope that this will again move up the occupancy and ARPOB.

The third is Centers of Excellence. We've made investments in Onco and neuro, but beyond that we've been able to attract the best doctors in these fields. And with that, we are seeing a growth of about 20%. I think we ended the year with close to INR 2,300 crore of revenue in Onco alone as an example. So, there's also cardiac. So, we've on-boarded 150 new doctors who you know this year is like a cost and really took away from margin. But as we go forward, these doctors have already started contributing, they will contribute. And some of them will be moved into our new hospitals so that we can achieve breakeven faster as we open these new hospitals.

Sanjiv Gupta: I will take HealthCo. So, a couple of things, if you look at FY 24, we grew by about 73% in the digital business versus FY 23. As far as the current fiscal year is concerned, FY 25, we're looking at anything closer to 50% growth from the current levels of INR 2,700 crore for GMV that we did in the previous year. Majority of the growth is going to come from the pharmacy. And we have got certain action plans, which talk about increasing the private label, increasing the assortment and also looking into the omni way of acquisition of the customers and then looking in to the entire Apollo ecosystem to ensure that we have more access to the customers, and we have more number of transactions over there.

Diagnostics will continue to grow. And apart from this, we are also banking on insurance and digital therapeutics. These are the 2 verticals that should give us a decent margin. And apart from the fact that corporate partnerships across the Apollo ecosystem is, was something which we believe that current year will give us the growth momentum.

As far as the front-end or the offline stores are concerned, we closed with about 6,000 stores in FY 24. We added about 500 stores in the previous year. We strongly believe that we would add another 500 to 550 stores in the current fiscal year. And would see anything between 20% to 21% as the growth in the offline business. That's from the Apollo HealthCo.

Kunal Dhamesha: And just since we are kind of discussing the offline pharmacy, the growth in the quarter 4 is quite below the run rate, right? And we have highlighted that there is some liquidation at the front-end stores. So, can you quantify that impact, had that not been the

re, what could have our growth within that?

Obul Reddy: Last year FY23, as you are aware, we have added about 1,100 stores in view of that. And also, to increase the stock in our existing stores, we have increased the stock by about INR 248 crore. Against this, in the current year, the stock increase is only about INR 97 crore. We have not added anything in the existing stores, only for the stores opened during the year, we added the

stock. This is because of the store-level stock rationalization, which consciously we planned and worked. That is the reason. So, there is a delta of about INR 150 crore lesser stock at the front-end, which means lesser sales for the AHEL's pharmacy distribution. This is the only factor that has influenced the growth. And as we move into the next year, we are factoring about 22% growth in the front-end, which should reflect in the back end.

Kunal Dhamesha: And sir, the 248 stores that we have opened, that would have also led to some more inventory filling in those stores, right? So that should have also positively impacted...

Obul Reddy: Not 248 stores. It is about 550 stores we opened during the year, wherein we've talked about INR 80 crore worth of stock. Against last year, where we opened 1,100 stores, where we stocked about, say, INR 150 crore. Apart from it, in the existing stores, we have increased the stock last year through the rationalization of SKUs, et c., by about INR 90 crore.

So overall, from the beginning to the year-end, there is an INR 248 crore increase in the stock in FY 23 over '22, whereas a similar number in '24 against

'23 is only INR 90 crore . That difference is a delta that reflects in the sales of back -end because APL is the exclusive , I mean, AHEL is the exclusive supplier for the APL.

Kunal Dhamesha: Okay. So , there is inventory rationalization at the API level, right?

Obul Reddy: Yes, almost. That impact is about INR 150 crore on the sales of back -end pharmacy distribution.

Kunal Dhamesha: And when you open a new store, typically, how much month of inventories do you carry or is pushed into a new store?

Obul Reddy: We generally start the new store with about anywhere 10 to 12 lakhs per store stock when we operationalize this store and then over a period of 3 years move to 18 to 20 lakhs.

Moderator: The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: If I look at the occupancy number in the last year, we haven't seen too much of an improvement year -on-year. I know ALOS has sort of moderated so that benefits volume. But we have sort of guided to that 70% margin over time.

One, when do we think we get to that 70%? And what according to you will take us there? I know we've invested in all of these doctors, et c. But is this enough to give us confidence to get to the 70% occupancy level?

Suneeta Reddy: Yes, I think the doctors are important. But second, I think the insurance penetration, including into the tier 1 and 2 cities is really going to improve occupancy. And I have to state at this time that in our metro cities, we're about 70%, average is 69%, for some hospitals already at 80%. So , when they are at 80%, their ALOS is at 4. So , there is a direct correlation between ALOS and occupancy, and therefore, that's why we say that we focus on the volumes. And we're quite sure that in this year, we have budgeted 68% to 70% volume.

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So, we are really focusing on improving volume, improving case mix, and that's why we have on -boarded these doctors. We've also got tie -ups with private insurance that will make sure that it cre

ates access with tier 1, tier 2 cities

besides the metro. All of these efforts will result in even higher occupancy.

Neha Manpuria: And the doctor hiring, I know we hired 150 doctors, which you said some of them would be shifted to the newer hospitals. But since we're commissioning 4 other hospitals next year, wouldn't that hiring again happen next year? And wouldn't that be an offset to the margin expansion that we see from the existing beds?

Suneeta Reddy: No, no. There will definitely be some hiring at that time. But I think what we're trying to establish here is that last time we added beds, we were slow to see breakeven. But at this time, with the established beds, we're quite confident of seeing faster breakeven because we've identified certain key doctors for all o f this specialties, and they are getting trained within their core.

Neha Manpuria: Understood. And my second question is on 24/7, the breakeven guidance that we have made, what sort of a GMV should we get to, to get to that breakeven in 6 to 8 quarters? And how should that mix of the GMV look? I mean, how much of that should come from th e newer areas like insurance and digital therapeutics? Am I thinking about this the right way or that breakeven is also dependent on costs being lower versus what we're doing now?

Sanjiv Gupta: So I think first , today we are at about INR 700 crore of GMV if we look at Q4 numbers, specifically INR 700 crore . Now, as we look into 6 quarters, what we are targeting is the total GMV for that quarter to be in the range of about INR 1,700 crore in that. And how the composition will change is that if I look at current composition of INR 700 crore , it's about 46% is the pharmacy. This particular division should give us about 55% in this quarter from now. And we are also looking at about 10% of INR 1,700 crore to come from newer segments, which is the digital therapeutics, insurance and the demonetization. Some parts of it within the insurance and monetization has already started. We are into a run rate of about INR 20 crore annually as we speak, which is about INR 2 crore to INR 5 crore currently per quarter. So, 10% mix will come from the newer segment, about 55% is from the pharmacy and rest all diagnostics, consultations and all these ones will continue to be here.

Now, we are also mindful to the fact that , this is Plan A that you get into a topline of about INR 1,700 crore of GMV. And the current take rate, if you look at this, is about 4%, which is the commission that we call it. And the 4% will go up to 8% in 6 to 7 quarters from now. While we keep , we've got detailed plans

and such results to ensure that we achieve that target. So mindful that at some point in case we think that there's a little bit of lesser volumes coming into the business, we will have to take care of the expenses. We will also rationalize and optimize that. The plan will be also to look into the extensive payer's mix because (Inaudible 0:23:07 min.) at INR 190 crore of quarterly expenditure, we brought it down to INR 140 crore as before and speaking to that day. But that will be dependent on how growth is coming up. So, I think net-net, it is a

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sum total of both the things, where we are looking to working on the growth drivers as well as we are looking into the cost side.

Moderator: The next question comes from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: So, a couple of clarifications on the number of beds. Just wanted to know if the Rourkela has been added? Is it operational already?

A Krishnan : Yes, it has got operationalized, and it's part of the numbers here.

Bino Pathiparampil: Okay. Because in the Eastern region, I am seeing only about 50 beds added to the operating beds in your

presentation.

A Krishnan : These beds were added now, operationalized , it is gradually getting operationalized. It will get higher with time.

Bino Pathiparampil: Understood. Second, in western region, I can see the operating beds increased by 60, but I don't know of any new facility added there. Where is that increase coming from?

A Krishnan : Indore is part of Western region, and that is something that we have added in the 60. So , we have an expansion in Indore that we have done recently. And so that's the number which is coming there.

Bino Pathiparampil: Sir, in that case, the northern region has gone up by 100 beds plus. Where would that be coming then from?

A Krishnan : So, Lucknow also added in Northern. In Northern, if you look at it, Lucknow added by almost around 40 , so actually, Northern includes Lucknow and Indore. So , to your point, 60 and 50 of , 60 beds of Lucknow and 50 of Indore actually got added in Northern. So , if you look Western region, we had this Ahmedabad where we had bought over this , one of these facilities called CBCC, which was a 50:50 JV. And that is something which is now getting consolidated. It's a small JV. So , the beds have come into that.

Bino Pathiparampil: Understood. And just to follow up just on the answer to another question previously, Suneeta mentioned about some tie-ups with insurance companies to drive up volumes. Could you elaborate a bit on what sort of tie-ups are these? How does it work?

A Krishnan : So, if you look at the insurance, there are multiple things that we are doing. One is the insurance itself is broken up as retail as well as corporate. So , on the retail side, we have started working with a lot of insurance brokers and agents as well because they have strong connects also in the markets who are also enabling patients to take decisions at the time of a need. So, at one end, we are working with some of them to see how we can , we have started working on that front very recently, so it's something that we are seeing whether we can get traction on that. The initial this has been good, inflows that we are getting has been good.

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The second has been corporate. On the corporate side, we are directly working with large corporates across each of the units because from that perspective, the decision-making is really the employee at the time of the need. And he has to really think of Apollo when he needs to come in. So , we are working with them, and I think there is a team now across each of the geographies to see how we can improve our penetration there. So , these are the two things that we are doing. With the insurance companies, there is very little that we can particularly do because they are not able to really channelize patients to a particular set of hospitals.

Suneeta Reddy: But I think with the new regulations, the coverage scope has expanded. They've included robotic surgeries, which were not allowed before. They've included some digital therapeutics. They've included stem cell , some of the oncology procedures, neuro guided navigation. So, several new clinical

procedures have been added.

Moderator: The next question comes from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on the margin for the hospital services or Healthcare Services this quarter. Several comments have been made around new doctor hire, marketing, IT expenses. Also, one-off fixed asset impairment of INR 12 crore , INR 120 million, right? So, has it all come through? Is that impairment also about the EBITDA 9%, and if you could say how we're going to recoup some of this in the next fiscal , sorry, fiscal '25?

A Krishnan : The asset impairment is below the line because that's one depreciation of assets, which ha

ve been aged, which is something which came as part of the audit that we did, the year -end audit. The first one, which is about the new doctor's hires, marketing and IT is all above the line. And this because the IT expense was kind of a onetime reset that we had to do when we moved into the cloud and also migrated from one provider to the other. So given that, there has been a shift in the cost line, and we don't think , we don't see that increasing from here on. So, as you see, some of this revenue increase coming into the next year, you will see that it will get defrayed with the higher base of revenue. The new doctors hire, obviously, it's a clear decision to hire doctors to improve revenues and increase them in sp ecific specialties, and that's started playing already, but we will see a fuller benefit of it by Q2. As we speak, in Q1, we are seeing some benefit in some geographies already. Marketing, again, it is a bit of increase that we did on purpose on digital marketing. Again, we saw that benefiting us. But with that benefits on the revenue, we have now , that also has been a quantum jump, but we are not seeing a jump from here on, and we're also calibrating some of the other non -digital expenditures on marketing as we speak. So , because with all these 3 efforts, we are quite confident that the margins in the next year should go back to the earlier margins and even higher.

Shyam Srinivasan: Yes. Krishnan, if you could quantify, like when I look at 24% for fiscal '24, are any guidance that we are offering?

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A Krishnan : Our target is to get to 25%. That's what we have as internal targets. Let's see how we are able to get there by end of the year.

Shyam Srinivasan: Yes. So, you're not giving that as a full -year target, but just as maybe a quarterly one, perhaps?

A Krishnan : That is correct.

Shyam Srinivasan: Understood. The second question is just on AHLL, specifically on diagnostics. We have now improved to like 10%, 11% margins. Quarterly margins have been even higher. Anything that we are calling out in terms of how should we look at the path forward? I remember in the past; we've had an INR 1,000 crore revenue target as well. So , are we in track for both, those two things?

Sriram Iyer: First of all, yes, our margins have improved this year to 10.8%, which is a 3.2% jump over the previous year. And we clearly have goals as well to hit INR 1,000 crore , which will take a couple of years. The good thing is we are continuing to cycle a 20% growth now for third year in row. And we are quite confident and bullish of continuing to keep growing the topline at that rate and at a better margin level as well. So, we have initiatives. One is the higher volumes that comes into play will give us the operating leverage. At the same time, we have tightened our unit economics and taken a lot of initiatives that is helping us improve our margins. To reach to INR 1,000 crore as well, we have our investments outlined, and we are planning for aggressive growth, so we are very bullish on diagnostics. And we are on path, as I say, the next couple of years to get to the INR 1,000 crore number.

Ashish Maheshwari: And just to add to what Sriram said, we have also been able to improve our gross margin by 3% on the diagnostic side. We have moderated our overall cost. We have changed our methodology from dry to wet, and that has also helped in our gross margin expansion. So, as Sriram said, when the revenue would scale up, then the fixed cost absorption will also get better. And hence, the projected EBITDA margins will also significantly go up.

Shyam Srinivasan: Helpful. Just one sub -question on diagnostics. What's our B2B, B2C split of the diagnostic venture? And is there any targets that we're givin

g even for the margins side, please?

Sriram Iyer: I think currently our B2B mix is about 25% to 30%. That's our B2B mix. The rest of the things are from a B2C and digital put together. So, we expect the B2B business to be around 1/3rd of our business and the rest 2/3rd to continue to come from B2C and B2B. In terms of margin, we should be around mid-teens. That is what the target is, to be around mid-teens in this financial year.

Moderator: The next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets India Private Limited.

Damayanti Kerai: My question is on your cluster-wise performance. So, ma'am, you said for occupancy, you're targeting somewhere 68% to 70% for next fiscal. When I am looking at region-wise occupancy, I guess, AP, Telangana and Western region are 2 clusters where occupancy seems much lower than the network

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occupancy. So, would you like to call out any specific region? And will improvement in these 2 clusters will be meaningfully adding on to your 68%, 70% occupancy for next year?

A Krishnan: I think you got it right, actually. I think these are the two reasons that we are really focusing on for the occupancy because in AP and Telangana, if you look at it, even in Hyderabad, we have two smaller hospitals in two different areas and the large hospital is already at close to 70%, which is the Jubilee Hills one, 69%, 70% we are at. But if you look at the other two hospitals, we are at a number which is 50%, et c. So, we are working specifically on those areas to see how we can take that to at least 63% in the coming year, especially in the AP and Telangana region. And also, there, of course, we have places like Vizag, where we have seen a good traction, which is now coming, and we are now the leaders now in Vizag. And we should hope that Vizag should also help us get the overall occupancy higher. The second point, as you rightly said, Western is a bit of a drag because of Nasik. That's one that we are not very comfortable taking the occupancy at a higher level. But Mumbai and Ahmedabad is something that we are quite, Mumbai is doing well, but Ahmedabad has some options to go up. And that is something that we are working on. So, these two geographies definitely are important to get our overall occupancies higher in the overall when we take it to 68% to 70%.

Damayanti Kerai: Okay. Just a clarity, although Nasik is a small unit, but what is the main issue there, like why is the drag?

A Krishnan: I think, though it is a small market, unit, there are multiple players in that market who have come in already. I don't even understand why so many players are contending in that small market. And multiple different players have come in. The second point is we are not taking scheme patients, et c., because we don't want to, the market is also a bit of a low-paying market, and then, you have a scheme. So clearly, I think it's a market that is not a very, we're not investing there. We just want to make it stop. It's reasonably, it's just getting breakeven. I think we would just continue to grow it from here and see what we can do better.

Damayanti Kerai: That's helpful. My next question is a bit of clarity on your margins. Did ma'am mention that for next year there will be 150 basis point improvement in margin despite like four new facilities coming on board?

A Krishnan: So, most of the facilities are coming really by Q4, right. If you look at the facilities, and one of that could probably even go to Q1. So, I think, pretty much, I think we are looking at the full-year margins to get to expand by at least 150 bps is what we would like to do full year. As I said, 25% is the target that we have for the end of the year also.

Damayanti Kerai:

Okay. Last question is on your AHLL business, especially on Specialty Care. So, when I look at revenue, it's very similar in fourth quarter and the third quarter. But in terms of margin, I guess, this quarter, it's around 5% compared to 13% in sequential quarters. So, what has happened? Why like so much of swing?

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Sanjiv Gupta: So one of the things on Specialty Care is that we had a lot of centers that were coming up last year. And some of the centers will have gone live in this quarter of this financial year. So, a lot of costs had got baked in, and of course, the

revenue had not come. So that was one of the reasons. And in this financial year, you will see improvement in margins coming in the Specialty Care business. So that is one of the reasons that happened in the Specialty Care.

Ashish, you want to add anything else?

Ashish Maheshwari: Yes, maybe. So, on the secondary care Specialty business, there are 4 centers; Electronic City, Rajajinagar, Indirapuram. For first 3 centers were having only costs. They were the relaunches or the launches in the current year, in FY 24, and could not contribute to revenue. Then, we had two relocations, one in Jaipur and one in Pusa Road. Also, these relocations did not contribute to the revenue. In terms of quantification, we had a fixed cost due to these changes of almost INR 29 crore, but there were no, there were minimal revenue impact, so this really impacted the overall margin.

Damayanti Kerai: Okay. So that exercise is done now, and things should pick up from here on.

Ashish Maheshwari: Yes. So, the relocation as well as the launch is complete. In FY 25, we will have revenues for the full year. There are certain last minute and the closure of the launches in Electronic City, which is still in progress, but definitely during FY 25, we'll, for the major part of the year, we'll have the revenues also coming in.

Moderator: The next question is from the line of Siddhant from Tusk Investments.

Siddhant Kanodia: Generally, we have seen that the mature centres have an EBITDA margin of more than 20%, 21%. But our centre in Delhi, it's a mature centre with high ARPOB and very high occupancy, still we are doing margins around 15%, 15.5%. So, what are the reason for this low margin? And any targets for FY 25 in terms of the margins for the Delhi centre?

Suneeta Reddy: So, the Delhi centre, there's a lot of free work. So, if you added back the free work, it would definitely be in the 24%, 27% margin.

Siddhant Kanodia: Okay. And previously, it was mentioned that we are looking to do capex at the Delhi centre. So, what is the progress so far on the capex front?

Suneeta Reddy: For the capex we have done, what we've done currently is to install the ZAP machine. We have to do, create this parking, the one-side parking to be compliant for us to add further beds. So, we are looking at completing the parking requirements so that we would get additional FSI. So currently that...

Siddhant Kanodia: Okay. And after the parking issue, what is the amount of beds you are looking to add at the Delhi centre?

Suneeta Reddy: The amount of beds.

Siddhant Kanodia: The number of beds, yes.

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Suneeta Reddy: Yes, yes. We're looking at 100 beds.

Siddhant Kanodia: Okay. And the leasing was also due at the center. Has that been taken care, the land lease?

Suneeta Reddy: That has been taken care of, yes.

Siddhant Kanodia: Okay. And regarding the capex mix, since it's a JV, do we need the approval of the center?

Suneeta Reddy: We have the approval of the Government.

Moderator: We will take the

next question, which is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Can you help us understand our payer mix for FY 24 for the Healthcare Services business?

Suneeta Reddy: Yes. Currently, we have 39% is cash. Over 40%, 43% is insurance. 7% of our revenues are international patients. And we still continue to work with State Governments in 4% to 5%.

Kunal Dhamesha: Okay. And last year, the Government patients because as far as I remember, last year, it was more like 45%-45% (inaudible 0:42:08 min.) and international and (inaudible 0:42:11 min.) of it. So, has there been range for it?

Suneeta Reddy: So, if you look at different hospitals, it's actually different. But at an aggregate level, this is, percentage has not dropped.

A Krishnan: The PSUs are 7%, outside of the Government if that is what you are asking.

Public sector units are 7%, 3% is government, so between the 2 it is 10%.

Kunal Dhamesha: Okay. 2 institutional, we have 10%, like PSU and Government.

A Krishnan: That's correct.

Kunal Dhamesha: Okay. And then how do you see improvement in this in FY 24?

Suneeta Reddy: I think we are broadly okay.

Kunal Dhamesha: Okay. And in terms of number of...

A Krishnan : Insurance can grow a bit faster from here on also because that's something that we will see happening.

Suneeta Reddy: And we're focusing on international patients, so hopefully that will improve to 10% of total revenues.

Kunal Dhamesha: Sure . And what would be our total number of doctors as of the end of FY 24?

A Krishnan : We'll come back to you, please. We don't have the numbers off hand.

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Suneeta Reddy: It's 6,100 doctors, I think.

Kunal Dhamesha: Okay. I will get in touch with Krishna Kumar on that. And then the last one on the GMV, since we are expecting (inaudible 0:43:41 min.) for 24/7, when should we start seeing that at jump? Because for that , for us to meet that guidance, we should see a step jump in the GMV, right? Will it (inaudible 0:44:57 min.).

Moderator: Sorry to interrupt, but your line for you seems to be breaking up in between.

Sriram Iyer: I got the question. Maybe I can give the answer. I think what Mr. Kunal is trying to check is that the GMV growth from which quarter we start looking at increase. I think Q2 onwards, we should start looking at increase. And this is only because of the reason that we set our model in Q3 of the last fiscal year, which impacted our Q4 and a little bit of Q1. Hence, we strongly believe that from Q2 onwards we'll start seeing the meaningful growth, and that is how directionally we will hit the numbers that we're looking at for six quarters from now.

Kunal Dhamesha: Sure. And let's say, in all our business growth, how much we are baking in, in terms of the effective seasonality or the quarter 2, quarter 3, which generally remains stronger, right? Are we expecting a seasonality to be above the seen last year? What are we baking in, in our base case?

Suneeta Reddy: For hospitals, we are looking at a better quarter, so March, April, May in spite , I think the only detriment returns would be with extremely hot weather, which is persuading some people from traveling. But, otherwise, we are looking at a better quarter.

Moderator: The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala : Just two questions from my end. First is on the court case, which was in our public litigation, which has been filed with regards to curbing the hospital price , rates at which the surgeries are performed. So , any update on that front? I mean, you could provide if it is helpful.

Suneeta Reddy: Yes. I think the Supreme Court said that hospital rates are a function of market rates. And in a place where

you cannot , in a market where you cannot control the input rates, specifically calling out doctor fees, you cannot really put a fix on what the pricing should be. It should be determined by markets probably. We also made a mention of lawyers are not bound to charge by a certain size, and that same principle should be applied to doctors as well. And having said that, we told the Government to come back on September 10. But I think it was a very good outcome for the private sector. Abdulkader Puranwala : Got it. Just a second question, again, on this. So, there have been talks about some revision to happen on the CGHS rate. I mean what could be the impact on Apollo? Or what is the kind of benefit, which...

Suneeta Reddy: No, no. I think I was referring to the same when I said this was , so earlier the

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court had mandated the Government to come out with the pricing policy, at which time the Government came back with suggestions from several of the States because health is a center and a State subject, where the State said this cannot be implemented. And therefore, this was taken to the Supreme Court. And the response to the Supreme Court , to this was that, yes, it cannot be, it has to be determined by market forces and cannot be done by the Government , in fact.

A Krishnan : Our exposure to CGHS is very limited.

Suneeta Reddy: Yes. And the good that the CGHS rates go up, I believe there definitely is a possibility.

Moderator: The next question is from the line of Siddhant from Tusk Investments.

Siddhant Kanodia: So, at the 100 beds at the Delhi center, what will be the timeframe?

Suneeta Reddy: The timeframe for that should be about 2 years.

Siddhant Kanodia: Sorry, 2 years?

Suneeta Reddy: Yes.

Moderator: The next question is from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: Just one question on the online pharmacy distribution, is INR 238.7 crore number comparable to INR 199 crore number that you have driven in this presentation? Because last year, you had reported sales of around INR 253 crore. So, I was just wondering what changed here, what are the restatements we've made here?

Sanjiv Gupta : Yes, this is the comparable number. And INR 238 crore, which is 35% revenue to GMV, this is in line with INR 199 crore, which is reflected in Q4 FY 23. So, this is, revenue to GMV ratio is correct in both these chances, and they're apples to apples from that time, you can compare them.

Kunal Randeria: Okay. But maybe just for my benefit, can you explain what's the difference between the INR 253 crore that you had reported last year and INR 199 crore that you have restated?

Sanjiv Gupta : I will have to come back to you on this particular point on not giving the right answer for this. But if you can connect offline, I'll be able to help you with this thing.

Moderator: Ladies and gentlemen, that was the last question. I would now like to hand the conference over to Mrs. Suneeta Reddy for closing comments. Over to you, ma'am.

Suneeta Reddy: Thank you, ladies and gentlemen, for joining this call. As we enter FY 25 on a very positive note with all three business verticals having clear plans for revenue growth, enhanced profitability and strong operations in, we strongly

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believe that we have the most comprehensive and integrated medical offering in the region and our best position to deliver the strongest value proposition to our consumers and (inaudible 0:51:02 min.) . We look forward to your continued interest and support.

Once again, thank you all for joining the call, and we look forward to

connecting

with you in the next quarter. Thank you.

Moderator: On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

Disclaimer: This document is a transcript and may contain transcription errors.

While the transcript has been edited for clarity, the Company takes no responsibility for such errors. Efforts have been made to ensure a high level of accuracy, and any figures that may have been inadvertently mentioned have been reviewed and corrected as necessary.

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Apollo Hospitals Enterprise Limited

Transcript of Q 1 FY25 Earnings Conference Call

August 14, 2024

Moderator : Ladies and gentlemen , good day and welcome to Apollo Hospitals Limited Q1 FY 25 earnings conference call. As a reminder, all participant lines will be in the listen -only mode , and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star and then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you, sir.

Mayank Vaswani: Thank you, Neerav. Good afternoon, everyone, and thank you for joining us on this call hosted by Apollo Hospitals to discuss the financial results for the first quarter of financial year 2024-25, which were announced yesterday.

We have with us today the senior management team represented by Mrs. Suneeta Reddy - Managing Director; Mr. Krishnan Akhileswaran - Group CFO; Dr. Madhu Sasidhar - President and CEO of the Hospitals Division ; Mr. Madhivanan – CEO of Apollo HealthCo ; Mr. Sriram Iyer, CEO of AHLL; Mr. Sanjiv Gupta - CFO of Apollo HealthCo ; Mr. Obul Reddy, CFO of the Pharmacy Division; and Mr. Ashish Maheshwari - CFO of AHLL .

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties in our investor presentation. Documents relating to our financial performance have been circulated earlier . These have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks . Thank you, and over to you, ma'am.

Suneeta Reddy: Thank you, Mayank. Good afternoon , everyone , and thank you for taking time to join this call. I trust that all of you have received the earnings documents, which was shared earlier today.

We are delighted to report a strong start to Fiscal Year '24-'25 with our performance in Q1 FY25. We have seen robust performance across all of our business segments , despite the headwinds of election cycles and heat waves , culminating in strong revenue growth and improved profitability on a year-on-year basis. In prior quarters , we had indicated that efforts towards improving volumes by augmentation of medical teams which are now demonstrating results.

Against this backdrop let me walk you through the financials for the quarter:

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Our Healthcare Services business witnessed a strong 15% year-on-year revenue growth to Rs. 2,637 crore .

Within this revenue from insurance patients saw a year-on-year increase of 17%. There is increased consumer movement towards high-quality providers, and we will strengthen this by deepening our partnership with corporate and our retail outreach. IP volumes grew by 11% year-on-year, with growth across all markets. High-end tertiary care specialties like cardiac sciences, oncology, neurosciences, and gastro sciences, which are our areas of strength, grew at a healthy rate. Higher secondary specialties too witnessed robust growth due to patients preferring high-quality organized sector care, enabled by insurance products.

Overall, occupancy across the group

has risen to 68%, an increase of over 600 basis points on a year-on-year basis.

ARPOB increased a little over 2% year-on-year to Rs. 59,073. We believe that ARPOB growth will improve over the next few quarters with stronger growth in surgical volume and better case mix.

Our consolidated revenue grew by 15% on a year-on-year basis to Rs. 5,086 crore . Revenues from Apollo HealthCo were at Rs. 2,082 crore in quarter 1 , growing at 15% year-on-year. Given the strong efforts for stock liquidation in the front-end stores, combined pharmacy business revenue grew by 16%.

44 new stores were opened this quarter. The election season resulted in lower store expansion due to a slight delay in approvals. We expect the pace to pick up in the

subsequent quarters, which should also improve the growth momentum of the business going forward.

The private label and generic business of the Omni Channel Pharmacy was at Rs. 16.1 of total pharmacy revenue. Our digital platform, 24/7, added 2 million new users. The platform GMV was at Rs. 695 crore, representing a 9% growth over the same period last year.

Revenues from Apollo Health and Lifestyle also grew by 15% year-on-year to Rs. 366 crore in Q1 FY25.

Consolidated EBITDA was at Rs. 675 crore, registering a 33% increase year-on-year.

Within this, the Healthcare Services EBITDA was at Rs. 622 crore, registering a growth of 15% year-on-year and Healthcare Services margin was at 23.6%. Our investment in clinical talent, marketing costs are helping to drive this double-digit volume growth. We believe enhanced volume growth, improvement in case mix and payer mix, and a focus on optimizing discretionary cost will drive margin expansion by 100 basis points over the next 3 to 4 quarters.

The offline pharmacy distribution in Apollo HealthCo recorded an EBITDA of 139 crore representing a year-on-year growth of 11%. The digital platform cash losses

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excluding ESOPs were at 97 crore, a significant reduction from 152 crore in the same quarter last year, demonstrating the Company's effort to drive profitable growth.

Apollo HealthCo has therefore reported an EBITDA of 23 crore, extending its trajectory of positive EBITDA.

AHLL recorded an EBITDA of 31 crore, delivering 33% year-on-year growth and an improved margin of 8.4% compared to 7.3% in quarter 1 last year.

Consolidated PAT was at 305 crore, growing 83% year-on-year within the health care businesses.

The outlook for FY25 continues to be promising. We are dedicated to driving growth and enhancing profitability through a series of strategic initiatives.

In Health care services, we are expanding our market boundaries and uncovering latent demand through better use of technology and business intelligence tools. We are intensifying efforts to increase surgical volumes at our Centers of Excellence, supported by an expanded medical team and advanced procedures like CAR T-cell therapy and ZAP-X.

Our plan to operationalize 4 new hospitals, adding 1,500 beds in key markets, is progressing as planned. We anticipate operationalizing new facilities in Gurgaon, Kolkata, Hyderabad, and Pune within the next 5 quarters.

We are on track to achieve break-even for Apollo 24/7 digital segment within the next 6 to 7 quarters, supported by strong growth in GMV and optimum portfolio mix.

Transaction integration and healthy growth in offline pharmacy distribution revenues will drive Apollo HealthCo towards its stated strategic intent in the next 3 years.

Our diagnostic business

is also poised for strong healthy growth driven by

investment in new capacity, expanded test menu, and an improved margin.

On that note, I would like to hand it over to the moderator and open the line for questions. I have our CFO – Krishnan, with me, Dr. Madhu – CEO of the Hospital Division, Sriram Iyer from AHLL, as well as Obul Reddy, Sanjiv, and Madhivanan – the CEO of Apollo HealthCo, who are all here with me to take your questions. Thank you.

Moderator: We will now begin the question-and-answer session. The first question is from the line of Binay Singh from Morgan Stanley.

Binay Singh: My question is on the volume side. We have seen a nice pickup in the inpatient volumes. At the same point, we are also seeing a sharp pickup in the insurance coverage, like in the last call we called it out to 42%, 43% is insurance cover, this time it's around 47%. So, are they two in some ways linked or this is more coming from our efforts to revive some of the smaller hospitals in Hyderabad and some efforts in Karnataka that we earlier talked about? That's the first question.

A Krishnan: So, I think we are seeing insurance growth actually happening across. You know, it's not necessarily corrected only to the urban locations, etc. And we are seeing it across both tier 1 and tier 2 locations as we speak.

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So, what's happening with insurance clearly is that one is earlier as we said, while

we were getting most of the high-end cases earlier, we are also getting a lot of secondary care cases with the insurance business penetration. And there has been a deliberate effort that we have taken to penetrate into corporates, etc. in most of the regions.

So, we are seeing that benefit, and that benefit is yielding in better occupancies. And some of that obviously has resulted in a bit of a lower ARPOB, but the more important thing for us is the benefit that's coming from occupancy, which is sustainable and it's going to help us into the next few quarters as well.

Binay Singh: So, whenever the understanding is correct, right, this insurance pickup of 200-300 basis point that we have seen quarter-over-quarter is in a way leading to better occupancy and at the margin lower ARPOB. A link to that, when we talk about ARPOB 7% outlook for the year and we have said that we do expect the case mix to improve in the coming quarter, is that already visible to us as we are sort of getting out of this election season and moving out of the monsoon season?

Suneeta Reddy: Yes. I think yes.

A Krishnan: July has been good, and I think we should be able to see better pickup in this quarter.

Binay Singh: That's great. And lastly, just on the diagnostic side, I do understand there is a little bit of seasonality in quarter 1. Anything else you want to call out for the sequential margin drop that we have seen in the diagnostic side?

Sriram Iyer: Yes, as rightly pointed out, it was a bit of a seasonal impact that we had, and I think we have also made certain investments on adding specialty manpower and focusing on some of the segments that we were not present earlier. So, certain investments have gone on those in the quarter 1 and starting this quarter we will be able to see a much higher growth rate, both on the revenue and on the margin front.

Moderator: The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: On the hospital business, ma'am, you mentioned 100 basis points margin expansion over the next three or four quarters. Plus, the fact that it see

ms like increasing

insurance penetration could be a headwind for ARPOB. Would this margin expansion essentially depend on our ability to improve occupancy from here, which we usually do see given first quarter is seasonally slow? Is that the right way to sort of look at the margin expansion that you have talked about?

Suneeta Reddy: No, I think there are three levers for margin expansion. The first, of course, is payer mix, and we have spoken about that and how insurance has really helped us in this sense, contributing to not only volumes but margins. And the second is, over a period of time, we will be focusing on international and therefore better ARPOB. The third, of course, is case mix, where this quarter, you know, there was less of surgical volumes and electives, and we believe that in the next three quarters, we will see an improvement coming from that. And fourth, there is a tariff distribution of 4% that will play out within the next 4 quarters. So, all of this combined will give you an improved ARPOB, we are looking at an ARPOB increase of 7% for this year.

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A Krishnan: Another point that we have to let you know is, you know, ARPOB is not a headwind for insurance. That's something that I think it's important that I should correct you here. Clearly, the average revenue per patient that we have from an insurance is equal to or higher than the average revenue per patient that we have from walk-ins. The reason being some of the insurance patients opt for higher category of rooms, as we have said in the past. ARPOB is just a derived number, which is a combination of both occupancies and ALOS. So, I think from the perspective of volume, you know, you are seeing the volume growth and if it's volume into value, we are not going to be seeing any deterioration in our revenue pick up because of insurance. On the contrary, it will only be positive. So, the ARPOB has to be seen in the context of medical, case mix etc. It's not because of insurance. It's not a head wind for term insurance.

Neha Manpuria: So, in this quarter the lower ARPOB is essentially because of the lower surgical mix.

A Krishnan: Yes, you can say that, or you would rather say that better higher medical mix. It's how you put it.

Suneeta Reddy: Yes, this quarter was higher medical mix.

Neha Manpuria: And my second question is on the 24/7. It seems like our GMV is sort of on a sequential basis on a very slow growth. I understand we have tweaked the discount rates and that's probably impacting the GMV momentum quarter-on-quarter. How should we think about balancing how we get back GMV growth with the target of achieving breakeven? Because I thought you also mentioned in your opening remarks that we need to see GMV growth to achieve that breakeven. So, just trying

to understand how we will balance both and what we are trying to do to improve that GMV traction.

Madhivanan B: See, even in the last quarter we have spoken about how our operating model was changing. The earlier GMV growth was being substantially driven by a very high percentage of marketing spends in acquiring new customers. Over the last two quarters, we have changed that operating model from a pure digital kind of an origination to a much stronger omni approach. We have studied it very well and we realized that this works very, impacting even in multiple geographies across the world. Our omni model is a very, very viable model for both our offline business as well as for online business. So, this transition is in phase.

I just wanted to say that

our number of new customers is slowly increasing. Give us around another quarter or so for us to get back into a growth path. It's not that we have de-grown, but our growth rate has been much in the range of maybe around 4% to 5%. You would expect it to be much bigger. You will see that happening in the next two quarters. In terms of initiatives, we are working very strongly on our loyalty program because this is what again drives GMV in a big way. So, a common loyalty program between both the offline pharmacies and the digital pharmacies is contributing substantially well and our revenues from customers who operate on both sides is doing well. So, I would ask for forbearance over the next, maybe this quarter and the next quarter before the growth story starts picking up in a sustainable manner.

Neha Manpuria: So, more like fourth quarter of fiscal '25 is when we should see a step up in growth?

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Madhivanan B: I can actually say maybe even in Q3, but yes, to be on the safe side, Q4, but at the same time, even the operating expenses are being kept in control. Six quarter profitability, we should be on course.

Neha Manpuria: And what about the new businesses, sir? I think we talked about that also in the last quarter, the new areas that we are focusing on. Is that gaining traction?

Madhivanan B: Yes, so two things. From a regulatory perspective, one of the big initiatives is going to be the insurance business, primarily health and term. But we are in the process of getting our approval from IRDAI. We expect it to happen by October. Till that time, the approach is more of a monetization module. So, we have been experimenting with multiple journeys. The moment we get the corporate agency, we should be able to launch the business in a very full-fledged manner. And the difference between people who sell insurance in the rest of the industry versus us would be, it would be incorporated within our own existing customers, as well as within the incremental flow. And we expect this to be, our cost of acquisition to be much lower, and the story should play out. So, insurance expects the licenses to come by October. And therefore, the last quarter, you will start seeing some good traction across two to three products. And on the digital therapeutics, we have had a bit of a change in the model. We are working very closely with both the hospitals and the clinics, as against doing it purely digitally and health facilities. So, here again, the omni approach will be picked up. So, we are doing one or two experiments, and we expect that business es also to pick up. So, in cross-collaboration within AHEL, AHLL, and AHL, we work together. So, these are the two businesses that we are focusing. One more channel which we are going to be pushing, which will help us give us a GMV, is the corporate channel. Again, we are working in a very synergistic model with the hospitals group and the clinic group, and finding out how we can be the digital front end to maximize corporate business. So, all the three areas, corporate, insurance, and digital therapeutics, within the group will drive value for the digital front end.

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Just continuing on the GMV growth, I think we had earlier put out the guidance of around 50% growth. And since we are now expecting the growth to start from quarter 3, quarter 4, is there a revised guidance that we are putting out for this GMV for 24

17?

Madhivanan B: No, we are not putting. We are on core. It's just that, see, originally, we were expecting that Q2 closures will happen, maybe a month, year or so. So, we are very much on. So, we are not revising the guidance from our perspective. And if there is any other input you have, both in terms of our operational profitability as well as we are on course of whatever we have been committed. Sanjiv, if there is anything else you want to add, then please.

Sanjiv Gupta: You know, I think you said it right. At this stage, we do not believe that we could be

revising the numbers. I am reminded of the fact that we talked about 50% growth, over 2,700 of growth of GMV that we did in the last fiscal year, which makes it to about 4,000 crore. I think at this stage, at least all of us strongly believe that there is no reason for us to revise the numbers.

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Kunal Dhamesha : Which line of business do you expect to ramp up so fast out of two, three lines that we have now, which would drive the growth? Is it still the pharma delivery would drive growth, or any other line of business?

Madhivanan B: No, so pharmacy continues to be our flagship. So, in fact, the last month, we did one of the highest numbers ever. So, that's why we feel the trajectory is back. While overall, the industry from the digital side did not grow up, but overall pharmacy delivery is increasing. The online part of it has not been growing in a big way. Because I think all players, we have been tracking I think the discounts and trying to build a sustainable model with breakeven being one of the primary targets. So, pharmacy will continue to grow. Like I told you, both the corporate side of the business and the new customer acquisition will continue in a hybrid model. The second big area where we are seeing a very nice lift is our consult business. From an average of around 100,000 consults, which we used to do, the OPD consults, around the last quarter, this quarter we have been clocking anywhere in the range of 150,000 to 170,000 consults. What this results in is a feeder into both the pharma business as well as the diagnostic business. Because it's the recommendation that drives the story. And so, these two businesses are also seeing an uptick. So, diagnostic business being seasonal, like Sriram highlighted, we again clocked very good numbers last month. And we expect that numbers to continuously keep growing forward as well. So, primarily driven by pharma, supported by consults, and diagnostic following in its way.

Kunal Dhamesha : And one on the hospital business, the volume growth is good this quarter. And it has also led to a peak in occupancy. But the same has not played out in terms of operating leverage, in terms of profitability. And ARPOB has also been much slower, which we expect to grow faster now. And we have taken an ASP. So two, three questions here. One is the ASP hike of around 4% with the State. That will be across channels, or is it just a cash patient or insurance patient? And why has the higher occupancy not given higher profitability?

Suneeta Reddy: I think the higher occupancy not driven higher profitability will start playing out in the next few quarters. Like we said, we have absorbed a lot of costs. The second thing that we had mentioned is that we had a lot of medical admissions this quarter, and therefore, medical admissions resulted in an increase of ARPOB of only 2%. So, moving forward, we think that you will see the benefit of operating leverage.

A. Krishnan:

And we did see the EBITDA margins. If you are looking at Q1 over Q1, just to remind you, if you look at Q4 of last year, our EBITDA margins was at 23.1%. That is because we had added doctors and marketing costs. So, from the 23.1%, we are seeing it come to 23.6%. So, from a quarter over quarter basis, there is a 0.5% improvement in EBITDA margin that we have seen. This is because of this operating leverage. You should, as we said, we are looking at a 100-basis point increase, which should come over the next few quarters. We had said last quarter that we had seen some increase in our IT costs, because we have increased at least 0.25% to 0.3% has got impacted because of higher IT costs and cyber security, etc., which we have invested in. All of that has now gotten in the number that you have; it has got absorbed. So, going forward, you should see that happen as well.

Madhu Sasidhar: So, if I could just give some color to what AK said. We added about 102 doctors in the last quarter in Q1 that we brought on board with a very negligible attrition of about

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3 or 4 doctors. The quarter prior to that, we added 84 doctors. So, this has been a very active time of medical team expansion. Many of those doctors are starting to get more and more productive. So, as Ms. Suneeta said, we expect that operating margin to improve over the next few quarters.

Kunal Dhamesha : Sir, is this hiring happening at a much higher level in terms of senior doctors, etc., because we have a base of 5,500 full-time doctors, right? So, adding 100, 200 should not move our cost line items too much.

Madhu Sasidhar: No, so you are right. So, we are recruiting some of these doctors who are star doctors and leading providers in their space. These are not, I am not giving you junior doctor

recruitment that we don't count that in the consultant recruitment numbers.

A. Krishnan: There is an incremental 50 crore difference in the doctor cost.

Kunal Dhamesha : 180 doctor that we have recruited ?

Madhu Sasidhar: Quarter to quarter. Same period last year, there is a difference of 50 crore.

Kunal Dhamesha : And 50 crore is quarterly, so around 200 crore is the annual.

A. Krishnan: And you should look at the volume growth, right. I think it's important to look at margin growth, which we have said, but I think the important thing for us is the volume growth, which is something that we are very happy about. And the volume growth is more sustainable, and it can keep, and it can, it will definitely, the margin through operating leverage as well going forward.

Madhu Sasidhar: And the volume growth has you mentioned, we have driven that volume growth very intentional. We have been intentional about the markets that we have driven that volume growth in. So, that's why we believe it is sustainable as well.

Moderator: The next question is from the land of Madhav Marda from Fidelity.

Madhav Marda : I just had one question on the Apollo 24/7. So, if you look at the pre-OPEX EBITDA margin at 13.6%, which is reported in Q1, that seems to be moving up steadily through the past few quarters. Should we think that this number kind of has more legs to improve as we go through rest of FY25 and if you could help us understand what are the key levers for that number moving up?

Sanjiv Gupta: So, two things . One is that our mix on the pharmacy side is better versus the last year. It is at about 35% to 36% versus 32% same quarter last year. That helps in this margin accretion, which is at 13.6% versus 10.9% same quarter

last year. So, we

expect this to continue to move upwards. Would it be 25%, can't say at this stage, but we are looking at to hit a number closer to 20% in next 1 to 2 quarters.

And apart from pharmacy, as we move into new verticals, as Mr. Madhivanan talked about that we get into insurance, we get into digital therapeutics, along with the hospitals, these are the two and app monetization, which we just started. So, these are very, very margin accretive to the business and given it is a technology -driven services from that sense, the entire margin goes back to the EBITDA. So, I believe those verticals, those new segments will also add on to this. So, I think we should

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expect in next one to two quarters to hit this to around 18% to 20% and while pharmacy will continue to support this margin .

Madhav Marda : You are saying that 38.6% has legs to move closer to 20% this year itself as it moves through the rest of the year?

Sanjiv Gupta: Let's see that in next 2 to 3 quarters how it pans out, but the goal post is to hit 20% by this year end.

Madhav Marda : And just on the OPEX cost as well, it's at about Rs. 150 crore and we know we controlled it a fair bit. So, should we expect this to stay in this range for the rest of the year?

Sanjiv Gupta: I think operating expenses at current level would stay for a little depending upon the investment that we are doing into the new segment, and we should expect a little bit of further cost optimization to the current levels of Rs. 130 crore depending upon the investment that we require in the new segment and how much time do they take to start giving positive EBITDA . So, that is how I would see that some reduction , but there will be a little bit of investment into the new segment also.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal .

Tushar Manudhane: Sir, if I look at the region wise ARPO B growth , like except north , most of the other regions have been muted or in fact decline in the ARPOB . So, is this sort of peaking - out in terms of ARPOB growth and so not just at Apollo but maybe if you could help us understand at the industry level, and then so subsequently the growth would be more driven by the volume as reflected in this quarter ?

A. Krishnan: So, two things , right. From our side, we would still like to believe that the inflation of 4% to 5% is something that we will continue to do and hence that will start reflecting clearly. The other thing is the function of occupancy as well as ALOS as I said. So, here when we are getting , when we are focusing on higher volume growth etc. across and into using that to get secondary care cases as well as higher oncology , some of that reflects in a bit of a lower ARPOB because oncology has been seeing a stellar growth. We are the largest oncology operator in the country. It will help us get margins , but ARPOB may not reflect the same because if you look at a chemotherapy , the chemotherapy ARPOB is lesser than the ARPOB of hospital of our surgical etc. But what happens is the flow through is visible for us. So, you know , ARPOB should not be looked at from the perspective of margin expansion. It's only

again I am saying that it is something that you should understand this more as a lever , but if you look at the volume growth and the value growth so long as it's there and if you are seeing the volume growth, we have always guided that we should be looking at mid-teen kind of overall revenue growth so long as that is visible, and we should see the margins exp

and.

Suneeta Reddy: I think it is important to recognize that in metro s, our ARPOB is very healthy averaging around Rs. 70,000. So, clearly , it is because we also have tier 2 and some tier 3 that you are seeing the average ARPOB which is around Rs. 53,000. So, clearly , there is room for expansion, and we will continue to look at that. But please understand that this is an average of tier 1, metros and tier 2.

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Tushar Manudhane: And from a bed addition perspective given the way the projects are going to pan out, it seems next four quarters at least we will have to have improvement in the occupancy on the existing beds and then subsequently the additional beds would help drive growth. Is my understanding correct?

A. Krishnan: That is correct.

Suneeta Reddy: That is correct.

A. Krishnan: And next year we are seeing good additions in some of our existing markets also like Calcutta and Gurugram also in some way for us is an existing market , right? Because we have Delhi , which is not getting consolidated , but we know Gurugram very well and that will also come next year. So, we are quite, I think next year would see us getting good number of bed addition.

Tushar Manudhane: So, directionally , while we are improving the profitability at the existing sites, but these OPEX coming on in FY26, so if you could just help understand how the profitability would look in hospital business for FY26?

A. Krishnan: We have a good big base of EBITDA now. So, that is a very important thing that you should look at because if you look at by end of the year, we should have significantly large EBITDA base and hopefully by that time, we should have also seen the 100-bps expansion in margin that we have been guiding to have gotten into our books. From there on, given that we are looking at relatively, we do not think that the losses will be significant. So, we would think that 100 to 150 bps lower from that level since the maximum that we will see the EBITDA margin fall.

Tushar Manudhane: And on the offline pharmacy side, like we have added 44 stores. So, this year probably what could be the run rate of store additions?

Obul Reddy: It will be around 500 to 550 this year as well as planned. The Q1 is lower because of the elections and other delays. Overall , it will be 500. We will be on track for that.

A. Krishnan: Q2 has already started to ramp up well.

Tushar Manudhane: And any scope of improving the margins for offline pharmacies from like 7.6%?

Obul Reddy: This is at the back -end Apollo Pharmacy distribution level that margin , that is a structured margin. As you know, Apollo HealthCo supplies only for Apollo Pharmacy's front-end business. So, it has a fixed retail margin to be retained at this level. So, that if you notice, last 8 quarters operated within a range of 25 basis points as we see the margin expands in the front-end.

Moderator: The next question is from the line of Abhinav Ganeshan from SBI Pension Funds .

Abhinav Ganeshan: I just had two very brief questions. First one was your occupancy has gone up 600 basis points year-on-year. So, can we assume similar kind of occupancy for the rest of the year?

Suneeta Reddy: Yes, we can. If not, improve it.

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Abhinav Ganeshan: That is really helpful. Second thing is, as a part of your opening remarks, you had talked about expanding in few locations like Pune and Gurgaon. W

hich are the other

locations that you had spoken about?

Suneeta Reddy: So, the first one is going to be Gurgaon. Then comes Hyderabad. Then will be Kolkata and Pune. These are the four that we are focusing on. We also have brownfield expansion in Mysore, which is 140 beds.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Just want to understand like what was the contribution from international patients during the quarter and what kind of impact you are expecting due to change in Bangladesh, which I suppose is one of the largest markets for you in terms of

international business contribution?

Madhu Sasidhar: Yes. So, Bangladesh is about 30% in terms of our international, but as a percentage of our total revenue is about 2%. We have seen some throughout the first quarter and into this quarter as a combination of elections and the more recent political issues, some drop in volume. We are hopeful that this will come back very quickly. We are just monitoring the situation very closely.

Damayanti Kerai: So, very broadly, this 8% to 10% kind of contribution from international patient is very much there for you.

Madhu Sasidhar: Yes. And there are other markets that we are looking at as well. So, over the past year to two years, we have been actively exploring other international markets as well, which will continue to expand during this time.

Damayanti Kerai: And one question on your volume pickup, which we have seen during the quarter. So, I understand like your efforts over the last few quarters is giving this strong volume pickup. But when I look at AP, Telangana and Karnataka, there is very strong pickup in the OP volume part. So, can you explain if there is some seasonal element there or like I guess, very strong, around 50% for Karnataka and then 40% for AP, Telangana in terms of OP volume pickup?

A. Krishnan: So, these volumes are mostly on-site health checkup volumes in the outpatient volumes, which has come in. Because we have been doing a lot of, as we said, as part of our insurance penetration, etc., that we have been planning, there has been a lot of corporate outreach or efforts that we have been making. And you know so, one of the corporate outreach has also been health checks. And we have seen good increase in some of those health checks in the corporate. So, that results in newer registrations into our system, which has been something that we have been focusing last year on both Hyderabad as well as Bangalore and that whole region, which is why you are seeing a higher increase in the outpatient volumes there.

Damayanti Kerai: So, this could be feeding to your IP volume pickup, right? So, what kind of IP growth you are expecting, say, for this year? 11% in Q1 definitely a very strong number to look at but for full year, what are your expectations?

A. Krishnan: Yes. So, this sort of primary care outreach does feed into our inpatient volume. If you were to aggregate all of it together, roughly about 4% across the board is an inpatient

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conversion number that we see. So, we will continue this level of outreach because, as you said, it is accretive to both our diagnostic volume, outpatient diagnostic volume, as well as our inpatient volume. So, we wouldn't be able to guide you for the overall volume growth, but I think we have said that we should be seeing good volume growth for this year. This year's focus is volume growth, and we are continuing to focus on that.

Damayanti Kerai: And my last

st question is clarification on your EBITDA margins for next year. So, with four units coming on board, did you mention from the level which say, like, we will end up in FY25, there will be 100 to 150 basis point loss at max because of the new beds coming in?

A. Krishnan: So existing should ideally improve, as we said, 100 bps in this quarter and this year by next three quarters, and hopefully go up a bit more in the next year. And then there will be some reduction because of the new units coming by 100 to 150 bps.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just going back to the platform GMV growth of 9%, right, the Pharma AOV has grown 15%. So, is it the other businesses like consultancy, right, or diagnostics that have declined? So, what explains the slower growth?

Sanjiv Gupta: Yes, while the exact numbers of the pharmacy have a lot of value or grew which is 1,072 versus 935 a year back, I think Q1 has always been a slight, we have seen that IPUP transactions also fall down from the online side a little bit during Q1, but I think especially diagnostic and the other side of the business is concerned, it was okay. So, I think the drop is only with respect to the other side of the business, but otherwise, yes, and that is the reason you see a muted growth of about 9% in this quarter versus the last year.

Shyam Srinivasan: And Sanjiv, what gives us the confidence that we can do 50%, 60%, 70%, 80% percent in the remainder of the year?

Sanjiv Gupta: I think, see, this is what Mr. Madhivanan also talked about that one is that we strongly believe that the change of the model, which resulted into moving to an omni acquisition of customers, a lower discount that is happening, these two things had an impact on our pharmacy for a good 5 to 6 months in the past, but as we see July, numbers have started picking up and we believe that it was this change in the

operating model , you know whatever the impact , that will impact only till current quarter end or maybe some bit in the next quarter . So, this is one point . So, pharmacy will start picking it up as we move forward .

Second thing is that we also have play on the corporate and partnerships , which is yet to kick in. As you understand that, typically these partnerships take a little bit more time to fructify. So, I guess , some bit of growth will also happen in Q3 and so that is the second element . And apart from the fact that we have got the insurance and other segments to also kick in and Q1 has always been a muted quarter for us even in the last fiscal year also versus prior to that. So, I think we still have a confidence that the 50% growth over the previous year's numbers is what we look at in the current year. And I think at this stage , as we also gave the answer to one of the questions on revising the guidance , if there is any, I don't think there is any worry

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at this stage. We have got many initiatives which are working in parallel , and we strongly believe that we come back on the growth side.

Shyam Srinivasan: And lastly on this one, the EBITDA margin for the 24/7, 4 to 6 quarter even just on that piece ; I mean that Apollo 24/7, there was a breakeven guidance of 4 to 6 quarters that was announced I think last quarter or maybe before. Are we sticking to just that piece also breaking even?

Sanjiv Gupta: Yes, we are sticking to that guidance and if you notice in the past also, when prior to the digital segment , which we call it Apollo 24/7, we had suggested

that the entire

Apollo HealthCo will get breakeven in Q4 FY24 which happened in Q3, one quarter in advance. And similar to that, we also gave the guidance in Q4 that digital business will get breakeven in 6 to 7 quarters. We are sticking to our promise . The entire management team and down below is working towards that common goal. So, at this stage , I think all of us need to believe to this point that digital segment will also break even in next 6 to 7 quarters.

Madhivanan B: So, if I can come in for just 30 seconds, see, we are reasonably confident that the combination of growth in a calibrated way along with the breakeven goals that we have sort of given guidance to, we will stick to it. And that's where , and effectively, we are moving away from, like I told you, I am just reinforcing it once again , from a very high cost of customer acquisition model into a much more stronger omni - channel combined with loyalty. So, that is what will facilitate growth, not at an exorbitant operating expense model. So, both on the EBITDA side as well as on the growth side, we are reasonably on course , and we expect this particular change to time, and we are seeing the positive result. So, we stick to our analysis.

Shyam Srinivasan: My last question is just on the diagnostic piece, while our network has grown 16%, our topline has grown 8%, you mentioned in the remarks about seasonality , but seasonality Y-o-Y should not matter , I presume. But anyway, if you could just tell me what happened for the slowdown, if I were to look at industry peers, we have seen them grow between 12% and 14% and despite the expansion , we seem to be growing slower.

Ashish Maheshwari: In Q4, we had a few one-off gains , which led to a margin expansion as well and had improvement in margin which was earlier . Two of those Q4 one-off gains were related to one, of course , being the year-end closing. We had a reversal due to any kind of turnover related discounts that were factored during the course of the year, and once the year-end closing happened and when we knew the final numbers , then the margin accretion and the improvements and the reversals had an impact. That was approximately around Rs. 2.6 to Rs. 3 crore , number one.

Number two, we also had an impact due to the year-end reversal on account of any kind of leave encashment and PLI, which is on the accounting and the Ind AS side, because during the course of the year, the accruals are based on the current manpower trends , and at the end of the year, there is actually a valuation that takes place. That impacted to almost like Rs. 1.5 odd crore . In case if we factor that in, our margins were almost in similar lines.

Secondly, the seasonality factor , which Sriram also earlier said during the call, in during Q1, we have made some new expansions wherein the revenue kind of

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potential that we expect , the costs are more versus the revenues are not quite there in the Q1 which we expect to come in Q2 onwards and hence the overall impact is also factored in Q1 which was not there in Q4. And this is the prime reason why we

have had the decline compared to the last quarter .

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie Group .

Kunal Dhamesha : Just one on the ARPOB growth, just trying to understand a bit more. So, we are saying that the medical mix was higher in this quarter , but then quarter 2, quarter 3 again is a higher medical mix because of the overall monsoon seasonality , right? So, just trying to understand that how c

onfident are we in achieving the 7% ARPOB

growth ? And secondly , the inflation hike of around 4%-5%. Is it in a particular channel? Is it for cash patient or is it for insurance patient? How should we think about it? Is it also hospital focused more in metro or non-metro? That would be helpful.

A. Krishnan: I think , I would again want to repeat. I think maybe I am happy to do one call outside.

ARPOB is not as much a lever for our EBITDA growth etc., because average revenue per inpatient is gone by 4%, inpatient volumes have gone by 10%. Volume into value equal to revenue growth. ARPOB is just the perspective of how each of our beds, you know what the revenue per occupied bed is . And so long as we get incremental volumes and if the ARPOB of that incremental volume is a bit lower, it shouldn't really matter for us. And that is exactly what I said when we are penetrating into corporate s, you are penetrating into insurance, penetrating and getting more chemotherapy volumes due to oncology etc., some of that ARPOB is, even if the ARPOB is bit lower, it shouldn't matter for us. So, the focus is not on ARPOB. ARPOB is just a derived number. Let's see how it goes during the year. It will be higher than the current levels, and I don't want to guide specifically for that 7% either, though we know that that is something that we will get over a period of time. So, this year, I think it is a volume driven growth, and we should continue to focus on that.

Kunal Dhamesha : And one, when I look at the slightly larger picture across our segment, I think there has been talks about getting corporate business, large accounts etc. So, is it fair to say that there has been some incremental allocation towards getting that B2B business and would it have any impact on our cash patient over a longer end? Because this B2B business in any industry generally comes at a lower profitability , right? While it may increase the volume initially , but then it can actually counter the cash patients etc. So, how should we think about this corporate piece which we have been talking across hospital business as well as online business as well?

A. Krishnan: When we say corporate , I think it's important that you understand that when we talk of corporate or private corporate for that matter , it is coming , what increases for us is the insurance business . People covered through private insurance , as you know , that's a market which is growing by 35%. We are seeing significant penetration in the overall coverages . And when we say corporate, we are talking of large corporates like HDFC, ICICI , the likes of all the big fours etc., and the IT companies all of them are covered through private insurance . But the outreach through corporates is more something that we outreach to the employees of the corporate. The employees then come into our systems through their insurance coverages . So, we don't see any risk on this at all. To that contrary , as I said, the average revenue per patient on an insurance patient is higher than some of the average revenue per patient on walk-

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ins today because walk-ins also, because they are conscious about paying out-of-pocket , so they select for the lower bed categories like general ward , sharing etc. But mostly , when it comes to insurance and private corporate s, we see them opting for single rooms and higher . So, I don't think that's a worry at all to these . As I said to the contrary , average revenue per patient on insurance is higher . So, this is not a corporate tha

t you should think like in the other businesses . This is a corporate which is getting channelized through insurance.

Moderator: The next follow -up question is from line of Damayanti Kerai from HSBC.

Damayanti Kerai: My question is on 24/7. So, you mentioned more cost-effective ways of, say, marketing etc., has helped you to improve profitability and lower customer acquisition cost is one of such cost. So, can you quantify bit like what is the current customer acquisition cost for you compared to, say, a year back , and how different is from your next competitor ?

Madhivanan B: So, let me highlight it from two perspectives . We used to spend anywhere in the range of Rs. 150 to Rs. 180 crore in terms of marketing expenses alone , which included customer acquisition through CRM , through prepaid, through Google etc., etc. That number , as we speak , we have brought it down to around Rs. 80 crore both

in plan, and we are, in fact, doing much better than we plan. So, on a cost for acquisition, it depends on the different kinds of customers. And today, the new opportunities that we look at, which is through our omni-channel etc., the cost can be as low as 250 bucks and can also go to the range of around 300, 400 bucks. So, we are operating in that range. So, that is one thing. But typically, what happens when you go through a digital sourcing mechanism is, you virtually pay for the same customer again and again. In this case, we do the combination of originating customers through Omni, attaching what we call as a subscription program called Circle, which ensures that we have continuity. So, both these combinations have given us very, very good numbers and this advantage, I believe, is only available to the Apollo Group because we already have 6,000 outlets, we have hospital. So, we have multiple sourcing points. And what we are realizing is, customer in today's world, no customer is 100% digital. It's always hybrid, omni, and so that is what is helping us. Compared to this, some of the other competitors are primarily, in my mind, are focusing more on the digital acquisition continuously. So, I think if you can get this model right, we have enough of leg room for growth and at a reasonable calibrated cost without impacting our cost.

The second part of it is we are exploring delivery. It is sort of combat, I would not say sort of combat, but to ensure that we are able to take care of that acute customers which is people who buy in an emergency, we are experimenting with different models of delivering our medicines within a much faster time frame for which we will be charging. So, that's another experiment which is also bringing us new customers in certain specific zone. So, between both these put together, our cost of acquisition as well as cost per order on an ongoing basis is showing a downward trend.

Damayanti Kerai: And my second question is also if you can update us on what kind of discount you are offering as of now on your pharmacy online, offline channel and compared to industry, like where do you stand?

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Sanjiv Gupta: So, on the online side, we have anything between 13% to 13.5% as a discount which happened in the last quarter and if I compare it against the industry standards, industry standards could be maybe 150 to 200 bps higher than what we offer at this stage, but there also they have lot of cuts for the new customers and the repeat customers. But we strongly believe that, as always, since last three, four years, we always have been on the lesser side

versus the competition. On the offline side, discounts are very much close to 11%.

Damayanti Kerai: So, somewhere on a blended basis, somewhere 11%, 12% is a discount from your Pharmacy segment.

Sanjiv Gupta: Yes, that's right.

Moderator: Thank you very much. I now hand the conference over to the management for closing comments.

Suneeta Reddy: So, thank you, ladies and gentlemen, for joining this call. As we conclude this discussion on Q1 FY25, we are optimistic about the trajectory that we have set across all business verticals. With clear strategies in place for revenue growth, profitability enhancement and operational excellence, Apollo Hospitals is better positioned than ever to deliver unmatched value to our patients and our stakeholders. A comprehensive and integrated health care offerings supported by a robust pipeline of initiative and expansion will place us at the forefront of the industry. We appreciate your continued interest and support, and we look forward to sharing further progress with you in the coming quarters. Thank you, and good afternoon.

Moderator: Thank you very much. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

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Call: Nov 2024

(no extractable text — likely a scanned image PDF)

Call: Feb 2025

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Apollo Hospitals Enterprise Limited
Transcript of Q 3 FY25 Earnings Conference Call
February 11, 2025

Moderator : Ladies and gentlemen, good day, and welcome to Apollo Hospitals Limited Q3 FY 25 earnings conference call. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you, sir.

Mayank Vaswani : Good afternoon, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals, to discuss the financial results for the Q3 FY25, which were announced yesterday.

We have with us today the senior management team, represented by Mrs. Suneeta Reddy – Managing Director; Mr. A. Krishnan – Group CFO; Dr. Madhu Sasidhar – President and CEO of the Hospitals Division; Mr. Madhivanan Balakrishnan – CEO of Apollo HealthCo Ltd.; Mr. Sriram Iyer – CEO of AHLL; Mr. Sanjiv Gupta – CFO of Apollo HealthCo Ltd.; and Mr. Obul Reddy – CFO of the Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks, and uncertainties on Slide #2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy : Good afternoon, everyone, and thank you for taking time to join this earnings call. I trust all of you have received our earnings documents, which we shared yesterday.

We are pleased to report the strong performance in the Q3 FY25. Our results reflect an all-round growth with all 3 business segments reporting mid-teen revenue growth. This has been accompanied by clear progress across key operating metrics, and a sharp rise in profitability. While we continue our strategy to focus on volume growth, we have strengthened our focus on high-end specialties, such as Cardiac Sciences, Oncology, Neurosciences, which augurs well for our overall revenue intensity as well as growth in margin profile for the future.

Our efforts to provide tailored solutions to diverse patient needs has enabled us to create value across all segments. We have been consistently investing in medical

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advancements across specialties ahead of peers, which provides us with a clinical platform and differentiation across specialties and will enable us to sustain our growth momentum as we build out our new facility over the next 12 months.

Our Healthcare Services business has delivered a strong 13% year-on-year revenue growth to INR 2,785 crore, sustained operational progress driven by favorable trends in centers of excellence, case mix and payer mix has helped partially to offset seasonal trends and headwinds. Revenue from cash and insurance patients saw a year-on-year increase of 15%. Collectively, these segments accounted for 83% of inpatient total revenue.

These outcomes reflect the success of our strategic efforts to optimize our payer mix. We would like to emphasize that this performance is despite a seasonally muted nature of the quarter as well as

a decline in footfalls from Bangladesh. Revenue from international patients other than Bangladesh rose by 19% year-on-year, while the Bangladesh drop has resulted in an overall 1.5% drop in revenues.

While IP volumes grew by 5% year-on-year, our focus on specialties, Cardiac, Oncology, Neurosciences, Gastro Sciences and Orthopedics, represented by the acronym CONGO, grew at more than double the pace. Volumes grew at 10.35% with revenues in Conco growing by 25%, Neurosciences 23%; Gastro 20%. Overall revenue growth at 17% in the Specialty segment. This has been achieved through

strategic clinician recruitment and clinical program development in key markets. Group-wide occupancy stood at 68% in Q3 FY25 compared to 66% in Q3 FY 24. Overall, ARPOB grew by 8% year-on-year, reaching INR 60,839. We believe key levers such as high surgical volumes, and enhanced clinical case mix and improved payer mix will continue to drive ARPOB growth in the future.

Our financial results; our consolidated revenue grew by 14% on a year-on-year basis to INR 5,527 crore, consolidated EBITDA was at INR 762 crore, registering an increase of 24% year-on-year.

Within this, the Healthcare Services EBITDA was at INR 671 crore, a growth of 14% year-on-year. Healthcare Services margins remained robust at 24.1%. Revenues from Apollo HealthCo were INR 2,352 crore in Q3, growing at 15% year-on-year. Revenues from Apollo Health & Lifestyle also grew by 15% year-on-year to INR 390 crore in Q3 FY 25.

Apollo Health Co reported its first ever quarterly profit in Q2, has further strengthened its performance, delivering a PAT of INR 32 crore in Q3 against a loss of INR 28 crore in the same quarter last year. Meanwhile, AHLL has maintained its strong year-on-year growth momentum with all growth drivers aligned. We have delivered a 52% year-on-year increase in consolidated PAT, reinforcing the significant potential of our integrated offerings. The Offline Pharmacy Distribution business in Apollo HealthCo recorded an EBITDA of INR 159 crore, representing a year-on-year growth of 19%. The Online Pharmacy Distribution and Digital business in Apollo HealthCo recorded an EBITDA of INR 38 crore, excluding 24/7 operating costs, representing a growth of 51%. 24/7 operating costs were at INR 141 crore. Apollo HealthCo has therefore

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reported an EBITDA of INR 57 crore in Q3 FY 25, registering a strong improvement from INR 2 crore in Q3 FY 24.

AHLL recorded an EBITDA of INR 34 crore, delivering a 32% year-on-year growth, and an improved margin of 8.8%, compared to 7.7% in Q3 last year. Consolidated PAT was INR 372 crore, growing 52% year-on-year.

Within the Healthcare Services business, we have delivered a ROCE of 29% with balanced ROCEs across all geographies, the metro, the Tier-1 and the Tier-2 cities. Private label and generics revenues were at 16.4% of total pharmacy revenue. Our digital platform 24/7 added 2 million new users. The platform GMV was at INR 760 crore, representing a growth of 11% over the same period in the previous year. The number of daily active users has grown by 25% over the same period last year.

As we reflect on our progress through Q3 FY 25, we are encouraged by the strong momentum and the strategic initiatives shaping our growth journey. Our unwavering commitment to clinical excellence, expansion on high-growth areas, continued innovation and patient care, solidify Apollo Hospital's leadership in the health care sector.

With new facilities coming online in the coming year and ongoing infrastructure enhancement, we are well positioned to capitalize on emerging market opportunities. Alongside these efforts, we remain focused on optimizing operational efficiency and enhancing profitability

to drive sustainable long-term growth.

On that note, I would like to hand it over to the moderator and open the line for questions. I have Krishnan – our CEO; Madhu Sasidhar – CEO of the Hospitals Division; Sriram Iyer – CEO of Apollo Health and Lifestyle; Madhivanan – CEO of Apollo HealthCo; Obul Reddy and Sanjiv from Apollo HealthCo with me to take all your questions. Thank you.

Moderator: We will now begin the question-and-answer session. Our first question comes from the line of Binay from Morgan Stanley.

Binay Singh: Good set of numbers across our businesses. Broadly, I had 3 questions. Firstly, on 24/7, now we are also adding insurance to the platform. So, what kind of a GMV growth are we expecting? Secondly, on the hospital, it's a busy FY 26 for us. If you could talk a little bit about how is the commissioning scheduled, which quarter should we expect what? And lastly, just on the Microsoft partnership that we've announced. Earlier also, we had a partnership on the Cardio side. It seems the scope of this partnership is wider. Which parts of the business do we expect benefits or positives to be seen first?

M. Balakrishnan: So, thank you for the question. We are a bit early at this point of time as far as our insurance foray is concerned. We've got all the necessary approvals from IRDAI, such as the entire corporate agency license, and certain other securities/digital clearances. As of now, our GMV is around INR 3.5 crore for the full quarter, because we have

been focusing purely as a marketing setup, and the product is only group health insurance. In this quarter, we will be having at least 3 life insurance companies and 3 health insurance companies, which will get enabled on our Apollo 24/7 platform. And from April 1 onwards, we expect the numbers to start building up. So, the number of

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planning as far as insurance is concerned in terms of GMV is still under process. We will be able to give you the exact numbers around March. So, just bear with us in this quarter, but all the necessary formalities are done, and we should be able to take it up. More than the GMV, this has helped our margin expansion in a much bigger way, because insurance as a business, because we'll be focusing on retail, health insurance and retail term. The margin percentages in these kind of products is reasonably large.

Binay Singh : And any commentary on the outlook on future revenue growth or GMV growth for this segment?

M. Balakrishnan : For the insurance alone?

Binay Singh : For overall, 24/7.

M. Balakrishnan : Okay, so, I might as well handle it right now. So, if you remember our last 2 quarters' articulation, our emphasis has been on trying to get a good quality of business. What I meant by that, and this is primarily from the Pharmacy side, e-Pharmacy side, we were always dependent on a very marketing spend driven kind of a growth, which was coming from digital marketing. However, the digital engines are extremely stringent in terms of remarketing abilities, thereby making the cost of acquisition for a customer for Pharmacy, very highly prohibitive. So, over the last 2 quarters, we changed our operating model and as we speak now, our emphasis is on actually acquiring customers who will be with us for a longer period of time, examples, chronic, moms and babies, where we get much stronger recurring revenue. That's what I mean by quality of the business, number one.

Number two, there is a much larger synergy between the Offline business and the Online business, which is resulting in much lower cost of acquisition. We call it the omni business. Over the last 2 quarters, we have seen the traction going up. So, on one hand, our paid marketing has come down. Other hand, our omni customer is

slowly building up.

Third, the Circle program, which I spoke to you in Q1, at the end of Q1 report, is really beaming up pretty strongly. On the offline, the trajectory rate is very high. We have almost 5 million customers who are paid for a Circle Loyalty Program, and they are bringing. Our next challenge is to how do we bring them on to the online platform and that we are seeing happening in a good way.

From a predictability perspective, we exited the month of December at INR 120 crore. We believe that around INR 140 crore on e-Pharmacy would be our new normal at least going forward, and we intend to build it. But again, I told you, I don't think we will be looking at extraordinary growth. It would be much more sustainable growth with margin, because our profitability by the end of Q2 and Q3 is what is our primary focus. On the other 2 businesses, again, for consulting business, for example, it's shown a very nice revenue trend. More than the GMV, we are getting a much positive revenue play. We hope to sustain it. Maybe in the month of March, post our annual operating plans, we will be able to share with you much greater numbers.

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Suneeta Reddy : With regards to the hospital expansion, we are on track to open 3 facilities in FY 25, '26, second half. By the end of FY 26, we will open Gurgaon and Hyderabad. We do believe that in all of these facilities, we will achieve breakeven very quickly, because we already have an existing presence and brand Apollo is well known.

Madhu Sasidhar : And I think the third question was about Microsoft partnership. I can take it if you want me to?

Suneeta Reddy : Yes, sure.

Madhu Sasidhar : So, the question regarding Microsoft, compared to the previous partnership, which was narrow, on cardiac risk prediction. This, as you pointed out, is a much more broader partnership. It aims to bring technology, some of the newest technologies in AI, to the bedside. And it's both an efficacy and an efficiency thing. There will be a slight improvement in efficacy due to clinical decision support, better accuracy, better patient quality, but we also expect a substantial improvement in physician and caregiver productivity as part of these technologies.

Binay Singh : Okay , thanks team.

Moderator : Our next question comes from the line of Neha Manpuria from Bank of America.

Neha Manpuria : Just a little bit on the Hospital business. If I were to look at the cluster -wise data, there seems to be a fair bit of moderation or muted trends in both the Tamil Nadu cluster as well as the West cluster. I just wanted to understand the reason for the weaker inpatient volume growth in these 2 clusters.

A. Krishnan : So, in Chennai, one of the important things that you should remember is this Bangladesh effect, right . We have said that as an overall, as a Company , we have seen a 1.5% revenue impact because of Bangladesh. And most of Bangladesh was coming into Chennai and Tamil Nadu. So, which is why if you look at it, at the Chennai level, it's almost a 3 -plus % impact, which is there , and which is showing up in that region that you're seeing. Otherwise, if you look at the revenue intensity in Tamil Nadu, it has actually done much better, which is why if you look at Oncology , we have grown very well in Oncology. Proton has done well. Transplants is picking up well. So, some of the high -end CONGO specialties has really helped us achieve higher revenue, right, at 8% compared to the volume. Volume would have again been 3 -plus % at least had Bangladesh been better.

On the Western cluster, again, we did some moderation in the Ahmedabad region, we had some moderation of CGHS cases that we were earlier doing, which is the reason that that's part of the Western region. But going forwa

rd, we are seeing a very

good uptake happening in Navi Mumbai. And we are hoping that into next year, we should see Navi Mumbai doing much better than what we have seen this year.

This year, we have actually done very well, as you have seen in AP, Telangana, which is again something that we wanted to focus on, and that's the benefit that we are seeing coming into the existing cluster, which we told, if you remember last 2 years, we have been telling you that we are first pruning some of the other cases and then hopefully, some of the paying cases will ramp up and which you have seen us do, which is the exact plan that we have for Western region.

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Neha Manpuria : Understood. And in terms of the Chennai and Tamil Nadu cluster, given that it's a fairly high -margin cluster for us, how do we plan to replace this volume loss or lower inpatient flow, particularly given we don't know when that situation is going to normal ize. In the past, we had talked about corporate tie -ups, etc. when we added capacity in Chennai. So, is there more room for us to tap into those to improve occupancy in the Tamil Nadu, Chennai cluster?

Madhu Sasidhar : Yes. So , absolutely , and I think some of that answer was implicit in what A . K. just told you as well. So , we see more capacity in our Chennai cluster. We see that some of the synergies between the units and between our group companies can build this occupancy potential. We will also replicate what we have done in some of the other markets, including the AP, T elangana market, which is to continuously drive our case mix towards a more complex cases that require multidisciplinary care. As you know, some of the most complex care in the country is offered through our Chennai units. So, we believe this is an opportunity for us to continue to expand with higher care contracts.

Suneeta Reddy : But having said that, we're also looking at different markets to support growth, among them are Indonesia, Iraq, Iran, Sri Lanka. And I believe the flow of patients has started to come from these markets and the Middle East and Africa.

Neha Manpuria : Understood. My second question is on 24/7. You've talked about the collaborated growth and margin strategy for a few quarters now. At what point do we actually achieve the balance? I mean, at some point of time, shouldn't we start expecting the cost to go up as we are focusing on growth from these new areas? Is there more room for us to actually cut costs to achieve that breakeven, or we need to see the GMV growth and the revenue momentum imp rove for that to happen?

M. Balakrishnan : Yes, thank you for the question. So, we will operate on both levels, but I would request all of you to bear with us for one more quarter. By the end of Q3, our recalibrated model of what is a right kind of cost structure for the next 2 years would be more or less ready. I deally, we would have been ready to give you an answer right now , but the 19 -minute proposition that we did , in Q2 we started off ; Q3, we have rolled it out to 4 cities. Effectively, this has pushed back our unit economics breakeven by maybe around a quarter. But it is actually resulting in reasonably good demand. We are able to get some new customers in these markets. We just need to get our unit economics right. As far as discount story is concerned, I think we have also recalibrated it to a reasonably good number, which I think is sustainable. Maybe another 50 basis points

is something which we 'll keep working on. So, we expect the numbers to start slowly moving up. From April onwards, you will see growth coming in. Cost, t here is a little bit more of some rationalization that we will end up doing. And like we always stated, by end of Q2 of the next year or positively in Q3, we will be

able to break even on the digital side. And if the insurance scales up much faster than we anticipated, maybe we'll be able to give you a positive surprise. So, growth will come back. But I would say maybe this quarter, you will see Q4 of this year slowly building up. But again, let me also highlight it will not be on par with the quick commerce kind of growth. We will be much more sedate , but consistent and it will be profitable and sustainable growth. Neha Manpuria : And sir, at what level of GMV are we talking about achieving that breakeven in the end of second quarter. In your view, what's that number, that would...

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M. Balakrishnan : Between INR 900 crore to INR 1,000 crore all put together. A big chunk will still come from e -Pharmacy . We expect e -Pharmacy to validate at around INR 180 -odd crore , and between the hospital consult business and Diagnostics and insurance, I would say around INR 900 crore to INR 1,000 crore , give or take, INR 100 crore depending upon how we are able to manage our cost structure. So, we are already at INR 760 crore , INR 780 crore , we will slowly inch forward .

Neha Manpuria: That is helpful, sir. Thank you so much.

Moderator : The next question comes from the line of Damayanti Kerai from HSBC.

Damayanti Kerai : Just coming back to the 24/7 platform. So, I just missed it , at what level you will be achieving breakeven in terms of GMV, you said INR 900 crore to INR 1,000 crore ?

M. Balakrishnan : No, I would say don't look at it from that perspective. We are committed to deliver it like I said, either end of Q2 or Q3 of the coming financial year will be a positive, for which I need to have around INR 170 -odd crore in Pharmacy into 3 , so around INR 900 crore Pharmacy plus Diagnostics plus consult put together. By that time, we will have insurance also contributing, not necessarily from a GMV perspective, but more on the margin side. So, our revenue side of the story will play out. So , I would still say you can take around INR 900 crore , INR 950 crore as my primary GMV number, if that is what the benchmark that you want to drive .

Damayanti Kerai : Sure. So, in terms of service, obviously, we are looking at this insurance piece very keenly. But if you can also discuss like what other value -added services you can put into this platform apart from what we already have here, which can really help you to improve on scale in coming quarters?

M. Balakrishnan : Now, that's a very good question , thank you for asking. So, see, while insurance will start off as primarily a margin expander , because we have a very strong base, we have a reasonably good understanding of the medical records. So, unlike other people, we are not going to be pushing insurance. Our intent is to give the right product to the right people. We will start off with that. That's what I meant by saying 3 life insurance companies and 3 health insurance companies with standard products. But by the end of maybe Q1 of next financial year, we intend to get into OPD, which will actually be very much in synergy, both with our e -Pharmacy business as well as our e -consult business , because then this will not only be creating a base, which helps us to build a margin structure on the insurance product, but also acts as a consistent demand generator for our Pharmacy . We have experimented this with 1 or 2 corporates, and we believe. So, that's the first value -add is that .

The second value -addition that we have been building is what we call as the 'My Family Doctor ' , which is a bunch of general practitioners between the Hospital , the Clinic and Apollo 24/7. We offer some very base level services , so that we can then refer it back to the hospital. So, these 2 engines will be value -added. So, we

won't

remain as primarily an e -commerce platform, which is focused on GMV, but exploring a full comprehensive health care and insurance w e'll be able to support.

We are also looking at a few partnerships with financial services institutions like SBI, ICICI, etc. which will help us to get a much larger base of customers without actually incurring a huge cost. So , we have had some very good traction in this quarter, and

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we will be expanding this. So, value -added services in the form of insurance OPD, My Good Doctor ' facilitation , and more partnership with financial services companies that we don't have to incur the cost, but we will have a revenue strength .

Damayanti Kerai : Okay , just a clarification. You already have these partnerships with some big corporates, right, which you earlier mentioned. So , what exactly you offer, say, like you are tying up with SBI, ICICI, how it will help your 24/7...

M. Balakrishnan : What we are realizing is the difference, the online proposition for Pharmacy , given the fact that the overall market is very much driven by discounts. One path is to walk the discount track which impacts your unit economics in a very negative way. But one way is to stick to our core Circle program , and work with banks and work with insurance companies to find out whether they would be able to offer additional discounts or additional value propositions, which enables them to bring their customers to our platform. This is what I meant.

From an earlier contribution of such discount level partnerships or scheme level partners, which used to be the range of around 2% to 3% of our contribution, has actually moved up to 13%, and we believe this will even pick up further. So, for example, we are working on a co-branded card with SBI Cards. This will be a big base for us, very similar to the Amazon ICICI Card Program. Think of a health card, this is going to be called a Health Card, which will dramatically help both our offline Pharmacy business as well as online, and we'll have spillover benefits on to our both Diagnostics and hospitals as we make the proposition stronger. So, these are the kind of propositions, not simple corporate partnerships alone.

Damayanti Kerai : Okay , understood , that's clear. My second question is on broader hospital business.

So, ma'am mentioned that like you are looking at other markets to really offset for disruption happening on the Bangladesh floor. But like we still are uncertain when things will come back fully. So, meanwhile, like some of the new facilities will also come on board next year. So, how should we look at the hospital margin trajectory? So, right now, I think we are in mid -20s, but maybe considering these factors, what could be the way ahead?

Suneeta Reddy : No. So, for the current period, we're at 24.1%. I think moving on next year, we should only be able to improve our margin. When we open these hospitals, because of the calibrated opening of operating beds, I think the margin impact will not be more than 100 basis points. So, net-net, we should be at around 24%.

Damayanti Kerai: Okay , thanks.

Moderator: Our next question comes from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan : Just on the overall occupancy trends. We did 68% for the quarter; year -to-date, 69%. We're about 400 bps higher than last year. So, if you could help us understand over the medium term, where could we likely see it? I know you have an expansion plan as well in '26 and '27, but there are headwinds also from the Bangladesh on, say, for example, the Tamil Nadu region. So, how should we look at occupancy rates over the next 12 or 24 months? Can this still keep trending upwards?

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Suneeta Reddy : So, while there are headwinds from Bangladesh, I think we have to remember that Apollo fortunately is present in many markets. And this gives us access to different catchments. Currently, we are at 68%, but we are hopeful that we should improve to 72%, 73% before the new beds open out. And by then, we would have found some level of replacement for the Bangladesh revenues and volumes , in terms of other foreign markets. Madhu?

Madhu Sasidhar : Yes. So, I think to add to what Ms. Suneeta said, I would say broadly that occupancy is probably not a perfect metric to measure us by , because there are so many numbers that go into it. For example, especially in our top 6 units, where we have a significant occupancy and a bed constraint issue, we worked very hard to drive down the length of stay. So, the right metric is inpatient volume. And as we've stated before, while the volume overall has grown up by 5.4%, CONGO volume is up by 10.3%. And even though we are doing more complex cases, we've actually, as a group, driven down our length of stay from 3.34 to 3.29 , and this is very intentional on our part. A lot of this is the operational efficiency that Ms. Suneeta talked about. Some of this is enabled by Health IT. So , we'll continue to work on our efficiency, so that we operate in the right level of occupancy to make us efficient and for patient experience.

Shyam Srinivasan : That's helpful. And just a second question on the ARPOB dynamics, right . So, we have seen an improvement in Q3 related to a year -to-date run rate of about 5%. We have seen 7%, 8%. So, is this now sustainable, we think, in terms of should we go back to our historical 6% to 7% kind of CAGR on ARPOB? And some of the changes that we have tried to either in terms of mix or pricing, will that now sustain in the path forward?

A. Krishnan: Yes, Shyam, I think that's correct. You should consider it to be around 6% to 7%. I

think that's a fair number to consider going forward.

Shyam Srinivasan: Understood, sir. Thank you, all the best.

Moderator : The next question comes from the line of Rajit Aggarwal from Nilgiri Investment Managers Private Limited.

Rajit Aggarwal : In your Retail Health & Diagnostics and Digital Health & Pharmacy distribution, just to understand your medium-term plan or strategy, the way you are looking at these businesses. Let's say in 3 years' time, how do you think these businesses would have ramped up, Retail Health in terms of turnover and margins, and Digital Health in terms of margins? Just for my understanding how to build something for the next 3 years, I am not asking for a specific guidance as such.

Suneeta Reddy : So, Retail Health, physical formats is with Apollo Health & Lifestyle. So, I'll ask Sriram to start answering that question, and Madhi to chip in on Digital.

Sriram Iyer : Yes. So, this question is specific to Diagnostics or overall Retail Health?

Rajit Aggarwal : Overall Retail Health.

Sriram Iyer : Yes. So, as you saw in AHLL, right, so we have 3 verticals, right. Primary Care and Diagnostics is one, then we have Specialty formats, and then that comprises the

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entire AHLL. So, we are growing at about 15% YTD. And I think, as Suneeta madam had also shared, and YTD also, we are at 15% growth. We look to keep the growth rate between 15% to 18% for the next couple of years, primarily driven by the momentum in diagnostics. So, diagnostics is one area, along with clinics, where we are betting on for the future. Clinics also has a lot of diagnostics revenue within the setup. And these are the 2 business units, we will be really pushing a lot to get

us to

the 15% to 18% growth mark for the next few years. That will help us draw our top line and also subsequently help us drive the better margins as well.

Rajit Aggarwal : Right, sir. Just as a follow-up question on this piece. For us to grow 15% to 18%, that would mean adding a good number of centers and clinics. Do you see that kind of an expansion happening? And also on the margins piece, what would be a 3-year your aspiration level to reach, would it be closer to our healthcare vertical or would it be somewhere halfway up to the healthcare vertical?

Sriram Iyer : So, first is that definitely, we'll be adding more centers in diagnostics as we continue to grow. We have an expansion plan to add more labs and open more centers. In primary health clinic as well, we work closely with hospitals to look at opportunity to add more clinics. So, these will be the formats, we'll be expanding.

Currently, coming to your question on margins. Currently, we are at a 9.2% EBITDA this year. And we definitely, over a longer-term view, we should expect a 2-percentage point in a year, at least to come in, and primarily being driven because of a stronger growth coming in from diagnostics and clinics, and the business mix changing more towards Primary Care and diagnostics.

Rajit Aggarwal : So, you said 2% improvement per year?

Sriram Iyer : Per year. That's right.

Rajit Aggarwal : Okay, thank you. And on the digital vertical, and Pharmacy.

M. Balakrishnan : On the Digital, our aspiration is to grow at an average of 20 plus kind of growth rate. Let me tell you how we intend to do it. Today, most of the businesses that we are doing is primarily coming from the top 6 cities in our country, which is the top 6 metros, that's almost 80%, 85% of our business comes from this. So, the reason why we are trying to get our core model in place is to see how we can replicate it in maybe smaller markets. So, these are the cities. So, over the next 2 years, we intend to move that 6 to at least around 25-odd markets. And I say 25 really core businesses. That's one way of looking at it, which would result in a 20% kind of a growth.

The other way is, today, we contribute around 15% of the Pharmacy business, which is the Offline Pharmacy, roughly around 15% of the overall Pharmacy business. In the cities that we operate in, we are almost 30%. And as the trend grows more towards Digital, we will build it up. So, this story will play out again in the remaining 20 odd markets that we speak about.

On the e-consult business at the hospital, we are in the range of around 12% to 15% of the total consultation bookings that happen. We intend to build that up and become a much more integrated partner with the hospital and with the Clinics.

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The diagnostics business has been a bit flat. We hope to again expand that dramatically. So, between these 3 businesses, which are our core business, we are

reasonably confident of doing around 20% growth rate. I'll come to the margins separately. The Insurance business, which we hope to begin in the coming year, would start maybe a bit slowly, but by the end of Q3, we should have a recurring revenue, which will be contribution to the margin structure. They might not support us on the growth, but on the revenue side, it would be a big flow. For margin, can I ask Sanjiv as to how the margin structure will play out, please?

Sanjiv Gupta : Yes, thanks , Madhi, for this. So, yes, on the margin side, I think we have been steadily increasing our margin. If you look at Q3 FY 25, we are at about 13.8%, and corresponding quarter FY24, it was 10.4%. We strongly believe that as we come closer to Q2 end FY 26 or Q3 FY 26, when we intend to become breakeven . And as Madhivanan talked about insur

ance and various other value -added services to kick in, we should be hitting anything between 18% to 20% as a margin during that quarter to support the breakeven intent of the organization. So, that is where I would see that steadily, we'll be increasing our margin quarter -on-quarter , and you can expect anything around 18% in next 3 to 4 quarters.

Rajit Aggarwal: Alright, sir. Thanks a lot.

Moderator : The next question comes from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha : Ma'am, we have shared that we'll be opening the new beds in a calibrated manner. So, if you could help us understand of the 1,737 beds planned for next year for commissioning, what would be a range of bed, which would be operationalized to start with? And how should we expect the ramp -up as in, how should those number of operationalized beds ramp -up over FY 26, '27 from that 1,700 beds?

A. Krishnan: At a broad level, you can expect 50% of it to come into next year and other 50% to come into FY 27. So , we would start as we said, that Pune and Calcutta and Delhi, Delhi which is the cancer hospital, those 3 will start first, followed by Gurugram and Hyderabad. And broadly, you should expect 50% to come in next year , and 50% in the year after that.

Kunal Dhamesha : Sure, sir. And is there any operational cost , already baked into our P&L with respect to the 50% operational beds that we are expecting for next year? Or will it be coming in...

A. Krishnan: Some of that, we have started building in anticipation of some of these beds. Like if you look at Hyderabad , in Delhi, we have been adding some doctors , etc. , because some of these we should be able to leverage them even into the new hospitals.

Kunal Dhamesha : Sure, sir. And one bookkeeping question. What has been the pharma Average order value (AOV) this quarter?

Suneeta Reddy : INR 1,001.

Kunal Dhamesha: Thank you, and all the best.

Moderator : The next question comes from Nitin Agarwal from DAM Capital.

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Nitin Agarwal : Sir, 2 questions. One is, on the hospitals with the newer capacities coming in, what kind of revenue growth should we look at for the next, say, 3 odd years now?

A. Krishnan: Let us come back to you on this. Allow us to come back , because there is time, and we are clearly looking at mid -teen growth to continue, hopefully, even now. That's the momentum that we are working on , and the new hospitals should add on. Let us come back to you on that separately.

Nitin Agarwal : Okay. And secondly, on the Apollo Healthco business , on the Digital , we're doing currently about operating cost of about INR 110 crore or thereabouts per quarter. So, these costs as the scale of the business increases, do they stay around here, or they also increase proportionately from here?

Sanjiv Gupta : Yes. We actually expect it to rationalize a little bit more, the new structure, maybe a little bit of a tweaking on our manpower cost. But at the same time, we will also be investing into the Insurance business. So , I would say the rate of deceleration in costs will come down. But it's not going to be a straight -line graph. So, we can expect that we will try to get a much better return for the cost that we are already incurring.

Nitin Agarwal: So, this stays more or less around this current level , when the revenue ramp up...

Sanjiv Gupta: Yes, more or less around the current levels, because I don't intend to spend again on marketing and any kind of variable expenses, so most of the fixed we will try to sweat it out more rather than that.

Nitin Agarwal: And just one sort of clarification. So, the year 3 target that we put out for the Healthco business in its restructured form with an INR 25,000 crore revenue number, what timeframe are you

u looking at for that to achieve?

Sanjiv Gupta : Yes, I can take this question. So, we are looking at about INR 25,000 crore of revenue and about 6% to 7% of EBITDA in FY 27. That's the guidance we have given. And if you notice, earnings call deck, we have also given memo accounts and including Keimed . And you will notice that we are already at a run rate of roughly INR 17,000 crore with a 6.5% margin, if I exclude the digital losses , given the fact that digital losses would become breakeven in next fiscal year, which is FY 26. So , FY27, we should be hopeful of hitting from INR 17,000 crore to INR 25,000 crore with a 7% to 8% EBITDA line.

Nitin Agarwal: So, that's a full year number by FY 27, got it.

Sanjiv Gupta : That's right, sir. That's right.

Nitin Agarwal: Thank you, so much.

Moderator : The next question comes from Prashant Nair from AMBIT.

Prashant Nair : I just had a question on your expansion projects that you have outlined in your presentation. For FY26, can you give a slightly more granular breakdown as to which of these projects could get commissioned in the first half of the year versus second half?

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A. Krishnan: Yes, which is what we said. If you look at the Pune , the first one, which is there in the presentation, Pune, this should come in the first half. Calcutta should again come in the first half. Hyderabad will come more closer to the end of the year. Gurugram, again, closer to the end of the year. And Malleswar am and Mysore, we are still working on the dates. We have not yet given you the numbers, days or when it will come on. And Defense Colony, Delhi will come early into next year. That's the cancer hospital.

Prashant Nair : All right , okay. And for the ones which are expected over the next 3 to 4 years, any of those could come through in FY27 or '28?

Suneeta Reddy : These will come in '25, '26 in '26 .

A. Krishnan: So, half of this, as we said, operationally, this will come in '26 , and then other will be in '27. All of these have large revenue potential s, right . If you look at it, Gurugram, Hyderabad, Calcutta, Pune, all of them have large revenue potentials. The others will take 3 to 4 years as we have said , because we have started work in OMR. We have just about to start work in Worli in the next 3 months. So , from the time we start work in Worli and all, it will take at least 3 years. So, you should look at next 3 to 4 years as we have said for commissioning all of the others.

Prashant Nair : So, 3 to 4 years out from where we are now?

A. Krishnan: That's correct.

Suneeta Reddy : 3 years from now.

Prashant Nair : Yes, thank you.

Moderator : The next question comes from the line of Harsh Dubey from Financial ly Free.

Harsh Dubey: Just wanted to understand, ma'am, we were talking about the IRDAI license, and we said that in October, we will be receiving that. So, what's the update on that?

M. Balakrishnan : We have already got the license, like I told you . We got it in the last month , but there are other regulations which we have to clear. So , we have got that also. So , we will be rolling out a product in the month of March to begin with. And like I said, by April, we will have at least 3 Life Insurance , and 3 Health Insurance businesses up in place.

Harsh Dubey: Okay. And also , on the GMV. So, first of all, I would like to understand what exactly GMV is , because there's some doubts for me in that. And also, we had a target of INR 4,000 crore of value for the GMV in the 24/7 business. So , can you please give me some answer to that?

M. Balakrishnan : When we started the financial year, we were exploring how to touch around INR 4,000 crore . As we speak from Q2 onwards, like I said, we had a major recalibration in the way we are approach

ing the business. Instead of GMV, our focus shifted to ensuring that we are able to build EBITDA -neutral model by Q3 coming financial years. So , we have given ourselves 5 quarters. So , we are on go al. So, let me just explain to you the GMV concept. So, GMV is a contribution of 3 lines of business, as we speak.

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First is the e-Pharmacy , this is the total Digital business that we do purely on Apollo 24/7. This is driven by our 19 -minute proposition as well as delivering 90% of the medicines within the same day. This forms the biggest component of our GMV. In the month of December, we exited with around INR 120 crore , and we are seeing that number t rending towards INR 135 crore to INR 140 for Q3. So, that's the trajectory

that way.

Our Diagnostic business is in the range of around INR 30 crore to INR 35 crore every quarter. We intend that to grow next financial year.

And then we have the Consult business, which is predominantly driven by both the Outpatient business as well as the Inpatient business that we are able to drive. This has been a bit flat this quarter because of muted growth.

So, all put together, instead of INR 4,000 crore, with a much better quality of business, we will be ending up at around INR 3,100 crore. And we will recalibrate our business on these lines. Like I told you, the emphasis is on breakeven. And once the insurance comes in, I think our revenue mix will look much better.

Harsh Dubey: Okay. And just one more question on the CAPEX that we are planning for INR 1,700 crore. As you said that Pune, Calcutta and Delhi will get commissioned in H1 FY 26.

Am I reading it correct, that Delhi will get commissioned in H1 FY 26?

Suneeta Reddy: Yes.

Harsh Dubey: Okay. So, on that, so for INR 1,700 crore the CAPEX that we are doing, does that also include the working capital for the hospitals that we are talking about?

A. Krishnan: No, not so much. It has got the pre-op expenses in that, but...

Harsh Dubey: Sure. Thank you.

Moderator: The next question comes from Marsal, an Investor.

Marsal: So, my question is regarding this Online Pharmacy. As we can see, this business is bleeding and on top of this, like there are a lot of other startups in the market. So, it's very difficult to get profitability and get the market share. On top of this, we are spending a huge amount on the ESOPS. Like this quarter, we have put an expense of INR 26 crore. So, we understand that the market dynamic is not in our hand, but ESOP is in our hands. So, why don't you stop the ESOP, especially in the Pharma business? So, our loss is not increasing much.

M. Balakrishnan: So, thank you for the question, sir. So, first and foremost, I completely agree with you that in the digital e-commerce business, it's a very deep investment business where you are putting in a lot of money, and therefore, there are a lot of startups, which are not making money. And that's what we said over the last 2 quarters, we have recalibrated the model, so that we don't have to go for growth at any cost, especially at the cost of the bottom line. So, we are very much on trajectory when it comes to building the EBITDA profitability. So, as we have told again and again, over the next 3 quarters, we should at least be into a breakeven model and build on model going forward. Number one.

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Number two. Like I said, our business is not just e-commerce. It's actually a combination of a health care module, where we will be doing all the 3 lines of businesses: e-Pharmacy, which is Online Pharmacy; E-Consult, much more deeper integration with the

hospital; and third, on the Diagnostics side and radiology, etc.

we'll expand. So, we are reasonably confident in turning around and building it.

On your cost structure for ESOP. This business is a very technology-driven business. If you notice, we have only around the 300 to 400 core team members who are driving this agenda. And as we are recalibrating ourselves, in fact, we will be further trimming down the entire employee force. So, from our best way to retain very good tech people at this point of time in the onslaught of quick commerce, whether it be Bangalore, whether it be Gurgaon, is to somehow retain our people. And we have given our first ESOP around 4 years back when the Company had started. At the end of the 4-year period, there was a small increase that we've put, because if I lose my employees, that's a big drain. And it is completely governed by our NOMREM Committee. And this is with a clear-cut intent of retaining our core talent and not losing out, and we will build it, and we'll be able to make up for it.

Marsal: Sir, I am saying that we are giving every quarter like for YTD we already have an expense of INR 62 crore. So, at least put a brake on this one. You have given a lot of ESOP on this one. So, my point is that, please put a brake on the ESOP in the online Pharmacy business, number one. And number two, how are you sure today that within 2 to 3 quarters, you will be EBITDA positive. Like are you just increasing market share. Or are you putting more like discount a lot of things out there. And third thing, why don't you start the distribution of Pradhan Mantri Janaushadhi medicine also, because we can see that in the Pradhan Mantri Janaushadhi scheme medicine is available at half price, but the locations of those medicine shops are not nearby. So, like if, if you also cover the Pradhan Mantri Janaushadhi in Online Pharmacy, maybe you can get more market share.

M. Balakrishnan : So, we'll take all your suggestions on board , sir. We will work on this very closely. Thank you.

Marsal: So, please stop the ESOP, at least.

M. Balakrishnan : We will certainly consider. Thank you.

Moderator : The next follow -up question comes from Harsh Dubey from Financial ly Free.

Harsh Dubey : Sir, I was asking on the Onco , Cardio and Neuro margins. So , we know on the overall hospital segment, the margins. But , just wanted to understand how our Onco, Cardio , and Neuro segment is doing?

A. Krishnan: So, I think all of them have seen more than double -digit growth, as we said, even in the overall volume s. So, the margin of medical radiation and surgical that we have. So, generally, the EBITDA margins are north of 35% in this, given that it is high intensity of radiotherapy , etc. also that we have. Neuro, again, the whole important thing is the ARPP on a neuro is typically higher, the average revenue per patient, which enables us to have a higher operating leverage, resulting in more than Company margins. So, broadly, some of these are the reasons that if you saw our

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overall margins have gone up despite a drop from Bangladesh , which has been a 1% volume and 1.5% impact on the revenue.

Harsh Dubey : Right. And just wanted to also understand, so we are seeing many of the hospitals are coming, especially for Onco business. And just a follow -up question on the Bangladesh patient also. So , do we like charge more to the Bangladesh patient or do we have it in line with the Indian patient. So , what's the take on that?

A. Krishnan: So, Bangladesh , any foreign patients, we have been having some premium to be normal, which is around 15% premium. So, that is how it has been. And 15% to 20% premium has always been done for all foreign patients. So, that is where we have had this

loss on the revenue of 1.5% for us, though the volumes were down by 1%. But hopefully, we have seen the bottom on that , and we should, going forward, at least see it continue at the same trend or hopefully increase next year, we'll have to see that.

Harsh Dubey : Right. On the greenfield that we are doing in Pune, Calcutta and Delhi, just wanted to understand , when do you think that the margin suppression will go out of the P&L, and then we'll be able to see some recovery going forward like in FY 26 or '27, what kind of would that be?

A. Krishnan: Calcutta and Delhi , both we would expect that we should be able to EBITDA breakeven in less than 12 months. In fact, both of them are existing markets, and the Calcutta Hospital is already full at over 80% occupancy. We are starting this at the other side of Calcutta, which is almost 17, 18 kilometers away. So , we have a very strong brand also there. So, we are quite hopeful that ; we have the set of doctors that's going to join in also. That is going to be oncology program as well in the new hospital. So , we are quite hopeful that we should be able to break that even in less than 12 months. It's also in the heart of Delhi. In fact, it's a Defence Colony that we are starting. We know the set of doctors who are going to join us. It's a focused woman cancer center, and we have a clear plan on , program in place on how we want to augment that.

Moderator : The next follow -up question comes from Damayanti Kerai from HSBC.

Damayanti Kerai : Just a clarification . In Q3, you booked other income of around INR 64 crore , which is much higher than around INR 37 crore, INR 38 crore in first quarter and second quarter. Can you explain this, please?

M. Balakrishnan : There was this interest from mutual funds and also one -off interest, which we received from an income tax refund of almost around INR 20 crore . That was the reason.

Damayanti Kerai : Okay. Otherwise, rate is similar to like what we saw in the first, second quarter?

M. Balakrishnan : That's how it should be, yes.

Damayanti Kerai : Okay. Thank you.

Moderator : Ladies and gentlemen, we would take that as our last question for today. I now hand the conference over to the management for closing comments.

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Suneeta Reddy : Thank you, ladies and gentlemen, for joining this call. As we conclude our discussion on Q3 FY 25, we are encouraged by the strong momentum across all business segments. Our comprehensive and integrated health care offerings, marked by a robust pipeline of initiatives and calibrated expansion plan, continue to strengthen our leadership position.

We are excited about the progress we are making in HealthCo with sustainable and high-quality growth in revenue, accompanied by margin expansion. The Keimed merger is advancing well, and the team is on track to achieve breakeven in the digital segment too within the next quarters. With a strategic focus on driving revenue growth, enhancing profitability and advancing operational excellence, we remain well positioned to deliver exceptional value.

We sincerely appreciate your continued interest and support and look forward to sharing our progress with you in the coming quarters. Thank you, ladies and gentlemen.

Moderator : Thank you. On behalf of Apollo Hospitals Limited , that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Disclaimer: This document is a transcript and may contain transcription errors. While the transcript has been edited for clarity, the Company takes no responsibility for such errors. Efforts have been made to ensure a high level of accuracy, and any figures that may have been inadvertently mentioned have been reviewed and corrected as necessary.

Call: Jun 2025

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Apollo Hospitals Enterprise Limited
Transcript of Q4 FY25 Earnings Conference Call
May 31, 2025

Moderator : Ladies and gentlemen, good day, and welcome to Apollo Hospitals Limited earnings conference call. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you, sir.

Mayank Vaswani : Good morning, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals to discuss the earnings for the 4th quarter and full year or financial year '24 - '25, which were announced yesterday.

We have with us today the senior management team represented by Mrs. Suneeta Reddy – Managing Director; Mr. A. Krishnan – Group CFO; Dr. Madhu Sasidhar – President and CEO of the Hospitals Division; Mr. Madhivanan Balakrishnan – CEO of Apollo HealthCo; Mr. Sriram Iyer – CEO of AHLL; Mr. Sanjiv Gupta – CFO of Apollo HealthCo; and Mr. Obul Reddy – CFO of the Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide # 2 of the investor presentation that has been circulated earlier. Documents relating to our financial performance have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Good morning, everyone, and thank you for joining our call this Saturday. I trust that all of you are well and have received our earnings document, which we shared yesterday.

I am happy to share at the outset that in FY '25, we have achieved two significant milestones. We crossed INR 20,000 crore in consolidated revenue and came in at INR 21,794 crore. Alongside, Healthcare Services revenue crossed a milestone of INR 10,000 crore and came in at INR 11,147 crore. We believe these milestones reflect the scale and depth of our operations and give us the impetus to move ahead. Our performance in Q4 FY '25 reflects continued strength in execution, underpinned by resilient operating metrics and sustained growth across specialties and geographies. This has helped us to conclude FY '25 on a very strong note with healthy

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momentum and double-digit growth across all 3 verticals of Healthcare Services, Apollo HealthCo and AHLL.

Our Healthcare Services business continued to benefit from strategic investments in high-end specialties and a sharper focus on optimizing both the payer and case mix, all of which contributed to steady improvement in ARPOB and margins. Our investments in clinical programs, medical talent and digital transformation have further strengthened Apollo's position as a leader in integrated healthcare delivery. Apollo HealthCo has now delivered a third successive quarter of profitability after breaking even in quarter 2 earlier in FY '25. AHLL has sustained its growth trajectory, supported by ongoing expansion in diagnostics and primary care formats. This performance positions us well for sustainable growth and enhanced profitability as we step into FY '26.

Against this backdrop, let me share some of the key highlights of our performance for

the quarter :

- Consolidated revenue grew by 13% year-on-year to INR 5,592 crore. Within this, the Healthcare Services business delivered 10% year-on-year revenue growth to INR 2,822 crore in quarter 4. This is after a 2% impact on account of patient flows from Bangladesh. Revenue from cash and Insurance segments saw a year-on-year increase of 11%. Collectively, these segments accounted for 83% of our inpatient hospital revenue.

Inpatient volumes grew by 4% year-on-year. Our focus specialties, cardiac, oncology,

neurosciences, gastro sciences and orthopedics continued to expand at a healthy pace, recording a volume growth of 8%, which reflects in our ARPP as well as margin despite the setback from the Bangladesh patient flow. In addition, higher secondary specialties saw robust growth too, supported by the expansion of insurance coverage in the recent years, which has increasingly enabled patients to access urbanized healthcare services.

Group-wise occupancy stood at 67% in quarter 4. Occupancy in metro cities was at 70%. ARPOB grew 7% year-on-year, touching INR 63,569. We believe that the key driver such as increased surgical volumes, a higher complexity case mix and further incremental improvement in payer mix in select geographies will continue to support ARPOB growth going forward.

Apollo HealthCo reported revenues of INR 2,376 crore, growing 17% year-on-year in quarter 4 on the back of omnichannel pharmacy growth and double-digit growth in GMV. Revenues from Apollo Health and Lifestyle increased by 11% on a year-on-year basis to INR 394 crore during the quarter.

Consolidated EBITDA was at INR 770 crore, registering an increase of 20% year-on-year. Within this, the Healthcare Services EBITDA was at INR 686 crore, a growth of 16% year-on-year. Healthcare Services margins remained robust at 24.3%, reflecting the high proportion of tertiary care in the mix. Pharmacy Distribution business within Apollo HealthCo recorded an EBITDA of INR 162 crore, a growth of 21%. In the Digital segment, cash loss were at INR 80 crore compared to the INR 111 crore in Q4 FY '24. In addition, there was an accelerated ESOP charge of INR 45 crore in this quarter. Therefore, Apollo HealthCo reported an EBITDA of INR 36 crore in Q4 FY '25, up 3x from INR 12 crore in Q4 FY '24.

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AHLL delivered an EBITDA growth of INR 47 crore, representing a 32% year-on-year growth with margins improving 12% from 10% in quarter 4 last year. Consolidated PAT was at INR 390 crore, growing 54% year-on-year.

Within Healthcare Services, we delivered a ROCE of 27.5% with balanced ROCEs across all geographies, metro, Tier 1 and Tier 2. Private label and generics revenue were at 15.6% of total pharmacy revenues. Our digital platform 24/7 added 2 million users. The platform GMV was at INR 795 crore, representing a growth of 11% over the same period last year.

I now come to the summary of the full year performance for FY '25 : - FY '25 consolidated revenue was at INR 21,794 crore, a year-on-year growth of 14%. Healthcare Services revenue was at INR 11,147 crore, a growth of 13%. Apollo HealthCo reported revenue of INR 9,093 crore, a growth of 16%.

AHLL revenue grew by 14% to INR 1,554 crore. Full year consolidated EBITDA grew by 26% year-on-year to INR 3,022 crore. Healthcare Services EBITDA grew by 15% to INR 2,701 crore. This represents a margin of 24.2%. Apollo HealthCo reported EBITDA of INR 168 crore after considering noncash impact of INR 108 crore for the year. AHLL EBITDA grew 32% to INR 154 crore.

Consolidated PAT was at INR 1,446 crore, a growth of 61% year-on-year. Healthcare Services PAT was INR 1,426 crore, an increase of 25%. Within this, we have ended the year with a robust set of finan

cials.

As we look ahead, we are on track to operationalize our previously announced facilities in Gurgaon, Pune, Kolkata and Hyderabad later this fiscal year. In addition, we are happy to announce a significant expansion in the Sarjapur micro market in Bangalore with an addition of 700 beds in 2 stages. Stage 1, the acquisition of an existing 200 -bed hospital, a leased facility where we can commence services within 2 months.

Stage 2, establishing a 500 -bed greenfield hospital in very close proximity for which the land has already been purchased. Within this, we will operate a 700 -bed integrated facility and will take our bed strength in Bangalore to 1,500 beds, along with our strong network of clinics, Spectra and Cradle facilities.

We are also pleased to announce capacity expansion of 160 beds in our existing Jubilee Hills and Secunderabad facilities in Hyderabad, which will be operationalized this year, along with our planned expansion in Gachibowli, which will take our overall bed strength to 1,500 beds in Hyderabad.

With this, we are committed to spending over INR 8,000 crore for the additional 4,300 capacity beds over the next 3 to 4 years. We have already spent INR 1,000 crore for land acquisition , (Inaudible - 0:11:19 min.) and project development. Balance project cost to be incurred over the next 3 to 4 years is around INR 6,000 crore.

We have significant cash and cash equivalents on hand and generate healthy free cash flows in excess of INR 1,000 crore per year after accounting for routine items. We believe we can fund this portfolio expansion, primarily through existing funds and internal accruals.

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We continue to execute on our strategic priorities, strengthening our core healthcare services, building clinical differentiation, expanding our digital retail healthcare footprint, and enhancing patient outcomes through rigor and innovation. With strong fundamentals across the business, plan is in place to mitigate the headwinds caused by disruptions in inpatient flows from Bangladesh and several facilities set to be commissioned in a phased manner. We are confident of sustaining our growth trajectory in FY '26. As always, our focus remains on creating long-term value for our patients, partners and stakeholders, while driving operational excellence across the board.

On this note, I would like to hand it over to the moderator, and open the line for questions. I have our CFO, Krishnan with me; Dr. Madhu Sasidhar, CEO of the Hospital Division; Sriram Iyer, from AHLL; Madhivanan, Obul Reddy and Sanjiv from Apollo HealthCo with me to take all of your questions. Thank you.

Moderator: We will now begin with the question-and-answer session. The first question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is on 24/7. I wanted to understand that some of your competitors have tied up with quick commerce platform to deliver medicines in 10 minutes to 15 minutes. So how do you see competition ahead for your E-pharmacy business? And also, for 24/7, on a stand alone basis, when do you see cost breakeven in terms of higher ESOP charges, which we saw in the fourth quarter?

M. Balakrishnan: So to answer the first question, as far as the quick commerce alignment is concerned, as of now, we do not have any plans of working with any of these. We have received multiple offers, but we believe that it is not in the best interest of what we are trying to put forward. We already have an alliance with Amazon over the last 2 to 3 years. So that we are continuing to build, wherein we will try and work towards how to make that as a sustainable growth engine.

As far as the response to the quick commerce companies are concerned, this particular phenomena start

ed around a year back. Since then, Apollo 24/7 has also launched a 19-minute proposition, which today contributes to almost 30% of our total GMV. So we believe when it comes to medicine as against a 10-minute proposition, something in the range of 19 minutes makes a lot of sense as it drives emergency purchases, and at the same time ensures that we are completely in line with all the regulations that are needed from a healthcare perspective. This particular 19-minute has already been rolled out in our top 6 cities.

As we progress, we intend to extend it to the next 10. So as we speak, we have no intentions of tying up with anybody, because that basically relegates us to a back office a function, which one of our group companies, Keimed is already doing. So that's something which will not work. And over the last 2 quarters, this particular approach of ours has been sustained.

When the quick commerce initiative started, we actually saw a little bit of a drop in our health essentials, the FMCG products that we sell. That has come back again reasonably well, and we think we will continue to stick to our guns of building our own capabilities. That was on point 1.

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Point 2, as we have always commented, we stick to a path of converting into a cash breakeven between Q3 and Q4. The last quarter, that is Q3, Q4 sequentially for FY '25, it was a bit flat in terms of bringing down our expenses because of 1 or 2 one-time expenses. But our trajectory to continuously reduce cost at the same time increase our net revenue on the Pharmacy business, which effectively drives the turnaround is on course. Our unit economics is steadily increasing. And if I were to see the trend for quarter 1 of this financial year, we are very much on that path of meeting cash even between Q3 and Q4.

Sanjiv Gupta: I will take the last question of yours, which is related to the ESOP cost. As Suneeta ma'am mentioned, we had accelerated ESOP cost coming in Q4. So that is one of the reasons that we have a higher cost in Q4. However, for the current fiscal year, which is FY '26, we are looking at about INR 100 crore of overall ESOP cost in line

with the previous year. However, the next year, it is going to be almost 1/3rd of the cost. So much of the cost is going to be only till this year -end, current fiscal year -end. Damayanti Kerai: Okay. Actually, one clarification, which I was looking on the quick commerce space. So aren't you a bit worried that your competitors might be taking a larger chunk of the volume because of this 10 -minute delivery or so. I was looking for some clarification on that part.

M. Balakrishnan: So let me clarify. So that's what I said, over the last 2 to 3 quarters, this particular trend essentially impacts us on the OTC products , because the processes that are required to ensure that we are completely on board when it comes to various regulatory requirements , I think Apollo Pharmacy and Apollo 24/7 are in a much better position to validate that. And while we saw some numbers going down, we are back on course. So we are watching this space very carefully. A lot of these players are doing some experimental efforts in Bangalore. Our Bangalore numbers are holding up. In fact, we continue to grow there. So we will watch it. So we are very much aware about it. We are not being stone -headed about it. We will keep looking at it. But at this point of time, the equations do not justify that , and we believe we are on the right path.

Damayanti Kerai: Sure. And my second and last question is on the Healthcare business. You have maintained margins close to 24% or so. So how should we look at this trajectory given a couple of new units are coming up in '26 and beyond?

Suneeta Reddy: I think the thing to keep in mind

is that the existing units will continue to generate very strong cash flows and therefore, stronger EBITDA margins. The second initiative of ours, which is to lower the cost will somehow bridge the gap between losses that we might incur in the last quarter of the year. About 140 basis points estimate is what we see, and 80 basis points improvement will come from cost s. The balance 60 basis points will come from improved revenues , because of the initiatives that we are taking within the hospital from improving the payer mix to complexity to improving ARPOB , and higher occupancy . All of these will generate very strong cash flows , and therefore minimize the impact of new hospitals.

Damayanti Kerai: So very broadly, you can maintain margins around 24% , despite new beds coming in , closer to that?

Suneeta Reddy: This year, yes.

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Moderator: The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Just a follow -up on the Digital business. Could you give us an update on how the new businesses are doing? I think we introduced insurance in March. Has that started reflecting in GMV in the quarter or that's yet to come by? And a follow -up question on the Digital business is if I look at the pre -op margins, they seem to have moderated in the quarter. So when do we start seeing that improving to achieve that cash breakeven in the second half that you mentioned?

M. Balakrishnan: Sorry, I was having a little bit of a disturbance. Can you repeat the question , please ?

Neha Manpuria: Yes. So, I asked about the new business segments in digital. I think we introduced insurance in March. So I wanted to check if that's already started contributing to the GMV in the quarter or that should come through. What's the progress you are seeing? And a follow -up on the Digital business is on the pre -OpEx EBITDA margins , - when do we start seeing improvement in that number , because that seems to have moderated quarter -on-quarter.

M. Balakrishnan: Okay. So, on the first bit of it, the Insurance business, we officially started from the 1st of April, when we became a full -fledged corporate agent. Before that, we were a marketing agent. Once we have got the license, our number compared to last quarter has already doubled. So, we are on course this quarter to move at around INR 6 crore to INR 7 crore at a top line basis.

On an annualized basis, we have plans to exploring in the range of around INR 75 - odd crore. This particular business gets driven primarily by the number of insurance partners who we are able to add to our portfolio. As of now, we have 4 operating , - while we have signed up around 12, 4 of them are already operational, which is Niva Bupa, ICICI Lombard, Star Health and HDFC Ergo.

In this quarter, even before June, we would have another 4 people coming in, and this has a direct impact. So this is a combination of digital, telecalling and certain POSP on the field. So we expect the first quarter to close around INR 6 crore, INR 6.5 crore, and then this will continuously progress so that we exit with around INR 8 crore to INR 10 crore on a monthly basis.

So we are very much on the course . The technology is getting built up , the fields are all coming in. But like I said in the earlier investor call also, this year, we do not expect

it to contribute to our bottom line. But at the same time, we are not expecting any kind of losses. We will break even on this business on an end-to-end year. Next year, there would be a considerable/significant increase in the margin that this business will cover.

On the second line of business, more than as a stand alone, it is actually a contribution to our Pharma business. This is our SBI Card s. It got launched just around this month.

And we are seeing some very good interest. We already have some very good results, we already have some people who have registered and expressed their interest. We hope to get into a reasonably strong number, which will help us to get some fee income. But more importantly, it will actually drive us towards ensuring a sustainable business on both the pharmacy as well as the diagnostic side. So, these two are on course.

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And as far as your second item is concerned, Q3 and Q4, in fact, our margins compared to the last quarter are slowly increasing. I will ask Sanjiv to sort of add on to it. So we are on a consistent trajectory, both in terms of cost reduction or unit economics, which at the same time last year was in the range of around INR (-60) negative is already down to around INR (-27) on the digital side. And the unit economics continues to improve. As our AOE is stabilizing, our cost of delivery, which is roughly around 9% of my total operational expenses at the unit economics level, is also coming down, and we are reasonably confident that we will be on that path. So both on the revenue side as well as the cost of delivery side we are bringing down. At the same time, our discount structures, which used to be reasonably higher, is also stabilizing in the range of between 13% to 14%. So, all these three things should start seeing, - this quarter 1 you will see upward trajectory.

Neha Manpuria: Yes, that's helpful. My second question is on the hospital margins. Ma'am, is it fair to assume that a lot of the capacity that we are adding will probably come through only in the later part of the year, based on your commentary that the losses will only be visible in the 4th Quarter. So we don't expect any of these hospitals to actually start in the first half of the year.

Suneeta Reddy: So even in the first half, some of them which are expansions, for example, Secunderabad Wellness, which is an existing hospital, we don't expect significant losses. Pune, which is a new hospital, is where we could see some small losses. We are not really expecting to see huge losses. In the second quarter, definitely not, we would have added another 150 to beds, 200 beds. But by the end of the year, yes, in the 4th quarter, there will be significant bed addition.

Neha Manpuria: And both Jubilee Hills and Secunderabad are coming through in this year itself?

Suneeta Reddy: Yes.

Neha Manpuria: Okay, thank you.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal.

Tushar Manudhane: So, just a clarification on the operating beds, while most of it, - as operational beds are expected to come from Q4 FY '26, and this is where that impact of 140 bps. Is my understanding correct?

K. Akhileswaran: That is correct. So what will happen is by Q3, you will see us operationalizing Pune, you will see us operationalizing Kolkata, the Defense Colony and the first Sarjapur acquisition that we have said. All the four of them should get operationalized by Q3.

That's the internal target that we have.

Tushar Manudhane: Got it. And just on GMV, if you could sort of clarify what kind of growth one should think of with these new business initiatives for FY '26.

M. Balakrishnan: This is for Apollo 24/7.

Suneeta Reddy: Yes.

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M. Balakrishnan: Okay. So, we are planning on the digital side alone, a GMV increase of 25% to 30% on an annual basis. We are already seeing that trajectory happening. We closed Q4 with INR 795 crore GMV, which is a combination of pharmacy consult and the hospital IPOP and diagnostics. We did not see a big

growth in our IPOP business. So we are working towards that. But pharmacy, we are very much on course.

If you remember last time, I spoke that our new normal is INR 140 crore, - in the first 2 months we are already at around between INR 135 crore to INR 136 crore on

pharmacy alone, which is just on the direct platform. You combine the Amazon platform as a part of that, we are very much on course. So 30% growth in GMV, like I said last time, around INR 1,000 crore is when we expect this breakeven to happen.

Tushar Manudhane: Got it. Considering the cash breakeven for Apollo 24/7, maintaining the EBITDA margin in the Healthcare Services side. So if I sort of combine this at a consol level, so what kind of EBITDA growth one should think of for FY '26?

K. Akhileswaran: I think, as you know, we don't guide you for any EBITDA growth. You are right in your assumption that you will see EBITDA expanding because of HealthCo clearly helping us in the overall EBITDA. You will have to model both the EBITDA separately, and you can look at the numbers. As we said, at a broad level, we would like to maintain our Healthcare Services numbers, including the new hospitals for the coming year.

Tushar Manudhane: Okay, thank you.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just a question is on hospital growth. If you could guide us on how we should look at growth for the current fiscal, fiscal '26? And also, if I were to look at Q4, actually growth slowed down for the Hospital business, 10%. You attributed at the start to 2% from Bangladeshi patients. Is it the one that has to have the most impact, if you could clarify.

K. Akhileswaran: That's correct. If you look at the overall hospitals growth, Bangladesh, the impact was pronounced in Q3 and very pronounced in Q4. Clearly, the Bangladesh business has for the whole year impacted our revenues by at least INR 100 crore and which is 1.5% for the full year. And somewhere, if you look at our EBITDA margin also, had Bangladesh been there with us, we would have been in line with what we had guided to you of at least closer to another 50 basis points on the EBITDA as well, could have been possible had Bangladesh continued, which was in line with what we had committed to all of you a year back.

So going forward, I think we have always said that we would like to look at it organically at a low -teen growth is what we would like to focus on low- to mid-teen on the organically itself on the Healthcare Services. And the potential for new is all these beds coming together, it will start contributing significantly from FY '27. And we are hoping that in FY '27, we can add over INR 1,000 -plus crore of revenues from the new hospitals itself.

Shyam Srinivasan: So Krishnan, there is no mitigation for Bangladesh, right? So we should assume it's not coming back in '26.

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K. Akhileswaran: I think we have mitigated it with the higher CONGO -T and other things. Madhu, if you want to chip into that Congo -T revenues.

Madhu Sasidhar: Yes. So a couple of things that we have done, we have focused on the quality of our revenue. As Ms. Suneeta alluded to earlier, there's a much higher contribution of CONGO specialties to our revenue. We have also pivoted to other international markets, and we continue to do that. Those markets have increased substantially year-on-year, and we will continue to focus on those markets for next year as well.

Suneeta Reddy: And also in terms of local market share, I think that we are seeing improvement in local markets and the fact that the quaternary care hospitals that we have in the metros are really bringing in patients from surrounding markets. And this is why we are witnessing a high o

ccupancy in these markets. So we will continue to work with the surrounding localities, and the cities around us, and this flow will continue into the next year.

Shyam Srinivasan: Helpful. The second question is on the 80 basis points and 60 basis points you called out. So I am not sure that you articulated what are the cost measures and I think 60 basis points is ARPOB increase, like I think the Congo and other things. So, if you could just qualitatively give color on what are we cutting on costs and what are some of the improvements we foresee on ARPOB?

Madhu Sasidhar: Sure. I think we spoke about this a couple of calls back. We have invested substantially in technology. And now we are starting to see the application of that technology and improvement in productivity. So, some of these are related to how we deploy our workforce and improve the productivity of the workforce. We are being very careful with how our expenses on materials costs and our understanding how we can improve sustainability as it relates to materials, when we take out the high complexity cases. So I think what is important to keep in mind is that while the complexity of our cases has increased, we have still, despite that, been able to expand our margins. So we are very focused on both the material cost as well as the productivity of our workforce.

Shyam Srinivasan: Thank you.

Moderator: The next question is from the line of Abdulkader Puranwala from ICICI Securities.

A. Puranwala : A couple of questions on your HealthCo business and on Keimed. So, starting with Keimed, I think this year, we have seen close to 22%, 23% kind of revenue growth, while the margins on a year -on-year basis have dipped a bit. I like to have your thoughts on what has happened in Keimed, which is driving this kind of growth. And while we were expecting some better margins, the reason for this margin slip to happen on a Y -o-Y basis?

K. Akhileswaran: Sanjiv, would you have visibility now?

Sanjiv Gupta: To an extent, we can give answer to this question, sir. So, I think Keimed, you saw a slight dip in the margin you know for or EBITDA for the current fiscal year or for the previous fiscal year , only because of the reason that onetime expenses related to the entire acquisition of various subsidiaries into the Company as well as you know the

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overall there were a little bit of higher legal fees for the year. So that is one of the reasons that has knocked out about a few basis points. This is one point. And the second is that I think during the year, we also saw a certain mix undergoing change and resulting into a few drop. But as we move into this year, I think the current fiscal year, you would see a better margin, not only by virtue of the onetime or t he things not being there in this year, but also a focus on certain different categories of the products would also mean better EBITDA margins for the Company . So certainly, from this fiscal year, you would see we are going back to the better EBITDA profile.

A. Puranwala: Understood. And secondly, on the annual CAGR guidance of close to 24% from 25% to 27 %; - while this year, I think our Pharmacy business would have grown close to 16%, 17% and top of that, Keimed would have added. If you could throw some light on how are we planning to achieve that 24% growth for this combined business, taking it to close to INR 25,000 crore?

Sanjiv Gupta: Yes. So a couple of things. One is that while we still believe that we will be able to hit about INR 25,000 crore in FY '27 , but at this stage, the clear visibility comes around INR 24,000 crore. I think somewhere in the Q3 or Q4 of FY '27, the run rate should be in the range of about INR 25,000 crore. And as we said in the previous calls also, the

growth across all the businesses are very strong. When it comes to Keimed, we grew nearly 20% and a variety of reasons for that growth coming in over there. As far as the pharmacy front -end business is concerned, last year, we saw a slightly dip in the growth only because of the reason that a couple of things happened ; - one is that we had the general elections in the country. So many of the approvals took time. Secondly, we had most of the stores coming into the fag end of the year versus equal or a symmetrical quarter -on-quarter increase or new openings. So I think those things have passed , and some bit of private label work that we saw happening in Q4. I think current year, as we see the Pharmacy business, the front -end business should be able to grow anything upwards of 20%. And of course, Digital business, Mr. Madhivanan talked about a growth of about 25% to 30%. All this put together, we strongly believe that we should be able to hit a number close to INR 24,000 crore for FY '27. And somewhere in Q3, Q4 of FY '27, the run rate should be hitting or crossing INR 25,000 crore.

A. Puranwala: Understood , thank you.

Moderator: The next question is from the line of Ashish Kumar from Ampersand Capital investment Advisors .

Ashish Kumar : I only had a small clarification regarding Bangladesh you have spoken about. So, could we say that the worst is over and on the base of FY '25 would not have much impact next year?

K. Akhileswaran: So you will see one more quarter impact, which will continue in Q1 , because clearly, Q1, we didn't see any impact in last year of Q1. So the impact has been most pronounced in Q4. And after that, you should see it go away.

Ashish Kumar: Okay , thank you.

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Moderator: The next question is from the line of Dheeresh Pathak from WhiteOak Capital.

Dheeresh Pathak: Just to clarify, so in the presentation, you show ESOP charges only under 24/7 and online pharmacy distribution. So in the firm, only employees working on this vertical are getting ESOPs , and senior employees in other verticals are not getting ESOPs?

Suneeta Reddy: No, no. We do have a plan for ESOPs for senior employees , and for our doctors. So

there is a plan, I think, when the first quarter is over, we will announce that plan.

Dheeresh Pathak: But as of now, the last so many quarters, I see only ESOP charge under that vertical. So as of now, only those employees in the firm are getting ESOPs.

Suneeta Reddy: See the Board has basically cleared a plan. We will work on the implementation and the impact and share it with you the next quarter.

Dheeresh Pathak: Understood. And just a second clarity on this. The ESOPs when the chart that you showed, these are ESOPs given of the listed entity or the HealthCo, which is now merged with Keimed. So which company's shares are they given?

Suneeta Reddy: HealthCo.

Dheeresh Pathak: HealthCo, okay, understood. And one more question, so in the HealthCo, there is online pharmacy distribution, and there is offline and online distribution, and then 24/7. So when I just look at the EBITDA for the full year, there is a Slide # which talks that pre 24/7 cost, EBITDA is INR 140 crore right? Now this INR 140 crore would have a take rate on the INR 3,000 crore GMV and would have EBITDA margin on online pharmacy distribution.

Now based on earlier conversations, if I take out the take rate on INR 3,000 crore of GMV on the platform, then there isn't much EBITDA left for online pharmacy distribution EBITDA. So is that fair understanding? And how are you allocating the cost? Because see, you have an offline distribution and online distribution sharing a lot of infrastructure common among the two. So how are you distributing the cost between off-line pharmacy distribution and online pharmacy distribution?

Sanjiv Gupta: Yes. So see, it's a common platform for the entire thing, as you rightly said, that the same platform is working towards fulfilling pharmacy, diagnostic, consultations, insurance now and subscription packages, so on and so forth. So it is extremely difficult to allocate expenses for 5, 6 verticals which are firing as of now. This is the first point.

Second, the way we have presented in the earnings call also. What we look at is that an overall year basis, we had revenue of about INR 1,080 crore with an EBITDA margin of 13.1% for the last year versus 11.4% of FY '24. So, there is approximately 200 basis points of upward movement. And we have been guiding on this point that 13.1% in this year should be in the range of anything upwards of 17%, 18% with addition of insurance, with addition of various other levers of growth in margin that the Company is working on. I think we believe that is the way to look at it. However, in case you would like to understand the allocation, maybe we can take this point offline and work out something and then can discuss it with you for sure, sir.

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Dheeresh Pathak: Yes, would love to do that. Thank you so much.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark.

Harith Mohammed: Can you clarify the timelines for completing the Keimed merger?

Suneeta Reddy: 15 months.

Harith Mohammed: 15 months from now, okay. And the guidance that you have given for FY '27 for the combined business, Apollo HealthCo plus Keimed, you clarified on the top line part.

But on the margins front, the guidance is 7% to 8% for FY '27. I am assuming it's the exit rate. But from the current level of around just over 3%, that's a significant expansion. So if you can give some color on the drivers for this?

Sanjiv Gupta: Yes, let me just take this question. So, see, if you look at today, when we closed Q4

or the full FY '25, we had 3.2% and our guidance is 7% plus. Now this 3.2% also carries ESOP cost as well as the operating losses at 24/7. And in case, as I said, that ESOP cost is going to be tapering down by almost 1/3, they will come down to 1/3 from next year onwards, which is FY '27. And digital losses will also be made breakeven; we believe that our revenue growth as well as various other things that we are doing in the digital segment would make the digital P&L breakeven. If I remove these two things, if I assume these two things even today and remove these losses and the ESOP cost, the new number that comes for the EBITDA percentage is 6.4%.

So that means clearly, once the digital losses are off, the ESOP cost is rationalized to a lower number, we should be seeing blended with offline pharma and Keimed to be in the range of about 6.5% to 7%. This is one point. The second important point is that themselves, both these businesses are also bettering their gross margin, net margin and the expenses line, resulting into the EBITDA percentage being better.

While offline pharma, we are seeing 7.4%, there's a clear room of expansion of another 100 basis points over there as we progress into next year. And in the Keimed also, since the last fiscal year, we had one-off expenses, which I talked about a while ago, we believe that at least 40 to 50 basis points upward movement in Keimed

EBITDA is also on to the table. These two things put together, plus the digital, I think 6.4%, or 3.2% blended should certainly be hitting a 7% number.

Harith Mohammed: Okay. Just to confirm, the breakeven that you plan to achieve in online pharmacy distribution and 24/7 by the end of FY '26, that's excluding ESOP costs?

Sanjiv Gupta: That's right.

Harith Mohammed: And then my second question is on, the 24/7 GMV, which

is around INR 800 crore for

the quarter, online pharmacy. And within online pharmacy, how much is prescription drugs versus, let's say, OTC, FMCG type of formats, which I believe are more vulnerable to quick commerce competition?

M. Balakrishnan: Our ratio is roughly 80 -20. The sense that our Rx medicines and other part of the OTC products contribute around 75% to 80%. Health essentials, FMCG business like moms and babies, et c., contribute another 20%. Like I told you, we did see a blip in the last Q2, Q3, but we are back to our normal numbers. In fact, we are seeing many more categories opening up such as nutritional supplements, which will work out . So in spite of the quick commerce , we should be able to hold on our numbers.

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Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Just following a little bit on the previous question. On the 24/7, you talked about 17% to 18% EBITDA margins. So on the operating expenses, how do you see the operating expenses playing out? They were about like INR 480 -odd crore this year.

Do you see them coming off in absolute terms?

Sanjiv Gupta: So I think certainly, there is a room of reduction for about 15% to 20% on the expenses. So I strongly believe that while the expense for the whole year has been in the range of about INR 480, but current fiscal year, we should be seeing anything less than INR 400 crore or maybe INR 400 crore to INR 425 crore. That should be the number on the expenses side. And you would appreciate that running a business would require certain expenses. And obviously, beyond a certain point, expenses will not fall down. And I think the overall business that we have created so far, the overall investment that we have done into the branding, technology and lot many other things will help us scale our business. Insurance will help us build our margin profile. And expense lines, anything around INR 425 crore , we should be able to bring it down too, and that should help us hit the breakeven in Q3, Q4.

Nitin Agarwal: And this is a number which stays more or less with inflation increases going forward? That's the way to think about it?

Sanjiv Gupta: Yes.

Nitin Agarwal: And from a GMV to revenue conversion, what should we look at with the new services as a number?

Sanjiv Gupta: So currently, we are at roughly 37%. And I think my estimate says that we should be able to hit closer to 45% to 47%. That should be the revenue to GMV conversion ratio.

Nitin Agarwal: Actually, We are looking at 45% to 47% GMV to revenue conversion, margins inching closer to 20% in FY '27, and operating expenses staying around INR 420 crore, INR 430 crore level. That's the way the P&L should shape up for 24/7.

Sanjiv Gupta: Yes. Absolutely.

Nitin Agarwal: And sir, secondly, on the Diagnostic business, there's been some Y -o-Y slowdown in this business, clearly, at least especially in the last quarter. And there has been a very sharp improvement in the Specialty Clinics business. So, two things. One is, if you could explain these two. And third, what is the overall plan for AHLL? Strategically, where does it fit in the overall scheme of things for Apollo?

Sriram Iyer : So I think I will take your first part, yes, in diagnostics, there has been a slowdown. I think one of the things is we had to reset the model of our franchisee business. And I think for that, we had to do some changes with respect to the pricing and the commission modeling. So obviously, we will do that change. And hence, there was a bit of a slowdown. but we have successfully managed to build a model which is more sustainable and profitable going ahead. And I think the entire last 4 quarters, there was a lot of work around different channel

s and getting the cost under control. If you

look at diagnostics over the last 2 years, we have improved EBITDA by 4.5%. So that was a journey that was taken. And I think now we are done with that, and you will get

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to see starting this quarter, a high -teens growth in terms of diagnostics. That's point

one. The second point, you are right. On clinics, definitely, our focus is on working very closely with hospitals and really driving ProHealth and the specialty checks by a further number. So we have done a very good volume growth last year, and we are continuing to see a very good uptick in our ProHealth volumes even in this quarter. So that will continue to be our focus on clinics. I think with respect to AHLL, while we have a range of business units and each one of them has a separate target segment, we are really doubling down on diagnostics and on primary care clinics. And as I said, clinics is all about being in the catchment, working closely with hospitals to drive the Ips, and of course, drive more preventive health in that particular catchment. So we are confident about driving the high -teen growth as we start this year.

Suneeta Reddy: Just to add a line on Clinics, this is probably the only conceptually clinics that are created with GPs and some specialists that will look after primary care. Probably the only model in India that has this primary care focus. So we believe that in the long term, we will be able to take it to scale, get better revenues, but funnel into AHLL will also be significant.

Nitin Agarwal: If I may last one on that. On the Diagnostic business, your peer set is around in the mid-20s EBITDA margins. Where do we see our Diagnostic business really ending up over a period of time and the next 3 years there about?

Sriram Iyer: So definitely, I think we are also looking to a number of 20%. I think we are about a couple of years away. But steadily, as I said, we will be looking forward to grow about 2% to 3% this year on our diagnostic margins. Now compared to our peers who have already expanded and have a full volume, our focus this year is to drive volume growth, because we have our significant expansion right now in this financial year. So, when we go and expand then the volume comes in. And the operating breakeven is about 18 to 24 months for every new lab that we expand. So we definitely have a target of 20% in mind. That is a couple of years away. The focus is on driving volume growth; focus is on improving lab expansion and on sustainably growing volume and profitability.

Moderator: The next question is from the line of Prashant from Unived Corporate.

Prashant Kshirsagar: Can you share the timeline for the Company's Gurgaon project, a reference of which was made in the opening remarks? And secondly, can you share the progress made in the Worli project, Mumbai, which was announced earlier?

K. Akhileswaran: So the Gurgaon project should be more in Q4, more towards the end of Q4, around March is when we are expecting that to be commissioned. And that would be the time frame when we should look at even the hospital in Hyderabad around that same time frame. Worli, we are getting all approvals, and we are hoping that we should start to dig ground after the rains. That's the planning.

Prashant Kshirsagar: Okay. And my second question is regarding the associate Company, Indraprastha Medical. Ma'am, in the earlier conference call, you had mentioned about the capital expenditures to be made in that. And has the capital expenditure been completed? Or is it in progress as of now?

Suneeta Reddy: No, it's in progress. And that will be made by Indraprastha Medical.

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Prashant Kshirsagar: So you had mentioned earlier about parking, neuro center and the expansion of rooms. So which part is pending in that?

Suneeta Reddy: I think we have not yet started, but we have all the drawings and it's in the process of getting the approvals.

Prashant Kshirsagar: You have not got the approval for capital expenditure, okay.

Suneeta Reddy: No, no, we have got approval for capital expense. We have got approval for FSI. We are just going through the submission of drawings.

Prashant Kshirsagar: And what should be the time line for that then, for the expenditure?

Suneeta Reddy: It should take 2 years.

Prashant Kshirsagar: Okay. 2 years from now, from May '25. Okay, thank you.

Moderator: Next question is from line of Sriram, an individual investor.

Sriram: My question is on the market structure of the pharma industry. Both retail and distribution today are fragmented. So where do you see the long-term trends for each of them? Or what is the level of consolidation that can happen in India?

M. Balakrishnan: So, two things. Let me handle the online business first. So like I said, online today contributes only 15% to our own Pharmacy business. And if you have to take the much larger industry, it continues to remain fragmented, there are only 2 or 3 major players besides us, one is Tata 1mg and PharmEasy. All of them are in a consolidation mode. So this is where I think the Apollo Pharmacy story plays out to its advantage of looking at a very strong omni model.

And internally, we would like to keep a target that which we can be focused on, which is how does the 15% of contribution to the overall Pharmacy business moves up to the range of around 30%. I think that's when the significant our ability to retain customers, offer an omni proposition will work.

As far as the Physical business is concerned, the Apollo Pharmacy story still plays out. It's 6,600 -plus outlets, the footprint that we are creating. There are still some markets which are available such as the western part of the country, to some extent, the North, which we will continue to grow. We almost open one outlet a day. So we are still looking at anywhere between 375 to 500 -odd outlets is still in the plan. So we see this consolidation happening. In between, we do keep getting offers, other offers or other chains, which we evaluate very, very carefully. But at this point of time, we are not seeing any major synergies which could accrue given our road map. So that's something very much not there.

We still feel that this business will continue. If you have to look at our growth itself, it's 17%. But if you look at only Rx, Rx was actually growing at more than 23%, 24%. The degrowth or the slowdown in the growth actually happened on the FMCG and other health essential side. So, we are very bullish that the Rx business will continue, and we would stick to our plan of opening up stores, increasing our digital penetration and looking at the synergies between these two businesses. And now with Keimed also

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coming into the scope, there is enough of supply chain synergies, which we should be able to take advantage of.

Suneeta Reddy: There has been a slowdown in the FMCG sector that has been there throughout the country. So I think it was reflected in AHIL's performance.

Sriram: Okay. So, my question is, - so there are about 600 stores for you and the organized chain, correct me if I am wrong, might be less than 20,000, right? But then there are about 9 lakh pharmacies today in India. So I am just trying to understand the big picture, let's say, in terms of the total addressable market for the offline channel, how many store count can we have,

let's say, 5 to 10 years down the line?

Obul Reddy : As we have already informed the market, we have been adding about 600 stores every year, and we will continue to add that. But as you rightly observed, organized market, our share today is very insignificant against the total market of 9 lakh retailers. It will take a long time to get a market share of that. But today, given our scale, we are at about 8% of the market. We should aim for 20% in the next 5, 6 years with the network addition, and also entering into the new geographies where we are not present. And for your other question, - Distribution business will remain separate from the retail, because distribution business is geographically organized in India, and we don't see any shift happening in that. So that business continues to be with its character separately.

Moderator: The next question is from the line of Rhythm Sharma from Deloitte.

Rhythm Sharma : My question is regarding Apollo Cradle, the Mother and Child segment of Apollo. I just wanted to get an understanding what would be the current revenue mix like within the Gynecology, Pediatrics or any other segment that we offer? And what is the growth rate that we are seeing for the Apollo Cradle and expansion pipeline, whether it would be a greenfield or brownfield projects, what are we aiming at?

Sriram Iyer : So basically, if you look at our current mix, our current mix is primarily around OBG and around women's gyn ecological surgeries and so on and so forth. Our mix of pediatrics is quite low right now. But we are happy to share that we just inaugurated our latest flagship Cradle Royale in Electronic City in Bangalore a couple of weeks back, where we have launched the PICU setup. This got the pediatric ICU, and we will be doing a lot of pediatric cases there. And all the complicated pediatrics will then be seamlessly handed over to Apollo Hospitals. So, I think in the coming year, we will expect the pediatric mix to grow about 10%. Right now, it is primarily focused on OBG and gyn. Alongside said that we have around 12 Cradles pan -India and electronic city is our newest addition. At this point of time, we are really focused on driving higher utilization. We are present mostly in Bangalore, Delhi, Hyderabad and Chennai. And these are the cities that we wish to stay within Cradle right now and drive higher traffic for both OBG and as I said, for all the women -related gyn ecology and other hysterectomies we typically do in Cradles.

Rhythm Sharma : Thank you for this. Just a follow -up question on the same. What would be like if I talk about the doctor payout structure and the corporate OD, what would be the major contribution in the revenue segment? And if you can highlight what would be the payer mix that you can see even in this particular structure right now?

Sriram Iyer : So you want to know the doctor payouts as a percentage of this one, is it?

Rhythm Sharma : Right .

Sriram Iyer : Can you repeat that?

Rhythm Sharma : Yes, I just wanted to confirm about the doctor's payout structure. Like within industry , we can see a particular rate of 30% to 50%. So how does Apollo Cradle have this particular structure and also to discuss about the corporate expenditure within this particular segment?

Sriram Iyer : Yes. So basically, as you know, in this industry, there's a mix of structure that we have. We have full -time doctors, and we also have consultants who come and operate. So , we have a structure where there's a mix of both the structure that operate . And you are right; our payout structures are around 20% to 30%. Those are the numbers at a weighted level average. These do differ by city. And as I said, these do differ whether we have a full -time doctor, we have a

visiting consultant or we have

a FFS model. So, we have all these 3 models working out. And our endeavor always is to have the best, and we continue to be the best brand. I am also happy to share that we just received the Times ranking award yesterday for being the best Cradle for third year in a row. So Cradle continues to be the most loved brand in the Mother and Child segment in the country.

Rhythm Sharma : Thank you so much for this.

Moderator: The next question is from the line of Abhishek Agarwal, an individual Investor.

Abhishek Agarwal: Yes. So my question is around the brownfield expansion, specifically the Secunderabad brownfield expansion. So in the disclosures for the 80 -bed capacity addition, the project cost is estimated to be INR 545 crore, which comes around INR 7 crore per bed , and is way higher than the...

Suneeta Reddy: No, no.

K. Akhileswaran: No, it's INR 54 crore. I think there is an error in the Slide there , in the earning slide.

Abhishek Agarwal: Yes, yes, Slide # 24, yes.

Suneeta Reddy: Yes. There is an error on slide no . 54.

Abhishek Agarwal: Understood.

K. Akhileswaran: Sorry. We will correct this.

Abhishek Agarwal: Understood , no worries. Thank you so much.

Moderator: We will take the next question from the line of Madhav Marda from FIL.

Madhav Marda : Just one question from my side. On the 24/7, when you spoke about the margins going to 17% to 18%, that was in FY '26 itself or that's more like by FY '27 , because I think the number I am referring to is 13.1%, which we did in FY '25.

Sanjiv Gupta: Yes, this should be in this year. This increase should happen in this year, yes.

Madhav Marda: Okay. And sir, the sales to GMV, you said can get to 45% or 40% to 45%. I missed that number.

Sanjiv Gupta: Should be near 45%.

Madhav Marda: That is also this year itself?

Sanjiv Gupta: That's right.

Madhav Marda: Okay. That's a pretty sort of accelerated jump in profitability. So that's all being driven by the insurance vertical scaling up or there are other initiatives that you could flag , because that's a pretty sort of solid improvement in profitability, right?

Sanjiv Gupta: No, I think 2, 3 things. And Madhi, please add in case I am missing out anything. So, one is the insurance, which is obviously a high margin profile business. Secondly, we also started working with many pharma companies to get the app monetization done. So digital inventory is being suitably used for showcasing FMCG and OTC brands and so on and so forth. So that's the second one. Third, there is an overall reduction in discounts that we believe while partnering through some of the Banks on the card side, the payment mode side. So , these three things put together should give us a very high strength on the margin, plus our own margins on the Pharmacy, Diagnostic and the Consultations and the current set of business streams has also evolved. They are better for the current fiscal year versus the previous year on the strength of the scale. So all these things put together should help us get a better margin profile for the current year.

M. Balakrishnan: Sanjiv, has summarized it pretty accurately. I think all the 3, Insurance business will obviously have a big chunk, but the top line at this point of time, the GMV for insurance is not significant. By the end of Q4, we should be in a good position. And what we

highlighted, there is a very strong focus on monetization of the various digital assets that we have. That's getting us some very good traction. So we have created some unique digital assets on the lines of what quick commerce and Amazon do. So that's the second contribution. And our partnership with the Banks and other financial institutions effectively helps us

keep our discounts under control, while increasing the GMC. And third, our omni proposition that we do between both offline and online has been growing at a very good speed. We will keep enhancing that proposition. And this is all in addition to the margin expansion that we expect to happen in pharmacy as we go towards breakeven.

Madhav Marda: Got it, thank you.

Moderator: The next question is from the line of Vivek Agarwal from C ITI Group.

Vivek Agrawal: Actually, my question is related to Northern region as far as your Healthcare business is concerned. So what I understand is that this is the region where impact of Bangladesh patients should be minimal. But despite that, the revenue growth is slightly subdued. If you look at this quarter, it's around 9% and full year it's around 11.6%. So what explains this? Although in some of the other regions, you are continuing to do well.

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Suneeta Reddy: I think north Q4 is traditionally subdued because of the winter months. So there is definitely, if you ...

K. Akhileswaran: So especially in Lucknow and those regions, we see Q4 has always been a cyclical low, and also there was this effect of Kumbh this time in the quarter. So it was a combination of both that we saw an impact. But otherwise, we are back on track in Q1.

Vivek Agrawal: Understood, sir. Thanks.

Moderator: I now hand the conference over to the management for closing comments.

Suneeta Reddy: So thank you, everyone, for joining the conference, in spite of this being a Saturday. But as you know, Apollo works 24/7. As we conclude our discussion on Q4 FY '25, we are pleased with the continued momentum across our core and emerging businesses. Our integrated healthcare ecosystem, which includes hospital, diagnostics, pharmacies and digital platforms continues to deliver on both growth and efficiency.

We are making steady progress on our expansion road map, with multiple new facilities set to be commissioned over the year. In Apollo HealthCo, we remain focused on driving profitable growth, and the team is firmly on course to achieving breakeven in the Digital segment as guided.

The Keimed merger process is also progressing well, which will further enhance scale and margin resilience in our Pharmacy business.

We thank you for your continued interest and support, and look forward to updating you in the coming quarters.

Moderator: On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Disclaimer: This document is a transcript and may contain transcription errors. While the transcript has been edited for clarity, the Company takes no responsibility for such errors. Efforts have been made to ensure a high level of accuracy, and any figures that may have been inadvertently mentioned have been reviewed and corrected as necessary.

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Apollo Hospitals Enterprise Limited

Transcript of Q 1 FY26 Earnings Conference Call

August 13, 2025

Moderator : Ladies and gentlemen, good day, and welcome to the Apollo Hospitals Limited earnings conference call. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" and then "0" on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India.

Mayank Vaswani : Good afternoon, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals, to discuss the financial results for the first quarter of FY26, which were announced yesterday.

We have with us today the senior management team, represented by Mrs. Suneeta Reddy – Managing Director, Mr. A. Krishnan – Group CFO, Dr. Madhu Sasidhar – President and CEO of the Hospitals Division, Mr. Madhivanan Balakrishnan – CEO of Apollo HealthCo Limited, Mr. Sriram Iyer – CEO of AHLL, Mr. Sanjiv Gupta – CFO of Apollo HealthCo Limited, and Mr. Obul Reddy – CFO of the Pharmacy business. Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks, and uncertainties on Slide 2 of the Investor Presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks.

Suneeta Reddy : Good afternoon, everyone, and thank you for taking time to join this earnings call. I believe you would have received our earnings documents, which we shared earlier yesterday.

We are pleased to start FY26 on a strong footing with a healthy momentum and double-digit growth across all three verticals : - Healthcare Services, Apollo HealthCo, and Apollo Health & Lifestyle. The performance in Q1 reflects consistent execution across key geographies and business segments, resulting in broad-based growth, resilient operating metrics, and improved profitability.

Consolidated revenue grew by 15% on a year-on-year basis to Rs. 5,842 crore. Within this, Healthcare Services business has delivered 11% year-on-year revenue growth to Rs. 2,935 crore. Revenue from insurance patients saw year-on-year increase of 13%, while revenue from cash patients grew by 16%. Collectively, these segments accounted for 84% of our inpatient hospital revenue.

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Surgical revenues grew by 14%. Our focus on specialties, cardiac, oncology, neurosciences, gastro, and orthopedics, which is strong, maintained a strong momentum, growing revenues by 13%. Group-wise occupancy stood at 65% in Q1 FY26.

This quarter, we are announcing a change in our reported metrics. We have traditionally reported average revenue per occupied bed. From this quarter, we propose to discontinue this metric from our report. ARPOB blends pricing, length of stay and occupancy into one figure. Higher ARPOB can sometimes reflect higher operational efficiency like higher acuity mix and better bed turnover rather than fewer patient revenue. Growth in ARPOB can result in due to reduction in ALOS or increase in occupancy which are purely operating metrics and are not related to realization per patients. Unfortunately,

health sector stakeholders were sometimes misrepresenting ARPOB growth as price or tariff growth to consumers and the public at large and implying that hospital inflation is higher than it actually is. This narrative ignored the impact on cost of advancements in technology, therapeutic procedures, such as robotics and minimally invasive procedures, which significantly improve patient outcomes. Instead, we draw your attention to average revenue per patient, ARPP, which we believe is a more accurate measure of our realization. It isolates what we are able to realize per patient, irrespective of how long they stayed or how efficiently

beds were used. It reflects true pricing path and case mix strength. More complex cases leads directly to more ARPP.

ARPP in Q1 FY26 was Rs. 172,282 and grew by 9% through a combination of better institutional tariffs, case mix and regular tariffs increase for inflation.

Moving ahead, Apollo HealthCo reported revenues of Rs. 2,472 crore, marking a 19% year-on-year growth. Apollo Health and Lifestyle revenues increased by 19% to Rs. 435 crore during the quarter.

Consolidated EBITDA was at Rs. 852 crore, registering an increase of 26% year -on-year. The Healthcare Services EBITDA was at Rs. 718 crore, registering a growth of 15% year -on-year. Healthcare Services margins remain robust at 24.5 %, a year -on-year increase of 88 basis points.

The Pharmacy Distribution business in Apollo HealthCo recorded an EBITDA of Rs. 167 crore, as against Rs. 139 crore, higher by 20 % year -on-year. 24/7 EBITDA losses were at Rs. 73 cr ore, significantly lower than Rs. 116 crore in Q1 FY2 5.

This is the result of conscious work on cost management and closely tracking unit economics in each line of service. As a result, Apollo HealthCo reported an EBITDA of Rs. 94 crore in Q1 FY26 , up from Rs. 23 crore in Q1 FY25 . This is an excellent outcome, and we are well on track to achieve break -even in the digital business by the end of this fiscal.

Apollo Health and Lifestyle delivered an EBITDA Rs. 40 crore, representing 31% year-on-year growth, with margins improving to 9.3 from 8.4 in quarter 1 last year.

Consolidated PAT was at Rs. 433 crore, g rowing 42% year -on-year.

Within Healthcare Services business, we have delivered a ROC E of 38.4, with balanced ROC E across our geographies, the metro, Tier 1 and Tier 2. Private label

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and generics revenues were at 14.6% of total pharmacy revenues. Our digital platform 24/7 added 1 million new users. The platform GMV was at Rs. 682 crore, representing a growth of 23% over the same period last year.

We are making good progress on our hospital capacity expansion plans, our dedicated women's oncology center in Delhi, multi -specialty hospital in Pune, acquired hospital in Bangalore, and our multi -specialty hospital in Kolkata will be operational in FY26, a capacity addition of 700 beds. Plans on others and ongoing projects remain on track with phase d operationalization in the coming quarters. The strategic reorganization of our omni -channel pharmacy and digital health business is proceeding as planned , and we expect to complete all steps within 18 to 21 months, a timeline that we had earlier announced. The scheme is expected to conclude by the end of FY27, by which time the business is expected to achieve a run rate of Rs. 25,000 crore of revenue with 7% EBITDA margin on a combined basis. Our Q1 FY26 performance underscores the resilience and agility of our integrated healthcare ecosystem. We have continued to advance our strategic agenda, deepening our leadership in core healthcare services, scaling our digital retail platforms, and further enhancing patient outcomes through clinical excellence and innovation. We are well -poised

to capture emerging opportunities and sustain our growth momentum through FY26. Our unwavering focus remains on delivering long -term value for patients, partners, a nd shareholders.

On that note, I would like to hand over to the moderator for questions . I have with me Krishnan , our CFO, Dr. Madhu Sasidhar , CEO of the Hospital Division, Sriram Iyer , CEO of AHL L, Madhivanan , CEO of Apollo HealthCo, Obul Reddy and Sanjiv from Apollo HealthCo with me here to take all of your questions.

Moderator: We will now begin with the question -and-answer session. Our first question comes from the line of Damayanti Kerai from HSBC .

Damayanti Kerai: Congratulations for the good set of numbers. My first question is on 24 /7 where we have seen significant improvements in profitability. So, as we can see, our operating costs, excluding ESOPs, were down almost 16% quarter -to-quarter. And I believe that is the key reason for better profitability. But at the same time, when we look at GM V for the platform, it is down around 14%. So , how should we look at your GM V growth ahead? And then you mentioned you will be cost neutral by end of this fiscal itself. So, I just want to have more clarity on how should we look at the GM V for 24/7 platform. And additionally, if you can update on some of the newer services which were added to the platform like insurance and corporate tie -ups, etc.

Madhivanan B: Thank you for the question, Damayanti . So, two things. Your assumption is correct. On the cost front, one of the biggest things which was driving cost was our unit economics, which is a combination of our customer acquisition charges, the kind of

discounts and the lifecycle cost that we incur , and the cost of delivery. At a broader CM1 marginal contribution , we are almost at break -even for pharmacy. And this is what contributes the biggest. So , a big chunk of the cost reduction that you are seeing is coming from there, which is helping us. Over the next two quarters, our pharmacy would be way above the line in terms of unit economics. And therefore, we

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will be in a much better position. And this trajectory continues ; - we continue to keep spending less on marketing , our customer acquisition is growing at a good healthy pace of around 200,000 new customers coming to the portfolio and we are able to retain our customers who come in through our very strong circle program and the constant improvement in service.

Specifically, on the GMV bit, if you had to notice, our GMV is contributed by three lines of business. The first one, driven by pharmacy. The second, diagnostics. In both these cases, the correlation between GMV, revenue and margins are very straightforward.

However, in the business that we drive for the hospitals, which is both IPOP business as well as the various consult business, compared to last year, we had a redefinition of how we generate GMV. That is why we have restated some of the GMV numbers which we can explain in detail. But what is effectively done is, earlier we used to take the total GMV which was contributed by Apollo 24 /7. Incrementally, from this year onwards, we are focusing only on the new customer business that we are generating. That is how our revenue structures are aligned. So , for the investors and the analysts, I would request that we focus a little bit more on the revenue drive as far as hospital is concerned , the hospital IPOP is concerned, both hospital as well as clinics. And for diagnostics and the GMV business will continue. So , the restated numbers will basically indicate that we have grown on a sequential basis at an 8% , whereas on a year-on-year basis, we have moved on 23%. So , it is a simple technical readjustment. The underlying business assumption

s remain the same.

And from a future perspective, you know, earlier we used to say that Rs. 800 crore to Rs. 900 crore of GMV is what will sort of take us to a break even . That number may be around by another Rs. 50 crore to Rs. 100 crore , I would say , maybe the day we start touching Rs. 800 crore , some of these things will start playing out because of this readjustment.

Damayanti Kerai: Just to clarify, you mentioned pharmacy, diagnostic, no change in how you book the numbers .

Madhivanan B: Correct.

Damayanti Kerai: But it is the IPOP, the hospital consult, et cetera where there was some adjustment. And now we should look at the incremental delta which will come in this part of the business, right?

Madhivanan B: Yes. And we have reinstated both last year. When we talk about the growth numbers, we reinstated according. It reflects the growth in a much more stronger manner.

Damayanti Kerai: So, the reflected number is on like -for-like basis, what we are seeing there?

Madhivanan B: Yes.

Damayanti Kerai: My second question is, some of the e -com platforms, I believe they have gone very aggressive in last few months in terms of stepping up their presence in not only the OTC , wellness products etc ., but on the prescription medicines supplies also . So, how

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do you see competition in this space from some of these e -com players where I see , I guess , lot of aggressive push coming in last few months ?

Madhivanan B: I wouldn't say last few months. All this action has started just around from this month onwards which is two quick commerce players jumping into the prescription business. We will watch. But in my mind, this is actually a better development for the overall industry.

Today, the digital part of the business in the markets that we operate in, especially for pharmacy, if I have to take that as a benchmark, operates at around 12 % to 15% of the offline pharmacy business. We genuinely believe in some of the , as the market matures, this number should settle at around 25 % to 30%.

Incremental players coming in with aggressive strategies, my mind will expand the digital market and give greater amount of trials. And here, I think Apollo, given its very strong trust, very strong supply chain capabilities, and the ability to offer always good quality, trusted medicines should hold on in the long run. We are not going to be in

this war for acquiring customers at any cost but build a much more stronger sustainable business under the direction of our Board ; if I look at this market expansion.

Damayanti Kerai: Yes, so, this is good for the industry, and you believe you are in a better position, you have a winning proposition in this space despite entry of more competition.

Madhivanan B: Yes. Our operating model is to build a sustainable model wherein at a unit economics level and growth will happen. And this is where the advantage of having an offline pharmacy of almost 6,500 plus outlets in sync will work much better. So, we constantly reorganize our supply chain network. We believe there is a lot of growth still available in the Tier 2 and the Tier 3 cities, and we are focusing on that as we go forward, as we walk towards our end-to-end break-even.

Moderator: Our next question comes from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: First one on the average revenue per patient growth of 9% year-on-year, if you could provide the growth drivers for this in terms of case mix, payer mix and price increase, if you can split this 9%?

Madhu Sasidhar: So, if you were to average out the tariff, which is the price mix I think you referred to, the landed effect is probably in the 4% to 5% on a year-on-year basis. So, the remaining comes from a combination of case mix,

primarily on case mix. And most of

this has been driven by our Congo specialty. So, by Congo, we refer to the cardiac oncology, neuro and gastro and orthopedics. They have independently contributed to that delta, especially gastro sciences and orthopedics, which have grown at about 16% and 17% on a revenue basis year-on-year. Even within the Congo specialties, when we look at each one of the specialty, the average revenue per patient, which is an indication of the severity and complexity of the case, that has grown in healthy double-digit numbers across each one of our Congo specialties.

Kunal Dhamesha: Sure. So, basically half and half is a way to put it.

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Madhu Sasidhar: At least half and half, yes. But on a volume basis, most of our volume growth, almost all of it, I would say, in this quarter has come from the Congo specialties.

Kunal Dhamesha: And more cash as well, right, which is typically slightly higher in terms of as compared to insurance?

Madhu Sasidhar: The payer mix growth has been highest in insurance on a volume basis. It is about 7.8% followed by our cash, which has also grown by about 5%.

Kunal Dhamesha: Thank you for the clarity. And ma'am suggested that we will be operationalizing around 700 beds. Is that the correct understanding of the 1,577 beds that we are slated to add this year?

Suneeta Reddy: Yes.

Kunal Dhamesha: And when would the next tranche of, let's say, 800 plus beds shall we expect to be operationalized?

Suneeta Reddy: The following year.

Kunal Dhamesha: FY27, okay.

Suneeta Reddy: Yes.

Kunal Dhamesha: And the last clarification from my side on this GMV change. So, in Q4, we had guided for revenue to GMV ratio of around 45% to 46% per full year versus 36% in FY25. So, now with this redefinition, is there an update to the guidance or that guidance bring in the redefinition of the IPOP, GMV revenue conversion?

Madhivanan B: No, I wouldn't change that. I think we can go with more or less that kind of a tracker.

Like I told you, we are focused more on the revenues. So, the revenues will have been, there is no change in the definition of the revenue. In fact, it will continue to grow. So, I wouldn't make any changes there and continue.

Kunal Dhamesha: And breakeven is, you said Rs. 800 crore to Rs. 900 crore revenue.

Madhivanan B: At the GMV, revenue, we should be able to, on the digital side, we will be hovering between Rs. 90 crore to Rs. 100 crore. So, once we touch that number, that is how.

What I told, Rs. 800 crore is the GMV, that is a good indicator.

Moderator: Our next question comes from the line of Neha Manpuria from Bank of America.

Neha Manpuria: The first question is on the hospital business. Now that the Bangladesh patient flow disruption is in the base, is it fair to assume that the hospital growth rate which has been in the 10%, 11% will inch up to the mid-teens along with the bed expansion?

Just trying to understand when we go back to the mid-teen growth that we are used to seeing in the hospital business.

Suneeta Reddy: Yes, Neha, it will certainly go up. I think you will see also an improvement in the existing assets as well as the addition of new hospitals.

Neha Manpuria: And this is , because if I look at the occupancy, the occupancy does not seem to be showing an improvement. Obviously, we have also seen a decline in ALOS . So, I am just trying to understand how much of it would be, you know, how will it be reflected in terms of operating metrics of occupancy? How should I look at that for the existing network?

Suneeta

Reddy: I think you can look at existing delivering close to 13%, 14%. An additional 10% coming over a period of three years from the new facilities because there is headroom for growth in the existing. So , definitely, we can look at 13 % to 14 % on existing and additional 10% coming from new beds in the next two to three years.

Neha Manpuria: And the margins on these, like you had mentioned before, there is still room for margin expansion on the existing network before we , I mean, we will have losses.

Suneeta Reddy: Yes. I think there is room for margin expansion. We have a plan to move it to 25%. And as we look at the losses from new hospitals, total impact over a two -year period could maximum be around Rs. 150 crore. And the reason why the impact will be minimal is that we do have large cash flows as opposed to earlier when we did our expansion. So, you know, strong operating cash flows and margins coming from existing and with some losses but no huge impact on EBITDA.

Neha Manpuria: And my second question is on the digital business. What would be the GMV for the full year based on this rebasing that we have done? And also, if you could give us some color on the new businesses, how they are tracking , the app monetization and insurance, how they have been tracking ?

Madhivanan B: Sanji v, can you give the projected GMV?

Sanjiv Gupta: Yes, thanks , Madhi . I will take the first question . So, we closed Q1 with Rs. 682 crore . We strongly believe that we should be hitting a number of about Rs. 3,000 crore to Rs. 3,200 crore for the full year as far as the new way of looking into the GMV. And this should represent roughly 25 % to 30% growth versus the previous year.

Madhivanan B: Yes, thanks for that. So , we will continue to grow between 25% to 30% growth. And we have seen our first q uarter on schedule. There are certain businesses which we are tightening to ensure that our revenues are on the upswing.

Specifically on the new businesses, insurance business, which I have spoken to you about, has started taking, you know, getting some legs. For this quarter, we finished with a Rs. 5 crore number as against, we are targeted actually to do around Rs. 7 crore , we did 5 crore because some of the technological integrations with the partners are still in process. As this is the first time that we are building it, it is taking a little bit of a time. But a very encouraging sign is that we are getting a reasonably large number of digital customers who are buying very strong products from partners like Niva Bupa , Care and Star Health, we have started off with he alth, life and the other two will progress.

Various of the corporate partnerships, we are working together as a one Apollo story, wherein as one full entity, we are approaching larger corporates to build a much stronger story. The SBI story has started off on the right turn. We are in very early

stages again. It is giving us good quality customers on a repeat basis. We will continue to build on these lines of businesses.

The insurance business will continue to take a little bit of more investment, both in terms of people and technology. But we are reasonably confident by the end of the year, it will not take away from our breakeven objectives.

Neha Manpuria: And what about the app monetization?

Madhivanan B: Oh, ye s. That is also doing as per track. While it might not be on the lines of quick commerce, but given very, very specific, we are working in line with people with, whether it be the weight loss drugs, some of the other mega pharmacy brands, very strong FMCG brands who are a part of the mix. That story is also picking up nicely. So, we should be on the course for monetization.

Neha Manpuria: That is helpful , si

r.

Moderator: Our next question comes from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on the hospital business . When you guide for organic growth of about 13% to 14%, we did 11% for this quarter. So just want to understand what should change ; - is the ARPP growth going to remain as high or are you going to see better volume growth in the path forward?

And just if I were to look at your occupancy for this quarter, and when I compare to the peers, I know there could be a geographical difference, but it is one of the weaker numbers. So, just want to understand for the path forward, how are we looking at even utilization improvement? And what should help us get back some of the growth? I know you talked about Congo, but maybe does it mean we have lesser occupancies when we do more Congo? So, I am just trying to see, is the business model changing? Suneeta Reddy: No, I think that there is a huge focus on volume. We have focused on Congo. I think that we, you know, having a network that is all over India, we will also focus on secondary care and therefore the volume should improve.

As you consider payer mix, I think we are strengthening our alignment with the local markets, as well as with corporates, with PSUs, and hope to see an uplift in international patients coming in in the second and third quarter as we found new markets. So, all of this should contribute to higher occupancy, which you will see in the second quarter and continue to grow in third and fourth.

A. Krishnan: Shyam, also I would like to quickly let you know that clearly there has been an uptick in our overall robotic surgeries that we are doing in the system, minimally invasive work, et c., that we have continuously focused on, which is why if you look at the ALOS, ALOS is down by 6%, right. Clearly, it is a large number compared to a 3-point, you know, and that number clearly, versus 3.23 of Q4, we are now at 3.06. Clearly, the ALOS is one of the reasons for the occupancy, which is why the ARPP is good, the margins are good. You know, it gives us headroom for growth also in our existing metros, which is good till the time some of the new hospitals come in.

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So, that is something that we are quite committed to. You know, volume growth while you are seeing headline volume is 3%. As we said, insurance and walk-ins are at 5% and 7%. And 3% is also because of the IPS. If you adjust for Bangladesh, it would have been at 5%.

So, clearly, yes, you know, there are reasons for our particular for this quarter, but going forward, we should go back into that ten growth is what we have committed to.

Madhu Sasidhar: And if I can just add to that, add to what Ms. Suneeta and Krishnan said, a lot of the occupancy number that you see has been intentional. It is a 6% reduction in length of stay, which is almost like adding 550, 600 beds, right. What this gives us is the extra capacity in our chassis to focus on supply side. So, the question as to where will this volume growth come from is that we have been very closely paying attention to that supply side, which is adding clinical talent. So, you will see some of that coming to bear, especially in our metro market, driving that volume growth. So, it will be in high-complexity care mostly, but it will be driven by some addition on the supply side.

Shyam Srinivasan: Yes, very helpful. Just a second question on AHLL. You know, it is kind of all over the place, if we can use one word, right. We have topline growth, but no EBITDA growth in some and we have, so, maybe diagnostics is one I noticed, which is looking odd. Also, I think primary care and only where we have seen EBITDA margin expansion is specialty. So, maybe just a summary on AHLL, please.

Sriram Iyer: So, I think, see, our focus has been to drive, we have seen a very good growth on diagnostics on volume. When you look at the diagnostic margin in quarter 1, I think I just want to inform that we opened the Central Reference Laboratory in Chennai on 30th April. This is state-of-the-art fully automated DigiLab spread across 45,000 square feet. So, we obviously had certain one-off validation cost, which is there on the EBITDA that is baked into quarter 1. If I normalize that, my EBITDA margin in diagnostic is at about 10.3. So, that is the number. But obviously, since we went live, there are these one-off costs that typically happen when we go live, which is more for making the lab live and controlling calibrations. Having said that, yes, we have seen a good increase in margin on specialty. And even on primary care, there has been a focus to grow revenue. And the margins will come in the coming quarters. We are confident about sustaining these growth rates further as well. Hope that clarifies.

Moderator: Our next question comes from the line of Raman KV from Sequent Investments.

Raman KV: I have two questions. I just want to understand this is with respect to the 24/7 pharmacy business. There was a strong uptick in terms of the margin pre-operating cost 24/7, pre-24/7 operating costs with respect to the online pharmacy distribution segment. I just want to understand what drove this strong uptake in terms of margin. And can you give any guidance for Apollo Healthcare from perspective in terms of revenue growth and margin improvement for the next 1-2 years?

Sanjiv Gupta: So, maybe I can take the first question. So, our margin for the quarter 1 has been

15.4% versus Q4 of 12%, which is about 340 basis points up. Usually, this is on account of the, you know, one renegotiated rates with all the service providers, which is across diagnostic and on the healthcare services, IPOP.

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Second, also, because of the better unit economics on the pharmacy side, the margin rates have been better. Plus, Q1 also saw insurance uptake, which Madhivanan talked about, that in Q1 we had almost Rs. 5 crore of insurance, while in Q4 it was roughly Rs. 1 crore to Rs. 1.5 crore, and higher upside on the insurance commission also boosted the margin apart from the fact that the monetization also helped us. So, all in all, all the revenue segments had a better margin profile for Q1 versus Q4. And sum total of that is what you are seeing is about 340 bps higher in Q1 versus Q4. Raman KV: And the growth guidance part?

Madhivanan B: So, the GM V of 3,200 that we spoke about, we continue to be on course. Like I told you in our first quarter, we are very much as per our AOP target. Typically, our new customer acquisition continues both for pharmacy, as far as our diagnostic business is concerned, we are at a new normal. So, that business is also picked up. This quarter should be good. So, we are on course for our breakeven on Q4. I would not want to look beyond that at this point of time as we look at some of these other lines for businesses.

Raman KV: And my second question is with respect to the demerger. So, currently, under Apollo Healthcare Hospital Enterprises, there are three segments. One is the hospital business. Another one is the Apollo HealthCo, which is the pharmacy business. And the third one is AHLL, which is with respect to diagnostics, primary care. So, my understanding is you are demerging the pharmacy business into a new entity. Is that correct? And can you also explain, - how will the merger with Keimed Health be beneficial in terms of synergy for the pharmacy business?

A. Krishnan: That is correct. You know, you are right in your under

standing. What will happen is

Apollo HealthCo., as you are seeing in the current form, is what is going to be demerged and will be listed separately by Q4 of FY27. By the time it will be listed, the NCLT process of merging Keimed into this business would also have come into play. Clearly, we have a proforma P&L. You can see the pro forma P&L, which has been shared as part of the investor relations, as part of the earnings deck. You would see that this current year, current quarter, we are at Rs. 4,430 crore of combined revenue, including Keimed.

Keimed supplies to 60,000 odd pharmacy stores, apart from supplying to Apollo as well. So, you will have a combination of a consumer-focused business, which is the entire pharmacy, which we are doing with the 6,500 stores. The online, digital, as well as the Keimed business. So, all the three will be part of this number. So, run rating currently at Rs. 17,000 crore of revenue, which is what we have said that by Q4 of next year, FY27, by the time that this will also get listed, we should be run rating at Rs. 25,000 crore of revenue with a 7% EBITDA margin. So, there are synergies of the Keimed business, which I will allow Madhi to chime in. The hospital-based pharmacies continues as part of hospitals, which I am sure you know, but it is something that, remember, what we are spinning out is only the retail pharmacies and the online out of that.

Madhivanan B: Very quickly on the synergies. See, while as the digital business becomes unit economics positive, and with this merger, we actually believe Keimed, besides being a part of the physical distribution business through us, there are a lot of integration

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synergies that we believe, whether it be technology, whether it be people, whether it be cost.

And what typically e-commerce and quick commerce players put it on the dark stores, today we have 22 dark stores. Once Keimed comes into the picture, we have the ability to find more synergies of setting up more economical dark stores across the country. So, our expansion beyond the top 6 cities to the next 10 and to the rest of India can become much more smoother with a greater integration between the two entities.

On the product side, we believe our ability, like somewhere in the results we spoke about, a 14.6% for PL in generics. There is a huge scope available to drive that number, not just within the Apollo chain as well as Apollo Digital, but also across the various Keimed franchises. So, technology, data, people, and product categories, all

the four synergies should start playing out as this merger comes in place.

Moderator: Our next question comes from the line of Avnish Burman from Vaikar ya.

Avnish Burman: I just have a question on Keimed . You mentioned that the HealthCo plus Keimed would reach about 7% EBITDA margin by the end of FY27. And Keimed margins, as I understand, is about 3.2% for the full year of FY25. So , I just wanted to know that how much increase in Keimed EBITDA margins are you baking in when you guide for 7% for these overall margins by Q 4 FY27?

Sanjiv Gupta: Yes, so it would be roughly 40 basis points. So , 3.1% should go up to 3.5%.

Avnish Burman: And the levers for this 40-basis point, I mean, is it coming mostly from the gross margins or operating leverage?

Sanjiv Gupta: I think majority of this will come through the scale. So , that means it will come from the margin side. But we do have the operating cost leverage as well as certain efficiencies that would come in. So , there should be some benefit from there also.

Avnish Burman: So, if I understand this correctly, 40 basis point improvement in Keimed margin is coming from you are saying both gross margins as well as the scale

benefits.

Moderator: Our next question comes from the line of Tushar Manud hane from Motilal Oswal Financial Services .

Tushar Manudhane: So, just one clarification on GM V aspect. So , you said there is redefining of GMV , and these are new customers to be taken into consideration. So, practically , if the same customer is coming for newer indication, then so effectively that will not be counted for GMV. How do you think about it?

Madhivanan B: Correct. I think you hit it right. Just let me give you an example. Last Q1, we reported a GMV of 695. That number stands at 553. It is because roughly 50% of the GMV that we drive for the hospital business is from existing customers and the balance 50% is from new customers who we bring to the overall Apollo Group , either through pharmacy or any other mechanism.

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Earlier, we used to get the revenue understanding was on the overall basis. Given the new business framework arrangement and the new agreement, our emphasis on maximizing businesses for the new customers , that is why that is the GMV that we are reporting. For the other customers, which is our existing customers, we have a servicing charge, which is much more predictable. So , this is the model that we are following. That is why the numbers. So , the GM V comes down, but the revenue story remains intact and ke eps hopefully growing at what we are present.

Tushar Manudhane: But then the GM V will comprise any which ways both, right? Either you put it as a share, or you put it as a new customer.

Madhivanan B: But for this statement purposes, because we would like to have a direct correlation between the incremental revenue , we have restated it accordingly. So , the new one primarily focuses on the new customers.

Tushar Manudhane: And only that new customer related share would reach out to HealthCo from Apollo Group, is that?

Madhivanan B: From a GM V perspective, yes. Revenue is all encompassed.

Tushar Manudhane: And that percentage, GMV, overall, combining the other aspects of GMV, the GMV to revenue ratio is 45 % to 46%.

Madhivanan B: Yes, we can maintain it. That is how we intend to bring down our cost. So , we are not making any guidance change in that.

Tushar Manudhane: Secondly, on the offline pharmacy, the margins have been pretty stable at 7.7% , while on a long -term basis increasing private label or let's say , the non -pharma product would dry, but anything further to, any scope to further scale up this EBITDA margin ?

Obul Reddy: This is at HealthCo level, it operates on a fixed model because we sell to the front -end business on a cost-plus basis. So, on a combined basis, it is fixed. This quarter, it is about 25 basis points more than last quarter. So , we should see that increase happening with the volume and scale. And one more thing to observe , last two quarters, we are at a stable private label number because of the repositioning of some products and introducing the new products. Now we have a separate head coming to drive that as an initiative . We should see the margin expansion in the coming quarter.

Moderator: Our next question comes from the line of Vivek Ag rawal from Citigroup .

Vivek Agrawal: This is related to hospital margin. Given that there is an expectation of around Rs. 150 crore of losses in the new hospitals put together over the next couple of years , so does the Company have enough room to absorb the entire losses? I am just trying to understand how to look at the Company's overall hospital margins since '26 and '27, whether they can remain stable, can move up or down , if you can help us understand?

Suneeta Reddy: I think with Rs. 2,700 crore of absolute EBITDA, Rs. 150 crore impact will not be significant. A.K., if you want

to elaborate on that.

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A. Krishnan: Yes, so we are already, as you have seen the EBITDA of the quarter, we are at Rs. 700 crore. We are averaging at Rs. 2,800 crore as we speak. It will only go up from here on. And on that scale of EBITDA of Rs. 2,800 crore, having Rs. 100 crore, Rs. 150 crore EBITDA would not be a big number. And most of these are in existing markets. And we have clear plans and visibility of doctor additions, clinicians that we want to join, we are already targeting, etc. And we will be breaking even in 12 months, as we have said, in most of these locations. So, you should see good growth coming and also very quick ramp up to double-digit margins after the 12 months also.

Vivek Agrawal: Yes, that I can understand. I am just trying to understand overall margins in '26 and '27, right? Whether that can remain stable or you are expecting a marginal dip?

A. Krishnan: It will be a marginal dip, right? 100 basis points marginal dip. But we will see expansion, as Ms. Suneeta said, the 24.5% EBITDA margins that we have reported, we hope to take it higher by 25 or 25 plus number. And from there, you will see a dip of 100 basis points. But overall growth will be visible after that.

Vivek Agrawal: And second question is related to international markets. So, first on the Bangladesh patients, so are you expecting any of the patient flow coming back? Or secondly, would you like to highlight which are the other markets, etc., that you are working on that can pick up and drive this international patient number? So, just trying to understand how to grow international patients?

Madhu Sasidhar: Thank you for the question. So, with respect to Bangladesh, the lowest volume was somewhere between November and December to early January. Since then, we have seen some increase in the volume, but it is nowhere close to the volume that we used to see at the same time last year.

But having said that, the complexity of cases, the sicker patients are coming to us. So, if you look at it on an ARPP basis, the value is higher per patient than what we used to see last year. We are actively engaged in that market, and we expect that the numbers will go up.

Outside of Bangladesh, we are very closely engaged in the markets in Africa as well as in the Middle East. We also have a strong presence and will continue to develop the markets in the CIS countries. I think Southeast Asia and further up are great interest to us. You know that we are partnering with a project in Malaysia to build a hospital. We are actively engaged in discussions in various countries, including Malaysia, in Brunei, as well as offering services in the Philippines. So, broadly, these are the markets where we expect to see significant volume through the remaining part of this fiscal year.

Suneeta Reddy: I just want to add to that that we have developed a new market, which is Iraq, and this is contributing to the growth of international patients with good healthy margins.

Currently, most of these patients are now going to Hyderabad, but there are some also coming to Chennai.

Vivek Agrawal: So, just last question here. So, what is the current contribution of international patients in the hospital revenues?

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Suneeta Reddy: Currently, it is 5% of revenue. We hope that it will be 7% of revenues by the close of the year, targeting 10% of revenues for the next year.

Moderator: Our next question comes from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: On the digital business, you know, we talked about a break-even by the end of FY26.

In your assessment, on a steady basis, once the business sort of gets into steady profitability situation, what kind of profitability this business can do?

Madhivanan B: I don't want to hazard a guess. Like you said, even at this point of time, our primary focus is to breakeven and build a sustainable model. We are typically seeing on a month-on-month basis anywhere between 2% to 3% growth. Because like I told you, our operating model does not believe in spending too much money on customer acquisition. And we still have a huge, the industry as a whole, especially the digital side, spends anywhere in the range of 15% to 17% as discounts, which the offline industry does not, which the offline sector does not drive. So, that is the second rationalization. The third bit is, there is always this challenge of increasing the average order volume to be in the range of around 1,200 bucks so that the cost of delivery is

maintained. So, I expect the growth on a year-on-year basis to be in the range of 20 % to 25%. Please do not expect astronomical growth. And if this unit economics comes through and we are able to keep our overheads at a constant level, which we are very, very bullish about as the merger plays out, because we will be able to rationalize quite a bit of our costs, then we should be able to continue to grow on a positive basis on the topline on a 25 % to 30% and the revenue will be a little bit more muted, at least for one more year. Because we also continuously invest in a lot of digital assets, which we will reinforce ourselves, whether it be our patient health records, whether it be our Ask Apo Ilo, which is our AI component. So, I would go a little bit muted on the revenue side, but we will remain profitable. Sorry, I don't want to put a number to.

Nitin Agarwal: That is understandable, thank you so much. And on the offline business, if you just give us a sense of how many pharmacies do we have, and what is the kind of growth rate, sort of annual growth rate addition that we are looking at, going forward?

Obul Reddy: We have, during the quarter, added about 120 stores, and as informed earlier, we are always planned for adding about 600 stores. Revenue growth of 17%, 18% is something that we can expect with the expansion plans in place.

Nitin Agarwal: And, sir, over the years, as you planned, is there any specific skew towards a particular geography that you are looking to build out more versus what is the current network?

Obul Reddy: We are going towards the Central region and South. For the Central region, we have a lesser number of stores. We are focusing on that area. And South is always our best bet to go into the next level towns.

Moderator: Ladies and gentlemen, as there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Suneeta Reddy: Thank you, ladies and gentlemen, for joining us on this call. As we close our discussion on Q1 FY26, we are encouraged by the strong start to the year and momentum across our core in emerging businesses. Our integrated healthcare

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system, spanning hospitals, diagnostics, pharmacies, and digital, continues to deliver balanced growth alongside operational efficiency.

Looking ahead, our priorities remain anchored in deepening our leadership in clinical excellence, enhancing operational performance, and capitalizing on new growth opportunities. We appreciate your continued interest and your support and look forward to updating you in the next quarter.

Moderator: On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you all for joining us and you may now disconnect your lines.

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the transcript has been edited for clarity, the Company takes no responsibility for such errors. Efforts have been made to ensure a high level of accuracy, and any figures that may have been inadvertently mentioned have been reviewed and corrected as necessary.

Call: Nov 2025

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Apollo Hospitals Enterprise Limited
Transcript of Q 2 & H1 FY26 Earnings Conference Call
November 7, 2025

Moderator: Ladies and gentlemen, good day, and welcome to Apollo Hospital Limited earnings conference call. As a reminder, all the participants' lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Devrishi Singh from CDR India. Thank you, and over to you, sir.

Devrishi Singh: Good morning, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals to discuss the financial results for Q2 and H1 FY26, which were announced yesterday.

We have with us today the senior management team represented by Mrs. Suneeta Reddy – Managing Director; Mr. A. Krishnan – Group CFO; Dr. Madhu Sasidhar – President and CEO of the Hospitals Division; Mr. Madhivanan Balakrishnan – CEO of Apollo HealthCo; Mr. Sriram Iyer – CEO of AHLL; Mr. Sanjiv Gupta – CFO of Apollo HealthCo ; and Mr. Obul Reddy, CFO of the Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risk and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide number 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been calculated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Good afternoon, everyone, and thank you for taking time to join our earnings call. I believe that you have received the earnings documents, which we shared yesterday.

Consolidated revenue was at INR 6,304 crore , a 13% year -on-year growth. Consolidated EBITDA was at INR 941 crore , registering an increase of 15% year -on-year. Within this, the Healthcare Services EBITDA was at INR 781 crore , registering a growth of 8% year -on-year and Healthcare Services margin remained robust at 24.6%.

We are pleased to talk about the sustained and strong momentum from quarter 1 into quarter 2, delivering strong growth across all three verticals, Healthcare Services, Apollo HealthCo and AHLL. Despite the seasonal impact from the onset festival period, focussed execution has enabled us to report broad -based growth and resilient operating metrics.

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Healthcare Services business has delivered a 9% year -on-year revenue growth to INR 3,169 crore . The revenue growth was driven by insurance and cash patients together accounting for 86% of the inpatient hospital revenue.

Q2 FY25 had a higher incidence of seasonal medical admissions, leading to a high base.

Medical admissions were lower in Q2 FY26. The low growth in medical admission was offset by a 14% increase in revenue from CONGO Speciality.

The reduction in number of patients from Bangladesh has had a 1% impact on Healthcare Services revenue in Q2 FY26.

Surgical volumes grew by 3%, supported by our continued focus on CONGO specialty.

Cardiac, oncology, neurosciences, gastro and orthopaedics - which remain a key growth engine , delivering a 6% volume and 14% revenue growth on a year -on-year basis. Group -wide occupancy stood at 69% in Q2 FY26.

As shared earlier

, we have withdrawn the ARPOB metric and introduced the average revenue per patient , ARPP as a more accurate measure of realization. ARPP in Q2 FY26 was 173,380 , recording a growth of 9% , through a combination of better clinical mix and regular tariff increased with inflation.

Within the Healthcare Services business, we have delivered a ROCE of 30.3% with a balanced ROCE across all geographies, Metro Tier 1 and Tier 2 .

Moving ahead, Apollo HealthCo reported revenues of INR 2,661 crore marking a 17% year -

on-year growth. Apollo Health & Lifestyle revenues also increased by 17% to INR 474 crore during the quarter.

Private label and generics were at 15.2% of total pharmacy revenue. Our digital platform, 24/7 added 3 million new users, taking the total to 44 million users. The platform GMD was at INR 723 crore, growing 16% over the same period in the previous year.

The pharmacy distribution business in Apollo HealthCo reported an EBITDA of INR 181 crore as against INR 153 crore, higher by 19%. Losses in the digital vertical was at INR 71 crore compared to INR 101 crore in the same quarter last year. As a result, Apollo HealthCo reported an EBITDA of INR 110 crore in Q2 FY26, up from INR 52 crore in Q2 FY25.

AHLL delivered an EBITDA of INR 50 crore, representing a 21% year-on-year growth, with margins improving to 11% from 10% in quarter 2.

During the quarter, the Chief Minister of Delhi inaugurated Apollo Athenaa, Asia's first dedicated cancer center for women in Defense Colony. The facility is designed to tackle the growing burden of breast and gynaecological cancers and integrates precision medicine survivorship programs and world-class treatment protocol. We also performed a soft launch of the Multispecialty Tertiary Care hospital in Pune. We continue to make steady progress on our other expansion projects across key metros including Kolkata, Hyderabad, Bangalore and Gurugram with commissioning lined up in the upcoming 12 months.

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The statutory processes for the restructuring within Apollo HealthCo and the Keimed merger continue to proceed as per plan, and we remain well positioned to capture the full benefit of scale within Apollo HealthCo.

Half year H1 FY26 Results : The consolidated revenue for H1 FY26 stood at INR 12,146 crore, a growth of 14% on a year-on-year basis, supported by balanced expansion across all three verticals. Healthcare Services revenue at INR 6,104 crore, up 10% year-on-year, driven by continued traction in high-end specialty and improving clinical mix. Apollo HealthCo delivered revenues of INR 5,132 crore, registering an 18% increase year-on-year. While AHLL grew 18% to INR 909 crore.

Consolidated EBITDA for the first half was INR 1,793 crore, reflecting a 20% increase year-on-year. PAT stood at INR 910 crore, up by 33%. This is a strong half year performance, which reinforces our ability to sustain growth momentum while maintaining financial discipline and operating efficiency.

Let me close by saying that our quarter 2 and H1 performance underscores to continue its strength of Apollo's integrated health care ecosystem and our commitment to delivering clinical excellence at scale. We remain focused on deepening our leadership in core healthcare services, accelerating digital in retail health and expanding our network through strategic capacity additions across key markets.

With a disciplined approach to execution and a clear road map for growth, we are confident of enhancing our performance in the coming quarters, and we continue to create long-term value for patients, partners and shareholders.

On that note, I would like to hand it over to the moderator and open the line for questions, I have Krishnan - o

ur Group CFO; Mr. Madhu Sasidhar - CEO of Hospital s Division; Madhivanan - CEO of Apollo HealthCo ; Sriram Iyer - CEO of AHLL, Obul Reddy and Sanjeev from Apollo HealthCo, we will take all of your questions. Thank you.

Moderator: We will now begin with the question-and-answer session. The first question is from the line of Binay Singh from Morgan Stanley.

Binay Singh: My first question is how to think about organic growth for hospitals. Keeping in light two points ; - first is that the Bangladesh impact incrementally will go away, so that will be 1% positive.

Secondly, there is lots of news on insurance pricing that hospitals are planning to keep steady for next year. So, keeping both these, could you comment on this and then guide on organic hospital growth for next year?

Suneeta Reddy: Yes. I think we are quite confident that we will get back into 30%. We say this because Bangladesh, at least 60% has started coming back in October and we believe that we will mitigate the impact of losing one territory. Also, we are exploring new markets, including the Northern markets in Uzbekistan, etcetera. So those that are surrounding markets, Africa, some with Indonesia, Iraq. New markets added to Bangladesh coming back will definitely give us an upward trajectory.

With regard to insurance, we continue to stay in dialogue with the insurance companies. And I think that there is a very good way forward because all of them represent different parts of the value chain, and we recognize how important it is for us to cooperate and work together.

A. Krishnan : And on the pricing front on insurance also, I am not sure about this commitment that you are talking of. Typically, as you know, in any case, our insurance contracts are once in two years. So, by that definition, itself we do not get a price increase every year. So, what happens is every two years, it gets reset. So, if you reset the contract this year, we, it is the same pricing applies for two years, while our inflation is always annual, which you know. So that is the way it has been working, and it is the same way that it continues to be. Certain contracts will come up for renewal this year, certain others will come up for renewal the next year. So, it is a rolling contract renewal that works across the system. It continues the same way.

Madhu Sasidhar: And Binay, if I can add, even within this quarter, you will see that there is, I think Ms. Suneeta spoke about it a significant improvement in the quality of the revenues. There has been an increase in the complexity of cases with substantial improvement in our CONGO specialties and high complexity cases. We have also invested a lot in the past two quarters in building solid programs and heavily into recruitment, especially in our mature units. I think these will all add to that organic growth that you were asking.

Binay Singh: Thanks for that detailed answer. Secondly, just one question on the presentation on Slide 16, where we talk about capacity expansion. Earlier we used to say FY26 commissioning number. Now we are putting it under FY '26-'27. So, is there any change in ramp-up plans just to check that for the next 2 years?

A. Krishnan : So, the way we are looking at it is this. Today, if you are looking at, and I will comprehensively handle this slide because there are a couple of questions also which will come up. One was six new hospitals is what we are looking at adding in the next year, this year to next year. This coming quarter, you would see us already add, started commissioning, we already soft commissioned the Defense colony Cancer Hospital. We have also soft commissioned Pune. So, you will see that Pune and Defense Colony, both we will start report

ing numbers from this quarter itself which is Q3.

Come to Q4, we will be starting the Sarjapur Bangalore Hospital as well as Calcutta. Come to end of Q4 or early Q1 is when we are looking at Hyderabad because there in Hyderabad, as you would have seen, the costs have gone up by INR 35 crore because we have now decided to add a comprehensive oncology program also there, which includes the radiation therapy equipment with results in us, and that will require some more time to build. So, which is why it will be more around Q1.

Even in Gurugram, it will be more around Q1 because we have enhanced the overall facilities and doing some more of private rooms, etcetera, in line with what the market demands. So, the way you should look at it is Q3, Q4, Q1. In that order, we will be looking at starting all these six hospitals. The balance brownfield, which is also something that we have now added. We have also started work on Jubilee Hills and Secunderabad now. So, all those brownfield

expansions will also come next year, mid of next year, etcetera, mid - to end of next year.

Suneeta Reddy: And in Bangalore, Malleswaram and Mysore expansion, work has started on both. So, you should see it happening. Next year, we will open out those beds.

A. Krishnan : So, by end of next year, all the census beds that you are seeing should be fully operational. Of course, as you know, we start 50% as we start and then keep ramping up. So next year is going to be quite a busy year for us. And you will see all of these fully commissioned by end of next fiscal.

Binay Singh: Thanks team, very clear.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is again on hospitals. So, between now and end of FY27, as you indicated, six hospitals coming up. So, I understand '26 Pune will be key addition and the majority will be coming in '27. So, if you can comment a bit on the impact on EBITDA margin trajectory due to the new costs coming up?

A. Krishnan : So, Pune and Defense Colony is this quarter, Sarjapur and Calcutta is also Q4. So, from an FY '26 perspective, FY perspective, these four hospitals come in FY26 itself. And Hyderabad and Gurugram will come into the next year. We continue to believe that next year, overall EBITDA losses from these hospitals should be around the INR 150 crore number, which is what would be the EBITDA losses from these hospitals, but we will come back to you closer to the Q3, Q4 to once we commission some of these hospitals, but we do not expect it to be higher than that.

Damayanti Kerai: And these losses include fully build-up costs, right ? Once you commission a unit, most of your doctor cost, your facility costs, etcetera, are already in place.

A. Krishnan : Yes, that is correct.

Damayanti Kerai: Okay. My second question is on your spend on Apollo 24/7. So, the last two quarters, I think somewhere INR 94, INR 96 crore kind of spend, which we have seen. So, what kind of headroom you have , to reduce from these levels? Or these are more sustainable cost levels to look at?

Madhivanan B: So Damayanti, when you say spend, it is a cost structure. So, a big chunk of the cost has been as an individual entity, we have reduced it quite a bit. So, this would be, in a way, a new normal .

But as we get into the program of aligning between the three entities, Keimed , Apollo Pharmacy, PD, and we will see a few more , a little bit more of synergies coming through as both the teams will merge. But our biggest expenses are primarily on the marketing side, so which has been , it is come to a very rational level.

So, you will see some more, but that will typically happen from the next financial year. Q3, Q4, more or less, we will

kill, there is nothing much more to bring down on the expense line . So, focusing on the revenue side of the story .

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Damayanti Kerai: And the target of achieving cost breakeven for 24/7 by end of this fiscal remains or we can...

Madhivanan B: We are on course . There might be one hiccup in the form because we are investing reasonably strongly on the insurance side of the business. We are seeing some very good traction. We have completed some of the integration with the health side, we are exploring the life. So, there might be a little bit of a hiccup here or there, but we are on course, as we speak.

Damayanti Kerai: That is helpful .

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar M: Again, just on Apollo 24/7 , on the GMV side . Probably 7,200, 7,300, sort of a stable for the last 3, 4 quarters while you already highlighted in terms of how the cost measures will help in getting the better profitability. If you could share your thoughts on, how do we think about improving the GMV?

Madhivanan B: So you have to look at the GMV from three perspectives. One, how is the pharmacy business growing . Within the pharmacy business itself, there were two levers . One, what we call as a platform revenue, which is primarily driven by Apollo 24/7 the app and the website, which has been growing at a very good pace of around 30% on a year -on-year basis. And on a quarter -on-quarter basis, we are in the range of around 5% to 7%.

In this quarter, we actually exited out of a few B2B businesses, because we were considering to walk out of them because of some of the bottom line related and our quest of trying to break even. So, this GMV reduction. That is why you are seeing that the 16% growth , that being primarily from the pharmacy side. So , that was one reason.

And the third was the GST, because earlier we used to report the total numbers on a total GST basis . Given the advantage that we have received as an industry, it has shaved off around 6% from the top line. It does not impact us on the bottom -line side. So that is the only reason. So, you will start seeing the increase on a quarter -on-quarter basis because now it is a new normal.

Tushar M: Got it. In terms of any other, let us say, business aspect, if you can, so the GST is more like a transitional period probably, but any other aspect in terms of , to drive the GMV ?

Madhivanan B: No we will continue to grow what we have our original guidance of growing between 25% to 30% on the overall. Our diagnostic business is picking up , in fact, we buck seasonality. So, we will be back on course again , on that side of the story. On the hospital side of the business, the business that we try to drive on the consults, we are relooking at our approach so that we can become much more relevant in the overall scheme of things as far as the hospital is concerned.

And the last one, the insurance will, like I told you, will take a little bit of time, but we have started clocking in some good numbers with just two cities, which is the NCR and Hyderabad . You will start seeing the new businesses picking up from Q4 onwards.

Tushar M: So just on the hospital side, the Karnataka cluster, I have seen IP volume decline . If you could

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share. ...

A. Krishnan : So, Karnataka region, there was a drop in the medical admission , significantly in that region , particularly. If you look at the drop overall of the 6% that we reported in this region, there is a medical volume drop of 15%, while surgical volume went up by 2% and cat

h also went up by

13%. So, I think it is really the seasonal medical admissions that we see in Q2 in Karnataka , we saw in Q2 of last year . Definitely was not there this year.

But otherwise, this is why if you look at the average revenue per patient in this region , it is gone up by 14% because the balance cases, which is surgical and cath, that makes us a higher ARPP compared to the ARPP of medical admissions. So clearly, that is the reason. So, the core continues to remain intact.

Madhu Sasidhar: Yes, that is correct. And if you trace those medical admissions back to last year, it was almost entirely a very bad dengue season, which we have not seen this year. So, I think it is a year - to-year seasonality , meaning volume that we do, especially in the high complex impact it continues to be strong. I think that growth will continue into this quarter.

Tushar M: That is helpful , thank you.

Moderator: The next question is from the line of Harith Ahamed from Aventus Spark.

Harith Ahamed: My first question is on margins for Keimed . We have seen some softness there of around 30 to 40 basis points decline on a Y -o-Y basis. In the past, you have talked about some restructuring and buyout of minority stakes, et cetera, within Keimed. Is that activity over? And how should we think about Keimed margins and especially in the context of the overall margin guidance of 7% by 4Q next year.

Sanjiv Gupta: Yes, you are right in saying this Q2, we had slightly drop in the EBITDA margins for Keimed , but this is only onetime integration and scheme related expenses, which got accounted in Q2. So, you did not see the same happening in , from next quarter onwards. And as suggested earlier also, over a period of time , you are looking at 20 to 30 basis points over and above 3.1%, which normally used to be Keimed EBITDA since last two, three quarters to be happening as we move forward.

As far as the overall 7%, which I think you are referring to the guidance of Q4 FY27, wherein we are suggesting about INR 25,000 crore of revenue run rate, with a 7% EBITDA. I think we are hopeful that we should be able to hit that mark. And progress is there quarter -on-quarter. This quarter, if I look at the H1 of FY26, we are at about INR 9,200 crore of turnover, which makes it to about INR 18,000 crore plus and EBITDA is 4.4% I think, once we get into breakeven, the margin should come closer to 7%.

If you exclude the digital losses, we are already at about 6.2%. And as Mr. Madhivanan suggested that somewhere in Q4 we will be very near to breakeven. So, I think after that, you would start seeing overall EBITDA also to be in the range of 6% plus. And one year forward, we should be in the range of about 7%.

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Harith Ahamed: Got it. My second question is on Specialty Care segment within AHLL. Growth has been a bit soft there and you called out competitive headwinds in the segment. So, trying to understand which verticals within Specialty Care we are seeing higher competition and what is the outlook here.

Moderator: The line for Mr. Sriram got disconnected .

Suneeta Reddy: Okay , could you repeat the question, we will take it.

Harith Ahamed: So I was asking about the Specialty Care segment within AHLL. We see a relatively modest growth there for that segment in this quarter. And in the presentation, you have called out competitive headwinds for the segment. So within , I understand there is multiple verticals like Cradle and Spectra within Specialty Care. So, trying to understand , which vertical we are seeing greater competition ? And what is the outlook here?

Suneeta Reddy: So, in terms of competition, clearly, the only one that has serious competition is Diagnostics because Spectra there is

no competition. And in Cradle , it is only where our Cradles are present , there is very little competition, except in Karnataka where Cloudnine, has a big market share. Having said that, I think that our focus will be on primary care. It will continue to be focused on growing diagnostics as well as growing the clinics, which is really primary care. Creating clinics with GP s at the center , to be able to look after communities and act as a funnel to Apollo Hospitals. So, this is what we are currently focusing on. And of course, dialysis continues to d well.

Harith Ahamed: Got it, madam. Thank you.

Moderator: The n ext question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Ma'am I think you mentioned that you expect growth to get back to 13% for the Healthcare Services business. This I assume would be organic growth and the expansion should add to this growth? Would that be a fair assumption?

Suneeta Reddy: I think over a three -year period, you will see that , there is headroom for growth within the system. This should result in 13% growth in the existing beds and an additional 5% coming from new b eds in the next 36 months.

Neha Manpuria: So, 5% will get added over the next three years. Would that be right?

Suneeta Reddy: Next two years, you will see another 5% coming.

Neha Manpuria: Okay , got it. And on the GMV growth in the digital business, could we get a breakup of what would be the split from GMV in pharmacy and insurance , new businesses? And how should we see this let us say, when the business achieves breakeven, what would be the mix of that GMV? Just trying to understand how the revenue and margin should move based on this mix?

Madhivanan B: Let me not give the exact breakup, but typically, the pharmacy business constitutes the biggest

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chunk out of this. And this is the one which has a direct impact on our profitability, because the unit economics is what drives this breakeven that we are working towards. So, any kind of a growth in GMV, which is on a profitable basis at a unit economics level, straight away contributes a breakeven. So, the way to read it , would be, that what percentage of our total GMV is being contributed through the e -pharmacy business . That is layer number 1.

The diagnostics business is again directly linked to GMV, albeit the numbers are not as large as that because the margin structure is reasonably better. So that constitutes the second portion. The third layer of business, which is the hospital business , is predominantly fee driven. We are more a tech services and a lead operator for the hospitals, being driven through the digital mechanism. So that is more or less independent of the GMV growth.

As far as insurance is concerned, this is more of the GMV equivalent in the financial services industry is gross written premium which is what we log in. So here again, because we are approaching a model wherein, we want to make insurance as affordable as possible. We are actually promoting a more EMI driven kind of a business in this model.

So here again, we will start reporting the collected premium into our numbers. So, these are the four layers . E-pharmacy the biggest constituent. Diagnostics is the second big one. The hospital contribution of the GMV is primarily driven by fees and therefore, will not make such a big play. And the fourth one is the insurance business, which will come to the party in another maybe a quarter or so. And like I said, 55% to 60% of the to tal pool comes from the pharmacy.

Neha Manpuria: And from a profitability perspective, the increase , I mean, for us to improve margins to, let us say, the 20%, the pre -operating margin for 20%, 25% would essentially then depend on pharmacy bus

iness growing? Or would it dip, as insurance picks up scale, that would be a bigger driver of the margin ?

Madhivanan B: Pharmacy is the current driver of the business, that is a reasonably established business. But the insurance at this point of time is still on a breakeven mode. Once that gets in the break even move, it starts contributing to the profitability in a much more disproportionate sense. But to read it for the next two quarters, I would focus on the pharmacy business.

Neha Manpuria: Understood , thank you so much.

Madhivanan B: I just want to highlight one more point. As we speak, just to give some confidence back to how our path to profitability is growing along , all the three lines of businesses that I spoke about , pharmacy, diagnostics and consult business at a C M1 level , has turned positive , in each one of them at an individual level. Our immediate objective in the coming quarters is to drive the CM2 and then the CM3. Like I told you, maybe the insurance business will take a little bit more time, but we will break that also up the curve .

Moderator: The ne xt question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: I know you mentioned about the seasonality and the reason why the utilization fell from 73% overall to 69%, and I get it. But I am just trying to look from a forward path perspective, right .

ALOS has been consistently declining. This is obviously as we are able to be more efficient in

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terms of our processes and stuff. But is there a theoretical lower limit where we reach on ALOS? I think that is question one.

And my larger point being, when will we start seeing better volume growth? Because utilizations have been below 70 %. And is there a plan where at some point of time, we needed to go to 70 , above 70 at all or our business model does not need to be like that ? I am just trying to compare ourselves with some of our peers whose utilizations are higher.

Suneeta Reddy: So, 70% is definitely a benchmark that we are looking at. ALOS has dropped by 7%. So, the use of new technologies, whether it is the cardiac where we have minimally resistant or robotics. So, this is really driving down ALOS and allowing us to discharge patients much faster. With regards to occupancy, I think that 70% is definitely the target that we hope to reach. Having said that, we are looking at improving payer mix.

We are focusing on the corporate, which earlier, I think we had not put enough focus there.

We also focus on retail as well as international coming back to us. With some of these initiatives, I think that we will move closer to 70%. Having said that, our metro hospital s have actually crossed 70%. And in October itself, we are looking at higher occupancy coming from metro. We have a separate plan for the non-metros.

Madhu Sasidhar: So, it is a good question, and I will take the ALOS question first. As Mr. Suneeta said, the benefit s that you are seeing are the results of us putting in driving our digital transformation, putting in electronic command centers in each of our hospitals and reducing variability. And I think we are probably not going to see sustained reductions in ALOS other than a few of our units that are starting to catch up.

I think where this puts us is in a place where we can drive utilization higher. We can more easily take on much higher levels of occupancy with less variation and more predictability, because the barrier to driving occupancy to much higher in acute care hospitals is being for best when there is high variability day in patient admission.

I think our digital transformation allows us to address this more efficien tly. So clearly, we are targeting much higher levels of occupancy. And in some hospitals,

it is possible to drive it to

80% and higher, especially when we are targeting more elective and semi -elective.

Shyam Srinivasan: Very helpful. And just a second question on the EBITDA loss guidance that you shared for the six hospitals. I heard it is INR 150 crore . I think you called it off at INR 150 crore . So, I just want to understand does this hit us in the full fiscal '27? Is it spread around just from our past experience 2015 to 2018, where some of the bed additions at that time led to , like very volatile margin trajectory? Is there something that you are doing this time differently , to ensure even some of the financial metrics are a little bit smoother, if I say so.

A. Krishnan : Surely, we are, these are all in existing markets. We go all in existing markets. So, if you look at places like Delhi, Hyderabad and Calcutta etcetera. We have a very clear plan on how we can ramp up. And as we have said, our internal target is to get all of them breakeven in 12 months. And given that it is all going to be if you look at it, it is going to be first two hospitals in Q3, followed by another two in Q4 and then Q1, there is a kind of , it is being spaced out a bit.

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And yes, in the first half of next quarter, there could be a bit higher than , it will not be equally spread, but then first half would be a bit higher and then it will come off. But we will start showing that separately in our earnings presentation as we have done in the past as well, so that you get the confidence that we are in track to achieve those numbers that we have cited.

Suneeta Reddy: And I believe that it wo uld no t be a significant impact on EBITDA margin, because we have strong cash flow s, EBITDA is supposed to be (Inaudible 0:38:45) as we open new hospitals.

A. Krishnan : And as Dr. Madhu said, the existing hospitals also, we have room for growth, and we are working on that. So that also should start supporting in the next year on the volume as well as the ARPP driven growth. So, we are working on both.

Shyam Srinivasan: Got it , thank you and all the best.

Moderator: The n ext question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: One on the insurance business. How are you selling the policies now? Is it mostly online?

Madhivanan B: It is a combination of all the three. Our first attempt is to try and sell it digitally because we have no intentions of going and spending on marketing spends and trying to get new customers. So, we are primarily focusing on this INR 1 crore plus customer and a INR 44 million registered customers. So, it is predominantly driven by digital. However, insurance being such a complicated piece, a small ticket item sells very well on digital. However, the moment the ticket size goes into the range of INR 20,000 to INR 30,000 people seek out an assistance.

So, we are building a capability of a call center, which is right now 300-seater. We intend to take up to around 500, as a capacity building and the productivity norms work out. So, think of it as a combination and omni model of leads and some business through digital and the corresponding ones through a call center mechanism. We are not in the process of putting any kind of a manpower on the field. If at all any, it would be more from a services perspective. But that is not the channel that we are focusing on.

Bino Pathiparampil: And do you plan to promote it through your physical pharmacy stores at all?

Madhivanan B: Here again, we will come out with some specific products such as, let us say, a vector insurance or a personal accident, which will be done more under the POSP model, but that is expected only around the next financial year. We would like to get the basics right first and

then go into some of those models in which case maybe out of the 7,000, we might identify

1,000 -odd outlets without putting a person in the place, we are not intending to increase the cost, but we will work very closely with the APL outlets with the right kind of outlets to sell products which can be sold as a part of the normal journeys and flows.

Bino Pathiparampil: Got it. And in HealthCo when you say the diagnostic business, it is sourcing of the diagnostic business, right? Your back end would be AHLL?

Madhivanan B: Correct. So, we are primarily into originating business for them, the final labs, etcetera, are

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with AHLL. So, think of us as a digital arm for AHLL

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: The first one on the hospital business. If you look at the EBITDA margin in this quarter has remained same despite strong ARPP growth of around 9%. Then you also suggested that the acuity mix is kind of positive. So, what is driving profitability to kind of remain same and since EBITDA margin in existing hospitals are not expanding despite the strong ARPP growth, how comfortable are we with offsetting the losses from the new units that we are expecting over the next four quarters?

Suneeta Reddy: So, with regards to the ARPP growth, we showed a 9%, which really indicates that we have really moved up in terms of complexity. To move up in terms of complexity and to ensure that we are prepared for the new hospital expansion as well as growing, getting new volumes into the old, there was considerable amounts above INR 67 crore spent on doctor hiring. I think we have created an organization structure that will make sure, that we will grow in terms of, we have improved sales and marketing, and there is certain costs attributed to that.

We have created teams that strengthen our projects division, as well as started recruiting for the new hospitals. So, we have the cost coming ahead of the opening, which is why it seems, it is at 24.6%. But going forward, we should see the benefits of all of this. And therefore, the impact of the losses in new hospitals will remain at INR 140 crore to INR 150 crore.

A. Krishnan: So also, if you look at it in a very different manner, last year same quarter, we did see an increase in spike in the margin because of the medical admissions that we spoke of. However, full year margins were at 24.2% full year. Whereas if you look at H1 of this year, we are at 24.6%. So, the way you can look at it is some, we have taken steps which is evident in terms of ensuring there is a margin flow through which happens because of the ARPP etc. We are not seeing it because of looking at it on a quarter-over-quarter basis. But when you look at it on an annualized basis, you will see that this year margins should be higher than last year reported margin.

Suneeta Reddy: And also, there is a plan to cut costs by about INR 120 crore. I believe that we have achieved about INR 60 crore of it. So clearly, this will continue to support the EBITDA margins in the range of 24.6% to 25%.

Kunal Dhamesha: And this INR 120 crore cost cutting, is it related to hospital business, and which are the areas within?

Madhu Sasidhar: So, it is a broad-based approach to managing our costs. We found opportunities in our materials management and supplies. Certainly, we are looking at how we utilize our HR more

efficiently, given that we have made huge commitments and huge investments in our digital technologies, therefore, that is driving a lot of efficiency in how we do our work. So, as we bring online new hospitals rather than the, we are actually looking at redeployment that is leading to better ef

ficiencies than how we use our HR. Beyond that, I think there will be some opportunities to reduce our ongoing costs in IT and some other costs as well.

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Moderator: The next question is from the line of Avnish Burman from Vaikarya.

Avnish Burman: My questions are on Keimed . I wanted to know that because of the GST change that happened in this quarter, how much sales did the business do, only the Keimed . How much sales loss was there?

Obul Reddy : You will see somewhere about 4% of the sales revenue coming down because of the, 12% items into 5% item, something like that. The entire 12% GST slab is removed and that consists of almost 70%. That moved to 5%, and we expect about 4%, 4.5% revenue drop .

Avnish Burman: 4% to 4.5%. Okay. And is the inventory back to normal at the customer end at the pharmacy end?

Obul Reddy : It is from day one, we have been managing that. We have planned it well and the transition was smooth that including our 700 frontend stores, we could implement this software and see the new billing is done at 7:00 AM on 22nd and everything is passed on to the customer as per the regulation .

Avnish Burman: No, my question was that in that interim period, we saw the inventory coming down at the retailer level. Is that back to normal?

Obul Reddy : That is back to normal.

Avnish Burman: Okay. And one question on the margin compression. We have seen Keimed margins getting compressed by about 40 basis points sequentially and Y-o-Y. Is there a reason for that?

Sanjiv Gupta: This is only one time . This is related to the scheme and integration related expenses, sir . And you do not see that happening in the next coming quarters. Even if those expenses are there, they will be very low. They will not be material to pull down the EBITDA percentile . But Q2 has happened because of these expenses.

Avnish Burman: Okay , thank you so much.

Moderator: The next question is from the line of Kritika Damani from Prospera Financial Solutions .

Kritika Damani: Congratulations on another strong quarter. I had a question about , like we have seen that the Healthcare Services has grown 11% in this quarter and your average revenue per patient is up 11%. So, the occupancy has eased up to 69%. So, in prior quarters, you have mentioned balance in occupancy and realization. Could you elaborate whether this quarter's average revenue per patient growth was primarily led by tariff revisions or richer case mix or structural shift in patient segments ?

Madhu Sasidhar : So, most of the tariff increases has already been realized previously. Almost all of our improvement in the average revenue per patient has come from an improvement in our case mix. And that is evident across all of our higher complexity specialties, including cardio, neurology, gastroenterology and orthopaedics . And it has also been broad based increase in ARPP primarily in our large hospitals in metro cities but across pretty much every single

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geography.

Kritika Damani: My second question is that you have highlighted that the artificial intelligence intervention in oncology, radiology and stroke care . Beyond your patient outcomes are you seeing any measurable operational efficiencies? Because how scalable are these models across your own hospitals?

Madhu Sasidhar : So thank you for that question. Yes, we are using artificial intelligence technologies very broadly. I think today, if you go on our website , there is an artificial intelligence agent that connects patients with complex medical conditions to the right doctor and al

lows them to book

appointments . That business, which is from the website grew from last year to the year by

318%, mostly due to the adoption of a very scalable AI technology that our patient consumer facing.

Internally, we have, as I said before, we have implemented command centers, digital command centers that have real-time intelligence, can see patient flows in each of the hospitals in real time. And on top of that, we are layering agentic technologies to autonomously anticipate patient issues and proactively solve them. So, I think there is tremendous potential for us to have better efficiencies through the use of AI technologies.

Kritika Damani: Thank you. That was very helpful.

Moderator: The next question is from the line of Lavanya T from UBS.

Lavanya T: Just one clarification. You mentioned that Q2, there was an increase in costs related to hiring for hospitals next quarter? So is it right to assume that this was related to the two hospitals, which are expected to open in Q3 and there will be a jump again in next quarter once the hiring for the other two hospitals to open in Q4 will happen. Is that the right way to think?

Suneeta Reddy: No. I think some of it is correct. For example, in Pune, there is a richer cost in the hiring of doctors and we are just about to do the full-blown launch in Pune. So, we do have to hire some of our doctors ahead of the opening. This will happen in Gurgaon as well. And I think that for this quarter, we have captured some of the costs. There will be a little additional cost that will come in March.

Lavanya T: Okay. the hospitals which are expected to open in Q4, there will be hiring which will happen ahead, right? So that should come in from...

Suneeta Reddy: Some hiring, yes. But it is not huge figure for this year or this quarter. So, it is not huge, but it is there.

Lavanya T: Got it. Thank you.

Moderator: The next question is from the line of Madhav from Fidelity. Please proceed.

Madhav Marda: Just wanted to check that we are at about 24.5%, 24.6% EBITDA margin in the hospital business, but we seem to have already added some of the opex for newer beds. Could you

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give some sense in terms of what the base network margins are adjusted for some of these costs that you are already building in for the newer hospitals?

And then those margins, like you said, you will report the opex for some of the newer hospitals separately, which I think, is a good idea. So, the base set of margins in your view, how do you think that could progress over the next couple of years, excluding the new beds?

A. Krishnan: So, in the base, we would like the approximate cost which is there in this quarter would be roughly around INR 10 crore in this quarter, but next quarter onwards, you will see it increase. But we will show you the split from the next quarter of our established hospitals and the new hospitals, both the revenue and the EBITDA such that it is clear for you on how it is the existing or the established is progressing, as well as the new is coming in. So that will clearly give you the perspective.

As of now, there are costs of roughly around INR 10 crore which is built in. And on the doctors side also you will see almost 5, in the doctor now. But you will see that being separated out from the next quarter. We are clearly hoping and working on ensuring that we should get the overall margins over the 25% in the next year and even higher on the established hospitals, which is at 24.6%, which is a combination of both revenue as well as costs that we spoke of.

Madhav Marda: So, the INR 15 crore cost, which is built in quarter 2, that is roughly 50 basis points, right, of our revenue, right? So basically, the base network is

above 25% in quarter 2.

A. Krishnan: So, you are right. You are right in the way that you are looking at this. But once we start splitting out from the next quarter, you will see the exact difference coming in.

Madhav Marda: And that 25.1%, which we are at in quarter 2, just that number, of course, excluding the new beds, that do you think there is a scope to expand those margins over the next one or two years?

A. Krishnan: Yes. Clearly, there is because there is here for growth. We are working on the clinical program. And clearly, we would, the internal target is to take it higher by 500 basis points.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Recently, there has been a significant hike in CGHS rates by the Central Government. So the question here is that - a) I mean, does these hikes in CGHS, I presume it will be followed by other Central Government agencies, does that change your perspective towards Government business? And two, how much impact does it really have on our business right now?

A. Krishnan: So that is a marginally better number that we have given. But from a base perspective, it does not change the views that we have been having on the business.

Moderator: We will take our last question from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Just on the CGHS rate revision, have we kind of done an exercise where we can, you can suggest what is the average rate hike that we have received or the government has given for

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the key therapies like onco, cardio etcetera.

A. Krishnan : So, the point is that I see there are certain specialties where they have increased the prices reasonably. So cardiac, onco, ortho etcetera will be reasonably better now. But the point is when you empanel yourself for CGHS, under the rule book, we have to take whichever patient they send to us or whoever comes to us. You can not deny admissions for the others. And it is not appropriate that we take that fruit unless the Government allows us to do that, then we could actually have selected some of these specialties and worked on those specialties. So, it is a bit of a difficult way of doing that. It is not something that we have been doing in the past either. So, then when you look at the overall average realization and compare it to the insurance business or even the cash tariff it is still a good 65% discount to the overall realization that we have. So that is, we do not make that kind of margins you know that.

Kunal Dhamesha: Thank you and all the best.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for the closing comments.

Suneeta Reddy: Thank you, ladies and gentlemen, for joining this call. I do want to say that the management is always available. I could see there were many more questions that needed to be asked. So please feel free to reach out to us. We are encouraged by the robust growth in revenue across all verticals. And the EBITDA we have delivered despite seasonal market factors.

Our integrated health care ecosystem spanning hospitals, diagnostics, pharmacies and digital platforms will continue to deliver balanced growth and improved asset utilization. Our footprint expansion will go on stream as planned and will add to strength to our clinical delivery, as well as to volumes and continue growth. We really appreciate your continued interest and look forward to communicating with you throughout the year. Thank you.

Moderator: On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you all for joining us and you may now disconnect your lines.

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cript and may contain transcription errors. While the transcript has been edited for clarity, the Company takes no responsibility for such errors. Efforts have been made to ensure a high level of accuracy, and any figures that may have been inadvertently mentioned have been reviewed and corrected as necessary.

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Apollo Hospitals Limited
Q3 FY '26 Earnings Conference Call
February 11, 2026

Moderator: Ladies and gentlemen, good day, and welcome to the Apollo Hospitals Limited Q3 FY '26 Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this call is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you.

Mayank Vaswani : Thank you, Yashashri. Good afternoon, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals to discuss the financial results for the third quarter and 9 months of FY '26, which were announced yesterday.

We have with us today the senior management team represented by Mrs. Suneeta Reddy, Managing Director; Mr. A. Krishnan, Group CFO; Dr. Madhu Sasidhar, President and CEO of the Hospitals Division; Mr. Madhivanan Balakrishnan, CEO of Apollo HealthCo; Mr. Sriram Iyer, CEO of AHLL; Mr. Sanjiv Gupta, CFO of Apollo HealthCo; and Mr. Obul Reddy, CFO of the Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties, which is on Slide 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy : Thank you, Mayank. Good afternoon, everyone, and thank you for joining us on today's earnings call. I trust that you have all received our earnings material that we shared yesterday. We are pleased to report a strong performance during what is typically a seasonally weak

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quarter. Importantly, we have sustained the positive momentum from the first half of the year into Q3, delivering double -digit top line growth across all three of our business verticals, Healthcare Services , Apollo HealthCo and AHLL.

On a consolidated basis, revenue grew by 17% year -on-year to INR 6,477 crore . Within this, the Healthcare Services business recorded revenue of INR 3,183 crore , reflecting a healthy 14% year -on-year growth. This growth was driven by a well -balanced mix, 5% from volume growth, 4% from case mix, and the remaining 5% from pricing.

Surgical volumes grew by 6% during the quarter, supported by our continued focus on CONGO -T specialties, cardiac, oncology, neurosciences, gastroenterology, orthopedics and transplant. These specialties remain a key growth engine for us and delivered a robust 16% year-on-year revenue growth. Group -wide occupancy stood at 67% in Q3 FY '26.

Insurance and cash patients together accounted for 83% of inpatient hospital revenues for quarter 3 FY '26, underscoring the strength and resilience of our payer mix. Average revenue per patient was INR 180,917 in Q3 FY '26 compared to INR 173,246 in Q2 FY '26, reflecting an increase in clinical intensity during the quarter.

Apollo HealthCo reported revenues of INR 2,827 crore , 20% year -on-year growth. Revenues from Apollo Health & Lifestyle increased by 20% year -on-year to INR 467 crore during the quarter.

Consolidated EBITDA for the quarter was at INR 965 crore , registering a robust growth of 27% year -on-year. Within this, Healthcare Services was at INR 719 crore , up by 18% with margins at 24.8%. Within Apollo HealthCo, the pharmacy distribution business recorded EBITDA of INR 195 crore compared to INR 159 crore last year, reflecting a 23% year -on-year increase.

Losses in the Digital business were at INR 67 crore . Cumulatively, Apollo HealthCo more than doubled EBITDA to INR 128 crore in Q3 FY '26 compared to INR 57 crore in Q3 FY '25. Cash losses in the Digital business were at INR 29 crore , the lowest in any quarter by far.

The private label and generics accounted for 15.53% of total pharmacy sales. Our digital platform, Apollo 24/7 added 2 million new users during the quarter and now serves over 46 million users. Platform GMV stood at INR 525 crore , a 28% growth over last year.

AHLL delivered an EBITDA of INR 48 crore , a strong 39% year -on-year growth with

margins improving to 10.2% from 8.8% in quarter 3 last year. With all three engines of the business performing well, evidenced by double-digit top line growth alongside accretive margin and profitability expansion, we reported a consolidated PAT of INR502 crore in Q3 FY '26, a growth of 35% year-on-year.

Turning to the 9-month performance. Consolidated revenue for 9 M FY '26 stood at INR 18,623 crore, growing 15% year-on-year, supported by balanced expansion across all three verticals.

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Healthcare Services reported revenues of INR 9,287 crore, up by 12% year-on-year, driven by continued traction in high-acuity specialties and an improving payer mix. Apollo HealthCo delivered revenues of INR 7,960 crore, registering a 19% year-on-year increase, while AHLL revenues grew 19% to INR 1,376 crore.

Consolidated EBITDA for the 9-month period stood at INR 2,758 crore, reflecting a 22% year-on-year increase and PAT grew at 34% year-on-year to INR 1,412 crore.

During the quarter, we operationalized 75 beds in our Pune facility. As we enter the next fiscal, we will be commissioning four new hospitals, one each in Hyderabad, Kolkata, Bangalore and Gurgaon, further strengthening our presence in key metropolitan markets with strong fundamentals. These facilities, along with the ramp-up in our recently commissioned Pune hospital, will approximately add 1,500 additional operating beds to our network, representing a significant step-up in capacity and

a clear runway for medium-term growth.

We expect to operationalize roughly half of this capacity in the upcoming fiscal year with the balance coming online early FY '28. This phased commissioning approach allows us to calibrate ramp-up efficiently, optimize capital deployment and drive occupancy-led operating leverage as demand scales.

Together, these additions position us well to capture growth opportunities in high-acuity care, deepen our market penetration and enhance long-term shareholder value. We have also made progress with respect to the regulatory integration process for the composite scheme of Keimed merger and demerger of Apollo HealthCo and remain well positioned to capture the full benefits of scale with the combined entity to achieve a run rate of INR 25,000 crore in combined revenues with 7% EBITDA.

Let me conclude by reiterating that Apollo's performance over the recent quarters reflects the depth, resilience and scalability of our integrated Healthcare ecosystem. Consumer interactions across all formats of care have increased and cross-format journeys are becoming more visible.

These results are a reinforcement of our patient-centric strategy. We have and will continue to invest ahead of the curve on in-hospital technologies such as robotics and the benefits of such investments are reaching the patient as evidenced by our growth in high-end surgeries. More importantly, we believe these results demonstrate the deep level of trust that our consumers place in Apollo. We value this trust and engagement and we will continue to sharpen our clinical differentiation, expand our capabilities across high-acuity specialties and strengthen our omnichannel healthcare platform to improve access, efficiency and high-quality care outcomes.

On that note, I would like to hand it over to the moderator and open the line for questions. I have Krishnan, our CFO; Dr. Madhu Sasidhar, the CEO of the Hospitals Division; Sriram Iyer, CEO of AHLL; Madhivanan, CEO of Apollo HealthCo; and Obul Reddy and Sanjiv from Apollo HealthCo with me to take all of your questions. Thank you.

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Moderator: Thank you very much. We will now begin the question-and-answer session. Anyone who wishes to ask a question may press star and one on the touchtone telephone. If you wish to remove yourself from the question queue, you may press star and two. Participants are requested to use handsets while asking a question. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

We will take our first question from the line of Binay Singh from Morgan Stanley. Please go ahead.

Binay Singh: Hi team. Congratulations on a strong set of numbers across businesses. I will first start on the hospital side. It is a very busy time for you, two hospitals you have ramped up and we also have Sarjapur and Kolkata around the corner. Could you -- last time we discussed about INR150 crore cost headwind coming out of this ramp-up. Any updated thoughts on that? How much of it is already built into these numbers? How much should we expect in the coming quarters? That is the first one.

A. Krishnan: So we will come back to you by Q4 one more time, but we continue to believe that INR150 crore is a good number for now. And we have started Pune and Athena just by the last month of the -- in the previous quarter. And we are hoping to ramp both of that up over the next 2,

3 quarters well.

Currently in the embedded numbers, we have approximately INR15 crore of losses in the overall reported numbers for Pune and Athena, which is part of the deferred numbers that has been reported now. As we go into next year, we are hoping that by Q1, we should be operationalizing Hyderabad, the Calcutta facility as well as the Bangalore, which is the Belenus one.

Gurugram would be more like Q2, because it is still, because of all these issues with the environmental re

lated delays, which we could not complete our construction. There is a 2, 3 months delay. So it will be mostly in Q2.

Binay Singh : Okay. So to an extent, some of the costs of that would start to come up in the March quarter, right, all your hiring costs and all?

A. Krishnan: March or more likely April.

Binay Singh : Okay. And secondly, just on the GMV of the Digital business. We have seen a sequential drop in GMV. And in fact, our revenue to GMV ratio also went up. Could you share your thoughts on that?

A. Krishnan: Madhi?

Madhivanan B.: Sanjiv, you want to explain the reinstatement of the GMV, please?

Sanjiv Gupta : I can take that question. Thanks, Madhi. Yes, it is a good call out. See, there are two things which has happened. One is that on 31st of September, we had a very large reduction in the GST on the pharmacy and the other products, which resulted into GMV impact of roughly INR30 crore to INR35 crore a quarter.

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And secondly, we had one channel of e-commerce, which was Amazon. We were supplying to Amazon and then we stopped that business or that segment somewhere in early Q2 of this fiscal year.

And for us to compare apple -to-apple, we had to remove these two. A cumulative impact of these two would be roughly INR75 crore for Q3. These are the only two adjustments that have been done to ensure that we compare apple -to-apple between the last year figures and current year figures. Thank you.

Binay Singh : Thanks, team. I will come back in the queue.

Moderator: Thank you. Ladies and gentlemen, in order to ensure that management is able to answer queries from all participants, kindly restrict your questions to two at a time. You may join back the queue for follow-up questions. The next question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Yes. Thanks for taking my question. Just extending the question on the Digital business. I see that the revenue growth here has also been low, sort of, I would not say lower, but there is been a moderation in revenue growth as well quarter-on-quarter. One, can you take us through what should be the revenue base for the digital business? What drove this moderation? And second, from a guidance perspective, do we still keep our guidance for cash EBITDA breakeven for the Digital business? I think it was supposed to be at the end of fourth quarter? Any update there?

Suneeta Reddy : Madhi?

Madhivanan B.: Yes ma'am . No. So, our revenue projections continue. So let me -- the entire focus is on trying to build a sustainable operating model. So, if you have to peel the onion a little bit, our pharmacy online business, which is our main stay of the GMV, has actually grown by 32%. There has been a little bit of pull down in some of the other areas, because of which our revenue at an overall level, GMV level, is at a flatter basis.

However, the quality of the business is improving. If I were to give you some highlights, our discount is stabilizing. Our average order value has gone up by almost INR 111 net of the GST, which has a positive impact on our unit economics. Our cost of delivery, which is one of the biggest component of our cost structure on the revenue side is also coming down. So while -- and our marketing spends, we have been extremely frugal in the way we are building the business.

But in spite of this, showing a 32% growth in our GMV and proportionate revenue is holding on. In fact, at a CM1 level, we were positive last quarter. We have made it even better in this quarter and the trajectory continues. There have been some little changes in the way we recognize the income/revenue for the hospital business, which we are recalibrating in terms of the attribution and the kind of campaigns that we are driving.

So there was a little bit of a pause which we will reactivate it again from the next quarter, but it is a full reoperating model

that we are doing. The third area on the revenue side, where we

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are facing a little bit of a mismatch, I would not say is the way our insurance business, which we started off is getting recognized. Till September, any collections, any gross return premium, which is the equivalent of revenue was coming to our commissions on a full basis. We are almost getting 80% to 85% of the total log -ins that we do. We were able to generate commission.

However, post September, given the GST change, which happened in the health insurance industry, there has been a bit of a mismatch between the business that we book and the collections that we do. That is impacting our revenue a little bit. So it is a mismatch in the sense that my revenue recognition is getting deferred over the next 12 months for the money that I am collecting.

So some of these changes is normalizing it a little bit. So we are on our course. On our -- the forecast about our ability to close it in Q4, given this insurance story, which has sort of pushed us back by around INR17-odd crore , given this mismatch of our revenue recognition, we will be able to close this loop in Q1 of FY '27.

Neha Manpuria: So, if I were to understand it correctly, you are saying that the digital -- sorry, the digital cash EBITDA breakeven is now probably pushed out by a quarter?

Madhivanan B.: By one quarter. By one quarter, yes. Because of this insurance mismatch that has happened. Otherwise, we are very much on course. That is why you see the minus INR29 crore , which, the cash EBITDA that you are speaking about, the biggest negative is coming from the INR17 crore negative from the insurance business, which has got deferred into the next year.

So we will recognize that income as we go along. And some of these revenue recognition modules that we follow in the hospital business is undergoing a change. So these are two things. So we hope to catch it up in the next quarter. But rest assured, all the primary indicators are on course.

Neha Manpuria: Sir, on the revenue front, if I were to understand it correctly, we will see this revenue plus the IP/OP that will get resumed from next quarter. Would that be a fair understanding?

Because this quarter's number is an aberration?

Madhivanan B.: Correct. Correct. Yes, we will reactivate those.

Neha Manpuria: Okay. So only thing that has changed in the insurance bit.

Madhivanan B.: Yes. That is the one which has a bigger impact. And there are some businesses wherein our revenue gets recognized at the end of the year, because some of them are back ended, that will also come into play as we go along.

Neha Manpuria: Understood. Okay. That is helpful.

Sanjiv Gupta : Just correcting one number. I think, Madhi said -- I wanted to say, INR7 crore of insurance impact into Q3, not exactly INR17 crore out of INR29 crore . Just as a correction.

Neha Manpuria: Okay. Got it. And my second question is, in the cluster numbers that we are giving, other than probably AP, Telangana and Chennai cluster, we have seen the IP volumes pretty much

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moderate in most other clusters year -on-year. So just wanting any color as to why we are seeing this trend, whether I see it in North, West or East cluster, there seems to be a moderation in the IP volume trends.

Madhu Sasidhar : Yes. So, I think you are seeing different phases of optimization. Especially in our West market, we have undertaken a lot of work to improve the quality of revenue. So I think volume by itself may not be the right indicator. We took a more holistic look into it, especially at the quality of revenue and the average revenue per patient. So especially our Navi Mumbai unit has been performing very well on that basis, especially with higher specialty care.

Neha Manpuria: All right. Okay. Got it. Thank you so much.

Moderator: Thank you. Next question is from the line of Karan Vora from Goldman Sa

chs. Please go ahead.

Karan Vora : Thank you for taking my question. My first question is with respect to the bed expansion. So I think we have mentioned other than Gurgaon, all the other greenfields or the larger units will come up by Q1. But just wanted to get a sense on like how many beds will be operationalized in Q1?

And how will the ramp up look like? I guess you will not operationalize all the beds, right?

So at a hospital level, how the ramp -up could look like over the next 6 to 9 months or even 1 year, that will be helpful for each hospital.

A. Krishnan: So, we will come back again by Q4 on this. But broadly, if you look at it, Sonapur is around -- Calcutta has 225 beds. Half of that should get operationalized in Q1. Hyderabad is 300.

Again, we think that at least 50% should get operationalized by Q1. Pune also, we have now operationalized 75%, but we think by Q1, we will add another 100 beds, which will also get operationalized. And Gurugram would be by Q2 200, 250 beds. Sarjapur, 150 beds, we are hoping that we should be able to operationaliz e 100 beds out of that.

Karan Vora : By Q1?

A. Krishnan: That is right. So roughly around 50% of these beds, right, 1,500 beds, if you look at it in these

-- we have Jubilee Hills expansion and Secunderabad which will come a bit later, which will be during the year. But of these four new hospitals, almost 40% to 50% will get operationalized by Q1.

Karan Vora : Got it. Got it. And like when we operationalize the remaining half in Q1 FY '28 or somewhere around that time, will there be any more start-up losses or fixed cost or elevations, which we need to keep in mind?

A. Krishnan: It will be there. So we will have to look at it as in a phased manner. So, we will see that Q1 should have -- so when we are saying at INR150 crore, there will be quarters which will have some higher losses also, right? There could be a quarter where there is a INR50 crore loss, which is coming in. But otherwise, we should look at overall at INR150 crore for the next year is how we would look at it.

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So it is difficult to guide now quarter-by-quarter. We will definitely, as we operationalize these, give you a broad guidance. But you can -- we have said that this is something that we are hoping that can be defrayed by the overall volume and revenue growth in the existing hospitals.

Karan Vora : Okay. Got it. And my second question is with respect to the ARPOB growth. So, I understand that we have moved away from ARPOB. But just if we back calculate ARPOB growth, it is healthy at around mid-teen levels. So just wanted to get a sense that what is driving this growth? And in the opening remarks, there was a mention of 5% price growth. But in the PPT, it is mentioned 3% price growth. So what is that mismatch, if you can just tie it down, whether it is 3% or 5%?

A. Krishnan: Sure. 3% was the tariff increase that we had taken during the year, whereas 5% is the effective price realization that we have done in this year, because there were some insurance contracts which got reset also during the year. So the tariff, what has been presented in the PPT is more the tariff increase, which was done in this year. So this is -- that is the difference between the two numbers. Your other point on ARPP, of course, we do not give ARPOB, you can compute the ARPOB, of course.

But because we do not do ARPOB because the reason is, again, we have OP, we have radiation therapy. We have other stuff also and a lot of daycare procedures also now come in. ARPP has gone up, which is a combination of, as we said, higher complexity cases that we have seen across and the CONGO-T growth and surgical growth. This is what has increased the ARPP at a broad level. Pricing realization in that has been 5%, the balance has been case mix.

Karan Vora : Okay. Got it. Just one last thing, if I can squeeze in. So Apollo 24/7 or the digital piece,

overall breakeven EBITDA breakeven, is there any target, not the cash, but even after including ESOPs?

A. Krishnan: Sanjiv, you have -- you want to give them a color of ESOPs for the next year?

Sanjiv Gupta : I think, it is better would be that when we meet again, somewhere in the Q4 earnings call, we will have a better understanding about all this as we get into AOP. But I think one point clearly that the ESOP cost maximum is getting consumed until this year. And after that, we will have a very less cost on the ESOP specifically. But overall question about including ESOP, I think, I would request just to wait for quarter. We are into the middle of annual operating exercise, planning exercise. We will be in a better position to tell next time.

Karan Vora : Okay. Got it. Thanks a lot.

Sanjiv Gupta : Thank you.

Moderator: Thank you. Next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil : Hi, good afternoon. Congrats on a great quarter. Just following up on the previous question, the price increase and realization increase that you are talking about, did that kick in Q3? Or

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was it already there in Q2 and Q1?

A. Krishnan: It was there in Q2 also.

Suneeta Reddy : But mostly in Q3.

Bino Pathiparampil : Mostly Q3. So, if I look at the kind of growth you delivered in Q1 and Q2 versus Q3, there is a big step jump. And from your competitors, we have not heard about a great season or anything in the healthcare side. So can I assume a significant part of that is coming from this realization improvement?

A. Krishnan: No. See, you have to remember that some of our competitors are in various different geographies. See, please appreciate that we are a pan-India player. If you look at the in-patient volume growth in Q2, we saw 2% volume growth, whereas in in-patient volume growth in Q3 is 4.5% versus last year. So there will be various different reasons on the way that our numbers can go versus people like who are more North centric or somewhere else

because each of the competitors, not many players are as pan -India as we are. So you have to keep that in your mind.

Of course, we have got the benefit of the price realization from Q2 onwards and which has moved into Q3. But the more important point that you should also consider is the fact that there has been a higher complexity of cases also that we are focused on in the CONGO -T specialties, and we should be able to sustain as we move forward.

Bino Pathiparampil : Got it. Second, in AHLL, we have seen quite a few number of additions in the number -- sorry, quite a few additions in the number of centers. Are these mostly on the diagnostic side?

Or is it all across?

Suneeta Reddy : Sriram?

Sriram Iyer : Yes. So most of these additions are on the diagnostics side. And we also launched two new clinics, one in Chennai and one in Hyderabad. Otherwise, most of these additions are on diagnostics.

Bino Pathiparampil : Okay. And when you say diagnostic, are these like full -fledged labs or just collection centers?

Sriram Iyer : No, these are -- obviously, we have expanded into new lab, new geographies also, but most of these are the infrastructure that you create in the existing geographies. So we opened the collection center and our company -owned outlets. So that is what you can see as expansion.

Bino Pathiparampil : Understood. Okay. And finally, an update on what is happening on the HealthCo side regarding the corporate action about demerger, et cetera. How is the progress on the Keimed side in terms of accumulation of the different pieces of business that were separate, et cetera, would be great?

Obul Reddy : We have obtained Competition Commission approval and SEBI approval. We have filed with NCLT. NCLT started -- it has listed and started the hearing. And we await for the next step.

Bino Pathiparampil : Okay. And Keimed was supposed to

buy out some of the associates, which were holding part of shares in different entities. How is that progressing? Is that all progressing on time?

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Obul Reddy : Keimed has progressing on time and Keimed has streamlined the entire subsidiary network, and they are 100% subsidiaries of Keimed, which is going to get -- I mean, Keimed is going to merge into AHLL.

Bino Pathiparampil : Great. Thank you very much.

Moderator: Thank you. Ladies and gentlemen, we request you to restrict to two questions at a time, please. You may join back the queue for follow -up questions. Next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai : Hi, Thank you for the opportunity. My first question is on your hospital business. So not specific to you, but I just want to understand your negotiation with the health insurance companies. Like how is it going? Because we heard some of your peers had some issues. So, from your hospital, either for existing as well for some of the new hospitals, how things are progressing on contracting or onboarding of health insurance?

A. Krishnan: It does not -- So we have a good relationship with all the insurance companies across. We have always been maintaining that and we have a central relationship with them as well. And as of now, basis, yes, there has been some delays in getting certain insurance approvals in some of the markets and which is why even last quarter, we saw some of that contracts getting pushed out for renewal. But with that said, I think we are on course, and we are seeing that we are fine going forward.

Damayanti Kerai : Okay. And these contracts are generally for 2 to 3 years or you are moving for some annual renewal contracts as well?

A. Krishnan: We would prefer annual, but as of now, it is still 2 years.

Damayanti Kerai : It is generally 2 years contract. And when do you start this empanelment process for the new hospitals, say four hospitals coming next year. So when do you start your negotiations to empanel the company there ?

Madhu Sasidhar : We start the negotiations much before we operationalize the hospital. To give you an example, some of the hospitals that Krishnan spoke about, some of the agreements have been in place or there has been an agreement by both parties on the terms. So we started much before operationalization, if that answers your question.

Damayanti Kerai : Yes. My second question is again on 24/7. Actually, I am not still very clear. So you mentioned the majority of changes happened on the pharmacy post GST changes. But when we look at other metrics, say doctors' consultation number or doctors and platform, diagnostic samples, all of these numbers changed compared to what we saw in the September number?

So actually, if you can help us understand what all changes were there when you are booking GMV? And if you can just clarify, you mentioned INR75 crore kind of number, which would adjust in the 2Q numbers, right? Some clarification will be great.

Sanjiv Gupta : Madhi, let me just take this question. I think what we are trying to say is a very simple point that, when you compare numbers across previous year, on the overall GMV side, there are

two factors which has undergone change. One is the GST and secondly, one of the channels that we closed. So obviously, when you compare GMV to GMV, this factor has to be taken into consideration while seeing growth things. So this is one change that has happened. Now as far as -- you also checked -- I think you asked this question also that total revenue for Q3 versus Q3 FY '25 versus Q3 FY '26, you are seeing a little lesser growth or 15% growth. I think the answer there lies with the Amazon channel, which is part of the sales for Q3 FY '25, which is not part of the sales in Q3 FY '26.

And if I minus that out, you would see a growth of 32% on the overall revenue versus 15%, which is

now mathematically can be calculated. So this is just, I would say, a small correction because of the GST came in somewhere in Q2, 21st of September, as I said. So we had to change the numbers for Q3 for comparative purposes. And somewhere in April month -- somewhere in June end, we closed the Amazon channel and hence, those numbers from Q2 had to undergo change. So this is just a change in the numbers consequent to these two things to arrive at a better growth factors.

On the diagnostics and the consultation side, there is no change. The business remains as it is. Whatever commission rates are there between the entities, those related party agreements are still good to go. There is absolutely no change.

And I think you also touched upon IP/OP GMV, I think IP/OP is one -- in the last earnings call, we did -- Madhi did mention this that the new arrangement with the hospital is more of a flat fee that we get on a quarterly basis. And it is like a booking or ensuring the tech platform for the entire Apollo ecosystem.

And that is where the commercials are between both the organizations. I think this should be helpful. In case you still think that you have got some doubts, maybe separately, we can connect and then we can discuss about this.

Damayanti Kerai: Sure. So just like in terms of annual GMV, any target we are looking for, say, a month -- this quarter, we did around INR425 crore. So will this be the run rate to look ahead? Or you think we can see much better numbers going ahead? Thank you. That is my last question.

Sanjiv Gupta: You can expect a consistent growth of around, say, 30% of the GMV for this financial year.

And once we are done with the numbers, we will come back for the next year plan.

Damayanti Kerai Thank you. All the best.

Moderator: Thank you. Next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, on the base hospitals per se, if I exclude the new hospitals, at the metros we are already at 70% occupancy, non-metro 62%, ROCE of 31%. So, if you could just elaborate in terms of what is further scope for these hospitals to sort of drive EBITDA going forward, maybe through CONGO or patient mix or let us say the payer mix? That is my first question.

Madhu Sasidhar: Yes. Yes. So I think, even in Q3 of last year, we had reduced a little bit. You are right, the occupancy at our metro units is fairly high. There are several levers that we are depending on. One is, I think there is a little bit more opportunity on length of stay reduction through operational excellence, a lot of investment in digital technologies that is helping us with that. The second is there is still some volatility, I think, day-to-day, week-wise and seasonal volatility, and we are looking at ways in which we can minimize that volatility. And the third is, of course, a consistent focus on case mix and especially in our flagship hospitals, making sure that there is an intentional shift to high-complexity cases.

Tushar Manudhane: Any broad number, if you could share like the seasonality impact for the quarter or, let us say, for the 9 months, how much it has been?

Madhu Sasidhar: Yes. It is very hard to tell because, as you know, it varies from year-to-year, right? And it especially shows up on the medical cases. Some years, we have a bad dengue year and that skews the occupancy number. So that is the unpredictable part of our business.

But there is a lot that we can predict, which is usually on the elective and semi-elective cases, the surgical cases and the high complexity cases like solid organ transplant, where we have made a lot of commitment and a lot of investments. Transplant, as an example, this quarter compared to last quarter is about a 50% increase in revenue at the group level.

Tushar Manudhane: Got it. And just secondly, on the

the overall combined Keimed plus offline plus online, we are at a quarterly run rate of INR50 billion. And the target is to reach INR250 million annualized by Q4 FY '27, which is effectively INR60 billion, INR62 billion. If I go by earlier quarter numbers, we are sort of growing at 4%, then what could be -- while we have explained that in detail in terms of the online pharmacy or the HealthCo, but any other factors which will drive this number to INR62 billion by Q4 FY '27?

A. Krishnan: Sanjiv ?

Sanjiv Gupta: I think all the business lines, whether it is the frontend or Keimed or online, as you rightly said, online pharmacy is growing by about 30%. We are seeing a decent growth in the other business lines also. I think where we stand today at Q3 annualized number, we are roughly at say, INR20,000 crore, and we have got five more quarters to hit the run rate of about INR25,000 crore, which is roughly about 25% from now to there.

Five quarters also, I think, if we continue to see the business with a growth trajectory of about 20%, 22% annually, we should be able to meet this number easily. At this stage, we do not see any challenge coming our way with respect to hitting the top line number. Thank you.

Tushar Manudhane: Got it, sir. Thank you.

Moderator: Thank you. Next question is from the line of Lavanya from UBS. Please go ahead. Lavanya, your line is unmuted. Please go ahead with your question.

Lavanya: Yes. Thank you for the opportunity. So I just wanted to check how sustainable you see the hospital margins, given the new hospitals being operational over the next couple of quarters?

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Madhu Sasi dhar: So, I think, we will continue to be able to maintain margins. We are carefully balancing the EBITDA deterioration in our new hospitals with how we are managing our existing hospitals.

Also, I think for the new hospitals, we are very, very focused on quickly ramping up to profitability by both making sure that our recruitment and our human capital costs are aligned with the occupancy numbers.

Lavanya: Just a question in this. Actually, here, even in this quarter, we have some new operational beds, but still if I see consol employee cost, it has been down on a sequential basis. Any specific reason there?

A. Krishnan: So in Q2, there were two one-offs which are there also. So if you look at Q2, there was a sick leave encashment provision that was required under the accounting standard that was a INR12 crore number. And there was also an additional cost of PLVP or performance-linked variable pay that we have in Q2, because July is when we do the -- we do the increment. So this was there in Q2, and that is not there in Q3. Otherwise, it is aligned with the numbers.

Lavanya: Okay. Okay. Got that. Just on the physical pharmacy, how do you see growth in terms of on sustainable basis, like store addition plus same-store growth overall, like how do you see it 18%, 20%? Or what is the level that you expect in terms of physical pharmacy growth?

Obul Reddy : Now, we are currently at about 20%, 20.5% on the total network. On the same-store growth, we expect about 18% and store additions will continue to be in the range of 600 per annum.

Lavanya: Okay. We should maintain this kind of run rate going ahead in near to medium term? Is that the right assumption?

Obul Reddy : Yes. We are confident of.

Lavanya: And 18% of the same-store, how should one should look at it? Like what should drive 18% to 20%?

Obul Reddy : 16% same-store growth as against the 20.5% overall growth.

Lavanya: Okay. Key drivers for the 16% is increasing private label or how should one see it?

Obul Reddy : Private label then, we even doing the refresh store, changing the model, changing the inventory, particularly with the -- dynamically . All those things add s to that growth.

Lavanya: Okay. Okay. Got it. Got it. Thank you. Thank you so much. All the best.

Moderator

r: Thank you. Next question is from Vivek Agrawal from Citigroup. Please go ahead.

Vivek Agrawal: Hi. Thanks for the opportunity. So one question on hospital ARPP growth. So 10% growth in 9 months looks quite impressive. So I just want to understand how sustainable it is. So can you maintain this 10% kind of growth? Or there is a possibility that it can come down? Thank you.

A. Krishnan: So see, we have been always guiding saying that we would like to achieve more volume growth, and we would continue to focus on getting the volume growth to a higher number.

And then the combination of volume plus the case mix focus, which Dr. Madhu already said.

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So that is the mix that you would get to.

So maybe it is -- this is a very dynamic number in the industry, right? As we focus on a certain clinical programs in a quarter and then depending on how the clinical programs ramp up, you will see the ARPPs come up accordingly. So we would like to believe that when it is a 12%, 13% organic growth that we look at of 14%, we would like to be half on the volume and the balance half on combination of case mix and pricing.

Vivek Agrawal: Just one question again on margin trajectory next year as you are guiding for INR150 crore kind of loss in the new units. So just want to understand, do you have levers in the existing network that can mitigate the impact of losses in the new units? Or are you seeing a significant dip as far as the margins or the hospital business margins cumul atively in the next year?

Suneeta Reddy: No, I think the lever that we have is asset utilization. So as we look at our assets and occupancy and even in spite of bringing down the ALOS, we have a headroom for lifting volume and asset utilization by another 8%. We will focus on this as we go forward into the New Year. The second, of course, we have not done any significant cost cutting, which we hope will bring us another 80-100 basis points. With this, we hope to minimize the losses coming from new hospitals.

Vivek Agrawal: Understood, madam. So just one more thing. What kind of a margin expansion that is possible, let us say, in the existing business?

A. Krishnan: At least 100 basis points is the margin expansion, which is possible in the existing business next year.

Vivek Agrawal: Thank you. Just last question from my side. It is on talent retention. So as we are seeing that hospitals in India are expanding capacities, it looks like that Apollo can become a hunting ground for big doctors. And very recently, we have seen one of the star oncologists in Delhi has been poached by one of your peers. So just trying to understand how you are tackling this issue.

Suneeta Reddy: Well, I think the reverse is also happening. So I think we should be aware that Apollo will continue to attract the best talent. Because, number one, I think we have created a platform that invests not only in technology, but in terms of reach, in terms of market and market share, we will continue to lead with market share. And I think this is what doctors want. Our systems are strongly embedded so that clinical outcomes at Apollo continue to be the best-in-class. And in terms of innovation research and collaborations with other hospitals, this will continue to drive very high-end procedures. This is -- I think this is what doctors would like to see. Madhu, you want to add to that?

Madhu Sasidhar: No, I think, Ms. Suneeta is absolutely right. And I will just share our experience in recruiting in the new markets that we have opened hospitals in like Pune or existing markets where we have new hospitals is that we have had a very good brand recognition and brand affinity with the doctors in the community that has made it easy for us to recruit.

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I think that you will see the sporadic cases of doctors leaving and sometimes it is for personal reasons that has nothing to do with the hospitals that

at we operate in. So I do not see that in the next year as being a problem, either retention or recruitment.

Moderator: Next question is from the line of Madhu Marda from FIL.

Vivek Agrawal: Thank you. That is from my side.

Moderator: Thank you. Next question is from the line of Madhav Marda from FIL. Please go ahead.

Madhav Marda: Hi, good afternoon. Thank you so much for your time once again. Sir, just on the margins again, if you look at our reported margins, 24.8%. I think you said INR15 crore of cost is already there for the new units. So base network margin seem to be 25.3% approx in quarter 3. You are saying that, that 25.3% has scope to go up by 100 basis points. So base network can be above 26% next year. Is that how we should read it?

A. Krishnan: Yes.

Madhav Marda: Okay. And then so basically, the impact of the losses from the new unit, the INR150 crore should come on this 26%, right? Basically on that, we should assume the drag?

A. Krishnan: That is correct.

Madhav Marda: Okay. Understood. Okay. And sir, just a second question is on the hospital business growth. Given the time line for the expansions that we have of the new beds coming in, how should we think about hospital business revenue growth next year on the base network? And then how much revenue can we add from the newer beds, which are coming in?

A. Krishnan: So we would not want to guide specifically around that, right, because it is forward-looking. So as you know that we are looking at -- we continue to look at seeing how we can at least be at the 12%, 13%, 14% growth on the existing hospitals, but we will see -- how the year starts and how we progress. And then we should -- the additional beds we should clearly add another 3%, 4%.

Madhav Marda: Understood. Thank you very much.

Moderator: Thank you. Next question is from the line of Avnish Burman from Vaikarya Change LLP. Please go ahead.

Avnish Burman: Yes. Hi. Good afternoon. Thanks for taking my question. I just have a couple of questions on Keimed. We saw some good margin expansion this quarter on a Q-o-Q and a Y-o-Y basis.

Can you just articulate what were the key reasons for that?

Suneeta Reddy: Sanjiv?

Sanjiv Gupta: So I think what you saw was the aberration in Q2 and as well as in Q1, where the overall EBITDA percentage was low because of the restructuring and the necessary legal cost associated with them. I think -- so now those are removed. And this is what we discussed in the last earnings call also that in Q3, you would see we will be coming back to operation of 3.1 upwards and 3.3% is what we have in Q3, but it is more as an aberration that got cleared

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in Q3 then plus the business efficiencies.

Avnish Burman: Okay. And because of the GST change that happened in September, did we see some kind of revenue push out from the 2Q to 3Q because I am guessing the retailers would be reducing inventories in 2Q and then building again in Q3. Does that happen? Or on a quarter-wise basis, it was just normal Q2 and Q3 operations in revenue?

Sanjiv Gupta: No, we did not see any such impact.

Obul Reddy: We have seen very good improvement in the FMCG and pharma slightly plus or minus sales at the same level, but FMCG consumption is very good, and we have seen good growth on that.

Avnish Burman: Okay, understood. Thanks. I will get back in the queue.

Moderator: Thank you. Next question is from the line of Kunal Dhamesha from Macquarie. Please go ahead.

Kunal Dhamesha: Sir, just one on the potential losses from the new units. You suggested that the first month from just two hospital is around INR15 crore, and we are guiding for a full year losses of INR150 crore. So, is it more conservative number because the annualized losses itself would be more like INR180 crore. I am sure it would improve in the existing units. But the 150 number, is it more conservative on the side, given we are also opening a greenfield in

Gurugram, which would definitely take some more time versus the brownfield that we are doing.

Gurugram, which would definitely take some more time versus the brownfield that we are doing.

Moderator: Kunal, can you mute your line please. There is some background disturbance.

A. Krishnan: See, we have two quarters of Pune to ramp up from here on before some of the other hospitals come on stream also. So that is one that should kind of benefit us as we move into the next year. And then you will start see some of the other hospitals coming in. And so it is going to be like three quarters of next year when you will be seeing the other new hospitals coming in. So that is what we are looking at.

And -- but I think broadly, as of now, we would like to keep it at INR150 crore. That is what we would like to keep it for now. And this number of INR15 crore is really for 3 months of Pune in terms of costs of Athena. The revenues have more been in the last 1 month or 1, 1.5 months, but the cost has been for 3 months. So you should -- so that is the way you should look at it. So now we will go with INR150 crore. We will see during the next year.

Kunal Dhamesha: Sure, sir. And second one on the GMV growth that we guided, we suggested that we would be ending the year with 30% GMV growth. So that means Q4 would be 30% or we guided for the 20%, 30% for full year FY '26?

Suneeta Reddy: Madhi.

Madhivanan B.: Yes. This is on a full year FY basis. We will obtain our 30% to 35% growth on the GMV basis. In this quarter, we remove around 4% to 5%.

Kunal Dhamesha: And sir, for 9 months, this number looks much lower, right, in terms of -- so you are adjusting

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the base for the GST impact and the Amazon impact -- for this 39%?

Sanjiv Gupta: Correct. Correct. Because the Amazon business, the reason why we walked out was because it was a negative business for us. So while it has impacted the GMV on the revenue side, it is been much more beneficial. So that is the new normal now.

Kunal Dhamesha: Yes. What was the full year number for Amazon last year for us to adjust the base?

A. Krishnan: Sanjiv, can you give the exact number, please?

Sanjiv Gupta: It was roughly INR160 crore.

Kunal Dhamesha: INR160 crore on a INR3,000 crore of GMV, right? So, yes, roughly, 5%.

Sanjiv Gupta: I think another important point that you need to keep in mind is that the -- what we are talking here about is the entire platform GMV to grow year-on-year, this year growth as well as for the next year. And when we are defining platform GMV, while obviously, we discussed about GST and Amazon impact.

In this platform GMV, we are not adding IP/OP GMV. And IP/OP GMV is -- and that is the reason you would see in the earnings call presentations also, we have highlighted this as separately. Because this is this -- in the earlier year, this used to be a variable pay model. From this year onwards, it has converted into more of a fixed fees to us, supporting the technology side of expenses that we are incurring for the group as a whole.

So I think going forward, our guidance would continue to be excluding IP/OP, what is the growth that we will be looking at. And I think Madhi in one of the questions did say that next year, you can expect in the range of about 30%. This year also, if I just look at some numbers, we should be in the range of about 28% growth for the full year versus previous year adjusted for the reasons that we just discussed.

Kunal Dhamesha: Basically IP/OP, GST and GMV. These are the three impacts that should address right?

Sanjiv Gupta: Perfect.

Kunal Dhamesha: Thank you.

Moderator: Thank you. Next question is from the line of Ronak Agarwal from Ithought PMS.

Ronak Agarwal: Yes, thank you for the question, madam. So, I just want a broad overview for the hospital. So 1,500 beds will be coming in the next year out of which 50% will be operationalized. So what is the 50%, 60% -- what is the time period which we are looking for, let us

say,

occupancy of 60%, 70%?

A. Krishnan: On the overall -- all the beds or half of the beds?

Suneeta Reddy: So we will open 750 beds next year -- sorry, in the coming year. And post that, another 750 beds. In 2 years, we should be breakeven on 1,300 beds that we are talking about.

Ronak Agarwal: So I am asking, like, let us say, we are opening 750 beds. So what is the occupancy rate we are looking in the first year itself?

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Suneeta Reddy: It will be around 40%.

Ronak Agarwal: Okay. Okay, madam. Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand the conference over to management for closing comments. Over to you.

Suneeta Reddy: Thank you all for your presence. I am sure that in the upcoming quarters, we will continue to remain focused on phase d capacity operationalization and the opening up of new beds in Healthcare Services and a continued ramp -up of revenue and profitability in AHL and AHLL. Disciplined execution leading to volume and value growth, sustained investments in clinical excellence and technology and market share gains across key markets will help us to sustain momentum over the medium term.

We are committed to delivering consistent performance while creating enduring value for our patients, consumers, partners and all of our shareholders. So, thank you all for your support.

Moderator: Thank you, members of the management team. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2026

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Apollo Hospitals Enterprise Limited

Transcript of Q4 & FY26 Earnings Conference Call

May 21, 2026

Moderator: Ladies and gentlemen, good day, and welcome to Apollo Hospitals Limited earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Mayank Vaswani from CDR India.

Thank you, and over to you, sir.

Mayank Vaswani: Good afternoon, everyone, and thank you for joining us on this call hosted by Apollo Hospitals to discuss the financial results for the fourth quarter and full year of financial year 2026, which were announced yesterday.

We have with us today the senior management team represented by Mrs.

Suneeta Reddy - Managing Director; Mr. A. Krishnan - Group CFO; Dr. Madhu

Sasidhar - President and CEO, Hospitals Division; Mr. Madhivanan

Balakrishnan - CEO of Apollo HealthCo; Mr. Sriram Iyer - CEO of AHLL; Mr.

Sanjiv Gupta - CFO of Apollo HealthCo; and Mr. Obul Reddy - CFO of the

Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties, which is on Slide 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks.

Suneeta Reddy: Good afternoon, everyone, and thank you for joining us for today's earnings call. I trust you have had the opportunity to review the earnings material that we had circulated yesterday.

FY26 has been a strong year for Apollo, marked by healthy growth across all three verticals: Healthcare Services, Apollo HealthCo and AHLL, alongside steady progress in our expansion agenda.

During the year, we also announced a strategic restructuring of the omnichannel pharmacy and digital health business, an important step towards sharpening focus on unlocking long-term value across our integrated health care platform.

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The NCLT convened meeting of the shareholders is now being called for on June 24 to obtain the requisite shareholder approval, consequent to which we are hopeful the demerger process is completed by Q4 FY27 as planned earlier.

In terms of financial milestones, I am pleased to share that our consolidated revenues crossed INR 25,000 crore for the first time, reaching INR 25,229 crore. Apollo HealthCo also surpassed INR 10,000 crore in annual revenues for the first time following a similar milestone achieved by our Healthcare Services business in FY25. These achievements underscore the scale, the relevance and the growing strength of Apollo's integrated healthcare ecosystem.

Turning to our performance for the fourth quarter: - Consolidated revenue grew by 18% year-on-year to INR 2,605 crore in Q4 FY26. Within this, the Healthcare Services business reported revenues of INR 3,268 crore, up 16% year-on-year. This performance was driven by a balanced mix of 7% volume growth, 5% case mix and the remaining 4% from price revisions.

Surgical volumes grew by 7%, supported by a continued focus on CONGO specialties, cardiac, oncology

y, neurosciences, gastro and orthopedics. The specialties remain key growth drivers for the business, delivering strong

revenue growth of 22% year -on-year in Q4 FY26.

Group -wide occupancy stood at 68% during the quarter with established hospitals now at 69% and hospitals in metro at 71% occupancy. This performance should also be viewed in the context of continued improvements in clinical productivity and operating efficiencies.

The average length of stay reduced to 3.19 days from 3.3 days in the corresponding quarter, a decline of 3.3%. The reduction was driven by significant adoption of robotics, minimally invasive surgeries, enhanced recovery and discharge protocols and stronger clinical pathway standardization across the network.

Importantly, the improvement in ALOS has been achieved by contributing to manage higher acuity and complex case mix, reflecting Apollo's alignment with global benchmarks in tertiary and quaternary healthcare delivery, where shorter hospital stays are enabled through technology -led interventions, evidence care -based protocols and superior postoperative outcomes.

Insurance and self -pay patients continue to account for 83% of inpatient revenues in Q4 FY26. Insurance revenues grew by 21% year -on-year, while self-pay revenues registered a growth of 13%, reflecting sustained demand across both segments. Average revenue per patient stood at INR 1,87,208 in Q4 FY26, an increase of 9% year -on-year, driven by an improvement in underlying clinical mix.

Within the Healthcare Services business, we delivered a ROCE of 25.4% for the year, supported by balanced performance across our network spanning metro, Tier 1 and Tier 2.

Apollo HealthCo reported revenues of INR 2,848 crore in Q4 FY26, representing a strong year -on-year growth of 22%. Revenues from Apollo Health & Lifestyle grew 24% to INR 489 crore.

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Consolidated EBITDA for the quarter stood at INR 1,011 crore registering a robust growth of 31% year -on-year. Within this, Healthcare Services EBITDA was at INR 781 crore, reflecting a 14% growth, while margins remained strong at 23.9%. Established hospitals, however, grew the EBITDA margin to 25.5% and EBITDA losses from new units in Q4 FY26 came in at INR 41 crore.

Within the Apollo HealthCo, the pharmacy distribution business reported EBITDA of INR 195 crore, an increase of 20% year -on-year. Within the online business, cash losses declined sharply to INR 16 crore compared to the INR 80 crore in Q4 FY25. Consequently, Apollo HealthCo reported EBITDA of INR 156 crore in Q4 FY26 versus INR 36 crore in Q4 FY25, a significant improvement in operating leverage and business efficiency.

AHLL delivered a margin of INR 75 crore, representing a strong 58% year -on-year growth with margins improving to 15.3% from 12% in Q4 last year.

Platform GMV of Apollo 24/7 was at INR 528 crore in Q4 FY26, a growth of 20% year -on-year. Digital revenues grew by 29% during the quarter on a like -for-like basis after excluding the closure of the Amazon corporate partnership. We reported consolidated PAT of INR 529 crore, higher by 36% year -on-year with significant improvements in PAT across all three verticals.

For the full year FY26 performance, our consolidated revenue was at INR 25,229 crore, representing a 16% year -on-year growth. Consolidated EBITDA was at INR 3,769 crore, reflecting a 25% year -on-year growth, while consolidated PAT grew 34% year -on-year to INR 1,942 crore.

This strong performance for FY26 is despite the lower seasonal medical admission alongside moderation in international patient volumes, particularly for Bangladesh. The performance reinforces our ability to sustain growth momentum, while maintaining financial discipline and operating efficiency.

As an

important development, we announced yesterday that Apollo Cradle and Fertility and Cloudnine would combine to create one of India's largest integrated mother maternity and fertility care platform. AHLL's mother and child fertility business are valued at INR 1,550 crore through a combination of cash and 9.9% equity stake in the combined entity.

AHLL will become the largest nonfinancial shareholder in the combined platform and will have Board representation in the combined entity through a nominee director. Revenue of the Cradle and Fertility verticals was at INR 450

crore in FY26 with an IGAAP EBITDA of INR 45 crore. The transaction has therefore been concluded at a multiple of 35x EBITDA. Spectra and the other assets of AHLL are not part of this transaction and will continue to remain in Apollo Health & Lifestyle. This collaboration will accelerate access to premium maternity, fertility, neonatal and pediatric care, while raising the bar on outcomes experience and continuity of care.

Apollo will bring its deep experience in this space to the combined entity to contribute to its growth. Apollo remains deeply committed to expanding the end-to-end women and child health care platform from prevention to high-risk pregnancy, neonatal intensive care, pediatrics and lifelong wellness through its own integrated health care ecosystem across geographies.

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During FY26, our expansion initiatives progressed well. We operationalized four new hospitals, Apollo Athenaa in NCR, Pune, Financial District in Hyderabad and Narendrapur in Kolkata with a combined potential operational capacity of 855 beds, which we are commissioning in a phased way with 185 beds now operationalized, the remaining 670 beds planned over the next 12 months.

As these hospitals continue to ramp up operations and scale specialties and clinical programs, we are also poised to commission 2 more hospitals, one in Sarjapur and the other in Gurgaon in the next 2 quarters. In total, these additions are approximately 1,400 operating beds, all in key metro markets. This will position us strongly as we move into FY27. And later, they represent nearly 25% capacity additions in these markets.

Alongside capacity expansion, we continue to deepen our clinical leadership through investments in high acuity specialties, oncology, robotics and advanced care pathways. Consumer engagement and technology adoption across physical and digital formats has also continued to improve with increasing integration across hospitals, pharmacies, diagnostics and digital platforms, reinforcing the strength of the One Apollo ecosystem.

On this note, I would like to hand it over to Krishnan, our CFO; Dr. Madhu Sasidhar, CEO of the Hospitals; Madhivanan, CEO of Apollo HealthCo; Obul Reddy and Sanjiv from Apollo HealthCo and Sriram Iyer from Apollo Health and Lifestyle.

Moderator: We will now begin the question-and-answer session. The first question is from the line of Binay Singh from Morgan Stanley.

Binay Singh: Congrats on good set of numbers. My two questions are, firstly, on the hospital losses that we had talked about, INR 150 crore. If you could update us on, are we tracking on to that number, which is the quarter where we see the most hit? And the second question is on the digital breakeven. Are we on track to deliver that in Q1?

Suneeta Reddy: So, on the first one, yes, I think we are sticking to our assumption that we will have INR 140 crore loss. Most of this will actually happen in the fourth quarter, where we would have opened out all of the facilities. For the second question, let me pass it on to Madhivanan.

Madhivanan B.: Yes, as we speak, we finished this quarter pretty well. And we believe that our growth story when it comes to GMV will continue,

both on the pharma side of the business and the diagnostics side.

On the hospital consults also, the trends have been positive. While Q1 would be a slightly more seasonal quarter for us, we expect that we should be very close to breakeven or breakeven very soon, so in Q1 itself. So, we are on course.

Binay Singh: Thanks for that team. Secondly, in the opening remarks, we talked about 1,400 beds, 185 already operational. Could you give us a little bit of a road map that by mid of this year, by end of this year, how many beds will start adding to the revenue bit?

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A. Krishnan: So as we speak, you know that this will be ramped up over the next 12 to 18 months, as we have said. As of now, it is 185, we are hoping that by mid of

this year, we will at least get this to 500, 600 beds before we even start Gurugram, which will then be a higher number.

So, the way you should look at it is because Gurugram will start more towards the end of Q2 and the hospital in Bangalore will actually start in Q1 as well.

So, we will be operationalizing most of this in the next 12 to 18 months, as we have said. By mid of next year, think of it as at least around 500, 600 beds operationalized.

Moderator: The next question is from the line of Neha Manpuria from BofA Securities.

Neha Manpuria: My first question is on the hospital growth. I mean the established hospital growth has improved to about 13%, 14%, if I were to strip off the new hospitals.

Should we assume that this growth improves given that we had low seasonality, the Bangladesh impact in FY26? And post the margin expansion that we have seen in the established hospitals, is there scope for more improvement from the 25%, 25.5% that we have done in the last 2 quarters?

Suneeta Reddy: Yes, I think the established hospital, so let me just come in. In terms of seasonality, there was a little bit of seasonality. But moving on to the next quarter, you will see an increase in occupancy. And in terms of, I think just moving forward, there will be an improvement in established hospitals margin. This will come from higher asset utilization as well as some cost reduction.

Madhu, you can add to that.

Madhu Sasidhar: So, as Ms. Suneeta has indicated, the lack of dependence on seasonality is the fact that a lot of this growth has come from our investments in clinical program building and recruitment. And this has been geographically across the Board. So, every one of our broad markets has performed very well. And it has been a very strong performance, especially in our Tamil Nadu market for Chennai and the rest of Tamil Nadu.

As an example of the high complexity work and the program building that we have done, in the fourth quarter, cardiac sciences grew by 19% and so did orthopedics by almost 20%, 19.9%. So, I think this is a strong flywheel, it gives us momentum, and these are not one-offs or seasonal. We also had some improvement in Bangladesh revenue compared to last year as well as some diversified income from some new markets in Africa as well as in Asia.

I think to your question regarding EBITDA, the strong performance in Tamil Nadu is very, very helpful. Those are hospitals with very strong performance when it comes to EBITDA. While we have improved EBITDA margins in our established hospitals, we think there is scope for continued improvement. We have improved our operating leverage, especially when it comes to some of the big costs here.

Neha Manpuria: Would you like to quantify how much more improvement we can bring through in established hospitals?

Madhu Sasidhar: So, we do not guide, but I think there is an opportunity. I will hand it over back to Ms. Suneeta.

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Suneeta Reddy: I think, AK, do you want to comment on that?

A. Krishnan: So, we have been saying that at least INR 100 crore to INR 125 crore impact is further possible. And that is going to be something that we are working on, on both costs as well as operating leverage. So, we think that the margins can sustainably get to, as of now, if you look at the Q4 margins, it is at 25.5%, and we are hoping that we should be able to sustain that for the year.

Neha Manpuria: Understood, sir. That is very helpful. My second question is on HealthCo. So, we exited, I think, FY26 margins are at 4.3%. And I see the slide now mentioned 6.5% to 7% as the exit margin for '27. Other than the digital breaking even and possibly having positive EBITDA, could you just help us understand how we get to that? I mean, what would be the key drivers to get to the 6.5%, 7% margin for fourth quarter?

Sanjiv Gupta: So I think this is what we have been discussing in the last couple of earnings call. We have a growth potential in private label coming out of the store front, which is the Apollo Pharmacy store. So, we strongly believe that further headroom is available on the private label, and that would be EBITDA accretive. Similarly, on the expenses side, also, you would see that digital also has started coming down on the losses. We continue to believe that Q1, we would be very close to the breaking even.

And apart from this, the growth itself would accrue, would provide a decent flow-through to EBITDA, so we strongly believe that all these things put together and various other initiatives to increase sales and other things should help us to achieve what we have been guiding the street since last 2, 3 quarters.

Neha Manpuria: So out of the 250-basis points margin improvement that we are expecting, bulk of it would be from the digital losses and the rest would be private label and finance cost, the integrated costs, coming down. That is a fair assumption?

Sanjiv Gupta: That is a fair assumption. Thank you.

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: First one on the hospital business. So, we continue to guide around INR 140 crore drag from the new hospital. And we are also suggesting that the drag is like the most in this quarter with almost INR 41 crore. So, if most of the operating expenses are already there in our number, then what is keeping us from commissioning the rest of the 670 beds, why are we kind of waiting till the next 12 to 18 months?

A. Krishnan: So, two points. I think it is important that we are not saying that this INR 40 crore is the maximum. Please let us appreciate that the peak can go up a bit, especially in Q2, etcetera and then it can come down. And then Gurugram will also get open. So, we have guided for the full year at INR 150 crore. Of course, we will have to see how each of the hospitals ramp up, and we are quite aligned to the fact that for the full year, we should still be around that INR 150 crore number. But the peak in a quarter can go up.

And we will operationalize some of these as we see ramp up. Some of these have only been soft commissioned. Let us appreciate that while we have done

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185 beds, barring Pune, which is 75 beds, which we have already commissioned, Kolkata just got soft commissioned. It is going to be fully commissioned in Q1. The Hyderabad was only recently commissioned and so is Delhi just around a quarter back. So, we will have to wait for some of them to commission fully because these are all preoperative costs that you are also seeing and the business will ramp up over the next Q1 and Q2 period.

Kunal Dhamesha: Thank you for that

t. And the second question for ma'am on the deal that we have announced for Apollo Cradle. My understanding was that we were doing quite well given that in this format, we had presence in our key markets. So beyond, let us say, merging and achieving the number 1 scale, what do you see in this deal? Is it the valuation that we are getting? Was it noncore for us? We did not see a lot of value. How have you looked at that?

Suneeta Reddy: No, I think it is a combination of factors. One is the valuation, which is clearly key to get a valuation of INR 1,500 for EBITDA at a multiple of 35 in this market is something that you cannot ignore. Second, the format that fits Apollo is the deeply integrated mother and child platform, which we will continue to grow in key markets. So, the mother and child format which we have in Chennai, which looks after the entire requirements of the mother as well as the child and is deeply clinical in terms of you know, we look after the child up to the age of 16. So, there is a huge difference in terms of value that you get from that, the revenues, the margins. There is a huge difference in all of these parameters. So, we will continue with that approach to mother and child.

The third is that, of course, that cash that we get, that INR 750 crore will be deployed in primary care. We have already built out a primary care platform that has clinics and diagnostics. But we believe that we have to be leaders in this platform because this will be the funnel to Apollo, and it will look after patients who are currently not in our system and who are not very sick. So, it is part of our strategy for preventive, for looking after all the requirements of all of our customers.

Kunal Dhamesha: And just lastly, madam, will our clinics will still continue to carry Apollo brand name, or the brand name will change?

Suneeta Reddy: Yes, yes. So, the clinics, everything else is Apollo brand name. Only the Cradle will carry that for 1 year and then it will become Cloudnine.

A. Krishnan: So, it has to be on the same page, the AHLL business, as you know is an INR 1,865 crore revenue business for this year. INR 450 crore is the Cradle number that has got Cradle plus fertility number. Out of this INR 1,865 crore this is

what has got transferred and combined into Cloudnine. So, it is only the Cradle and IVF business. All others continue. We are going to be doubly focusing on Apollo Primary Care and diagnostics as well as other specialty care, as Ms. Suneeta already said, which is the Spectra, dialysis and the others.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Karan Vora : So first question is with respect to the pharmacy business. So is it fair to assume that , I think we have mentioned this, but just double checking, the private label contribution going up is the primary or the sole reason why our offline pharmacy margins have been as strong. That is one. And the second

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subpart would be for the Apollo 24/7 reported breakeven, right? While cash breakeven, we are on track. Any colour on reported breakeven time lines will be helpful?

Sanjiv Gupta: So maybe let me just take the digital question first, madam, and then Obul sir can talk about the offline business. So, in the last earnings call also, we guided that the first milestone would be to have Q1 breaking even without ESOP costs. So that should happen in Q1. And we also suggested that by Q3, we should be able to break -even, including the ESOP cost. That is a second milestone we are looking at it. And from there upon, the journey is to make sure that we increase our EBITDA in absolute terms quarter -on-quarter. So, I think with that in mind, I think we should wait for these two milestones to hit first.

st. And then there upon we can further discuss about to what an extent digital business will add on to the absolute EBITDA on the AHLL.

Obul Reddy: The pharmacy offline business, as we have been seeing continuously margin expansion driven by the higher private label. And we are also working on the entire spectrum of purchase level margins and cost management, and we could see that improvement continues going forward.

Karan Vora : Okay. And also, where does this settle? What could the normalized or the more steady -state margin look like , is it 8%, is it 9%, 10% any colour there?

Obul Reddy: At a more mature stage with high level of private label, we could see somewhere between 8% and 9%.

Karan Vora : Okay , got it. And do we have any GLP -1 related benefit in our pharmacy business or any?

Obul Reddy: We are working on some two, three focus areas and that will be one area to work now. We have some specific focus on those areas. We will update you later.

Karan Vora : Okay , got it. Helpful.

Moderator: The next question is from the line of Banshi Desai from JP Morgan.

Amlan Jyoti Das : So firstly, just reconfirming , in terms of beds that will get commissioned over the next 2 years, should we assume most of the 1,400 beds which are yet to get operationalized, all of those will get operationalized by fiscal '28?

A. Krishnan: Yes, that will fully get operationalized by FY28. All of these are , let me re -emphasize that the structure is all getting constructed and it will all be ready as we commission it. Some of them other than the Phase 2 of Pune where the construction is on, all of these construction will also be ready by Q2 or beginning Q3 of this coming fiscal. So, there is no reason for us to not open all these up by FY28.

Amlan Jyoti Das : And therefore, if one has to think about losses, fair to assume then by fiscal '28, your cumulative losses, I mean, you could actually see breakeven or probably much lower number of loss?

A. Krishnan: Yes, at least a breakeven to begin with FY28.

Amlan Jyoti Das : All right. And then secondly, when I look at AHLL performance, obviously it has been very, very strong. So, within that, diagnostics particularly has done

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very well. So, if you could tell us what has driven this very strong growth and is this sustainable ? And secondly also on margins of AHLL , so 15% margins that we have seen, it is a very strong up move. So, any one -off element here or is this the new base?

Suneeta Reddy: Sriram, are you on the call? Okay, you take that AK.

A. Krishnan: So I think this is , we have seen that there has been a structural shift that we have seen in the overall diagnostics because they are looking at increased lab utilization as we speak. And also, within each of the markets, they are seeing how they can work on B2B business also along with the B2C that they are focusing on which is why the growth is something that they have been able to show and it will be sustainable.

Amlan Jyoti Das : We had alluded in the past that we have been reinvesting in this business. So fair to assume that we are towards the end of it or do you still see us reinvesting or growing the diagnostics business?

A. Krishnan: We definitely plan to grow the diagnostics business. We are amongst the top four players now in the country. The potential on diagnostics is significantly higher, having brought out IFC at the first stage and now with combining the Cradle business with Clo udnine, there is a strong intent to double down on the primary care and diagnostics in each of the metro markets and the non -metro markets as well because it is also a strong outreach for the Apollo brand and you will see us accelerating on th is front.

Amlan Jyoti Das : All right.

Moderator: The next

question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Two questions. First, in your new hospitals, which you have started in the last few months or so, what has been your experience in terms of onboarding insurance partners and then I think what we have heard from a few of your peers are the negotiations, etc etera, are taking longer. So, what has been your experience there?

Suneeta Reddy: So just to answer that, I think we have been quite fortunate in onboarding insurance partners , which is why we are seeing some traction. Of course, with the way that things are, it takes a little bit more time, but Apollo Athenaa has 6 high-volume insurers already supporting it. Our Pune Hospital has 3 of them and we have just closed out Star Health . In Kolkata also, we have got 6 high - volume insurers. Hyderabad has 4 high volume, and we are just closing out Care and Star, so it will be 6. An d including Bangalore, we have 3 of the top 8. So, we are all set in terms of partnership with insurance companies.

Damayanti Kerai: So, there has been no issue, right, unlike faced by some of other players, you said you have been very successful in onboarding these insurance companies in an exclusive?

Suneeta Reddy: I believe we have good partnerships, and we will continue to strengthen them.

Damayanti Kerai: Okay , that is good to hear. My second question is on your Apollo 24/7

business. So, while digital losses continue to reduce and you will likely achieve breakeven very soon. But if you can comment on the gross merchandise value trends, I think on a quarter -on-quarter basis, again, it is more or less similar to

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what we saw last quarter. And then if you can share some updates on the recent services there like hospital connect, insurance, etcetera, that will be helpful.

Madhivanan B.: So, this particular current year was driven primarily on an operating model change, which was effectively not to depend on sales marketing from a customer acquisition perspective. And I think we have been able to establish that engine wherein we are able to g et new customers between 20% and 30% without actually spending too much of marketing money. So that is an area which has got established on the pharma side of the business, which is our primary mainstay.

On the diagnostics side, again, I think both from a cross -pollination perspective as well as our ability to use what we call as an omni -asset has helped our diagnostic business what we do through AHLL again grew at around 25% to 30%, and we believe both these engines are sustainable without spending too much of the marketing money, we will continue. While we might not grow at 80% and 90%, between 25% to 35% is a growth in terms of new business we are confident of.

On the doctor -consult business, we have a much stronger framework within which we work with the hospitals and the clinics, and that is also an engine which is building up slowly. While this year, the growth was not very huge, we have reset it and that should also play o ut in the coming year.

On the cost front, while we have almost the base, we still feel there is a certain

amount of efficiency, which we will lean out and as we go towards the , as a combined entity by the end of this year, there will be some more cost elements which we will bring in.

So, both in terms of growth and in terms of EBITDA to the bottom -line level, it would be what I would call as a calibrated business. That is one area, actually two areas in which we continue to invest , and that they could be a bit of a drain because we treat all of them more as opex.

One is the insurance business, which we have started off with on a good booking on the health care. But like I told last quarter, there was a bit of a change

in the way the numbers play out. So, some of our revenue has got staggered to the back end. We believe in the next 2 quarters, that model will also stabilize.

And second area is we continue to invest in our technological assets, PHR, Ask Apollo, which is integrating with multiple partners. So that also remains the cost, but both of them are costs which will help us in the long run. So that is the broad commentary in the way we expect FY27 to happen. Happy to clarify any questions.

Damayanti Kerai: Yes, that is very helpful. Just I think a follow -up. How much more investment you look for these areas? You mentioned insurance and technology. So, any number which you can share?

Madhivanan B.: Understood. Sanjiv, if you can just say what has been our cost -structure that we have been working on and whether it is continuously coming down, but there is still a little bit of investment that is. Sanjiv, if you can just throw some

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light on it, please.

Sanjiv Gupta: Yes. I think if you are referring to our investment into AI technology as well as on the insurance side of it is concerned, so, we are expecting roughly between INR 6 crore to INR 7 crore on a quarterly basis. And that also is helping us a lot in reducing our costs. By the way, AI tools in the customer call center helps us to improve the productivity and reduce our expenses. Broadly speaking, I think that is the level of expenses that we are looking into these two sides of the business.

And you also checked on one point related to the margins to what an extent margins can go up in digital. I think it is too early to specify a particular percentile, but I think the way insurance business is increasing more of diagnostic business, we saw a very high growth in diagnostic business last year, and we continue to do with that, a better diagnostic mix, high insurance and more circle programs and that monetization will only strengthen the existing margin profile. A little too early to say that how we end up current fiscal year, but I think where we are standing today, we should be in a better place as you see quarter -on-quarter with respect to our numbers.

Moderator: The next question is from the line of Amey Chalke from JM Financial.

Amey Chalke: Congrats on the good numbers. The first question I have on Tamil Nadu region. This quarter, also for full year it has performed well. Going into next year, there is a limited bed addition or no bed addition basically. So, occupancy is already at 68% for the region. Plus, the volume growth for this year has been pretty tepid. So, the average revenue per patient will remain the sole growth driver for this region going ahead? Is there a scope for the case mix to improve?

Suneeta Reddy: Madhu, let me take this first. First, in terms of headroom for growth, there is definitely an additional 6% to 7% headroom for growth, there is definitely additional capacity, not just in our main hospital, but in the other hospitals in the Chennai region. There is another 7% uplift that can come from pure occupancy.

With regard to case intensity and improving ARPP, we will continue to focus on that. We have very high -end clinicians and probably the finest technology in the world in this region. So, you could see an improvement in ARPP and case mix. So, I think both on asset utilization, headroom for growth and ARPP, there will be continuous improvement.

Amey Chalke: Sure, ma'am ,thank you so much. And the second question I have, if we can provide the OCF and FCF generation for Hospital Services segment for FY26? And also, the capex, which we have given INR 1,980 crore for 1,000 beds for

FY27, should we assume that the entire capex for FY27 or some of it has already been spent?

Suneeta Reddy: AK, take that

, please.

A. Krishnan: So, the operating cash flow before dividend that we have generated this year is before the dividend is approximately around at the consolidated SCS level alone, it would be in the region of INR 1,850 crore. That is before the dividends. After the dividends, it will come in the range of INR 1,550 crore, which means

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that is the fully deployed, that is after our recurring capex which is used for hospitals, which continues at around INR 550 crore.

This also includes our taxes paid during the year as well as the working capital.

So, it is after all of this, which means the entire INR 1,550 -odd crore is really re-deployable for growth capex fully. And this is the number that we are quite comfortable continuing into the coming year also. We do not think that our number would be significantly higher than this number in the coming year for growth capex.

Moderator: The next question is from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: So first question on the financials. So, on a quarter-on-quarter basis, I see that you moved from net cash to net debt. So, the difference is almost INR 1,200 crore. So, I guess INR 400 crore would be a new capex. So, what would account for the remaining INR 800 Crore?

A. Krishnan: So, we also paid off IFC this quarter. This was the INR 1,250 crore payout that we had in this quarter, just to remind you, this came out from the Healthcare Services balance sheet. So, we did pay INR 1,250 crore to buy out IFC at AHLL, just to remind you. So that was one of the reasons for the spike in this quarter. Otherwise, now it will get normalized to the projects.

Kunal Randeria: Got it. And just one clarification. So, you said you will break-even in FY28 on the new beds that have been commissioned. But the remaining 670 beds will be spread over a period of 18 months. So, I am just wondering when 855 more beds will be commissioned or could be commissioned sometime in the next couple of years. So just want to understand the assumptions behind this.

A. Krishnan: So typically, at a 50% to 55% occupancy level for the entire stock of 1,500 beds is what we would be breaking even. You know in each of the hospitals at that level of occupancy, we expect to break even as we speak. And we are quite, given that each of these hospitals are going to be staggered in their expansions and there are certain hospitals which will get mature by Q4, not mature, I am saying they will probably become breakeven by Q4 of the coming fiscal and some will be still in that burn mode. In FY28, as of now, we believe that all the hospitals put together should be breakeven. There will be positives and negatives in that. A few negatives and a few positives. But then overall, as a new hospital cluster, we would expect FY28 to be breakeven.

Kunal Randeria: That includes Gurugram or Sarjapur all these hospitals that are going to be commissioned in FY27?

A. Krishnan: Yes, which includes that. But as I said, Gurugram itself may probably be in a bit of a loss position, but some of the others will become positive soon. Let us also appreciate that Sarjapur and all are good markets for us, existing markets in Bangalore, so is Gurugram and a market that we know of with good doctors. Hyderabad also is a market that we have already hired the right set of doctors, and we are quite confident that the you know overall ramp-up should be good over the next two, three quarters.

Suneeta Reddy: Ladies and gentlemen, if you could excuse me, I have a bit of an emergency. And I will hand over this call to AK to answer any remaining questions. We commit to stay connected with you. Feel free to reach us either by e-mail or

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Zoom. But the team remains committed to answering all of your questions.

Thank you for your patience, and I look forward to our next call. AK, I am handing it over to you.

A. Krishnan: Sure. We will take it forward.

Moderator: We will move to the next question, which is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Thank you for the follow up. AK sir, can you kind of consolidate all the outlook that we have provided business segment -wise for FY27 in terms of revenue growth and profitability?

A. Krishnan: That will be offline because clearly, each of the businesses are on different growth rates. As we said, at least at the highest level, we have said that the hospitals businesses, we should look at a mid -teen growth into FY27. We have shown a good quarter end. And into the coming year, you should see a gradual acceleration of the revenue with all the new hospitals contributing as we said. We have guided that the overall Healthcare Services margin should still improve from the current levels by at least 100 basis points into next year, which is the established units. We have guided you that INR 150 crore is what we would for now look at for the overall hospitals business next year as the EBITDA loss.

And AHLL, we are continuing on the growth momentum and Apollo HealthCo. We have already given an overall guidance on the pro forma which Madhi can reaffirm that we have said that by , in the pro forma basis, which we have seen a INR 19,000 crore of revenue for the pro forma basis, including distribution, we have said that we would be getting to a INR 25,000 crore run rate by Q4 of FY27 with a 6.5% to 7% EBITDA margin. And Madhi, you can confirm that as well.

Madhivanan B: Yes, I think we will reconfirm that. We are very much on the task. Like last year, we grew between 19.5%, reasonably confident of growing at 21%. And with the integrated, so on the topline bit, we are on course to be going towards 25,000, whether it be opening new outlets in our distribution side, some of our new stores mature enough. So, the trend is visible. But I think the biggest lift will come the moment the digital business breaks even and then starts contributing to the EBITDA story because if you have to look at the cost structure that is something which is reasonably and this growth should be able to sustain that. So we are, we would be sort of very confident that we are on course on the pro forma side that we have published.

Kunal Dhamesha : And sir, a follow -up. Can you update us on the timeline for this entire transaction on AHL and when will the new company will come into existence and when will the separation or the demerger will happen?

A. Krishnan: So, as we said already that the shareholders, this call is scheduled for June 24, where we should be, which is the NCLT process, convened process, where once we get the shareholder approval of AHLL, we would go back to NCLT and then the NCLT process should take it some 3, 4 months to close. And hopefully, by Q4 FY27, end of December or early Q4 FY27 is when Apollo Healthtech should get demerged and listed. That is the plan. And that

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Company , when it gets listed, you should be in a position to see that INR 25,000 crore of revenue by that quarter, annualized revenue.

Moderator: The next question is from the line of Lavanya from UBS Securities.

Lavanya: Just a clarification on 24 /7 when you mentioned Q1, Q2 reported profit, including and excluding ESOP, what is the run rate of ESOP expense that we are looking at for next 1 year?

Sanjiv Gupta: So roughly in the range of about INR 50 crore is what we are looking at the overall ESOP cost for the

current fiscal year. You might experience about INR 22 crore, INR 23 crore for Q1. But after that, we start tapering down. Overall, for the year should be in the range of about INR 50 crore.

Lavanya: Okay , got that. So sorry, if I missed, but I just wanted to check on this Cloudnine transaction, when are we expecting it to be completed? And in terms of reporting, when should it be reflected in our AHLL numbers?

A. Krishnan: Sorry, what was the question?

Lavanya: So, it was for AHLL and the Cloudnine transaction. So, post this transaction, when are we expecting it to be reflected in AHLL numbers?

Sriram Iyer: So as the deal has just got concluded yesterday, I think as next steps, we will have to go and apply it. And once we get the approvals, which we expect to come in next couple of months, after that, the formal commercial transaction

will happen.

A. Krishnan: So maybe by end of one more quarter. So, we expect the CCI approval to come in place and then the whole transaction should get consummated. By in Q2, we should have that reflected sometime in Q2.

Sriram Iyer: Yes, sometime between August, I believe August or September, yes.

Lavanya: Got it this was really helpful. Thank you so much.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: On the 24 /7 platform, can you give us a sense of some of the operating metrics which are there on user metrics? How does this compare with some of the other larger digital platforms, right, in the number of online transactions, number of users on a daily basis?

Madhivanan B: So, let me take the question. So typically, there are two peers that we compare ourselves with. One is the quick commerce, especially when it comes to the medical part of it. So, the most important thing that we look at is the new customers that we are acquiring on a month -on-month basis, and we compare it vis-a-vis our peers like Tata 1mg and PharmEasy. And again, like I told you earlier, what is important is not just the growth, which is coming through new customers, but the CAC at which we are acquiring it.

We are in a good position. We are able to do around 200,000 to 220,000 on a month -on-month basis. We expect this number to go up as we are building some very strong synergies with our 7,500 outlets, both in terms of originating the customer as well as building the proposition of delivery.

The second big, so first is the number of customers that thing . The second big

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thing is our ability to deliver at least 90% of our orders in the same day. We follow two metrics; one, which is called the same -day delivery, which is 90% and the insta delivery, which runs between 19 minutes to, let us say, an hour, that is another metric we try to bring it down. We try to target bringing it in the range of around 50% to 55%.

All this needs to happen with the cost of delivery on a constant downward trend. Again, we are reasonably on par with some of the peers. It has been showing a downward trend, and that is our unit economics as becoming more and more positive.

The final thing that we usually track is also the number of markets in which we have been able to provide our business. Suffice it to say, the 30% growth that we have been establishing is primarily coming from our top 5 markets. So, the top 6 metros minus Mumbai. So, these are the numbers. Once this model gets established, we have enough potential to roll it out into new markets and then play it out as we go along. In all this, unlike growth at any cost, we are very much focused on a sustainable model where in we will be able to justify. So, these are some of the broad metrics that we look at.

Our AOVs, which is average order value, etcetera, are on par with the industry, S

ometimes even better, given the fact that a big base of our customers is chronic customers. I will stop here. There are enough other metrics which we track, but these are the ones which would be fair enough to give a directional position.

Nitin Agarwal: And just to sort of build on that, on the daily consultations that you talked about of 15,000 thereabouts, how has this number trended? And how do you see this playing out? And how big a funnel is it for your medicine orders and sample collections?

Madhivanan B: That is what I was highlighting. So, two things. One, the digital consult has been where we compare ourselves with people like Practo, that business has been growing at a reasonable speed, say, around 15%, 20% growth. I think the bigger story will play out as we go along for the physical appointments that we help facilitate for the hospitals and clinics. That number has started showing some very positive trend in the last 1 to 2 months. We expect that engine to drive further.

The diagnostic business, however, derives more from our pharma business cross -pollination. So, people who are already very core customers of ours on the pharma business, we try to track them on the basis of the cross -pollination between pharma and digital. And both on the omni side, which is both digital and offline, that number is also showing a trajectory. So, in fact, last year, we

had a very big jump. We expect a similar jump along with AHLL to build that agenda as we go along.

Nitin Agarwal: And sir, on the point on diagnostics, we give a gross sales number, but how much would that be captive sales to the Apollo Hospital Group and how much would be net sales to third parties? I mean, outside of the hospital?

Madhivanan B: We do not do third party at all. Our business is primarily most of our business, 100% of the business is exclusively delivered through AHLL on the diagnostic

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side and the hospital consult through both clinics of AHLL and the hospital. So, we do not have any third -party business. So, the entire core business is driven by the Apollo ecosystem.

Moderator: The next question is from the line of Mitesh from Aditya Equity.

Mitesh: I had a question regarding the associated Company , Indraprastha Medical. There is a legal case which is going on with the Company , with the Delhi Government . So, does it have any implication on our holding in Indraprastha Medical, whether the case goes in our favour or goes against?

A. Krishnan: No, we have taken this up with the Government as well because given whatever they have said, we are quite confident that we should be in a position to take this to a logical conclusion because the lease is automatically renewable there. And the free bed obligation is also being done by the Company also. But as the shareholding anyway continues.

Mitesh: Okay. But any possibility of increasing , taking Delhi Government stake of 26%?

A. Krishnan: No, nothing as of now. Nothing for now.

Mitesh: Okay, thank you sir.

Moderator: Ladies and gentlemen, as that was the last question for today. I now hand the conference over to management for closing comments.

A. Krishnan: Thank you so much, everyone, for the call. As we come into FY27, we believe we are entering the next phase of growth with very strong fundamentals across all the three lines of businesses, a visible expansion pipeline and continued progress in clinical excellence, technology and digital health.

Our focus remains on keeping the consumer at the center, delivering superior clinical outcomes, improving access to quality healthcare across formats and creating sustainable long -term value for all stakeholders. If you have any questions, please reach out to us, and we are happy to answer any of them.

Moder

ator: Ladies and gentlemen, on behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

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